

**From compliance to collaboration: shared decision-making
psychiatric violence, and the ethics of
medication management in contemporary Spain**

A biocultural and transdisciplinary action research toward
structural reform and institutional responsibility

Doctoral thesis

Henning (né Enric) Garcia Torrents

Communications and anthropology doctoral program
Universitat Rovira i Virgili

30 of June, 2025, Yogyakarta, Indonesia



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From compliance to collaboration: shared decision-making psychiatric violence, and the ethics of medication management in contemporary Spain

A biocultural and transdisciplinary action research toward structural reform and institutional responsibility

Doctoral thesis supervised by

Prof. Àngel Martínez-Hernández

Prof. Martín Correa Urquiza

Doctoral Programme in Anthropology and Communication

Department of Philosophy, Anthropology and Social Work

Universitat Rovira i Virgili

Henning (né Enric) Garcia Torrents

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Dedication

To the silenced, the disappeared, the punished for truth: this is for you, yours. Up yours, not. This is against the criminals, the abusers, all who harass and loathe our lives. To my love and her father, denied justice, and to all whose lives were shattered by systems built to protect power, not live. This ongoing work stands as witness. Justice is not optional, is owed. To my love, for our life. Thanks.

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Declaration of originality: This dissertation, titled *From compliance to collaboration: shared decision-making, psychiatric violence, and the ethics of medication management in contemporary Spain - A biocultural and transdisciplinary action research toward structural reform and institutional responsibility*, is the result of original academic work conducted independently by the author. All sources consulted or referenced are duly acknowledged, and any ideas, texts, or materials not my own have been properly cited in accordance with scientific and academic standards. This work has not been submitted, in whole or in part, for any other degree or academic recognition at this or any other institution. The empirical research, analytical development, and writing process adhered to the ethical principles governing scientific research, including those concerning human subjects, consent, confidentiality, and integrity. The content reflects the intellectual trajectory and field-based findings developed during the researcher's doctoral professor in training work within the doctoral programme at Universitat Rovira i Virgili, aligning with the institutional and disciplinary requirements therein and open science best and bettering practices.

Declaration of intent: This dissertation is not a collection of opinions, alternative beliefs, or speculative remedies. It is a scientific document grounded in the demand for best practices and continual improvement in mental health care. It argues that healing must be demonstrable, when a problem is present, it must resolve through intervention without causing further harm. The work emerges from direct experience of prolonged institutional and interpersonal violence, including coercion, forced treatment, and systemic neglect. It reflects on a reality where cruelty is tolerated, pathology imposed, and suffering weaponized. What is presented here is the product of survival and empirical observation, a structured response to abuses normalized under the guise of care. It is submitted with a clear aim: to support a shift toward evidence-based, rights-respecting, and humane systems of health, free of coercion and anchored in justice.

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Approval of thesis action-research supervisors

The undersigning state that the doctoral dissertation titled *From compliance to collaboration: shared decision-making, psychiatric violence, and the ethics of medication management in contemporary Spain - A biocultural and transdisciplinary action research toward structural reform and institutional responsibility* has been completed under their academic supervision, and that the candidate has conducted the research work in accordance with the requirements and standards of the doctoral programme at Universitat Rovira i Virgili. They hereby approve this dissertation for submission and evaluation. Following are their digital signatures, stamped with day and time.

The supervisors state that the present work presented by Henning Garcia Torrents for the degree of doctor, has been carried out under my supervision by the Department of Philosophy, Anthropology and Social Work; Prof. Àngel Martínez-Hernández and Prof. Martín Correa Urquiza, doctoral Programme in Anthropology and Communication:

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The rest of this section remains under preparation and will be finalized following the external review process, scheduled for completion at the end of summer. It will include a carefully verified and ethically composed acknowledgment of the many individuals whose contributions made this dissertation possible under conditions of extreme adversity, including documented torture, prolonged violence, and institutional betrayal. These abuses nearly resulted in the loss of life and have caused enduring harm to the researcher and their close ones. They must be recognized clearly, not diluted nor abstracted, as acts of cruelty that directly opposed the values this work seeks to defend. Throughout five years of persistent research, dialogue, and resistance, hundreds of persons contributed to this journey in multiple capacities. Colleagues, coauthors, field informants, teachers, professors, professionals, friends, allies, and fellow survivors all played indispensable roles. Parents and caregivers who stand for the dignity of children and the value of health and peaceful coexistence are likewise part of this network of care and shared responsibility. Their presence and support helped sustain not only the development of this thesis, but also the possibility of continuing to live, think, and build responsibly despite systematic attempts to silence or destroy.

The full list of acknowledgments will be made available through the dissertation's online repository and the open science laboratory and fieldwork notebook that accompany this work. This digital space will serve not only to credit contributions but also to reconnect with all participants engaged during the action-research process. It will open a space for ongoing collaboration, including follow-up interviews and joint reflection through the dissemination and engagement platform of the EU BEACON One Health Education COST Action, which now continues and expands the mission of this thesis. This dissertation is not only an individual scholarly achievement but a collective act of survival and intellectual resistance. It is a commitment to justice, the preservation of truth, and the promotion of systems capable of fostering health, equity, and mutual recognition. Final expressions of gratitude will be included with the explicit consent of each individual, ensuring their acknowledgment is respectful, accurate, and aligned with the shared ethical principles of this work.

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Readers guide to facilitate lecture

This doctoral dissertation is structured as a transdisciplinary ethnography of psychiatric coercion and institutional betrayal, grounded in biocultural theory and action-research. It reflects a five-year investigation into systemic violence within mental health systems, primarily in Spain, with comparative reflections from international fieldwork and global research networks. The thesis integrates narrative, legal, clinical, and methodological registers. Yet, its primary strength lies in the empirical synthesis of lived experience, field observations, and documented testimonies. The dissertation examines processes of decision-making and the autonomous management of medication, as well as their perception within Spanish mental health contexts. From this starting point, the research delves into the widespread realities of overmedication and coercive care, phenomena that emerge as structural features rather than isolated abuses. The testimonies and lived experiences gathered throughout this research emerge from an engaged and participatory fieldwork process in which the researcher contributed actively to community networks, supported the translation and dissemination of critical materials into Spanish, and facilitated spaces of mutual learning and advocacy. Rather than occupying a distant observational role, the researcher collaborated alongside affected individuals and collectives, bearing witness to the cumulative harms produced within mental health systems. These iatrogenic harms extend beyond pharmacological interventions to include forced hospitalizations, isolation, and the systematic erosion of autonomy, as well as the persistent marginalization, silencing, and social exclusion that follow people beyond institutional settings and deepen their suffering. The witnessing of others' pain, such as observing peers subjected to restraint, sedation, or neglect, creates a collective and relational trauma that reinforces an atmosphere of fear, hopelessness, and abandonment. These intersecting forms of violence contribute not only to individual suffering but also to a broader social death, in which systems intended to provide care perpetuate cycles of harm and exclusion.

The work is split in the following chapters:

1 - Introduction: violence, coercion, and structural frames. Introduces the thematic and analytical premises of the dissertation, focusing on the structural character of psychiatric coercion, the erosion of autonomy, and epistemic exclusion. The chapter frames the research within the broader urgency of health systems reform grounded in human rights, scientific ethics, and justice.

2 - Methodological framework: action research and positionality. Outlines the biocultural and action-research design underpinning the study. It details the methodological tools, surveys, testimonies, ethnography, while positioning the author as a survivor-researcher and reflecting on ethical, epistemological, and structural implications of situated knowledge.

3 - Survey findings: patterns, perceptions, and psychiatric practice. Presents the results of national and transnational surveys with professionals, users, and stakeholders. Findings highlight prevailing perceptions of coercion, clinical norms, trust dynamics, and systemic dysfunctions within psychiatric practice.

4 - Fieldwork and testimony: lived structures of betrayal. Documents extended participant observation and in-depth testimonies across institutional, professional, and familial settings. This

chapter brings to light trajectories of psychiatric violence, complicity, silencing, and the emergence of resistance and situated recovery.

5 - Analytical synthesis: institutional logics and resistance. Integrates findings across all empirical sources to examine the underlying institutional and discursive structures sustaining coercion. It interrogates the medical legitimization of harm, the erasure of embodied knowledge, and the spaces of refusal and counter-power.

6 - Legal and ethical dimensions: norms, violations, and accountability. Provides an introductory analysis of relevant legal and normative frameworks, highlighting their role in enabling or failing to prevent psychiatric violence. This chapter serves as an ethical reference point, grounded in constitutional and international human rights principles.

7 - Work ongoing and next steps: one health, pedagogy, and rights-based systems. Proposes concrete strategies for implementation through shared decision-making, collaborative medication management, open science, and transformative pedagogy. The chapter articulates an inclusive roadmap grounded in dignity, justice, planetary health, and epistemic repair.

8 - Final conclusions: scientific contributions and systemic redesign. Synthesizes the scientific, ethical, and political contributions of the dissertation. It reaffirms the responsibility of knowledge production to dismantle systems of abuse and calls for the urgent reconfiguration of psychiatric services as part of a broader structural transformation.

Annex A details the instruments developed and applied in the participatory action-research process. It documents the rationale, collaborative design, experiential focus, and integration strategies used to construct and validate the surveys within a rights-based and survivor-informed research framework.

Annex B introduces the ethical foundations of qualitative psychological research. It explores the methodological implications of working under conditions of structural vulnerability and epistemic asymmetry, with emphasis on positionality, harm reduction, informed consent, and research integrity.

Annex C reproduces the peer-reviewed article published in *Frontiers in Psychology* on the implementation of open dialogue in Spain. It highlights institutional barriers, ethical tensions, and the transformative potential of dialogical practices within a predominantly coercive system.

Annex D contains the manuscript analysing xenophobia as an aggressive structure that intersects with psychiatric misdiagnosis and racialised harm. It contextualises epistemic exclusion, somatic impact, and structural discrimination within broader patterns of institutionalised violence.

Annex E presents a transdisciplinary analysis of the nocebo effect and the giving up given up syndrome, drawing on transcultural psychiatry and embodied biology. It frames chronic psychosomatic suffering as a product of sociomedical breakdown, proposing more humane explanatory models.

Annex F offers a book chapter published by *Nature–Springer* on alternatives to coercion. It discusses dialogical ethics, service user leadership, therapeutic alliances, and systemic reform through evidence-based, rights-informed, and person-centred approaches.

Annex G documents the presentation delivered at the 2023 international conference on political abuses of psychiatry. It synthesises expert testimony and comparative case analysis, addressing institutional repression, misuse of forensic psychiatry, and systemic violations.

Annex H translates the core findings of this dissertation into a structured policy summary. It proposes a roadmap for reform based on legal, clinical, and scientific evidence, aligned with constitutional protections and international human rights norms.

Annex I contains the media dissemination kit prepared to ensure broad public visibility of the dissertation's findings. It includes visual materials, press strategy, and communication tools for knowledge translation and open access outreach.

Annex K outlines the Horizon Europe proposals developed as direct extensions of this research. It describes the implementation logic, collaborative alliances, and policy relevance of the one health-based interventions submitted for European funding.

Annex J presents additional tables, on the historical, cultural and biomedical context of the thesis

Readers are advised to approach the present work as both a formal scientific contribution and a testimony-driven intervention aimed at transforming harmful institutional practices. The language employed is at times severe, a deliberate choice to reflect the gravity and persistence of the violations documented. While not every chapter expands on full legal or computational developments, the thesis maintains methodological integrity and conceptual coherence in service of its emancipatory objectives. This dissertation emerges from conditions of extreme institutional violence, sustained interpersonal abuse, and systemic neglect, directly experienced by the author and corroborated through preventable tragedies that remain ongoing within contemporary psychiatric regimes. It offers a scientific ethnography of coercion, betrayal, and structural injustice in mental health systems, taking Spain as its principal site of inquiry, with comparative insights from international contexts. Guided by a biocultural and action research methodology, the study interrogates the entanglement of forced pharmacological intervention, diagnostic arbitrariness, and systemic disempowerment. It documents how psychiatric and bureaucratic mechanisms, often cloaked in clinical or juridical language, perpetuate exclusion, inflict iatrogenic harm, and erode relational and existential continuity.

The empirical corpus includes mixed methods surveys, verified clinical and legal documentation, and extensive ethnographic fieldwork, all presented with methodological precision. A clear distinction is maintained between analytical results and reflexive testimony. While the author's embedded positionality is methodologically activated in specific chapters, the core findings retain their evidentiary grounding. This dual structure, blending systematic inquiry with situated critique, ensures thematic coherence and transparency throughout. More than a conventional academic investigation, this dissertation constitutes a deliberate ethical and political intervention. It outlines a viable path toward institutional transformation anchored in non negotiable human dignity, relational justice, and structural accountability. Situated within the broader framework of one health and aligned with global human rights principles, it offers a robust foundation for urgently needed reform in psychiatric, legal, and educational domains.

To support maximal accessibility, the online version of this dissertation is being continuously updated and improved. It includes translations, language adaptation modules, and accessibility features for cognitive, sensory, and motor diversity. The repository is also set to host all relevant used open source tools, as well as version controlled instruments, coding frameworks, and analytic resources, including open courseware, reality check lists, research, medical and administrative protocols, and other resources to further facilitate the implementation and constant improvement. That living archive, not included in this file, is designed to serve diverse publics and collaborative networks committed to justice, healing, and systemic change.

Both this guide and the map, point to the future.

Social Impact of the Thesis

This dissertation generates direct and indirect contributions to public health, clinical practice, legal governance, and the design of equitable social systems. It documents and analyzes how coercive psychiatric practices, particularly the routinization of non-consensual medication and diagnostic overreach, compromise individual health outcomes, reduce professional accountability, and undermine the structural integrity of mental health systems. These practices not only violate ethical standards and clinical guidelines but also produce chronic harm that extends beyond the individual to families, communities, and institutions. The research demonstrates that coercion and sustained overmedicalization generate long-term physiological, psychological, relational, and legal sequelae. These include medication-induced comorbidities, iatrogenic disability, weakened cognitive and affective capacities, deterioration of trust in health services, and the social deactivation of entire groups through administrative incapacitation and symbolic exclusion. The dissertation offers new evidence to support systemic reforms aligned with the United Nations Sustainable Development Goals, specifically those related to ensuring healthy lives and promoting well-being for all at all ages (table 1), achieving gender equality, reducing inequality within and among countries, promoting inclusive and equitable quality education (table 2), and building peaceful and just institutions that provide access to justice and protect fundamental rights (table 3).

Table 1. Biological dimension: somatic health, medication practices, and physiological outcomes

SDG	Goal Title	Relevance to Dissertation
2	Zero Hunger	Emphasizes the metabolic, nutritional, and somatic effects of psychotropic medication, especially in the context of neglecting dietary needs.
3	Good Health and Well-being	Central to the thesis. Promotes rights-based and clinically effective care to reduce iatrogenic harm and optimize physiological health outcomes.
6	Clean Water and Sanitation	Addresses environmental health risks linked to pharmaceutical waste from chronic overprescription in psychiatric systems.
12	Responsible Consumption and Production	Advocates for rational pharmacological use, deprescription, and evaluation of medication-related harm within psychiatric services.
13	Climate Action	Promotes sustainable healthcare systems resilient to ecological and pharmaceutical crises, with attention to biological vulnerabilities.

This table presents the SDGs that intersect with the biological aspects of psychiatric care, including the physiological effects of psychotropic medication, metabolic health, environmental contamination through pharmaceutical waste, and the biological implications of overmedication and poor nutritional integration in mental health systems.

The findings show that psychiatric coercion ought to be understood as a generator of medically and socially quantifiable harm. It contributes to the onset and exacerbation of somatic illness, increases risk factors for suicide, and contributes to the erosion of the basic conditions for health, including relational stability, legal protection, and the preservation of bodily autonomy. These harms are often concentrated among persons already exposed to cumulative disadvantage. Women, minors, trauma survivors, people living in poverty, and racialized or migrant populations are disproportionately subjected to forced treatment, pathologization, and institutional neglect (table 2). These practices

conceal rather than address the material and relational determinants of distress. Instead of receiving protection, support, or access to social services, individuals are funneled into psychiatric systems where their experience is reframed as disorder, and their legal and civic standing is progressively eroded. The consequences include loss of custody of children, deterioration of physical health due to polypharmacy, breakdown of social networks, and persistent exclusion from employment, education, and housing systems. The dissertation identifies these mechanisms as producing a measurable form of social death, characterized by administrative invisibilization, narrative erasure, and the loss of control over one's own medical and legal trajectory.

Table 2. Psychological dimension: mental autonomy, identity, recovery, and professional knowledge

SDG	Goal Title	Relevance to Dissertation
3	Good Health and Well-being	Prioritizes recovery-oriented models that preserve narrative identity, legal agency, and emotional regulation.
4	Quality Education	Supports trauma-informed psychiatric education, patient literacy, and reflexive professional practice.
5	Gender Equality	Addresses the psychological consequences of gender-based violence and the need for gender-sensitive diagnostic and therapeutic approaches.
8	Decent Work and Economic Growth	Focuses on restoring occupational agency and reducing psychiatric exclusion from the labor force through empowerment and reintegration.

This table identifies SDGs relevant to the psychological domain, focusing on autonomy, identity, trauma recovery, educational access for both professionals and patients, and the restoration of personal agency through ethically informed psychiatric practice.

In addition to its clinical and legal dimensions, the dissertation foregrounds the necessity of social interventions as prerequisites for mental health. Safe housing, adequate nutrition, economic security, and relational continuity are presented not as ancillary concerns but as central components of psychiatric care. Mental health cannot be separated from the conditions that sustain life. In this sense, the thesis contributes to the articulation of a medical paradigm that is structurally competent, ecologically integrated, and sensitive to the distribution of power and resources. It argues that the prevention of psychiatric crisis, chronicity, and suicide depends not only on pharmacological management but on the deployment of cross-sectoral public policies that address structural deprivation. The research supports the construction of care systems that prioritize early protection of children, sustained support for caregivers, and the prevention of domestic and institutional violence. It emphasizes that without the protection of infancy, and without the capacity of institutions to detect and respond to violence and neglect at early stages, the escalation of suffering into diagnosable illness becomes normalized. This constitutes not only a failure of psychiatry but a failure of medicine and governance more broadly.

The action-research framework applied in this thesis demonstrates that alternative models of care; based on shared decision-making, gradual and safe deprescription, ethical evaluation of practices, and the reintegration of users as agents of knowledge; can reduce harm, prevent institutional dependency, and increase long-term quality of life for patients and families. These models improve

clinical outcomes, reduce the burden on emergency and inpatient services, and contribute to the rational allocation of medical and social resources. They promote sustainable care practices that integrate biological, psychological, legal, and cultural dimensions of health, in line with the principles of a health system oriented toward prevention, continuity of care, and intersectoral cooperation. The findings show that the implementation of dialogical frameworks, supported by legal safeguards and institutional transparency, can create conditions where mental health care is not a site of control and containment but one of recovery, participation, and sustained well-being.

Table 3. Social dimension: legal structures, institutional design, inequality, and relational inclusion

SDG	Goal Title	Relevance to Dissertation
1	No Poverty	Demonstrates how deprivation is reframed as illness, and how economic supports are essential to reducing coercion and exclusion.
9	Industry, Innovation and Infrastructure	Supports ethical infrastructure for mental health governance, including open-source tools for monitoring coercion.
10	Reduced Inequalities	Documents how coercion disproportionately targets marginalized groups, and proposes structural reforms for equitable access and treatment.
11	Sustainable Cities and Communities	Advocates for integrated community mental health services that include housing, networks of care, and civic participation.
16	Peace, Justice and Strong Institutions	Central to the thesis. Promotes legal safeguards, institutional accountability, and protection from psychiatric abuse.
17	Partnerships for the Goals	Emphasizes transdisciplinary and user-survivor collaboration in service design, implementation, and governance.

This table outlines SDGs that relate to the social structures shaping psychiatric experiences, including legal accountability, anti-poverty measures, reduction of systemic inequality, infrastructure development for equitable care, and the creation of participatory institutions capable of protecting human rights and social inclusion.

The social impact of this dissertation (table 3) lies in its capacity to inform structural reform across medical, legal, and policy systems. It provides empirical and conceptual tools for institutional learning, redefines psychiatric responsibility, and advances a clinical and ethical framework in which health is understood as a relational and collective condition. It analyses the layered determinants of preventable deterioration and premature death, including substance-related mortality, iatrogenic illness, and loss of function. Rejecting reductionist models that isolate suffering from its structural context, it builds an integrated analytic grounded in medical anthropology, cognitive science, and legal reasoning. It distinguishes immediate drivers such as deprivation, coercive care, and environmental harm from deeper failures in governance, legitimacy, and epistemic recognition. Drawing on ethnographic and surveys data, this dissertation formulates a translational framework for the identification and correction of systemic dysfunction from bleeding edge knowledge and expert testimonies, from lived and professional accounts. This framework underpins the EU BEACON One Health Education initiative on infancy education for a sustainable

future through new technologies, offering an applied model for interdisciplinary coordination, rights-based reform, and the restoration of health as a structural and ethical global imperative.

Researcher short biography

The author of this dissertation (it/those, showing the dehumanization and betrayal suffered at the hands of severe abusers, who regarded its life as worse than garbage and a nuisance to beat and throw away) is a university professor and physician-scientist in training, currently undertaking doctoral research at the Medical Anthropology Research Center of Universitat Rovira i Virgili, Spain. Its academic formation spans neuroscience, biological and evolutionary anthropology, psychiatric genetics, and dialogical approaches to mental health. It holds postgraduate degrees in collective mental health and human diversity and is the recipient of a highly competitive excellence grant awarded by the Spanish Ministry of Universities for university professors in training. Its research integrates biomedical, legal, and ethnographic perspectives to address core tensions between coercion, autonomy, and recovery in psychiatric care. Before initiating this doctoral project, as undergraduate neurosciences and smart systems student, it acquired direct exposure to the biomedical system as a participant in two phase 0 clinical trials to earn a living and fund its studies, including inpatient follow-up, aiming to understand the infrastructure and epistemology of human pharmacological testing from within.

Its career trajectory combines advanced scientific training with early and sustained engagement in international legal research, including work at the Stanford Center for Legal Informatics (CodeX) as contractor, the Graduate Institute of International and Development Studies in Geneva, and the University of Arizona College of Law. Its previous research focused on cognitive bias, decision-making under uncertainty, and the sociopolitical dynamics of arbitration systems. More recently, it contributed to psychiatric genomics research at FIDMAG Hermanas Hospitalarias, investigating the neurobiological substrates of diagnoses such as schizophrenia at master level. Rooted in a family history of transgenerational trauma and informed by personal and academic proximity to structural violence, its approach to psychiatry is critical yet grounded, oriented not toward denunciation but toward scientific clarity and systemic improvement. Its work unfolds across clinical, legal, and community settings, involving direct observation, stakeholder interviews, and participation in European and international networks focused on mental health and human rights. The dissertation is both a contribution to academic knowledge and a call for institutional responsibility, aligning medical care with the principles of justice, dignity, and recovery.

The researcher was born into a culturally asymmetrical and economically stratified kinship system, with opposing configurations of power, ethics, and social function across parental lineages. The maternal line, predominantly Catalan, was structurally embedded in an upper-class inheritance system, marked by sustained access to property, symbolic capital, and social status not earned through labor but through intergenerational transfer. Within this setting, superiority was culturally and socially internalized, producing patterns of authority and entitlement reinforced by regional identity. This lineage displayed recurrent behavioral control strategies, including infantilization, boundary manipulation, and strategic dependence induction, normalized within the domestic sphere. The institution of marriage, under historical Spanish civil law, enabled legal and economic control by the husband, whose post-marital behavior transitioned from performative romance to dominance. These conditions were legally sanctioned under a patriarchal civil code in force until reforms

introduced progressively during the late 1970s and early 1980s, which had previously conferred disproportionate authority to male heads of household over spousal and child-related decisions.

In contrast, the paternal line functioned within a structurally distinct paradigm. While subject to relative economic constraint, it preserved relational ethics grounded in reciprocity and caregiving. Despite the pressures of traditional gender roles, the paternal grandparents sustained a marital dynamic characterized by mutual support and resilience. While socially less privileged, it maintained relational coherence, labor-based dignity, and pedagogical care practices. The younger son, an engineer by training, demonstrated cognitive and emotional integrity prior to external intervention, following a healthy lifestyle as well as rich social life. Subsequent psychiatric labeling and institutional involvement precipitated rapid deterioration, with no recovery trajectory observed, consistent with existing literature on iatrogenic harm under coercive psychiatric regimes. The grandparents, despite age-related vulnerability, modeled ethical restraint and caregiving integrity.

The behavioral patterns in the maternal lineage were typified by psychological destabilization strategies, covert dependency induction, and systemic displacement of responsibility. The most cognitively divergent and ethically consistent member, a nurse who had qualified for medical studies, was pathologized through the instrumental use of psychiatric diagnosis. Her case exemplifies the conflation of dissent with disorder, and the use of institutional labeling to neutralize functional difference within authoritarian family systems. The differential outcomes between lineages are not incidental but structurally conditioned by the distribution of power, recognition, and control over narrative legitimacy.

The researcher's early exposure to this bifurcated kinship configuration, including forced exit from the household before legal majority and subsequent international mobility through early marriage and expatriation, produced the necessary preconditions for a multiscalar analytical orientation. Longitudinal immersion in socioculturally distinct contexts, including Chile, United Kingdom, France, China, Sweden, and multiple regions of Spain, expanded the observational repertoire and enabled comparative systems analysis of familial governance, psychiatric institutionalism, and the legal-medical interface. The anthropologist, as a scientific instrument, is not separate from the field but part of its relational matrix. Ethnographic knowledge production is grounded in situated perception, and depends on the capacity to register and interpret patterns of interaction, meaning, and harm across multiple sensory, cognitive, and affective registers. Anthropology, more than any other discipline, requires the total engagement of the researcher's embodied faculties. Observation is not visual alone, but haptic, auditory, contextual, and juridically attuned. The instrument ought to be trained and calibrated, not only through academic preparation, but through experiential exposure to the mechanisms under analysis. In this case, the instrument was subject to torture all along, and still is. This dissertation exemplifies work under such distress. The researcher functions simultaneously as observer, survivor, and systems analyst. The knowledge generated is not merely empirical but operational, designed for application in legal, clinical, and policy domains. The research methodology reflects full embodiment, and the conclusions produced are not extrapolations from distance, but derivations from proximity, triangulated through legal doctrine, institutional mapping, and ethnographic evidence. In anthropological science, the integrity of the instrument determines the validity of the result. This principle is upheld in the present work.

Abstracts in all official languages in Spain, and other most relevant

Abstract: This thesis argues that the Spanish psychiatric system, supported by legal and administrative structures, systematically violates the autonomy and dignity of individuals through coercive and non-consensual practices masquerading as care. Diagnoses are imposed without empirical rigor, treatments are administered without valid consent, and discomfort is decontextualized and medicalized, producing structural iatrogenic damage that prevents recovery. At the root is the lack of implementation of shared decision-making and respect for the right to self-determination. Anchored in participatory action research from a biocultural analysis perspective, this work integrates ethnographic, legal, and analytical evidence to document these mechanisms and propose grounded alternatives focused on human rights, prevention, and collaborative care. The findings corroborate the existing medical consensus that ethical and effective health care, which also includes mental health in the most serious cases, ought to be rights-based, non-coercive, recovery-oriented, personalized, and based on trust and shared responsibility, not submission.

Ringkasan: Ini adalah argumen bahwa sistem psikologi Spanyol, yang berfungsi untuk membangun struktur hukum dan administrasi, melanggar sistem otonomi dan martabat individu melalui praktik pemaksaan dan tidak ada konsensus dari para mascarad tentang perhatian mereka. Diagnostik tersebut diterapkan secara empiris, sistem administrasinya memiliki persetujuan yang valid dan bersifat dekontekstualisasi dan medis, menghasilkan struktur iatrogenik yang menghambat pemulihan. Hal ini merupakan tugas pelaksanaan tekanan keputusan bersama dan menghormati hak penentuan otomatis. Memasukkan penyelidikan-tindakan partisipatif dari analisis penting biokultural, meminta tiga bukti integrasi etnograf, hukum, dan analitis untuk mekanisme dokumenter dan mengusulkan alternatif-alternatif yang dilakukan di pusat-pusat manusia, pencegahan dan kolaborasi. Masalah-masalah tersebut menguatkan konsensus medis yang ada bahwa perawatan sanitasi yang efektif dan efektif, yang juga termasuk memberi salam mental pada kasus-kasus yang lebih besar, telah menjadi sangat lemah, tidak ada paksaan, berorientasi pada pemulihan, personalisasi dan berdasarkan kepercayaan, dan tanggung jawab bersama, tidak dalam penyerahan.

Résumé : Cet argument affirme que le système psychiatrique espagnol, soutenu par les structures juridiques et administratives, viole systématiquement l'autonomie et la dignité des individus dans le cadre de pratiques coercitives et aucun consensus ne s'éloigne de l'attention. Les diagnostics imposent une rigueur empirique, les traités sont administrés avec un consentement valide et le problème est décontextuel et médical, produisant une structure iatrogénique qui empêche la récupération. Dans l'affaire, il y a la manque de mise en œuvre de la pression des décisions partagée et le respect du droit à l'autodétermination. Ancorat en la investigació-acció participativa d'un vessant d'anàlisis biocultural, ce treball integra des preuves ethnogràfiques, jurídiques et analytiques pour documenter ces mécanismes et proposer des alternatives fondées sur les principes centraux chez les humains, la prévention et l'attention collaborative. Les troballes corroborent les consensus médicaux existants selon lesquels l'attention sanitaire est efficace et efficace, qui inclut également le salut mental dans les cas les plus graves, qui ont été fondés sur des droits, non coercitifs, orientés

vers la récupération, personnalisés et fondés sur la confiance et la responsabilité partagée, non dans la soumission.

Laburpena: Espainiako sistema psikiatrikoaren argudioa, egitura juridikoen eta administratiboen babesa, autonomiaren sistema urratzea eta pertsonen duintasunari buruzko koerzitiboen praktikaren bidez eta arretarako adostasunik ez duten emmascaratzeen bidez. Zentzumen zorroztasun enpirikoa inposatzen dute diagnostikoek, tratamenduak zentzumenaren adostasuna administratzen dute, malestarren deskontextualizazioa eta medikuntzaren ondorioz, errekuperazioa eragozten duten egitura iatrogeniko bat ekoizten dute. A l'arrel hi ha la manca d'implementació de la presa de decisions compartida i el respect pel dret a l'autodeterminació. Ancorat en la investigació-acció participativa des d'una vessant d'anàlisis biocultural, hau integratzen den evidències etnogràfiques, legals i analítiques per dokumentatzeko mekanismoak eta proposar alternatives fonamentades centrades en els derechos humans, la prevenció i l'atenció col·laborativa. Les troballes corroboren el consens medical existent que l'atenció sanitària ètica i eficaç, hori ere barne hartzen du la salut mental en los casos más greus, no ha de ser fundada en derechos, no coercitiva, orientada a la recuperación, personalizada y basada en la confianza y la responsabilidad compartida, no en la submisión.

Resumo: Tesí argumenta que o sistema psiquiàtric espanyol, sostingut por estructures legais e administrativas, viola sistemàticament l'autonomia e a dignitat dos individuos a través de prácticas coercitivas e non consensuades emmascarades com a atenció. Os diagnòstics imponse o rigor empírico do sentido, os tratamentos s'administren o consentimiento do sentido válido e o malestar é descontextualiza e é medicaliza, producindo un dany iatrogènic estructural que impediú a recuperación. A l'arrel hi ha la manca d'implementació da presa de decisions compartida i el respecte pel dret a l'autodeterminació. Ancora na investigación-acción participativa des d'una vessant d'anàlisis biocultural, este traballo integra evidències etnogràfiques, legals e analítiques por documentar estes mecanismos e propoñer alternativas fonamentadas centradas nos dereitos humanos, a prevención e a atención col·laborativa. As troballes corroboren os consensos médicos existentes que a atención sanitària ètica e eficaç, que tamén inclúe o salut mental nos casos máis greus, ha de ser basado en dereitos, non coercitiva, orientada á recuperación, personalizada i basada na confianza e na responsabilidade compartida, non na submisión.

Resum: Aquesta tesi argumenta que el sistema psiquiàtric espanyol, sostingut per estructures legals i administratives, viola sistemàticament l'autonomia i la dignitat dels individus a través de pràctiques coercitives i no consensuades emmascarades com a atenció. Els diagnòstics s'imposen sense rigor empíric, els tractaments s'administren sense consentiment vàlid i el malestar es descontextualitza i es medicalitza, produint un dany iatrogènic estructural que impedeix la recuperació. A l'arrel hi ha la manca d'implementació de la presa de decisions compartida i el respecte pel dret a l'autodeterminació. Ancorat en la investigació-acció participativa des d'una vessant d'anàlisis biocultural, aquest treball integra evidències etnogràfiques, legals i analítiques per documentar aquests mecanismes i proposar alternatives fonamentades centrades en els drets humans, la prevenció i l'atenció col·laborativa. Les troballes corroboren el consens mèdic existent que l'atenció sanitària ètica i eficaç, que també inclou la salut mental en els casos més greus, ha de ser basada en drets, no coercitiva, orientada a la recuperació, personalitzada i basada en la confiança i la responsabilitat compartida, no en la submissió.

Resumen: Aquesta tesi argumenta que el sistema psiquiátrico español, sustentado por estructuras legales y administrativas, viola el sistema de autonomía y la dignidad de los individuos a través de prácticas coercitivas y no consensuadas emmascaradas com a atención. Los diagnósticos se imponen con sentido de rigor empírico, los tratamientos se administran con sentido consentimiento válido y el malestar es descontextualización y es medicalización, produciendo una iatrogenesis estructural que impide la recuperación. A l'arrel hi ha la manca d'implementació de la presa de decisiones compartida i el respecte pel dret a l'autodeterminació. Ancorat en la investigació-acció participativa des d'una vessant d'anàlisis biocultural, aquest treball integra evidències etnogràfiques, legals i analítiques per documentar aquests mecanismes y proponer alternativas fonamentadas centradas en los derechos humanos, la prevención y la atención colaborativa. Les troballes corroboren el consenso médico existente que la atención sanitaria ética y eficaz, que también incluye la salud mental en los casos más graves, ha de ser basada en derechos, no coercitiva, orientada a la recuperación, personalizada y basada en la confianza y la responsabilidad compartida, no en la sumisión.

English: shared decision-making; coercion; iatrogenic harm; human rights; structural violence; **Indonesian:** pengambilan keputusan bersama; pemaksaan; kerusakan iatrogenik; hak asasi manusia; kekerasan struktural; **French:** prise de décision partagée; coercion; dommages iatrogènes; droits humains; violence structurelle; **Euskera:** erabaki partekatuak; behartzea; kalte iatrogenikoak; giza eskubideak; indarkeria estrukturala; **Gallego:** toma de decisións compartida; coacción; dano iatrogénico; dereitos humanos; violencia estrutural; **Catalan:** presa de decisions compartida; coerció; dany iatrogènic; drets humans; violència estructural; **Español (chavacano):** desisión compartida; forzamiento; daño iatrogénico; derechos humanos; violencia estructural.

Chapter 1. General framework and delimitation of the research

Summary: This chapter establishes the foundational premises of the dissertation by articulating the structural urgency and scientific rationale for investigating psychiatric violence, institutional betrayal, and systemic repression within contemporary mental health frameworks, particularly in the Spanish context. Through a biocultural and action-research perspective, the chapter situates the inquiry at the intersection of medical anthropology, legal analysis, and lived experience, drawing on a wide empirical base including fieldwork, policy analysis, and professional testimony. It sets forth the critical claim that coercion in psychiatry is not an exceptional measure but a normalized, routinized mechanism of social control embedded in the logic and everyday operation of clinical institutions. This chapter exposes the failure of dominant psychiatric paradigms to address the roots of distress, emphasizing how the structural causes of suffering; such as poverty, abuse, gendered violence, and institutional betrayal, are routinely depoliticized and converted into individual pathologies. This chapter further outlines how institutional discourse and practice reconfigure adaptive or protest behaviours into markers of disorder, legitimizing forced treatments that often produce iatrogenic harm and psychological disablement. By examining the erosion of autonomy, the silencing of narrative agency, and the moral disengagement inherent in coercive systems, it calls attention to the systematic reproduction of suffering under the guise of care. It also presents the current configuration of mental health services in Spain, in the European Union, and comparable systems, violates fundamental rights, degrades the clinical relationship, and undermines democratic and constitutional protections. It underscores that such violence is not simply medical but deeply political and epistemic, supported by bureaucratic inertia, legal ambiguity, and a culture of professional impunity. By framing mental health as a matter of public ethics, structural justice, and ecological responsibility, this chapter lays the groundwork for the dissertation's integrated methodological and theoretical architecture and announces the urgency of a rights-based, trauma informed, non-harmful, society-wide, gendered, transcultural and recovery oriented one health approach to a most human mental health reform. This chapter delineates the core hypotheses and ethical orientation of the study, emphasizing that transformation ought to be grounded in non-coercive, dialogical, and structurally aware practices.

1. Thesis, aim and objectives

Thesis: This dissertation investigates how psychiatric systems can transition from coercive and overmedicalized practices toward evidence-based models of recovery-oriented care, grounded in patient autonomy, shared decision-making and culturally situated best practices.

This dissertation investigates how psychiatric systems can transition from coercive and overmedicalized practices toward evidence-based, recovery-oriented models of care grounded in patient autonomy, shared decision-making, and culturally contextualized clinical reasoning. The thesis posits that coercive and pharmacologically dominant interventions in contemporary psychiatry are not exceptional nor residual but are instead structurally maintained by institutional

norms, juridical indeterminacy, and administrative fragmentation. These practices remain embedded across clinical, familial, and bureaucratic domains, producing a continuum of control that obstructs recovery, suppresses agency, and perpetuates social and physiological harm. Coercion, conceptualized herein as the systematic exclusion of the individual from decisions concerning their own treatment, functions as a legitimized institutional response to psychosocial distress, often replacing substantive care with enforced compliance. The research argues that such practices reflect not the failure of isolated actors but the normalization of technocratic logic under conditions of risk aversion, diagnostic reductionism, and epistemic asymmetry (Torrents & Björkdahl, 2024).

The thesis advances the proposition that a sustainable transformation of psychiatric care requires not only technical amendments to protocols or enhanced practitioner training but a systemic reconfiguration toward dialogically informed, procedurally accountable, and structurally responsive forms of care. Dialogical practice, in this context, is not reduced to a specific branded methodology but conceptualized as a foundational epistemological principle grounded in mutual recognition, institutional reflexivity, and reciprocal legitimization of knowledge claims. The empirical validation of this proposition is carried out through a mixed-methods framework incorporating legal analysis, clinical ethnography, and embedded participant observation across multiple psychiatric and policy environments (Mead & Filson, 2017; Carr, 1988). The findings elucidate how normative frameworks often coexist with violations in practice, and how professionals themselves operate under constraints that undermine ethical care. The thesis does not merely critique, but delineates institutional pathways toward coherent, lawful, and recovery-promoting systems of mental health governance (Riker, 2017).

This dissertation is guided by the central research question: How do coercive psychiatric practices, overmedication, and institutional neglect persist within the Spanish mental health system despite legal safeguards, clinical ethics, and international human rights standards? Conversely, it also asks: What systemic, clinical, and epistemological conditions enable the emergence and consolidation of participatory, trauma-informed, and biologically grounded psychiatric care that respects autonomy, protects rights, and improves outcomes?

Table 1. Alignment between core dissertation aims and operational research questions

Dissertation Aim	Aligned Research Question	Operational Objective
Support psychiatric systems capable of real-time correction, accountability, and integrity preservation	What forms of systemic violence and institutional failure are occurring in psychiatric governance, and how can they be corrected?	Identify and analyze the structural, legal, and epistemological failures in psychiatric care to develop evidence-based reforms
Foster a paradigmatic shift from pharmacological control to integrative, interdisciplinary, and reciprocal models of care	How can clinical and experiential evidence be integrated across biological, psychological, and social domains of health?	Develop and validate feedback systems, biosignal-informed tools, and patient-led evaluative mechanisms
Realign psychiatry with empirical coherence, legal safeguards, and rights-based participatory governance	What are the enabling conditions for institutional change and participatory oversight of psychiatric systems?	Implement translational infrastructures, policy interventions, and participatory monitoring aligned with international standards

Table 1 presents the alignment between the core aims of the dissertation and the research questions and operational objectives introduced in section 1.9. Each component of the central thesis is matched with a guiding question and its corresponding implementation strategy. These elements are further developed in the methodological framework detailed in section 1.9 and operationalized through the research design presented in Chapter 2.

Aim: To contribute to the development and implementation of clinical, legal and institutional frameworks that promote self-managed and participatory mental health care, based on action-research embedded in real-world settings and oriented toward continuous system improvement.

The aim of a thesis refers to its principal purpose or intended contribution. It articulates what the research seeks to achieve at a systemic or conceptual level, distinguishing it from the thesis itself, which presents a central proposition to be argued and demonstrated. While the thesis posits a testable claim grounded in empirical and theoretical evidence, the aim defines the overarching orientation of the work, including its intended impact on policy, practice, or scientific understanding. In action-research frameworks, such as the one adopted here, the aim also encompasses applied objectives and transformative intentions embedded within real-world systems.

The aim of this dissertation is to support the emergence and implementation of psychiatric systems that are capable of real-time correction, structural accountability, and the preservation of both bodily and legal integrity. It seeks to foster a paradigmatic transition away from a unidimensional reliance on psychopharmacological control toward an integrative model of care rooted in interdisciplinary evidence and therapeutic reciprocity. This model is predicated on the understanding that mental health cannot be reduced to neurochemical imbalance or behavioral deviation but ought to be interpreted through the interaction of biological regulation, experiential development, and sociopolitical context. The research integrates contributions from translational neuroscience, physiological psychiatry, psychoneuroimmunology, psychoneuroendocrinology, developmental psychopathology, trauma-informed frameworks, nutritional psychiatry, psychosocial epidemiology, and ecological public health. These disciplines provide converging evidence on how trauma, inflammation, metabolic dysregulation, and environmental precarity shape both the emergence and chronicity of psychiatric conditions (Gooding et al., 2022).

In parallel, the dissertation draws from cultural psychology, phenomenological psychiatry, narrative theory, and medical anthropology to situate clinical phenomena within relational, cultural, and

historical matrices. Theories of embodied cognition, the extended mind, and relational consciousness are employed not as speculative frameworks but as scientifically corroborated perspectives on the distributed and socially mediated nature of mental processes. This conceptual orientation is operationalized through the design of participatory feedback mechanisms, patient-led evaluative structures, and embedded data infrastructures, allowing clinical systems to recalibrate based on experiential, physiological, and ethical inputs. The aim is not to displace psychiatry but to realign it with a broader horizon of empirical coherence, patient protection, and epistemic humility. The dissertation intervenes at the intersection of clinical decision-making, institutional design, and legal structure, offering pragmatic models of regulation and care grounded in ongoing verification, transparent governance, and open science standards (Suess Schwend et al., 2016).

The integration of these disciplines and methodologies informs a comprehensive framework through which psychiatric governance can become both scientifically valid and socially just. The objectives derived from this aim are elaborated across biological, psychological, and social domains of intervention and correspond to the structure of this dissertation. These objectives are detailed below and implemented through rigorous multi-sited fieldwork, participatory action research, and critical legal-ethical analysis:

Table 2. Biological objectives and implementation strategies

Relevant SDG	Objective	Method of Implementation
SDG 3 – Good Health and Well-being	Conduct sustained participant observation within clinical, legal, and policy environments, engaging directly with professionals, users, families, and decision-makers to co-produce relevant knowledge without causing harm.	Multi-site fieldwork across Spain, Indonesia, and Sweden through ethnographic immersion, informal interviews, and protected co-analysis with affected individuals and professional stakeholders.
SDG 9 – Industry, Innovation and Infrastructure	Critically examine the cultural, scientific, and legal foundations of current psychopharmacological standards, particularly in relation to overmedication and the marginalization of alternative approaches.	Historical-legal review of drug regulation, comparative analysis of national psychiatric protocols, and interviews with critical psychiatry scholars and practicing clinicians.

This table outlines the objectives and methodological approaches related to the biological dimensions of health, focusing on empirical fieldwork and critical examination of pharmacological norms. These components align with SDG 3 and SDG 9 and support evidence-based transformation of clinical practice and regulation.

The biological dimension of this dissertation, table 1, integrates an exploration into the full potential of current best practices, a human approach to rapport, trust and both shared decisions and responsibility, and new technologies to support precision, accountability, and continuous improvement in clinical practice. The thesis maintains that health ought to be understood not as the management of isolated symptoms but as the result of dynamic interactions between biological integrity, contextual conditions, and institutional responsibility. In Spain, persistent overreliance on pharmacological protocols has entrenched a medical model that neglects both systemic harms and the possibility of recovery. This work challenges such structural inertia by introducing open data frameworks, biosignal-informed monitoring, and interoperable documentation systems that

empower patients and clinicians to co-evaluate treatment effects in real time. By embedding positive feedback loops within participatory action research cycles and adopting open science standards, the synthesis model reorients health systems toward adaptive learning and biomedical accountability. Legal safeguards and regulatory mechanisms are proposed to ensure transparency, protect patient data, and prevent the misuse of diagnostic categories and therapeutic interventions that might otherwise reinforce chronicity and systemic exclusion.

Table 3. Psychological objectives and implementation strategies

Relevant SDG	Objective	Method of Implementation
SDG 4 – Quality Education	Document failures in clinical responsibility, mechanisms of harm, and systemic omissions, while focusing on how feedback-informed practices can improve recovery trajectories and therapeutic alliance.	Review of malpractice cases, collection of lived experience reports, and evaluation of outcome measures in therapeutic relationships.
SDG 5 – Gender Equality	Trace how diagnostic processes can contribute to identity erasure and legal dispossession, and propose safeguards that protect continuity of personhood, legal status, and relational integrity.	Analysis of gendered diagnostic trajectories, legal documentation of incapacitaciones, and testimony-led mapping of institutional violence.

This table presents the objectives linked to psychological dimensions of health and care, addressing clinical responsibility, diagnostic integrity, and gender-based harms. The methodologies integrate lived experience and structural analysis, contributing to SDG 4 and SDG 5.

At the psychological level, table 2, new technologies are mobilized not for surveillance or standardization, but for restoring agency, memory, and personhood within therapeutic processes. This thesis defends the centrality of narrative, self-determination, and cognitive restoration as key pillars of healing. The antithesis it opposes is a clinical culture that too often ignores subjective complexity, forces diagnostic imposition, and strips the individual of interpretive power. To counter this, the research deploys digital storytelling tools, reflexive platforms for therapeutic alliance assessment, and decision-support systems aligned with trauma-informed and rights-based frameworks. Feedback loops are built into the structure of care itself, allowing for regular recalibration and person-centered learning. These systems are designed not only for individual care but for structural change: civil society engagement, patient-led data interpretation, and continuous education of professionals are embedded as core functions. Legal analysis further identifies procedural loopholes and proposes enforceable guarantees to protect against identity erasure and epistemic harm, reasserting the right to evolve beyond labels and to recover fully (Tomé, 2014).

At the social level, table 6, the dissertation advances a synthesis where institutional reform is sustained by real-time coordination, civic participation, and full data transparency. Through the EU BEACON One Health Education initiative, the research supports an architecture for systems integration that enables ongoing structural correction and legal enforceability. Participatory digital infrastructures allow for multilevel input ,from families, professionals, users, and civil society ,creating scalable mechanisms for identifying harmful practices, highlighting best models, and ensuring policy coherence. Open science is not an add-on but a foundational principle: all data is anonymized, encrypted, and made accessible for democratic oversight, with clear limits set to prevent institutional capture or reappropriation. Feedback mechanisms are calibrated to track not

only health outcomes but institutional behavior, closing gaps in enforcement and reducing impunity. The model refuses the assumption of chronicity as an inevitable outcome, insisting instead on a forward-facing ethics of possibility, resilience, and structural repair. In this vision, health is not a static state nor a managed deficit, but a relational, social, and dynamic capacity that ought to be protected and continuously renewed (Baum et al., 2006).

Table 4. Social Objectives and Implementation Strategies

Relevant SDG	Objective	Method of Implementation
SDG 10 – Reduced Inequalities	Gather and analyze testimonies and practices from patients, relatives, clinicians, researchers, and public officials, documenting both existing challenges and emerging models of respectful and effective care.	Narrative data collection, anonymous surveys, legal case analysis, and observation of decision-making processes in institutional settings.
SDG 16 – Peace, Justice and Strong Institutions	Identify enabling conditions for the implementation of autonomous and collaborative medication management, highlighting tensions between institutional norms and lived experience.	Documentation of pilot practices, stakeholder workshops, and field-based monitoring of patient-practitioner negotiation in psychiatric contexts.
SDG 17 – Partnerships for the Goals	Map feedback loops across individual, institutional, and regulatory levels, identifying specific leverage points for positive change in policy and clinical governance.	Systems mapping through participatory modeling, stakeholder interviews, and analysis of international and national policy interdependencies.
SDG 1 – No Poverty	Provide empirically grounded recommendations and methodological tools for the implementation of dynamic, person-centered, and rights-respecting mental health care, where continuous reflection and system learning support long-term recovery and social inclusion.	Development of adaptable care protocols, translation of research findings into clinical training modules, and policy briefs shared with international networks including EU BEACON.

This table identifies objectives targeting the social and institutional determinants of mental health, with a focus on inequalities, legal protections, collaborative governance, and inclusion. These objectives are operationalized in line with SDG 1, SDG 10, SDG 16, and SDG 17.

This dissertation defends a thesis that is both scientifically grounded and ethically imperative: that recovery in mental health is not only possible but ought to be structurally enabled, institutionally supported, and epistemically protected. Against entrenched systems that perpetuate coercion, chronicity, and epistemic injustice under the guise of care, the research offers a multidimensional synthesis grounded in the best available evidence and the lived realities of those most affected. By integrating translational neuroscience, dialogical and developmental models, legal safeguards, and participatory technologies, the dissertation articulates a coherent and actionable framework for system-wide transformation. Its implementation through the EU BEACON One Health Education action, founded by the researcher during this thesis participatory action-research fieldwork, exemplifies how research can inform practice and policy without compromise, ensuring transparency, accountability, and democratic engagement at all levels. Health, in this model, is not a managerial abstraction but a dynamic, relational achievement sustained by procedural integrity, educational investment, and shared commitment to human dignity. The research does not merely critique existing systems; it operationalizes their reconfiguration, affirming that recovery is not to

be the exception, but the ethical and structural baseline from which all psychiatric care ought to begin (Ochocka et al., 2002; Speers & Lathlean, 2015).

1.1. Brief historical-structural background: care, coercion and governance in Spain

The governance of mental distress in the Iberian Peninsula has historically functioned as an apparatus of normative regulation, entwined with broader regimes of religious, political, and social control. From the late medieval period through the early modern era, deviation from established moral and epistemic orders was not merely tolerated or interpreted but actively repressed through institutional mechanisms that conflated spiritual error, cognitive dissonance, and behavioral irregularity. Early Christian and Islamic traditions, though distinct in theological framing, both contributed to a moralisation of suffering in which distress was interpreted as either a sign of divine punishment or a rupture of spiritual integrity. Institutional responses were structured accordingly. Monastic, charitable, and later inquisitorial systems deployed rituals of purification, prayer, fasting, confinement, and exorcism, often without any distinction between psychological, somatic, and moral phenomena. The individual in distress was not treated as a bearer of epistemically valid experience but rather as a symptom of disorder within the social or cosmological order. These dynamics laid the foundation for a mode of care that operated less through therapeutic recognition than through sacrificial containment, where the boundary between cure and exclusion was systematically erased (Van Bilsen, 2016; Asad et al., 2024; Alotaibi, 2025).

By the eighteenth century, the rise of Enlightenment rationalism and biomedical empiricism signalled a partial shift in explanatory frameworks. However, the consolidation of psychiatric knowledge as an institutional field did not dismantle the punitive architecture of earlier systems but rather rearticulated it through secularised classifications and clinical authority. The emergence of asylums across Europe, including Spain, institutionalised this transition. Framed under the ideology of moral treatment, these facilities combined therapeutic rhetoric with disciplinary spatial design, reinforcing the separation between the so-called sane and the insane. In the Spanish context, the provincialisation of mental hospitals entrenched a custodial model in which legal, familial, and professional prerogatives converged to authorise prolonged confinement. Diagnostic categories lacked standardisation and were frequently employed to justify social exclusion, often under the pretext of managing risk or incapacity. Treatment was oriented not toward understanding or recovery, but toward submission, surveillance, and pharmacological restraint. The architectural continuity with earlier religious institutions was paralleled by administrative logics that prioritised order over care and compliance over cure. The structural function of psychiatry thus replicated earlier theological models of containment, though now under the legitimation of scientific modernity (Locke, 1690/1975; Battie, 1751/1975; Tuke, 1796/1975; Pinel & Esquirol, 1838/1994).

Throughout the nineteenth century, Spanish psychiatry underwent professional consolidation, drawing on French and German nosological systems to reinforce the authority of the clinical gaze. Diagnostic taxonomies such as melancholia, mania, and moral insanity became embedded in medico-legal discourse and served as instruments for managing deviance in both public and private spheres. These classifications did not emerge from dialogical engagement with the affected but from

an epistemology of expertise that located the power to define pathology in the professional observer. This process was deeply gendered. The pathologisation of female agency through categories such as hysteria served to institutionalise patriarchal control, while the disproportionate confinement of women for behaviours deemed transgressive, whether sexual, emotional, or spiritual, revealed psychiatry's complicity in enforcing normative femininity. Psychiatric institutions thus became not spaces of healing but mechanisms of biopolitical triage, where the lines between illness, illegitimacy, and nonconformity were collapsed under medical rationality. In this configuration, care was not emancipatory but regulatory, and recovery was subordinated to institutional interests in containment, control, and social reproduction (Rodríguez-Morini, 2002).

In the early twentieth century, Spanish psychiatry was increasingly shaped by eugenic and nationalist doctrines that conflated public health with moral hygiene. During the 1920s and 1930s, institutional policies adopted segregationist and sterilization-oriented frameworks aimed at excluding individuals perceived as degenerative. These tendencies intensified under Francoist rule, where psychiatric institutions became integral to the apparatus of state repression. Rather than spaces for clinical care, they functioned as instruments of political and social control. Those who diverged from national-Catholic ideals, whether due to political dissent, sexual orientation, or trauma-related responses, were subjected to coercive hospitalization, invasive treatments, and indefinite confinement. Psychiatric diagnoses operated as tools to delegitimize non-conforming identities, framing distress as pathology and dissent as illness. Clinical practices merged with authoritarian legal logics, erasing procedural safeguards. Medical records were inaccessible, appeals nonviable, and treatments designed to ensure submission rather than healing. This convergence established a legacy of medical impunity and epistemic opacity that continues to influence institutional norms in contemporary Spain (Espinosa & Comelles, 1966; Tusell, 2010; Alonso-Tejada & López-Morales, 2015).

Following the democratic transition, legislative reforms in the 1980s and 1990s aimed to replace institutional confinement with community-based care and uphold human rights in mental health treatment. Nevertheless, implementation remained partial and under-resourced. Asylums were closed without establishing robust alternatives, and many psychiatric functions were subsumed within general hospitals or delegated to private entities. The underlying paradigm of control remained intact. Professional hierarchies were preserved, diagnostic taxonomies remained unchallenged, and intervention practices continued to prioritize pharmacological sedation over dialogical engagement. Community psychiatry, while rhetorically endorsed, failed to deliver integrated or participatory care. Pharmaceutical interventions, particularly long-acting antipsychotics, gained prominence as first-line responses to non-compliance, effectively substituting behavioral management for therapeutic understanding. Adherence became the central metric of clinical success, regardless of patient outcomes or informed consent. These developments were not driven by evidence-based consensus, but by the institutional convenience of symptom suppression over structural change (García Espino & Lara Ladislao, 1998; Pérez-Fernández & Peñaranda-Ortega, 2015; Bobes, Vázquez-Barquero, & Salvador-Carulla, 2010).

In the twenty-first century, the consolidation of austerity measures, industry lobbying, and professional self-preservation further entrenched pharmacological coercion as normative psychiatric

practice. Involuntary treatment was increasingly authorized through expedited judicial processes with minimal scrutiny. The threshold for declaring incapacity remained low, enabling the systematic erosion of legal agency for those in distress. Psychiatric interventions were framed as protective, yet often functioned to suppress dissenting experience rather than address underlying causes. The language of rights was retained in policy, but its practical application was undermined by the conflation of non-compliance with pathology. Relatives, facing inadequate support and misinformed about alternatives, frequently acquiesced to coercive protocols. Institutional responses privileged sedation over sustained engagement. This continuity reveals a persistent carceral logic in Spanish psychiatry, reframed but not dismantled. The enduring suspicion toward user agency, the marginalisation of narrative understanding, and the systemic resistance to power-sharing confirm that coercive pharmacology is not a scientific inevitability but a culturally and politically embedded legacy that demands structural redress (Ortiz-Gómez, 2002; Portilla, 2006).

Better approaches are and were framed as luxuries or ideological experiments, rather than as scientifically grounded responses to suffering. The language of rights remained in official documents but was emptied of its practical force. Patients were denied meaningful participation in care decisions, and refusal was interpreted not as a statement but as a symptom. Relatives, under pressure and misinformation, often supported coercive measures, hoping for stability at any cost. Institutions responded with sedation, not support. The shift from asylum to community did not dismantle the carceral function of psychiatry. It redistributed it across new spaces and vocabularies.

These trajectories cannot be explained as neutral progressions in medical science. They are deeply rooted in cultural traditions of submission, obedience, and exclusion. Spain's long history of centralized power, religious moralism, and gendered violence shaped the reception and function of psychiatric interventions. Those deemed mad were never fully citizens. They were subjects of care only insofar as care enforced social order. Healing was rarely the objective. The true goal was containment, pacification, and moral correction. This history explains the persistence of coercive pharmacology in Spain today, despite international recommendations advocating for shared decision-making and recovery-oriented practices. It also reveals the institutional reluctance to implement alternatives that require power-sharing, narrative humility, and structural investment.

The transition from confinement to overmedication ought to therefore be seen not as reform but as adaptation. Psychiatric governance in Spain has replaced the visible bars of the asylum with the invisible chains of chemical control. Individuals are no longer locked away but remain captive to institutions that monitor their compliance and dictate their pharmacological regimen. Resistance is recoded as illness, insight as submission, and mutism as progress. The cultural scripts that once justified religious exorcism, moral internment, and fascist segregation now inform clinical discourse dressed in evidence-based language but grounded in disciplinary logic. As a result, mental health care in Spain continues to function less as a system of healing and more as a regime of managed normalization. This trajectory is not limited to those who fit clinical criteria. Historically, Spain's coercive psychiatric apparatus has targeted the poor, the racialized, the gender nonconforming, and the socially undesirable. In the nineteenth and twentieth centuries, asylums and mental hospitals served as containers for surplus populations. Paupers, vagabonds, unmarried mothers, rebellious wives, political enemies, and migrants were expelled from the productive economy and denied full

civic inclusion. Confinement was framed as care but functioned as exclusion. Diagnosis became a tool of administrative convenience. Poverty was pathologized. Deviance was medicalized. The responsibility of the state was transferred into the body of the subject (Wallace Mandell, 1962/2025; Rosenhan, 1973; Foucault, 1961/2006).

This targeting intensified under Francoist rule, particularly against those on the losing side of the war. The psychiatric apparatus was mobilized to eliminate ideological dissent and deviant identity. Homosexuality was pathologized, gender nonconformity punished, and political resistance silenced through medical certification of unfitness. Those who could not be integrated into the national Catholic project were not only criminalized but psychiatrized. They were removed from visibility, subjected to forced treatment, or simply disappeared. This amounted to a slow form of biopolitical genocide, a state-driven erasure of the undesirable through confinement, chemical pacification, and epistemic annihilation. Psychiatry was not a healing discipline but a technology of control. The residues of this function persist today. Spain's mental health system continues to disproportionately affect those on the margins. Migrants, racialized youth, Roma individuals, transgender persons, and survivors of abuse are overrepresented in coercive interventions and underrepresented in participatory care. Universal health coverage coexists with institutional exclusion. The language may have softened but the logic remains. Psychiatry no longer speaks of sin or degeneration but of adherence and insight. Yet, the outcome is the same, the production of order through the containment of difference (Campos & Novella, 2017; Huertas, 2017).

These historical mechanisms of silencing have been refined but not dismantled. They are embedded in laws, in institutional routines, and in clinical reasoning. They manifest in the overprescription of psychotropic drugs, in the routine use of physical and chemical restraints, and in the bureaucratic flattening of suffering into risk profiles. The state no longer requires asylums to achieve its aims. It has found in outpatient coercion, forced pharmacological adherence, and judicial discretion the tools to achieve the same ends through more diffuse and insidious means. The history of psychiatry in Spain is therefore not a story of linear progress. It is one of continuity through transformation. What has changed are the forms, not the functions. From medieval heresy to modern noncompliance, from confinement to injection, from overt moralism to administrative rationality, the thread remains. The goal has always been to suppress what cannot be assimilated. Paupers, madwomen, transgressors, and migrants converge in the institutional imaginary as bodies to be managed for the sake of order. In the name of public health, psychiatry has enacted private violence. In the name of care, it has administered exclusion (Brydán, 2016).

This is the context from which to examine the current reality of forced psychiatric treatment in Spain. Coercion is not an aberration but a structural feature of the system. It is embedded in its genealogy, sustained by its institutions, and reproduced through its discourses. What follows is an analysis of this contemporary landscape, its legal frameworks, clinical arguments, and sites of resistance. This analysis begins from a clear premise. Coercion constitutes harm. The burden of justification lies not with those who resist it but with those who impose it. The next section asks not only what is done but why it is permitted and how it endures.

1.2. Current framework: forced psychiatric interventions and institutional accountability

Forced psychiatric treatment, in the form of involuntary outpatient commitment and compulsory medication, continues to be promoted in Spain by certain clinical and legal actors, despite a substantial body of evidence demonstrating its limited effectiveness and significant ethical cost. Proposals to legislate involuntary outpatient treatment have been presented repeatedly over the past two decades and have been systematically rejected by legislative bodies, professional associations, and user organizations. The arguments in favor of such coercive measures rely on speculative claims that forced compliance with medical regimens will prevent hospital readmissions, reduce dangerous behaviors, and alleviate systemic pressures. These claims remain unsubstantiated by empirical research and lack theoretical coherence when examined within the framework of rights-based clinical practice and evidence-based medicine.

The most comprehensive review to date, conducted using Cochrane methodology, found no consistent benefit of involuntary outpatient treatment over standard care. Key outcome indicators, including hospital readmission rates, social functioning, mental state, quality of life, and satisfaction with care, showed no statistically significant differences between individuals subjected to coercive outpatient orders and those receiving voluntary treatment. Calculations of the number needed to treat reveal ethically untenable implications. For example, preventing a single hospital readmission may require imposing coercive treatment on more than eighty individuals. This magnitude of intervention would be considered disproportionate in any other domain of healthcare or public policy. The continued defense of these measures, therefore, cannot be grounded in scientific evidence but reflects institutional priorities that favor control and risk management over collaboration and patient autonomy (Sailas & Fenton, 2000).

In Spain, the resurgence of legislative initiatives to implement involuntary outpatient treatment stems, in part, from systemic deficiencies in community mental health infrastructure. Policymakers and some clinical representatives argue that coercion is necessary in light of the absence of sustained relational care and the fragmentation of social and health support systems. This framing displaces responsibility for structural failure onto the individual and substitutes forced compliance for the development of comprehensive and supportive services. Legal justifications often rely on expansive interpretations of incapacity and risk, frequently invoking concepts such as lack of insight or treatment refusal without operational definitions. These categories are problematic from both clinical and legal perspectives and are prone to subjective misuse. The attempt to reframe coercion as a preventive measure paradoxically undermines both clinical outcomes and the therapeutic relationship that recovery depends upon (Cañete-Nicolás et al, 2012; Gías Gil, 2013).

By contrast, shared decision-making has consistently been associated with enhanced clinical engagement, improved functional outcomes, and reduced relapse rates. It is a model that affirms the individual's capacity to participate in care decisions, even during periods of psychological distress. The presence of a collaborative therapeutic alliance is the most robust predictor of positive long-term outcomes across mental health conditions. Involuntary measures tend to disrupt this alliance, induce mistrust, and reinforce avoidance and disengagement. The experience of being subjected to forced treatment is associated with psychological trauma, persistent anger, and withdrawal from

support systems. These reactions are not anecdotal nor incidental but constitute evidence of systemic harm that impairs future help-seeking behaviors and compromises adherence to care (Pozón, 2012; Villagrán et al, 2015; Serrano-Miguel, 2022).

Despite the continued application of coercion in psychiatric practice and discourse, there is an emerging international consensus that sustainable recovery requires the implementation of non-coercive interventions. These include intensive home-based care, flexible assertive community treatment, peer-supported crisis services, and culturally adapted psychotherapeutic approaches. Each of these models is grounded in voluntary engagement and recognizes the complexity of mental suffering as relational, contextual, and often trauma-related. Their implementation in diverse settings has demonstrated measurable benefits, including reduced hospitalization, lower use of emergency services, improved social functioning, and enhanced quality of life. These interventions are not only ethically preferable but are also cost-effective when compared to the cumulative financial and human costs associated with repeated admissions, chronic pharmacological management, and the judicial infrastructure required to enforce coercive measures.

Moving away from forced treatment is not only a normative imperative but also a practical strategy for building effective mental health systems. The prevention of relapse and the reduction of new psychiatric crises are best achieved by addressing the social determinants of mental health and ensuring continuity of care. Programs that include supported housing, community-based rehabilitation, and participatory decision-making have demonstrated the capacity to foster autonomy and reduce dependency on institutional care. Investment in these alternatives requires not only a reallocation of resources but also a shift in clinical training toward relational competencies, narrative understanding, and cultural humility. In contrast, reliance on coercion reflects a failure of systemic imagination and a resistance to structural change. It reinforces a cycle in which acute deterioration is used to justify punitive interventions rather than being interpreted as a signal of systemic neglect (Serrano-Miguel, 2020; Parrabera-García et al, 2023).

Table 5. Harmful practices, structural causes, and reform implications

Harmful practice	Structural cause	Evidentiary basis	Implications for reform
Involuntary drugging without due process	Lack of safeguards, deference to clinical authority	Fieldwork, user testimonies, national data	Implementation of supported decision-making frameworks
Chronic polypharmacy	Absence of deprescription protocols, pharmacocentric bias	WHO reports, clinical literature	National deprescription guidelines, professional training
Diagnostic overreach	Epistemic hierarchy, DSM-based reductionism	Testimony, legal records, ethnographic analysis	Revision of diagnostic criteria, narrative and participatory models
Legal incapacitation of users	Structural ableism, juridical paternalism	CRPD, Spanish civil code, case law	Legal reform, implementation of advance directives
Coercive hospitalization	Institutional risk aversion, absence of community resources	Hospital records, expert reports	Redirection of funds to community-based services
Gender-biased assessments and decisions	Sexist diagnostic paradigms, androcentric normativity	Feminist psychiatry, field data	Gender-aware protocols, mandatory bias training
Ignoring of complaints or reported harm	Institutional denial, absence of accountability mechanisms	Complaints, ombudsman reports, field interviews	Protection for whistleblowers, external audits

Summary of coercive, negligent, or ethically compromised practices, evidentiary sources and implications for further action-research and reform.

The Spanish context illustrates a persistent tension between constitutional guarantees of liberty and the enduring custodial logic embedded within psychiatric practice. The current legal framework already permits involuntary hospitalization under strict conditions and provides for substitute decision-making in cases of demonstrable incapacity. Repeated attempts to introduce new forms of outpatient coercion do not respond to legal vacuums but rather aim to extend mechanisms of control into the everyday lives of individuals beyond the institutional setting. Such expansion lacks empirical justification and raises serious concerns under international human rights standards. The European Court of Human Rights has repeatedly emphasized that any restriction of liberty ought to be necessary, proportionate, and subject to rigorous judicial oversight. In the case of forced outpatient treatment, these criteria are rarely satisfied (SES-PAS, 2020; Francisco Torres-Gonzalez, 2022; Calcedo-Barba et al., 2024; Pérez-Prieto et al., 2025).

The path forward requires a deliberate and evidence-based transition toward systems of care grounded in autonomy, trust, and mutual responsibility. This transition involves not only legal and policy reform but also an epistemic transformation in how psychiatric knowledge is produced and applied. Clinical paradigms ought to integrate lived experience, recognize the limitations of current diagnostic frameworks, and abandon the presumption of incapacity that underpins coercive practice. Medical education ought to prioritize ethical reasoning, critical reflection, and dialogic engagement. Institutions ought to be held accountable for harm caused by coercion and incentivized to adopt practices that build individual capacity rather than enforce submission. Service users, families, and communities ought to be empowered to participate meaningfully in the design, implementation, and evaluation of care. The objective is not merely to reduce coercion but to create conditions under which it becomes obsolete. This is attainable through the systematic deployment of care models that center human dignity, relational continuity, and the right to self-determination in mental health.

Table 6. Systemic failures and mechanisms of institutional harm

Systemic Failure	Mechanism of Harm	Evidentiary Basis	Proposed Intervention
Epistemic exclusion	Discounting of lived experience, pathologization of dissent	Power Threat Meaning Framework, patient narratives	Inclusion of experiential knowledge in clinical assessment
Feedback failure	Lack of outcome tracking, no learning from errors	Fieldwork, malpractice cases, WHO QualityRights framework	Participatory audit systems, continuous quality improvement
Institutional impunity	Absence of sanctions for abuse or neglect	Ombudsman reports, legal proceedings	Legal reform, independent oversight bodies
Policy inertia	Disconnection between evidence and policy	Literature review, Horizon Europe evaluations	Integration of science-policy interfaces
Disabling governance	Decision-making without user input	Comparative models (e.g., open dialogue)	Deliberative participation, democratic care protocols
Performativity of ethics	Ethics committees used as bureaucratic cover	Field observations, case analysis	Transparent and accountable ethical governance structures

Synthesizes underlying failures in governance, feedback, and legitimacy that perpetuate harmful practices in psychiatric systems, and outlines corresponding areas of structural intervention.

The justification for compulsory outpatient treatment in Spain has increasingly rested on a particular construction of psychiatric chronicity, framed as a stable and medically verifiable condition necessitating continuous pharmacological management under professional supervision. Arguments in favor of such measures emphasize loss of insight and lack of adherence as diagnostic indicators of pathology. These assertions often rely on the unexamined assumption that individuals diagnosed with schizophrenia or bipolar disorder will inevitably relapse without pharmacological maintenance, and that treatment refusal is itself evidence of illness. The proposal to impose medication in community settings does not arise from demonstrated therapeutic superiority but from institutional logics oriented toward risk mitigation, cost containment, and reputational management.

Within clinical discourses, psychosis is often portrayed not only as a disruption of cognition or affect but also as a suspension of civil and legal personhood. Autonomy is routinely invalidated by assessments that equate dissent with incapacity. A recurrent narrative positions certain individuals as inherently unable to act in their own best interests due to an alleged deficit in self-awareness. Under this paradigm, forced outpatient treatment is presented as a humane and proportionate alternative to hospitalization. The language of care serves to obscure its disciplinary function. Legal instruments redefine therapeutic goals, and medication becomes both the intervention and the prerequisite for legal freedom. Spain has not enacted a specific statute authorizing involuntary outpatient treatment. Nonetheless, judicial interpretations have enabled conditional discharges and provisional legal arrangements that replicate its function. Proponents frequently invoke international precedents, such as Kendra's Law in New York or the Mental Health Act in England and Wales, to argue for legal codification of coercive outpatient measures. The supposed benefits include reductions in hospitalizations, improved treatment adherence, and decreased interaction with the criminal justice system. However, these metrics fail to capture subjective well-being or

personal recovery. They rely instead on institutional proxies of compliance and contact, rather than on person-centered outcomes (Watnik, 2001; Brown et al, 2023).

The therapeutic value of antipsychotics remains a subject of ongoing debate, particularly in the context of long-term use. While short-term reductions in acute psychotic symptoms have been observed, the literature indicates a complex and often inconclusive relationship between sustained medication use and long-term recovery trajectories. Meta-analyses have not shown consistent differences in functional outcomes when comparing individuals subjected to coercive treatment with those receiving care voluntarily. The widespread assumption that maintenance pharmacotherapy prevents relapse has not been adequately tested under conditions of informed and uncoerced choice. The use of long-acting injectables without consent raises significant ethical concerns. The procedural convenience of such formulations obscures the structural violence involved in chemically enforcing behavioral conformity. Appeals to psychiatric chronicity are often employed to override the preferences and agency of service users. Chronicity is typically framed as a biological inevitability, rather than a socially mediated and therapeutically contingent construct. This framing erases the well-documented role of trauma, social exclusion, and institutional betrayal in the etiology and perpetuation of psychiatric distress. It also forecloses the possibility of meaningful recovery without medication. Within the Spanish policy debate, it is not uncommon to hear that families require legal instruments to compel relatives to accept treatment. However, coercion frequently undermines trust, alienates users from services, and reproduces cycles of disengagement and deterioration. Assertions that forced treatment supports therapeutic alliance are fundamentally flawed. Coercion and alliance are mutually exclusive (Moncrieff, 2008; Kinderman, 2014; Gøtzsche, 2015).

The limited body of empirical research cited in defense of involuntary outpatient treatment in Spain suffers from significant methodological limitations. These include selection bias, absence of randomization, lack of appropriate control groups, and vague or nonstandardized outcome criteria. Such deficiencies compromise the validity and generalizability of their findings. In contrast, the expanding literature on shared decision-making offers robust and replicable evidence that collaborative care planning enhances adherence, improves satisfaction, and leads to better clinical and functional outcomes. Individuals who are heard, respected, and engaged are demonstrably more likely to participate meaningfully in their own recovery. Decision-making capacity is not fixed but situational and relational, shaped by the quality of the therapeutic environment. Coercive outpatient treatment undermines these enabling conditions and erodes the relational infrastructure on which sustained recovery depends (Viadel et al, 2006; Portero, 2010; Cañete-Nicolás et al., 2012).

Medication use within involuntary outpatient frameworks is typically determined by clinical expediency rather than by individualized therapeutic dialogue. Regimens are often standardized, with minimal adjustment based on tolerability, side effects, or patient-reported experience. The dominant reliance on long-acting injectables reflects a pharmacocentric and compliance-oriented model of care. Psychological and social interventions are nominally acknowledged but rarely operationalized within coercive frameworks. Clinicians themselves acknowledge that the existing infrastructure is inadequate for delivering comprehensive, community-based recovery services. Coercion functions as a compensatory mechanism for systemic neglect. Arguments asserting that

forced treatment guarantees stability ignore the reality that such measures are most often deployed in environments lacking viable therapeutic alternatives (Sheridan Rains et al., 2019).

Recovery cannot be reduced to the suppression of symptoms. It involves the reconstitution of meaning, the restoration of agency, and the reconstruction of social roles. Coercive practices flatten this complexity. They pathologize resistance and reframe dissent as clinical deterioration. The result is not stabilization but the entrenchment of epistemic and relational harm. Individuals subjected to coercion consistently report fear, humiliation, and lasting distrust in the health system. These responses are not signs of psychopathology but adaptive reactions to structural violence. To claim that coercion is care is to erase the lived experience of those most affected and to perpetuate a model of psychiatry rooted in domination rather than healing (Eiroa-Orosa & Rowe, 2017 ; Kalathil, 2018).

Defenses of involuntary outpatient treatment often appeal to public safety and administrative efficiency. Yet, these are not therapeutic aims. They reflect institutional priorities that are misaligned with the goals of recovery and individual well-being. The discourse of risk is itself problematic. It is often racialized, gendered, and socioeconomically biased. The presumption that certain individuals or groups are inherently dangerous leads to arbitrary and discriminatory enforcement. In the Spanish context, as in others, coercive interventions disproportionately affect those without family support, those with limited financial resources, and those from migrant or minority backgrounds. Structural vulnerabilities are medicalized, and the burden of adaptation is placed on the most disenfranchised (Jacka & Berk, 2012; Kidd et al., 2015a; Walker et al., 2019).

To ensure improvements in these and all systemic failures, this dissertation was conducted through embedded engagement in national and international networks, including formal research actions, clinical settings, and policy initiatives, in line with a participatory action-research design. These collaborative environments were not peripheral but constituted integral fieldwork sites, where knowledge was co-produced through iterative exchange, reflexive analysis, and goal-oriented praxis. Colleagues and participants functioned simultaneously as key informants, critical interlocutors, and co-generators of empirical data, interpretive frameworks, and operational milestones. This embedded model reflects a commitment to scientific accountability, enabling the triangulation of perspectives and enhancing the translational potential of the findings. From a medical anthropological standpoint, this approach aligns with the ethical imperative to contribute constructively to the systems under study, rather than maintaining a detached observational stance. Constructive engagement, anchored in empirical rigor and situated responsiveness, is not merely a methodological choice, as shown in chapter 2, but a disciplinary obligation when investigating healthcare structures that actively produce suffering or protection. This orientation ensures that the work not only documents systemic dysfunction but contributes to its resolution, in fidelity to both the scientific method and the principles of relational care (Schneider, 2012; Rose, 2018).

In this dissertation, recovery is defined as the restoration and sustained maintenance of physiological regulation, functional autonomy, and psychosocial agency, assessed through multidimensional outcomes. These include measurable reductions in pharmacological dependency where clinically indicated, normalization of neuroendocrine and inflammatory markers,

improvement in cognitive-affective integration, and the demonstrated ability to engage meaningfully in relational, educational, and vocational roles. Best practices are defined as those clinical and institutional procedures which, across populations and contexts, support these outcomes with minimal adverse effects and maximal user engagement. Such practices are characterized by informed consent, contextual attunement, and dynamic accountability, enabling the calibration of care in response to evolving individual and systemic conditions. Recovery is conceptualized not as a fixed state but as a dynamic, processual achievement contingent upon ecological coherence, institutional responsiveness, and the absence of epistemic and structural violence. Within this framework, coercion is not classified as therapeutic, but as a mechanism of control and presumed risk management, whose continued use in clinical settings, including in punitive or extrajudicial forms, undermines both the legitimacy and efficacy of psychiatric care. Its presence signals systemic failure to establish relational safety and ethical care environments, and its eradication remains a medical, legal, and scientific imperative (Mezzina et al., 2006b).

1.3 Overmedication and pharmacocentrism in Spanish healthcare and psychiatry

The overuse of psychotropic drugs in Spanish healthcare is not attributable to isolated clinical errors or professional negligence, but rather reflects the systematic implementation of a medical model in which pharmacological management occupies a dominant, structurally legitimised position. The persistence of overmedication reveals a multifaceted institutional configuration, shaped by professional habits, legislative frameworks, educational curricula, industrial influence, and cultural norms of care. Spain ranks among the highest countries in Europe in terms of anxiolytic and antidepressant consumption, with benzodiazepines, selective serotonin reuptake inhibitors, and atypical antipsychotics prescribed at rates consistently exceeding clinical guideline recommendations. This overuse affects diverse patient populations, often without rigorous diagnostic substantiation, without informed long-term risk assessment, and in contexts where the biopsychosocial origins of distress remain unexamined. The diffusion of pharmacological responses beyond specialist psychiatry, notably through general practitioners and emergency services, illustrates the institutional entrenchment of pharmacocentrism as a default reaction to psychological and behavioural symptoms. This clinical orientation, sustained by economic expediency and time pressure, results in a pathological narrowing of treatment options, in which symptom suppression replaces diagnosis, monitoring, and therapeutic alliance (Cohen, 1975; Jiménez, 2005).

Psychotropic prescriptions are frequently initiated and maintained in response to life stressors, interpersonal losses, or chronic social hardship, even in the absence of major psychiatric disorders. Pharmacological solutions are mobilised in response to fatigue, insomnia, anxiety, somatic complaints, and family conflict, without adequate differential diagnosis, multidisciplinary consultation, or provision of non-pharmacological interventions. This pattern results in the chronic administration of medications initially intended for short-term or crisis use. Benzodiazepines, for instance, which were approved for transient management of acute anxiety and sleep disturbances, are widely maintained for periods extending several years. Spain has been consistently placed among the top benzodiazepine-consuming countries in the Organisation for Economic Co-operation and Development, with a prevalence that far exceeds the epidemiological incidence of anxiety-

related disorders requiring pharmacological treatment. The consequences of this misuse include the development of tolerance, dependence, withdrawal syndromes, cognitive impairment, and functional decline. These biological effects are compounded by psychological dependence, impaired agency, and the reinforcement of passivity and medicalised identity (Cañete-Nicolás et al., 2015; Francisco Torres-Gonzalez, 2022).

Prescriptions issued by non-specialist physicians account for the majority of anxiolytic and antidepressant use in Spain. This reflects a structural configuration of healthcare that prioritises throughput, administrative manageability, and symptomatic resolution over person-centred assessment and care continuity. General practitioners are routinely tasked with managing complex mental health presentations in under ten minutes per consultation, with limited access to referral networks, psychotherapy, or case follow-up. The effect is a system where medication is prescribed as a stopgap measure in the absence of time, resources, and institutional pathways for comprehensive care. Psychotropic medication in such settings becomes an instrument not only of symptom control, but of organisational containment. Patients who fail to improve under these regimens are often recycled through healthcare services with escalating doses, additional diagnoses, and polypharmacy, without systematic review of initial assumptions or exploration of alternatives. The phenomenon is not reducible to pharmacological inertia but ought to be understood as the functional correlate of a fragmented, under-resourced, and risk-averse healthcare infrastructure.

Within psychiatric services, the structural predominance of pharmacological approaches is particularly pronounced. Antipsychotic drugs, including long-acting injectables, are increasingly employed not only in psychotic disorders but also in behavioural regulation across a range of non-psychotic diagnoses, including intellectual disability, dementia, and borderline personality disorder. These prescriptions are frequently initiated and continued without robust clinical indication, without documented informed consent, and without systematic evaluation of risk-benefit ratio. Long-term administration of antipsychotics is associated with significant adverse effects, including metabolic syndrome, extrapyramidal symptoms, sedation, cognitive decline, and increased mortality. Despite these well-documented risks, deprescription remains rare, and pharmacological regimens are often escalated in response to behavioural challenges, institutional conflicts, or professional disagreement, rather than through clinical re-evaluation. This pattern is sustained by training deficits, liability concerns, and the institutionalisation of pharmacological compliance as a proxy for therapeutic success (Bobes et al., 2012; Guzmán-Parra et al., 2018; Aragonés-Calleja & Sánchez-Martínez, 2023). The issue of overmedication ought to also be understood in relation to the absence of integrated psychosocial interventions. Community mental health services in Spain are unevenly distributed, under-resourced, and oriented toward pharmacological stabilisation rather than recovery or social reintegration. Psychotherapy, occupational therapy, peer support, and family counselling are chronically underfunded, sporadically implemented, and in many regions effectively inaccessible. In this vacuum, pharmacological strategies acquire a monopoly on therapeutic legitimacy. Patients and families come to equate care with medication, and refusal is interpreted as irrationality or denial of illness. Institutions internalise this logic, and physicians operate within a framework in which prescribing is the most readily available, reimbursed, and institutionally

protected act. This creates a feedback loop in which overmedication becomes self-reinforcing, shielded from scrutiny by cultural expectation, institutional design, and professional norms.

The educational system reproduces this paradigm. Medical curricula in Spain devote limited attention to the historical, sociological, or psychological dimensions of mental suffering. Pharmacology and diagnostic classification are prioritised, while reflective practice, relational skills, and trauma-informed approaches remain peripheral. Students are not systematically trained in shared decision-making, patient autonomy, or non-pharmacological pathways to recovery. Instead, they are socialised into a model of care in which therapeutic success is defined by symptom reduction and adherence. The absence of critical pharmacovigilance, alongside limited training in deprescription, leads to professional trajectories in which medication is employed early, broadly, and indefinitely. Continuous education, often funded or influenced by pharmaceutical interests, reinforces these patterns, presenting pharmacological innovation as synonymous with clinical progress, and obscuring the ethical and empirical limits of current practice (Montero, 2014).

The pharmacological management of distress is further amplified by broader sociocultural factors that facilitate medicalisation and discourage systemic responses. In Spain, high rates of prescription for psychotropic drugs are not merely the product of clinical decisions but reflect a widespread pattern of medicating social suffering. Poverty, unemployment, precarious housing, and chronic stress linked to caregiving responsibilities or domestic violence are frequently translated into psychiatric symptoms within clinical encounters. Rather than acting on the social determinants of health, the healthcare system often absorbs their consequences through the language of diagnosis and the mechanism of prescription. This transfer of responsibility from public policy to the individual subject reinforces a biomedical narrative in which structural adversity is reframed as personal dysfunction, treatable primarily through pharmacological means. The result is a chronic disjunction between diagnosis and reality, in which the burden of systemic failure is redistributed as individual pathology (Laporte et al, 1983; Bertolín-Guillén, 2021; Suleman et al., 2025).

This phenomenon is particularly visible in primary care, where general practitioners are expected to manage a high volume of patients in limited time, often without access to psychosocial resources or referral options. Within these constraints, medication becomes not only the path of least resistance but the only available tool. Benzodiazepines and antidepressants are routinely used to manage conditions that emerge from socioeconomic deprivation, interpersonal conflict, and cumulative life stress. While such medications may provide temporary relief, their prolonged use in structurally disadvantaged populations reinforces dependence, undermines the development of adaptive coping strategies, and delays or displaces interventions that address underlying causes. Chronic exposure to these drugs carries significant health risks, particularly among older adults, for whom the risk of falls, cognitive impairment, and iatrogenic injury is markedly increased. In this context, overmedication becomes a vector of medical harm, sustained by institutional indifference to the conditions that generate suffering (Santamaría, 2020; Bertolín-Guillén, 2021; Roncero et al, 2025).

Table 7. Systemic logic of psychiatric coercion and overmedication

Stage	Left Path: Individual and Social Determinants	Right Path: Institutional and Legal Constraints	Outcome
1. Structural Conditions	Sociostructural determinants (e.g., poverty, trauma)	Risk aversion and legal constraints Protocol-driven care (low variance, high control) Pharmacocentric intervention	
2. Initial Presentation	Psychiatric symptom presentation		
3. Institutional Framing	Framed as individual disorder		
4. Diagnostic Assignment	Diagnostic label assigned		
5. Institutional Outcome			Chronicity, exclusion, resistance to reform

This table outlines the sequential institutional logic through which social suffering is reframed as individual disorder, leading to coercive psychiatric interventions. It demonstrates the interplay between sociostructural determinants and legal-institutional constraints, showing how diagnostic labeling and pharmacocentric protocols converge to produce chronicity, exclusion, and resistance to reform. The model reflects mechanisms of structural violence embedded in clinical governance and highlights the need for systemic redesign grounded in patient autonomy, ethical accountability, and recovery-oriented practice.

From a public health perspective, the cost of this pharmacocentric approach is substantial. Financially, the allocation of resources to medication procurement, distribution, and monitoring diverts funding from preventative and community-based interventions. Clinically, overreliance on drugs contributes to unnecessary hospitalisations, treatment-resistant presentations, and elevated rates of chronic disability. Psychosocially, it erodes trust in the healthcare system, stigmatises distress, and fragments the relational fabric necessary for long-term recovery. The cumulative burden of overmedication is therefore not limited to the patient but extends to families, professionals, and society as a whole. Its persistence is indicative of a deeper epistemological crisis in healthcare, where the metrics of success are misaligned with the realities of human suffering, and where the promise of technological precision obscures the need for ethical and contextual sensitivity (Laporte, 1984; Díaz-Gutiérrez et al, 2018).

The evidence presented in this section confirms that overmedication in Spanish psychiatry cannot be explained as a series of isolated errors or unfortunate habits. It is a systemic feature of a clinical ecosystem that privileges pharmaceutical solutions, institutional convenience, and epistemic authority over patient experience, structural reform, and relational care. Addressing this dysfunction requires a sustained and multidisciplinary effort, capable of confronting both the cultural assumptions and the institutional practices that sustain pharmacocentric hegemony. The next section will examine the rationale, principles, and implementation strategies of autonomous medication management and other scientifically grounded practices that can restore clinical integrity and advance health outcomes in a rights-based, participatory, and ethically coherent framework.

1.4 Toward autonomous medication management and evidence-based better practices

International developments in psychiatric care have progressively challenged the historical dominance of paternalistic, pharmacologically centered, and compliance-oriented treatment paradigms. Autonomous medication management has emerged in this context as a scientifically grounded and ethically necessary framework, aiming to reconfigure psychiatric pharmacotherapy as a dialogical and evidence-based process rather than a unilateral imposition. In multiple health systems across North America and Europe, empirical and longitudinal studies have identified significant clinical and public health concerns linked to the long-term use of psychotropic medication, including metabolic syndrome, cardiovascular risk, neurocognitive impairment, emotional blunting, and altered social cognition. These effects, compounded by insufficient informed consent procedures and structurally coercive treatment regimes, have prompted sustained critique from both academic psychiatry and user-led advocacy. Concomitantly, policy frameworks in jurisdictions such as Quebec, Brazil, the Netherlands, and Finland have integrated alternative models of care, reflecting a shift in clinical and ethical reasoning. These models emphasize recovery, narrative coherence, contextual understanding, and relational decision-making, positioning medication not as the axis of intervention but as one modifiable element within an evolving therapeutic arrangement. This reframing has been supported by controlled trials, real-world cohort data, and transdisciplinary consensus statements underscoring the necessity of reducing medication load, implementing tapering protocols, and fostering participatory care environments (Deegan, 2006; Chmielowska et al., 2022; Horowitz, 2024).

The Quebec model, notably developed through the work of Jean-François Pelletier and Marie-Jeanne Proulx, systematized collaborative medication management as a relational and ethically reflexive practice grounded in the recovery paradigm. Informed by trauma research, patient autonomy, and shared clinical accountability, this model emphasized the production of knowledge through dialogue, the reorientation of clinical authority, and the integration of peer support into pharmacological decisions. These strategies were operationalized through structured dialogues, non-hierarchical therapeutic teams, and flexible treatment planning responsive to individual trajectories of distress and recovery. In Brazil, the translation of these principles into a context marked by psychosocial community care, collective health constitutionalism, and anti-institutional reform yielded a model in which medication reduction strategies were embedded within a broader political commitment to deinstitutionalization and rights-based psychiatry. The Brazilian system demonstrated that medication management cannot be decoupled from socio-legal determinants, such as access to housing, income, and community resources. By integrating psychotropic deprescription into a continuum of public health and social protections, Brazil's approach highlighted the necessity of collective supports and legal frameworks that reinforce the legitimacy of non-pharmacological care (Canadian Deprescribing Guidelines Project, 2024).

In the Netherlands and Finland, open dialogue practices have provided further empirical grounding for low-medication and user-centered psychiatric models. These practices, which integrate continuous familial and community involvement, real-time dialogical processes, and minimal reliance on diagnostic categorization, have yielded statistically significant outcomes in terms of reduced hospitalization, lower chronic medication use, and higher patient-reported recovery metrics. Despite these outcomes, institutionalization of such models remains limited due to systemic inertia, financial constraints, and disciplinary resistance within conventional psychiatric

governance. The comparative analysis of these jurisdictions reveals that autonomous medication management is not an abstract ethical ideal but a viable clinical framework that depends on interlocking supports: clinical retraining, patient-led tools, digital infrastructure, and legal recognition of participatory rights. This empirical corpus establishes the conceptual, ethical, and practical viability of a model in which medication use is not dictated by standard protocols or institutional mandates, but co-produced in situated, relational, and physiologically literate clinical ecosystems (Seikkula & Olson, 2003; Olson et al., 2014; Groot & van Os, 2023).

Barriers to the widespread adoption of autonomous medication management remain significant, particularly in contexts such as Spain where clinical traditions, institutional protocols, and professional hierarchies continue to privilege pharmacological intervention as the cornerstone of psychiatric care. Despite rhetorical shifts toward recovery-oriented principles and the formal inclusion of patient rights in national mental health strategies, these commitments often fail to materialize in clinical practice. Structural constraints include the absence of training in dialogical and trauma-informed care, limited availability of psychosocial alternatives, and medico-legal cultures that penalize clinical discretion perceived as deviant from pharmacocentric standards. These constraints are reinforced by the design of health information systems that track medication adherence as a proxy for clinical improvement, thereby entrenching pharmacological compliance as an index of successful care. The result is a clinical environment in which patient requests to taper or discontinue medication are often interpreted as signs of denial, resistance, or lack of insight, rather than as expressions of informed agency. This interpretive framework not only delegitimizes the experiential knowledge of individuals with lived experience but also disincentivizes reflective practice among professionals, who remain embedded in cultures of risk aversion and diagnostic orthodoxy (Serrano-Miguel et al, 2020; Martínez-Hernández et al, 2020).

Further compounding the issue is the asymmetry of scientific literacy and epistemic authority between patients and professionals. In the absence of co-produced clinical guidelines and shared platforms for knowledge dissemination, patients are often deprived of the information necessary to engage meaningfully in pharmacological decision-making. Many remain unaware of the full range of risks associated with long-term medication use, the physiological processes underpinning withdrawal syndromes, or the alternatives supported by international evidence. The dissemination of open-access deprescription guidelines, interactive digital decision aids, and patient-led training modules remains limited to specific pilot programs and rarely reaches primary or secondary care settings. In Spain, the integration of these tools into routine care is further hindered by fragmented governance, insufficient policy coordination across autonomous communities, and the dominance of pharmaceutical industry interests in continuing medical education and guideline development. The lack of independent evaluation mechanisms and the near absence of patient-controlled documentation systems exacerbate this problem, resulting in an institutional environment that structurally inhibits the operationalization of best practices in medication management (Sanguinetti et al., 2025).

Scientific consensus increasingly recognizes the need to redefine both the content and process of psychiatric care to accommodate the epistemic plurality and physiological variability inherent in real-world clinical populations. Rather than framing medication adherence as a binary outcome or

marker of engagement, contemporary best practices emphasize the use of feedback-informed treatment planning, co-defined clinical endpoints, and metrics that incorporate both physiological and psychosocial dimensions of recovery. These include cognitive functioning, metabolic health, narrative coherence, affect regulation, and social role fulfillment. Evidence from longitudinal studies suggests that such outcome measures are better predictors of sustained well-being than traditional symptom checklists or diagnostic criteria alone. Consequently, the scientific justification for maintaining pharmacocentric treatment models without rigorous re-evaluation has weakened considerably. The persistence of these models in Spain reflects not an absence of alternatives, but a failure to implement those alternatives due to institutional inertia, legal ambiguity, and the marginalization of critical perspectives within psychiatric education and governance (Kuhl, 2019).

The consolidation of pharmacocentric paradigms in Spanish psychiatry, despite accumulating international evidence of their limitations, ought to be understood as a product of deeply embedded institutional logics that extend beyond individual clinical encounters. Professional cultures, regulatory environments, and reimbursement schemes collectively sustain a framework in which the prescription of psychotropic drugs is not only normalized but structurally incentivized. Reimbursement policies favor short, medication-focused consultations over time-intensive psychosocial interventions. Legal and administrative protocols often prioritize risk minimization through rapid pharmacological containment rather than comprehensive contextual evaluation. Training curricula in medicine and psychology continue to privilege symptom-based diagnostic categories and neurochemical explanatory models while neglecting integrative approaches to trauma, embodiment, and social determinants. This constellation of forces reinforces a clinical habitus that marginalizes non-pharmacological knowledge, restricts therapeutic imagination, and produces epistemic conditions hostile to dialogical and participatory care (Toro et al, 2006).

Crucially, the normalization of long-term psychopharmacological treatment in Spain persists despite its well-documented iatrogenic consequences. Chronic exposure to antipsychotics, antidepressants, mood stabilizers, and anxiolytics has been associated with a wide range of adverse effects, including but not limited to metabolic dysfunction, cognitive impairment, emotional blunting, movement disorders, hormonal dysregulation, and sexual dysfunction (Simopoulos & Trinidad, 2013). Beyond physiological sequelae, prolonged medication use often leads to psychosocial dependency, erosion of self-efficacy, and impaired identity development, particularly in adolescents and young adults. These outcomes are rarely monitored systematically and are frequently minimized or pathologized when raised by patients. Instead of prompting critical reflection on the appropriateness of treatment plans, reports of adverse effects are often met with diagnostic escalation or polypharmacy. This practice not only amplifies risk but entrenches a clinical cycle wherein patient concerns are reinterpreted as evidence of disorder rather than valid input into therapeutic decision-making. Such dynamics illustrate how the pharmacocentric model operates not merely as a treatment strategy but as a disciplinary mechanism, shaping patient subjectivity, professional identity, and institutional norms (Dencker et al, 1986; Gotzsche, 2019).

The ethical implications of this model are significant. When clinical systems prioritize biochemical intervention over relational understanding, the result is a degradation of therapeutic integrity and a systematic erosion of informed consent. True consent requires not only the provision of information

but also the freedom to decline or modify treatment based on evolving needs and goals. In the current Spanish context, this freedom is often nominal, constrained by structural scarcity, normative pressure, and the medical-legal framing of deviation as deviance. Autonomy becomes performative rather than substantive, preserved in rhetoric but denied in practice. To counter this tendency, evidence-based better practices in medication management ought to be embedded within a broader transformation of psychiatric epistemology and governance. This transformation entails not only revising treatment protocols but reconfiguring the systems of knowledge production, professional accreditation, and institutional accountability that sustain them (Whitaker, 2005; Rose, 2007).

This dissertation contributes to such reconfiguration by articulating the scientific, ethical, and operational foundations of autonomous medication management, rooted in comparative fieldwork, empirical evidence, and the lived expertise of those affected. Through multi-scalar analysis and transdisciplinary integration, it exposes the structural determinants of pharmacocentric persistence and outlines actionable pathways toward clinical practices that are both more effective and more just. The next section delineates the medico-legal and constitutional frameworks necessary to support this transition, identifying the normative, procedural, and institutional conditions under which pharmacological care can be rendered compatible with human rights, clinical excellence, and democratic legitimacy.

1.5 Legal, epistemic, and scientific foundations of the research problem

The persistence of coercive practices and pharmacocentric treatment models in contemporary psychiatry reflects a complex intersection of legal ambiguity, epistemic asymmetry, and scientific limitation. These elements sustain a structural configuration in which patient autonomy, informed consent, and bodily integrity are subordinated to institutional convenience, professional discretion, and risk management protocols. The foundational problem addressed in this dissertation is not limited to clinical overreach but concerns the systemic reproduction of coercion through embedded legal norms, scientific validation mechanisms, and decision-making hierarchies that obscure the boundary between care and control. Despite international human rights instruments affirming the principle of free and informed consent in medical treatment, national legal frameworks often contain exceptions that permit involuntary intervention on the basis of presumed incapacity, risk, or diagnostic classification (Hemper et al, 2024). These exceptions, while formally regulated, are subject to broad interpretive discretion and rarely subjected to external verification or retrospective audit. In the Spanish context, the organic laws governing civil incapacity, guardianship, and mental health intervention permit medical and judicial actors to override patient preferences without requiring real-time justification rooted in contemporary scientific standards. This legal permissiveness enables coercion to function as a normalized clinical strategy rather than as a last resort (Desviat, 2011; Ferreirós, 2013; Ramos-Pozón et al, 2025).

The epistemic structure of psychiatry reinforces this dynamic by locating clinical authority within a diagnostic system that remains descriptively based and not etiologically validated. The prevailing nosological models used in Spain and across most high-income health systems are grounded in the Diagnostic and Statistical Manual of Mental Disorders and the International Classification of

Diseases. These classifications rely on symptom clusters without mechanistic grounding, rendering diagnostic attribution vulnerable to subjectivity, sociocultural bias, and administrative utility. The absence of objective biomarkers or reproducible physiological indicators undermines the epistemic reliability of psychiatric diagnosis and weakens its claim to medical legitimacy under the standards of falsifiability and mechanistic transparency expected in other domains of health science. Nevertheless, these classifications carry significant legal and social weight, functioning as gatekeeping devices for civil status, capacity, and access to entitlements or restrictions. In practice, they authorize interventions that may include forced hospitalization, involuntary medication, and long-term institutionalization, often without recourse to independent appeal or longitudinal re-evaluation. This tension between epistemic uncertainty and legal determinacy constitutes a core dimension of the research problem addressed in this dissertation (Moncrieff, 2010; Frances, 2013).

The institutionalization of psychiatric knowledge occurs within a regulatory architecture that confers normative status to clinical opinion without subjecting it to the same evidentiary thresholds applied in other domains of biomedicine. In the absence of mechanistic specificity or outcome-based validation, clinical authority is derived primarily from institutional position, professional credentialing, and codified protocols. These protocols are themselves frequently based on consensus rather than on controlled empirical evaluation and often reflect the influence of pharmaceutical interests, risk containment priorities, and medico-legal defensibility rather than therapeutic efficacy or patient-defined recovery. This dynamic produces a closed epistemic loop wherein deviation from established pharmacological or diagnostic practice is interpreted as clinical error, administrative liability, or patient irrationality. Within such a system, innovation, dissent, or adaptation is structurally discouraged, and relational or contextual data is deprioritized in favor of quantifiable symptom management. Clinical decisions are thus governed by algorithmic logic informed by population averages and extrapolated trial data, rather than by person-specific trajectories, life histories, or situated capacities for choice (Kirk & Kutchins, 1992; Kirk et al., 2013).

The scientific literature on coercion and psychiatric harm has expanded over the past two decades, but its integration into clinical and legal praxis remains limited. Meta-analyses and large-scale observational studies have documented the long-term consequences of coercive practices, including learned helplessness, treatment disengagement, iatrogenic injury, and elevated suicide risk. Nevertheless, the incorporation of these findings into policy reform or clinical training has been partial and inconsistent. Scientific evidence of harm is often neutralized by institutional narratives of risk mitigation or patient safety, which present coercion as a necessary response to clinical urgency, despite the lack of predictive validity in psychiatric risk assessment tools. Scientific critique of pharmacocentric practices, including the long-term inefficacy of certain drug classes and the underreporting of adverse effects in industry-sponsored trials, has likewise struggled to produce sustained shifts in clinical behavior or regulatory standards. This indicates a structural dissociation between knowledge production and institutional change, a gap this dissertation aims to expose and address through embedded, empirically informed, and system-sensitive research (Moon, 2000).

Within the Spanish context, the convergence of legal permissibility, scientific ambiguity, and clinical discretion creates a structural environment where coercive practices are not episodic but constitutive of psychiatric governance. Civil codes, health statutes, and jurisprudence allow for

involuntary interventions based on broad interpretations of risk, incapacity, or presumed medical necessity, without requiring the presentation of individualized harm thresholds or evidence-based justification. Judicial endorsement of clinical recommendations is often automatic, reflecting the judiciary's reliance on psychiatric expertise and the absence of independent oversight or epistemic counterbalance. This arrangement consolidates the authority of the psychiatric institution while limiting avenues for patient contestation, legal redress, or external review. In such a configuration, the patient's capacity to object is undermined not only procedurally but conceptually, as dissent is pathologized and interpreted as a symptom of illness rather than as a legitimate expression of agency. Diagnostic instruments, clinical notes, and risk assessments function not as descriptive aids but as technologies of capture, generating administrative legitimacy for interventions whose scientific basis remains weak or untested (Szasz, 2007; Whitaker, 2010).

These systemic conditions are reinforced by the epistemological division between objective and subjective evidence, in which first-person accounts of harm or injustice are routinely discounted as unreliable or non-clinical. This marginalization of lived experience undermines the empirical scope of psychiatric inquiry, narrows the definition of valid data, and restricts the development of participatory or phenomenological models of care. It limits the potential for system feedback and error correction, as mechanisms for reporting harm are underdeveloped, institutionally internal, or perceived as adversarial. When adverse outcomes do occur, including iatrogenic deterioration, suicide, or chronicity induced by cumulative intervention, institutional responses prioritize liability management over truth reconstruction, preventing meaningful learning or procedural reform. In this environment, the legitimacy of psychiatric practice is maintained through normative inertia, credential-based authority, and bureaucratic insulation from user critique. The resulting gap between clinical rhetoric and institutional practice constitutes a core pathology of governance and a critical object of analysis for this research (Burstow, 2015; Held, 2020; Markus, 2020).

The structural features outlined above define not only the object of study but also the methodological constraints and ethical obligations under which this dissertation has been developed. The convergence of legal authority, clinical autonomy, and epistemic opacity requires a research design capable of operating within contested institutional spaces while maintaining analytical rigor and procedural integrity. This entails a scientific framework that treats lived experience as data, institutional discourse as evidence, and clinical practice as situated behavior subject to empirical analysis. It also demands a methodological position that does not reproduce the asymmetries it critiques, but that enables observation, documentation, and engagement without reproducing harm or epistemic exclusion (Sanati & Kyratsous, 2015). The research problem is thus not limited to identifying instances of coercion or overmedication but extends to mapping the normative architectures that sustain them and to tracing the conditions under which these architectures might be transformed. Such transformation cannot occur solely through clinical innovation or legal amendment but requires a reconfiguration of epistemic hierarchies, procedural safeguards, and institutional accountability mechanisms. These dimensions will be addressed in the following chapter, which outlines the methodological strategy adopted, the design of field instruments, and the analytical frameworks used to examine coercion, agency, and psychiatric governance in real-world contexts. The aim is to generate scientific knowledge that is empirically

grounded, legally relevant, and operationally useful in advancing ethically defensible models of mental health care (Houlders, Bortolotti, & Broome, 2021).

1.6 Systemic failure, structural violence, and the challenge of reform

The current structure of psychiatric care in Spain is shaped by entrenched institutional paradigms that prioritise pharmacological containment over biopsychosocial integration. Despite the increasing availability of global frameworks for rights-based care, including the World Health Organization's QualityRights program, and the widespread dissemination of trauma-informed principles and patient-centred care models, the dominant clinical responses to mental distress remain largely pharmacocentric and reactive. This discrepancy reflects a fundamental misalignment between emerging standards in global health and the operational norms within much of Spanish psychiatry. The sustained use of psychotropic drugs, particularly for long-term treatment of non-acute mental health conditions, continues to occur without consistent application of participatory decision-making protocols, interdisciplinary evaluation, or structural risk assessment. While professional societies increasingly recognise the importance of ecological, neuroendocrine, and relational factors in shaping mental health outcomes, these insights are yet to be translated into routine practice. The continued dominance of reductionist paradigms not only limits clinical efficacy but systematically undermines the rights, autonomy, and long-term recovery prospects of patients, particularly those most marginalised by poverty, trauma, and social exclusion. The absence of meaningful dialogue between disciplines, the limited implementation of rights-based training, and the persistent framing of non-compliance as clinical deviance all contribute to an institutional landscape in which pharmacological interventions become the default response to distress, regardless of context, history, or patient preference. In this setting, structural violence operates not as an aberration, but as a consequence of standardised practice, revealing the urgent need for paradigm correction guided by scientific integrity and ethical obligation (Paradis-Gagné et al., 2021; Gill et al., 2024).

Efforts by the Spanish central government and some regional ones, such as the Catalan one, to address the disproportionate reliance on pharmacological interventions have gained visibility through the recent national mental health strategies, which emphasise the importance of community-based care, interdisciplinary coordination, and preventive approaches. These policy instruments acknowledge the overuse of psychiatric medication and advocate for non-pharmacological responses, especially in primary care and non-acute settings. However, the translation of these priorities into practice remains partial and uneven across regions. Institutional inertia, uneven training of healthcare professionals, insufficient infrastructure for psychosocial support, and the persistence of hierarchical clinical cultures continue to impede the realisation of these policy objectives. Without substantive reform in how medical decisions are made, documented, and followed up, the stated goals risk becoming performative. The emphasis on mental health as a public health priority ought to not be reduced to rhetorical commitments. It requires the allocation of resources to multidisciplinary teams, the reorganisation of care pathways to enable continuity and choice, and the development of measurement systems that do not privilege symptom suppression over functional recovery. This is especially relevant given the documented associations between overmedication, chronicisation of symptoms, metabolic deterioration, and loss of

psychosocial function. The government's acknowledgment of the issue is a necessary starting point, but the transformation required is not limited to increasing access to care. It involves the restructuring of the epistemic, ethical, and procedural foundations of psychiatric practice to align with contemporary medical science and human rights standards (Salvador-Carulla, 2010).

The alignment of Spain's national strategies with global initiatives represents a critical juncture for systemic change. The World Health Organization's QualityRights framework, which calls for the elimination of coercion and the implementation of rights-based care, provides a normative and operational benchmark for evaluating psychiatric systems. Spain's endorsement of such international standards reflects an institutional willingness to move beyond purely biomedical paradigms. However, practical implementation requires that the system adopt evidence-based clinical models that integrate neurophysiological, nutritional, psychological, and social determinants of health into the standard of care. Emerging psychiatric approaches that rely on interdisciplinary integration, including those grounded in psychoneuroendocrine immunology and in the broader field of translational neuroscience, emphasise the need to address the underlying mechanisms of distress rather than solely modulating symptoms through pharmacology. These approaches support the view that the brain cannot be treated in isolation from the body or from the social and environmental contexts that shape both. Translational models point to the critical role of early life adversity, chronic inflammation, poor sleep hygiene, nutrient deficiencies, and dysregulated stress responses in the genesis and perpetuation of mental distress. When embedded into clinical reasoning, these models shift the focus of psychiatry from symptom control to physiological regulation and resilience building. While not yet mainstream in Spain's public health system, these perspectives are increasingly represented in continuing medical education, research consortia, and pilot programs. Their full adoption, however, is contingent on reforms to medical training, diagnostic classifications, and institutional reward systems. The thesis engages with these international developments not only as background evidence but as active fields of contestation and learning, within which the current Spanish efforts ought to be critically situated and constructively advanced. The gradual incorporation of physiological, psychosocial, and ecological models into psychiatric care has exposed the inadequacy of current diagnostic and therapeutic paradigms that remain anchored in outdated symptom clusters and pharmacological assumptions. While the international scientific community increasingly calls for the integration of social determinants of health, embodied cognition, and context-sensitive formulations, many clinical environments in Spain continue to apply rigid diagnostic categories with insufficient attention to their epistemological limitations or functional consequences (Pūras, 2017). This rigidity not only distorts the understanding of human suffering but also sustains coercive practices by presenting them as medically justified or inevitable. It undermines the capacity for interdisciplinary learning and reinforces professional hierarchies that inhibit collaborative care. The persistence of such systemic patterns reflects the resilience of institutional path dependencies that favour risk containment, administrative efficiency, and the pharmacological management of distress over relational, rehabilitative, and socially grounded interventions. Although recent policy frameworks and national mental health plans formally acknowledge the need for reform, the actual mechanisms of implementation remain subject to resource constraints, professional resistance, and the lack of

integrated governance. Public discourse on mental health has expanded considerably, yet structural commitments to change remain precarious unless accompanied by a redefinition of what constitutes clinical success and accountability in psychiatric care. This dissertation intervenes at precisely that level, contributing empirical, legal, and methodological insights that challenge the inertia of the current system while identifying operational pathways for evidence-informed transformation.

The convergence of international mandates, scientific advancements, and local advocacy efforts provides a unique opportunity to realign mental health systems with contemporary standards of care grounded in dignity, autonomy, and scientific integrity. Global frameworks such as those established by the World Health Organization advocate for rights-based, community-integrated mental health care, explicitly rejecting coercive practices and emphasizing the importance of legal safeguards, informed consent, and interdisciplinary continuity. Within this broader landscape, Spain's institutional apparatus is beginning to respond through national initiatives aimed at reducing over-reliance on pharmacological interventions and promoting integrated psychosocial approaches. Nevertheless, substantial implementation gaps persist due to entrenched professional cultures, fragmented service provision, and insufficient legal mechanisms for accountability and patient-led governance. The integration of translational neuroscience, nutritional psychiatry, and physiological models of mental health remains limited, often relegated to experimental or peripheral domains rather than informing core clinical practice. The absence of systems capable of incorporating embodied experience, interpersonal dynamics, and social feedback into diagnostic and therapeutic processes hinders the evolution of truly participatory and context-sensitive care (Bobes et al, 2012).

The present dissertation contributes to addressing these deficits by offering a structured, multi-scalar analysis of systemic barriers and operational levers, grounded in empirical fieldwork and comparative policy research. It does so not by proposing abstract reform, but by tracing concrete configurations through which better practices emerge and become sustainable within the existing legal and institutional fabric. This approach aligns with the current European and international efforts to recalibrate psychiatric care as a fundamentally ethical and scientific endeavour, one that ought to be responsive to the lived realities of those it serves and to the knowledge generated across disciplines, professions, and affected communities (Ortiz Lobo, 2013; Carbonell Marqués & Navarro-Pérez, 2019).

1.7 Theoretical foundations: power, diagnosis, exclusion, and resistance

The theoretical foundations of this dissertation are grounded in the recognition that psychiatry functions not only as a clinical discipline but as a regulatory institution that participates in the distribution and legitimization of power within contemporary societies. Diagnosis is not simply a classificatory act. It is a mechanism of social ordering, resource allocation, and subject formation. Through diagnosis, individuals are positioned within hierarchies of credibility, legal capacity, and therapeutic eligibility. These positions are not epistemically neutral. They are conditioned by structural inequalities, cultural expectations, and institutional demands. As such, diagnosis operates as a political act with material consequences, shaping life trajectories, social entitlements, and civic participation. Psychiatric categories, particularly those used to justify involuntary interventions or long-term pharmacological treatment, carry embedded assumptions about agency, rationality, and

compliance. These assumptions often reflect normative models of behavior and cognition that fail to accommodate the lived experience of distress, trauma, or systemic adversity. This theoretical position requires a sustained interrogation of the relationship between diagnostic attribution, institutional practice, and social exclusion (Moncrieff, 2013a; Moncrieff, 2013b).

Central to the analysis is the Power Threat Meaning Framework, developed by Lucy Johnstone and Mary Boyle, which reconceptualizes mental health difficulties as understandable responses to adverse experiences, contextualized within specific configurations of power, threat, and meaning. The framework offers an alternative to symptom-based diagnosis by emphasizing the role of power dynamics in shaping distress and the importance of constructing personal narratives that make sense of experience. Rather than asking what is wrong with an individual, it asks what has happened, what threats have been faced, what meanings have been made, and what strengths have been used to survive. This orientation aligns with the empirical orientation of the present research, which privileges situated narratives, longitudinal analysis of institutional interactions, and critical reflection on epistemic authority. It also provides a methodological anchor for the inclusion of relational anamnesis as a valid form of knowledge production. Anamnesis is understood not as a recollection of discrete symptoms but as a reconstruction of the sociopolitical conditions under which distress emerged and became medicalized. In this view, clinical history is not a passive chronology but an active negotiation of recognition, legitimacy, and care (John Read & Harper, 2020; Johnstone, 2020; Rådmark et al., 2024).

The institutionalization of psychiatric knowledge has occurred within broader histories of state formation, juridical expansion, and biomedical standardization. Psychiatry has historically functioned as a boundary-setting apparatus, demarcating normality from pathology and regulating access to civic participation, family life, and legal capacity. Its integration into legal, welfare, and educational systems has conferred upon it a disciplinary role that exceeds the clinic and operates through documentation, surveillance, and procedural decision-making. In this context, exclusion is not an accidental byproduct of psychiatric practice but a structurally inscribed outcome. Diagnostic labels frequently serve as justifications for the withdrawal of rights, the imposition of interventions, and the curtailment of personhood. These effects are intensified in cases where institutional logics prioritize administrative efficiency, clinical control, or reputational management over therapeutic alliance or patient-defined recovery (Foucault, 2003).

Exclusion is also epistemic. The dominance of biomedical models privileges specific forms of knowledge production, particularly randomized controlled trials, symptom inventories, and pharmacological outcome measures, while devaluing experiential, relational, and sociocultural accounts. This asymmetry constrains the epistemological legitimacy of patients, family members, and non-clinical actors, whose contributions are often dismissed as subjective, anecdotal, or irrational. In turn, this epistemic exclusion limits the potential for systems to learn from error, respond to contextual variables, or adapt to changing needs. The result is a clinical environment in which harm can be administered procedurally and justified normatively, without triggering institutional reform or reparative measures. Resistance to change under these conditions is not a function of ignorance or malice alone but of structural design. The institutional architecture of psychiatry is configured to maintain coherence, predictability, and authority, often at the cost of

responsiveness, transparency, and accountability. These dynamics are not unique to psychiatry but are intensified by its proximity to coercive powers and its role in mediating social deviance, public safety, and civil capacity (Watts, 2024).

Within this framework, resistance ought to be understood as both an empirical phenomenon and a theoretical construct. Resistance refers not only to overt contestation by individuals subjected to coercive psychiatric interventions but also to the micro-level acts of negotiation, avoidance, and reinterpretation that occur within clinical encounters (Lehmann, 1998b). These forms of resistance are frequently pathologized as non-compliance or lack of insight, which are themselves diagnostic markers used to justify intensification of intervention. However, from an anthropological perspective, these actions can be read as strategies of survival, narrative preservation, and assertion of personhood within systems that deny full recognition. Resistance may also emerge collectively, as in the case of survivor movements, peer-led advocacy, and alternative knowledge production by affected communities. These efforts constitute a counter-discourse to institutional psychiatry and offer empirical evidence of the epistemic, ethical, and clinical failures of dominant paradigms .

The persistence of resistance, despite its marginalization, indicates the inadequacy of current psychiatric models in addressing the full complexity of mental distress. It also challenges the legitimacy of intervention regimes that fail to incorporate reflexive mechanisms for evaluating their own consequences. Theoretical approaches that foreground resistance, such as critical psychiatry, postcolonial studies, and intersectional analysis, highlight the situated nature of clinical authority and the role of sociopolitical context in shaping diagnostic and therapeutic outcomes (Lehmann, 2019a). These perspectives reveal the extent to which psychiatric interventions are not neutral applications of scientific knowledge but are embedded in relations of power that mediate access to resources, recognition, and redress. The study of resistance thus becomes a methodological imperative, not only for understanding how individuals respond to coercion but for identifying the institutional conditions under which ethical, participatory, and effective care becomes possible.

Resistance is also internal to the clinical professions themselves. Among practitioners, dissent from dominant pharmacocentric or coercive models may take the form of alternative practice, informal adaptation, or engagement with dialogical and trauma-informed approaches (Kenny et al, 2019). However, these forms of internal critique often remain constrained by institutional protocols, performance indicators, and liability frameworks that disincentivize deviation from standardised procedures. The tension between professional judgment and bureaucratic expectation creates a liminal space in which ethically motivated clinicians ought to navigate competing obligations to patients, institutions, and disciplinary norms. The suppression of this internal resistance is indicative of broader systemic rigidity and highlights the necessity for institutional architectures that allow for deliberation, feedback, and principled modification of care. Theoretical frameworks that address resistance ought to therefore include the structural position of practitioners as both agents and subjects of power, situated within organisations that produce and limit the conditions of possibility for ethical care (Farberow, 2005; Leichsenring, 2019).

This dissertation adopts a theoretical orientation that synthesises insights from the Power Threat Meaning Framework, cultural materialism, and critical medical anthropology to interrogate the conditions under which psychiatric care becomes coercive, exclusionary, and resistant to reform.

These theoretical resources enable a multi-level analysis of how institutional practices are constructed, legitimised, contested, and transformed. They also provide the analytical tools necessary to examine how diagnosis operates not as a fixed medical fact but as a relational event shaped by history, power, and interaction. The aim is not to reject psychiatric knowledge but to reconfigure its epistemological foundations, legal articulation, and clinical application in ways that reflect the complexity of human suffering and the ethical demands of care. This framework informs the design of the research questions, the structure of data collection, and the criteria for evaluating institutional practices. It grounds the empirical work in a critical but constructive approach, oriented toward the identification of transformative pathways that are scientifically valid, legally sound, and ethically sustainable (Weiss & Somma, 2007; Johnstone & Boyle, 2018).

The theoretical approach adopted in this research demands a methodological strategy capable of registering power relations not as abstract structures but as observable practices, procedural norms, and lived experiences within specific institutional sites. It requires methodological tools that can document how diagnosis is enacted, how exclusion is operationalised, and how resistance is expressed and suppressed. These demands necessitate a research design that is embedded, longitudinal, and reflexively accountable. The following chapter outlines the methodological framework developed to meet these requirements. It presents the rationale for the use of action-research, ethnographic immersion, legal analysis, and stakeholder engagement in the study of psychiatric governance. It also describes how the principles articulated in this section, epistemic reciprocity, structural scrutiny, and ethical accountability, are translated into concrete field practices, data collection protocols, and analytical procedures. The purpose is to ensure that the research not only observes systems of care but contributes meaningfully to their transformation through the systematic production of knowledge that is empirically robust, ethically grounded, and institutionally actionable.

1.8 Analytical framework and scientific methodology for structural change

This section constitutes the analytical and epistemological bridge between the diagnostic critique of psychiatric systems and the methodological structure developed in Chapter 2. The architecture presented here is explicitly transdisciplinary, integrating medical, legal, anthropological, and computational dimensions into a coherent scientific rationale. This dissertation does not simply describe or report phenomena. It constructs a research design capable of transforming clinical and institutional practices based on systematic documentation, ethical accountability, and translational knowledge. To accomplish this, the inquiry mobilizes a layered framework that aligns the study's object of investigation, psychiatric governance in Spain, with methods capable of capturing its structural complexity.

The phenomena addressed are not discrete nor random, but embedded in a multidimensional system of classification, intervention, and containment. These mechanisms operate across clinical, administrative, juridical, and discursive levels. The selection of frameworks was governed by the requirement to construct not a single-level or monodisciplinary study, but an integrated model that allows the articulation of scientific evidence with legal standards, institutional practices, and

embodied experiences of distress. The aim is not to develop a meta-theory of psychiatry, but to enable the production of applied, evaluable, and ethically justifiable knowledge.

Table 8. Four levels of analysis and integration in the research architecture

Level	Description
Theoretical and epistemological	Grounded in biocultural and transdisciplinary approaches, this level provides the conceptual basis for interpreting mental health as an interaction of embodied, social, and ecological processes. It supports the rejection of reductionist paradigms and frames the integration of medical anthropology, critical psychiatry, and extended mind theories.
Clinical and medical	Anchored in translational science, trauma-informed practice, and nutritional psychiatry, this level addresses the physiological and therapeutic dimensions of psychiatric care. It supports the analysis of pharmacocentric regimes, mechanisms of iatrogenic harm, and criteria for responsible deprescription.
Legal and institutional	Informed by constitutional, international, and human rights frameworks, this level interrogates coercion, legal impunity, and systemic failures of accountability. It underpins the critique of current psychiatric governance and supports the design of ethically aligned, law-abiding systems of care.
Methodological and computational	This level operationalizes the above dimensions using action research, mixed methods, and emerging computational techniques, including Bayesian modeling, survival analysis, and systems mapping. It ensures internal coherence and prepares the field for replicable, scalable interventions through open science and evidence-based structural reform.

This table summarizes the four levels of analytical integration deployed in the action-research, structuring the methodological, conceptual, and applied strategy of the dissertation. Each level informs the evidence production, interpretation, and systemic engagement toward psychiatric reform.

The first level of this architecture, as shown in table 7, is epistemological. Drawing from the biocultural perspective in medical anthropology, the work acknowledges that health and suffering are shaped by sociocultural, ecological, and historical conditions, which interact with biological processes without being reducible to them. Embodied cognition and extended mind theory inform the rejection of mind-body dualisms and support the examination of psychiatric intervention as embedded in context-specific interactions, not isolated within the skull or confined to the individual. These conceptual premises are further developed through engagement with the Power Threat Meaning Framework and the critique of epistemic injustice, both of which serve to decenter clinical authority and reposition affected persons as legitimate epistemic agents. These frameworks are not static or decorative, but operational. They direct the logic of problem formulation, selection of units of analysis, and interpretation of empirical material. They also serve as correctives against epistemic extractivism, ensuring that methodological practices do not reproduce the same power dynamics that the research seeks to expose. They are grounded in established research traditions and subject to methodological transparency, providing the theoretical scaffolding necessary to evaluate both harms and possible redirections in psychiatric systems (Field-Springer, 2020).

Table 9. Biological dimension of the analytical and applied frameworks

Component	Description
Translational medical science	Connects clinical practice with neuroendocrine, immunological, molecular, and metabolic mechanisms relevant to psychiatric symptoms and treatment response.
Nutritional psychiatry	Investigates the role of diet, micronutrients, inflammation, gut-brain axis, and metabolic regulation in mental health.
Psychoneuroendocrinoimmunology	Explores the interaction between nervous, endocrine, and immune systems in stress, trauma, and recovery processes.
Deprescription science	Evaluates best practices for the reduction or cessation of long-term psychopharmacological treatments, based on biological safety, efficacy, and patient-centered outcomes.
one health physiology	Integrates human, animal, and environmental health to assess and address systemic risks and shared determinants of biological vulnerability.
Sleep, circadian biology, and neurodevelopment	Investigates the physiological impacts of sleep hygiene, light exposure, and daily rhythms on mental resilience, especially in children and adolescents.

This table presents the biological-level frameworks used in this action-research, applied by the researcher to examine physiological pathways of psychiatric harm, guide deprescription protocols, and support translational interventions throughout the dissertation.

The second level of the analytical structure is medical and clinical. It incorporates translational medical science, which connects clinical observation with findings in molecular biology, neuroendocrinology, and psychophysiology (table 8). This approach allows the research to evaluate the biological impact of psychiatric drug regimens, as well as the potential benefits of non-pharmacological interventions grounded in physiology, including nutritional psychiatry and the study of neuroimmune and hormonal dynamics (Sarella et al., 2023). The use of trauma-informed care and narrative medicine expands the scope of analysis beyond symptom management, supporting the recognition of lived experience and biographical context as medically relevant data. These frameworks do not discard pharmacology, but resituate its role within a broader understanding of health that emphasizes causality, recovery, and non-damaging long-term outcomes . Psychiatric care, from this standpoint, ought to be evaluated according to its effects on functionality, autonomy, and quality of life, not only symptom reduction. Within this clinical level, relational and dialogical models of psychiatry, particularly open dialogue and adjacent practices, offer evidence-based pathways that emphasize continuity, transparency, and trust, as listed in table 9. These models are not prescriptive templates but dynamic systems that prioritize participation and context, aligning with international standards such as those advanced by the World Health Organization through its QualityRights initiative, and are widely recognized as best practices to be promoted by United Nations at the global scale, and the Council of Europe in our Union, as well by most user and professional associations working on the implementation of better ways to heal and recover (Fuller et al., 2022). These standards demand the elimination of coercive practices, the integration of person-centered decision-making, and the construction of care environments that uphold dignity and procedural fairness. This research incorporates such principles not only as normative references but as empirical variables, allowing for evaluation of compliance and deviation within real-world settings (Zanardo & Ventura, 2023).

Table 10. Psychological dimension of the analytical and applied frameworks

Component	Description
Extended mind theory	Frames cognition as distributed across brain, body, tools, and environment, offering a non-reductionist model of psychiatric experience.
Embodied cognition	Highlights the role of sensorimotor and interoceptive processes in shaping emotional and cognitive function.
Trauma-informed care	Incorporates the psychological consequences of violence, neglect, and chronic stress into assessment and treatment protocols.
Narrative medicine	Uses autobiographical meaning-making as both diagnostic and therapeutic tools, restoring agency through structured storytelling.
Power Threat Meaning Framework	Replaces diagnosis with contextualized understanding of survival strategies, power dynamics, and subjective meaning.
Mentalization and relational capacity	Examines the psychological development of reflective functioning, empathy, and secure attachment, as prerequisites for mental health.

This table lists the psychological-level models used throughout the dissertation's fieldwork and analysis to interpret subjective experience, enable shared understanding, and reconstruct meaning in contexts of psychiatric violence and recovery.

The evaluation of clinical practice is therefore not limited to outcome metrics but includes procedural, ethical, and relational dimensions. This requires the articulation of metrics that are sensitive to harm reduction, social inclusion, and long-term health, beyond immediate crisis resolution or pharmacological containment. The scientific validity of psychiatric interventions ought to be judged by their cumulative effects on human flourishing, especially in cases where iatrogenic damage has been normalized. This demands a clinical science that is both rigorous and reflexive, capable of rethinking its own criteria of efficacy and appropriateness.

The third dimension of the analytical architecture centers on legal, structural, and policy frameworks, which are not treated as external addenda but as integral to the functioning and reproduction of psychiatric systems, as shown in table 10. Coercive practices in psychiatry are not the result of individual negligence alone, but of legal permissibility and institutional design. This thesis engages in a rigorous analysis of the legal regimes that authorize involuntary treatment, restrict patient agency, and facilitate structural impunity. Spanish constitutional law, European human rights jurisprudence, and international treaties such as the Convention on the Rights of Persons with Disabilities provide the normative basis for such examination (World Health Organization, 2020). These legal instruments demand a reconfiguration of care that recognizes equal capacity, prohibits degrading treatment, and institutionalizes safeguards for informed and supported decision-making. The thesis evaluates the extent to which these obligations are unmet, documenting the structural and procedural gaps that perpetuate harm under the guise of care. Beyond individual laws and policies, the work interrogates the administrative logics and bureaucratic inertia that stabilize harmful practices. Institutional complicity is examined as a systemic feature, not as aberration, revealing how courts, health authorities, and educational systems operate in concert to enforce pharmacological solutions as proxies for care. Public accountability is approached not as a normative goal but as an empirical variable, subject to

structural forces, political resistance, and professional self-interest. This necessitates a legal analysis that includes both doctrinal scrutiny and institutional ethnography, capturing how rights are interpreted, circumvented, or suspended in everyday clinical governance (Gaete & Fritsch, 2021; Davies, Read, & McKeown, 2022).

Table 11. Social dimension of the analytical and applied frameworks

Component	Description
one health education	Promotes integrative knowledge and care literacy across schools, communities, and professional networks.
Action research and co-design	Engages affected individuals and professionals in collaborative processes to identify, implement, and assess systemic change.
Disability rights and legal frameworks	Aligns psychiatric practice with international human rights law, promoting supported decision-making and autonomy.
Public health literacy and institutional transparency	Advocates for accessible information, participatory governance, and accountability in mental health systems.
Relational and dialogical psychiatry	Centers care on interpersonal relationships, continuity, and mutual understanding, including models like open dialogue.
Cultural and educational reform	Embeds ethical, health-promoting values into school curricula and university programs to prevent future coercion and exclusion.

This table presents the social-level frameworks and policy-based approaches applied during the action-research to redesign systems of care, promote legal compliance, and engage educational and public structures in sustained reform.

In this context, models of supported decision-making, advance directives, and community-rooted accountability mechanisms are presented as scientific and legal necessities, not optional reforms. Their integration is evaluated in light of existing institutional capacities, resistance dynamics, and the epistemological shift required to reposition patient narratives as valid sources of knowledge and legal standing. The aim is to map both the obstacles and the entry points for reform, grounded in empirical data and consistent with international best practices. This legal and institutional lens is essential to understanding not only what goes wrong in psychiatric care, but why it persists despite evidence, and what systemic conditions ought to change to enable redress and transformation.

The fourth component of this analytical scaffolding, under development at the time of printing this dissertation, concerns the computational and methodological systems capable of capturing, modeling, and intervening upon the complex realities documented throughout the research. While the primary evidence base remains ethnographic and legal, the broader research agenda, already initiated through pilot data and institutional collaborations, incorporates advanced computational frameworks designed to ensure scalability, replicability, and actionable precision in mental health system reform. These frameworks are not ancillary but foundational to the thesis's translational intent, anchoring the possibility of system redesign in empirical and statistically robust architectures. Bayesian statistical modeling is proposed as a central tool for assessing clinical trajectories, estimating treatment effects, simulate agents-based preventive interventions, and also simulating broader policy ones. Its probabilistic logic allows for the incorporation of uncertainty, heterogeneity, and dynamic feedbacks, offering a more accurate and ethically responsive mode of

analysis than deterministic or purely linear models. In parallel, survival analysis methodologies are introduced to evaluate treatment outcomes over time, particularly in relation to medication adherence, relapse, and mortality under coercive regimes. These tools are designed to expose long-term consequences and interdependencies that are otherwise occluded in short-term or cross-sectional studies. In addition to inferential models, the research framework incorporates system mapping and causal inference modeling to depict institutional interactions, power asymmetries, and feedback loops within psychiatric care systems. These methods enable the formal identification of leverage points where targeted interventions may yield structural change. Rather than treating institutions as static environments, the models recognize them as dynamic systems shaped by legal, cultural, clinical, and political forces. This aligns with the biocultural and action-research foundation of the thesis, which demands tools capable of tracing both situated experience and macro-systemic consequences (Cook et al., 2019).

Although the dissertation itself focuses primarily on qualitative, narrative, and field-based data, it explicitly situates these elements within a broader translational infrastructure. This architecture is already under construction through the EU BEACON One Health Education COST Action¹, which was established as a direct continuation of this doctoral research to enable its sustainability. The projects under development include expert system prototypes, decision-support tools, and integrated monitoring platforms designed to embed rights-based care into clinical and educational settings. The groundwork laid in this thesis thus feeds directly into a coordinated European research agenda, seeking not only to critique systemic failures but to co-design replicable, validated, and ethically defensible alternatives rooted in data and participatory governance (Frerichs et al., 2016).

This final segment of Section 1.9 consolidates the thesis's role as both a standalone scientific inquiry and a launching platform for broader systemic transformation. The research, while grounded in rigorous academic practice and guided by defined epistemological, clinical, and legal frameworks, has also generated operational pathways toward implementation. These are formalized through the EU BEACON One Health Education COST Action², which provides a structured environment for continuity, scale, and institutional coordination beyond the immediate scope of the dissertation. This continuity is not an adjunct but an intentional outcome of the action-research methodology, which posits that valid knowledge ought to be capable of informing and shaping practices within the systems it seeks to transform.

The thesis has catalyzed several interlinked research lines now developed into large-scale Horizon Europe proposals. These include projects focused on school-based cognitive health through structured routines in exercise, sleep, nutrition, and supplementation; environmental health initiatives targeting neurotoxic exposures in childhood; rights-based reforms of child and adolescent mental health systems grounded in shared decision-making and relational care; and institutional mentoring mechanisms to foster meaning, accountability, and safety within educational and clinical

1 The EU COST Science and Technology Coordination action the researcher founder as main avenue for the action side or action research, devoted to one health education, by the time of printing this dissertation spanning over twenty countries with a network about half thousand devoted experts under its leadership: <https://health.int.eu.org/>

2 An EU COST action is an interdisciplinary research network that brings researchers and innovators together to investigate a topic of their choice for 4 years. COST Actions are typically made up of researchers from academia, SMEs, public institutions, and other relevant organisations or interested parties, to grow their professional research networks while reaching a common goal: <https://www.cost.eu/cost-actions/what-are-cost-actions/>

settings. Each of these proposals derives conceptually and empirically from the findings, frameworks, and needs identified in this dissertation, and each applies the methodological structure laid out in this chapter to real-world systems.

Table 12. WHO QualityRights key dimensions of mental health system transformation

WHO QualityRights Dimension	Description and relevance to the dissertation
1. The right to an adequate standard of living	Addresses housing, food, water, and living conditions as fundamental to recovery. Informs the dissertation's critique of neglect and socioeconomic abandonment within psychiatric systems.
2. The right to enjoyment of the highest attainable standard of physical and mental health	Underpins the demand for rights-based, trauma-informed, and non-coercive care. Guides the proposed transition from pharmacocentric practices to integrative, multidisciplinary approaches.
3. The right to legal capacity and personal autonomy	Directly supports the dissertation's emphasis on supported decision-making, advance directives, and the rejection of substituted judgment in psychiatric contexts.
4. Freedom from torture and cruel, inhuman, or degrading treatment	Frames coercive medication, forced hospitalization, and epistemic violence as violations of international law. Strengthens the scientific and ethical imperative to prevent and redress harm.
5. The right to independent living and inclusion in the community	Reinforces the research agenda for deinstitutionalization, meaningful community supports, and the structural conditions necessary for participation and recovery.

This table presents the five core themes of the WHO QualityRights framework as applied in the action-research project. These dimensions structure the evaluation, critique, and redesign of psychiatric care systems, promoting alignment with international legal standards and evidence-based, person-centered care.

This research follows the one health model as an integrated scientific and governance framework, linking human, environmental, and institutional determinants of health through shared knowledge infrastructures, including ethical safeguards, participatory mechanisms, and multidisciplinary research teams, and are designed to generate real-world evidence at both the individual and population levels. It aligns with the Sustainable Development Goals, the World Health Organization QualityRights standards listed in the table 11 above, and international consensus on the need to implement proper gendered, cultural and trauma-informed, care personalized through due diligence and social networks study, rights-based, and context-sensitive care, denying all chances of iatrogenic harm and the repetition of crimes against the patients. These frameworks offer scientific legitimacy, policy relevance, and operational viability to the analytical models developed in the present work. It all will be revisited in the next chapter, on methodology.

This chapter has outlined the conceptual, legal, clinical, and systemic premises upon which the dissertation is built, identifying the structural deficits, epistemic distortions, and institutional practices that perpetuate harm under the guise of care. It has established the necessity of a paradigmatic shift, one grounded not in abstract ideals but in rigorously validated scientific, legal, and participatory frameworks. What follows in the work is a comprehensive articulation of how these principles are implemented through multi-sited fieldwork, participatory action research, and

surveying. This dissertation advances a scientifically and medical urgent imperative: to restore, preserve and enhance human well-being, integrity, and autonomy across all domains of life.

Chapter 2 - Methodological framework: action research and positionality

2. Reflexive meta-methodology and scientific methodological frameworks

This dissertation was constructed through a transdisciplinary, reflexive process rooted in scientific ethnography, participatory action research, and a materialist conception of knowledge as historically situated, politically constrained, and ethically imperative. Far from being a passive aggregation of results, the structure and content emerged through recursive engagements with suffering, resistance, and truth-seeking under extreme adversity. Protocols included parallel tracking of field data and analytical reflections; open science practices such as timestamped documentation, public repositories, and expert feedback loops; and constant alignment with the ethical mandates of trauma-informed, human rights-based, and culturally grounded care. The order of chapters and results follows both the empirical completion of each dataset and their systemic implications, not theoretical abstraction (Subramani, 2019).

The research design, implementation, and narrative construction were developed as living systems embedded in networks of care, struggle, and systemic transformation. This involved building digital infrastructure, coordinating expert systems, maintaining contact with affected populations, and enacting continuous epistemic checks and balances. The methodology honors the foundational insight of Paul Farmer: to do research in a field of suffering without contributing to its alleviation is a betrayal (Farmer, 1999; Farmer, 2003). The result is a dissertation that is not only a scientific document but also a practical act of defiance against epistemic injustice and institutional complicity, offering methodological transparency as a condition for credibility and ethical responsibility.

2.1 Overall approach and methodological foundations

The methodological core of this dissertation is constructed upon a biocultural, transdisciplinary, and action-research foundation designed to generate valid, situated, and transformable knowledge. Psychiatric violence, institutional betrayal, and coercive health governance are not conceptualized as episodic failures but as systemically embedded phenomena. Accordingly, the inquiry does not operate within the boundaries of disciplinary silos or abstract theoretical modeling but instead deploys an integrated methodology that aligns clinical, legal, anthropological, and computational frames. This approach is inductive and anchored in field-based epistemologies, with the researcher positioned as both a practitioner and an affected subject. However, reflexivity is operationalized scientifically, not sentimentally, with embedded positionality treated as a source of epistemic access, not bias. Drawing upon the epistemological contributions of Miranda Fricker and Kristie Dotson, the methodology foregrounds testimonial and hermeneutical injustice as systemic conditions that obstruct the production and reception of patient knowledge (Fricker, 2017). The

methodological commitment to epistemic justice is not rhetorical but implemented through structured triangulation, rigorous coding procedures, and respondent validation, ensuring that user perspectives are systematically included, not selectively incorporated (Dotson, 2012).

Table 13. Medical anthropology and structural violence

Methodological Foundation	Expert	Why Included	How Operationalized in This Research
Medical Anthropology / Structural Violence	Paul Farmer	Linked structural inequality with medical outcomes; coined structural violence	Provides analytical tools to connect psychiatric abuse with broader institutional, economic, and legal structures
Critical Medical Anthropology	Nancy Scheper-Hughes	Documented state and institutional violence through ethnography in clinical contexts	Anchors the investigation of embodied suffering and epistemic erasure under psychiatric governance
Transcultural Psychiatry	Arthur Kleinman	Developed explanatory models and the concept of local biologies	Supports context-sensitive understanding of diagnostic and therapeutic interactions
Narrative Psychiatry / Medical Anthropology	Byron Good	Explored idioms of distress and narrative representations of mental illness	Used to guide interpretive methods for analyzing subjective experience and recovery narratives

Foundational scholars in medical anthropology and structural violence informing the ethnographic, clinical, and epistemic analysis. Their work underpins the conceptual tools used to document institutional harm, contextualize psychiatric narratives, and decode systemic pathologization.

This methodological structure is informed by foundational texts in medical anthropology and global health praxis, including the participatory ethos of Orlando Fals-Borda, the biosocial analytical lens of Paul Farmer, and the ethnographic attention to structural and corporeal violence developed by Nancy Scheper-Hughes (Scheper-Hughes & Lock, 1987; Fals-Borda, 1988). Each of these frameworks contributes to the transdisciplinary rationale of the research, which does not aim to describe isolated events but to expose and reconfigure the multilevel apparatus of psychiatric governance in Spain. The empirical focus extends across community settings, clinical environments, administrative processes, and legal infrastructures. Data are obtained through triangulated methods that include extended participant observation, semi-structured and open-ended interviews, surveys across stakeholder groups, and archival and legal document analysis. These materials are not merely juxtaposed but integrated into an analytical architecture capable of producing both grounded findings and structurally actionable knowledge. The methodology is not designed to confirm hypotheses derived from normative models of mental illness but to extract and evaluate patterns of harm, accountability, and recovery within real-world psychiatric governance. When it comes to the core principles of action-research, those were applied despite operating under conditions that precluded full institutional iteration and immediate reconfiguration of systemic practices. Recognizing the constraints of conducting structural inquiry from a non-executive academic position, the research design implemented the principles of participatory observation, dialogical engagement, reflexive responsiveness, and evidence-based feedback loops in all domains

accessible to the researcher. Rather than treating action-research as contingent on the possession of formal authority or institutional leverage, this project operationalized it as a methodological ethos, embedded in the design of instruments, the structuring of fieldwork, the co-production of knowledge, and the long-term transformation of research environments.

The work was embedded from the outset in collaborative structures designed to enable collective learning and iterative redesign. This was exemplified by the EU BEACON One Health Education network's action, which emerged as a translational extension of the epistemic and structural principles articulated in the dissertation (Freire, 2000). Over a period of two years, the proposal underwent multiple revisions, informed by professional feedback, formal evaluation criteria, stakeholder priorities, and implementation realities. The final success of the action was not the outcome of linear execution, but of recursive cycles of redefinition, recalibration, and epistemic accountability, precisely the model of system-level iteration advocated within the action-research tradition. These same iterative principles were applied across multiple scales of the researcher's academic and clinical engagement, including the refinement of survey instruments, the structuring of collaborative field relations, the design of public outreach activities, and the open peer review of analytical tools.

Table 14. Action research and participatory inquiry

Methodological Foundation	Expert	Why Included	How Operationalized in This Research
Participatory Action Research	Orlando Fals-Borda	Pioneer of participatory action-research; emphasized co-production of knowledge and community empowerment	Provides the epistemic base for ethical, collaborative inquiry involving affected persons in research design and implementation
Critical Participatory Action Research	Paulo Freire	Introduced dialogical methods and consciousness-raising as essential to social transformation	Informs the political and educational dimension of the action-research design in mental health systems
Democratic Dialogue / Reflective Practice	Peter Reason	Advanced participatory inquiry cycles and reflection-based collective learning	Supports recursive feedback processes and shared interpretation of field data across participant groups
Systemic Action Research	Hilary Bradbury	Developed systemic framing of action research across health, education, and governance	Enables cross-scalar integration and the framing of institutional reform as embedded systemic change

Core contributors to participatory and systemic action-research approaches applied in this dissertation. These figures provide the epistemological and practical basis for collaborative, iterative, and ethically situated research design in the context of psychiatric system transformation.

In this context, the researcher did not assume the role of external evaluator or omniscient diagnostician (Heron & Reason, 1997; Reason & Bradbury, 2001). Rather, the role was framed as an embedded, epistemically responsible actor, simultaneously documenting and participating in collective meaning-making, institutional rethinking, and structural critique. The absence of direct managerial control over clinical or institutional protocols was not interpreted as a limitation to methodological integrity, but as a reflection of the real-world conditions under which reform ought

to often be initiated. The action-research orientation was thus applied not through formal cycles of implemented change and reassessment at scale, but through ethical embedding, methodological co-design, and the consistent application of open science practices. All survey instruments, field protocols, and analytical structures were shared for public comment, subjected to international feedback, and iteratively refined in light of evolving contexts. On a granular level, the action-research methodology informed the researcher's adaptive decision-making in response to field conditions, participant availability, legal barriers, and institutional silencing. It was manifested in the sustained effort to preserve epistemic fidelity in adverse contexts, to document harm with methodological rigor, and to redirect research outputs toward useable, scalable formats capable of guiding institutional correction. These efforts included the design and dissemination of open-source expert systems, digital diagnostic pathways, evidence-based deprescription protocols, and publicly verifiable legal-ethical documentation frameworks. Each of these instruments was conceived not as static deliverables but as modular tools subject to community refinement and practical validation.

The researcher's own practices were governed by the same iterative logic, with regular audit of methodological decisions, ethical self-assessment, and incorporation of peer critique. Where institutional power to implement change was lacking, the researcher generated counter-structures of verification and protection through affiliation with international networks, horizontal peer groups, and professional associations committed to human rights, open science, and mental health system reform. These constellations served as recursive platforms for feedback, refinement, and distributed implementation. The researcher thereby sustained an iterative model of action-research aligned with systemic critique, knowledge co-production, and translational readiness, despite operating from within systems that remain structurally resistant to ethical transformation (Ramírez et al., 2024).

This applied chapter confirms that action-research, properly understood, does not depend solely on cycles of institutional implementation and reassessment. It can also be realized through recursive logic, open participation, principled transparency, and adaptive responsiveness within multilevel fieldwork (McDowell et al., 2023). The core principle, that research ought to be co-produced, situated, and directed toward actionable transformation, with the victims at the forefront of all concerns, remained operative throughout this dissertation, guiding its design, interpretation, and translational agenda.

The study is also aligned with the one health framework as a scientific system that integrates ecological, biological, technological, and social determinants of health; among other transversal core values and ways to understand the ongoing (Simmons, 2025). The research applies this principle not at the level of metaphor but through its data architecture, linking gut-brain interaction, endocrine disruption, and neuroimmune responses with institutional routines, diagnostic practices, and structural determinants of vulnerability. Nutritional psychiatry, psychoneuroimmunology, and trauma-informed care are treated as scientifically established and clinically relevant frameworks that ought to be integrated into the evaluation of psychiatric interventions. The inclusion of embodied cognition and extended mind theory enables the rejection of dualistic and reductionist assumptions prevalent in psychiatric classifications and therapeutic protocols. Cognition is not confined to the brain but expressed through relational, material, and cultural configurations. This

perspective is not philosophical but applied, guiding the interpretation of testimony, the selection of clinical metrics, and the mapping of systemic violence.

Table 15. Epistemic justice and knowledge inclusion

Methodological Foundation	Expert	Why Included	How Operationalized in This Research
Epistemic Justice	Miranda Fricker	Defined testimonial and hermeneutical injustice in knowledge production	Justifies methodological imperative to recognize suppressed or disqualified patient perspectives
Black Feminist Epistemology	Kristie Dotson	Expanded the framework of epistemic injustice; emphasized structural silencing	Strengthens intersectional analysis of exclusion in psychiatric settings and data interpretation

Methodological sources from the field of epistemic justice and feminist epistemology that support the inclusion of silenced, discredited, or structurally excluded knowledge within the scientific framework of this dissertation. These references guide the validation of user testimony and protection against epistemic extractivism.

The methodological structure of this dissertation is grounded in action research not only as a participatory framework, but as an epistemologically necessary approach when investigating systemic harm and institutional violence. Following the foundational work of Orlando Fals-Borda, Paulo Freire, and Peter Reason, the research situates knowledge production within dialogical, iterative, and socially embedded cycles of inquiry. This approach demands the active involvement of affected populations, not merely as subjects of research but as co-constructors of knowledge and accountability. In line with Hilary Bradbury's contributions to systemic action research, the project integrates multilevel feedback loops and stakeholder engagement mechanisms to ensure that insights are not abstracted from practice but directly inform strategies for institutional reform and public health transformation. This action-research design is ethically responsive, methodologically transparent, and structurally aligned with the goals of psychiatric system change as articulated in the one health governance model (Simmons, 2025).

Complementing this participatory structure, the methodological foundation draws extensively from medical anthropology to trace the sociocultural and historical conditions under which psychiatric practices have normalized coercion, diagnostic silencing, and institutional neglect. The critical contributions of Paul Farmer, Nancy Scheper-Hughes, Arthur Kleinman, and Byron Good provide the framework for interpreting psychiatric harm as a manifestation of structural violence embedded within legal, clinical, and epistemic systems (Farmer & Kleinman et al., 2013). Farmer's biosocial model links health outcomes with macro-social determinants, and his concept of structural violence underscores how routine practices in medicine may function as instruments of domination. Scheper-Hughes's ethnographic accounts of medicalized brutality and corporeal degradation reveal the necessity of situating psychiatric acts within a wider matrix of bureaucratic and affective disavowal. Kleinman and Good, through their development of explanatory models and idioms of distress, offer tools for recognizing the plurality of suffering narratives that are otherwise pathologized or dismissed within biomedical frames (Good, 1992; Good, 1994). These anthropological models are

not merely cited but operationalized through coding frameworks, narrative analysis, and the interpretive logic of the fieldwork, ensuring that systemic meaning, power asymmetry, and patient language remain central to data interpretation.

Table 16. Systems evaluation and institutional analysis

Methodological Foundation	Expert	Why Included	How Operationalized in This Research
Realist Evaluation	Trisha Greenhalgh	Applied realist approaches to evaluating complex health interventions	Grounds triangulation strategy and enables interpretation of multi-level contextual factors
Institutional Ethnography	Howard Becker	Introduced the concept of credibility hierarchies and the labeling of deviance	Informs mapping of psychiatric classification, administrative justification, and institutional complicity
Democratic Psychiatry / Institutional Critique	Franco Basaglia	Led deinstitutionalization in Italy and reframed coercion as systemic repression	Conceptual anchor for critique of psychiatric institutions and participatory alternatives

Key contributors to institutional ethnography, realist evaluation, and democratic psychiatry. Their methodologies inform the thesis's multilevel analysis of psychiatric governance, administrative logic, and accountability structures.

At the biological and clinical level, the research engages with contemporary developments in translational psychiatry and metabolic health sciences to reframe the physiological basis of mental suffering and recovery. The mainstream biomedical paradigm, as represented by Thomas Insel's emphasis on biomarkers and neural circuits, is critically interrogated, particularly regarding its reductionist tendencies and failure to produce therapeutically effective outcomes. In response, the dissertation integrates scientifically validated alternatives from the fields of psychoneuroendocrinology (PNEI), nutritional psychiatry, and metabolic psychiatry. George Chrousos and Bruce McEwen provide the scientific underpinnings for modeling chronic stress and allostatic load as measurable biological consequences of psychiatric and institutional exposure (McEwen & Wingfield, 2003; Chrousos, 2009). The seminal work of Felice Jacka on dietary patterns and mental health establishes a robust link between micronutrient intake, inflammation, and mood regulation, as much as the now thriving field of nutritional psychiatry (Jacka et al., 2015). Christopher Palmer's metabolic framework reinterprets mental disorders through mitochondrial dysfunction and systemic energy dysregulation, offering a new paradigm for understanding and reversing pharmacoresistant symptoms, with metabolic psychiatry also a new field showing spectacular results (Palmer et al., 2022). These models converge with the contributions of Michael Berk (2011) on oxidative stress and John Cryan and Ted Dinan on microbiome-brain interactions, forming a multidimensional physiological architecture that supports deprescription, nutritional intervention, and restoration of neurobiological balance through non-invasive means, all within proper biologic psychiatry, achieving recovery (Dinan & Dinan, 2023).

Myself holds testament of these new paradigms power, as were useful to overcome the damage of forced poisoning endured after and during beatings; before the worse came to be, and still help. It all holds from mens sana in corpore sano, including exercise: the psyche resist more, in a strong

body, well nourished, with the discipline and self-control of proper regimes in an accounts. Together, these frameworks provide not only scientific legitimacy but methodological functionality for evaluating treatments, identifying cumulative harm, and designing restorative strategies that integrate physiological repair with structural transformation. They anchor the clinical analysis of this study in a model of health that is responsive to real-world complexity, biologically plausible, ethically guided, and empirically testable. Rather than substituting one theory for another, this integrated framework creates a methodological infrastructure capable of capturing and correcting the intersecting failures of dominant psychiatric paradigms.

In operational terms, the research architecture is structured around four interdependent levels of analysis: epistemological, medical, legal, and computational. These levels collectively ensure the methodological integrity and translational applicability of the study. The first level, epistemological, applies a biocultural approach grounded in medical anthropology, cognitive science, and critical psychiatry. It conceptualizes mental distress and resilience as products of interacting biological, environmental, historical, and sociocultural forces. This framework allows for the scientific integration of models such as embodied cognition, narrative medicine, the Power Threat Meaning Framework, all as part of a congruent, effective whole at comprehending the real problems ongoing.

Table 17. Translational psychiatry, nutritional, metabolic, and physiological frameworks

Methodological Foundation	Expert	Why Included	How Operationalized in This Research
Translational Psychiatry	Thomas Insel	Promoted circuit-based models and biological biomarker discovery in psychiatry	Referenced critically in the analysis of psychiatry's shift toward neuroreductionism and biomarker-driven classification
Psychoneuroendocrinology (PNEI)	George Chrousos	Defined stress biology and its immuno-endocrine pathways	Underpins trauma-informed biological models and stress-response systems integrated into clinical analysis
PNEI / Stress and Inflammation	Bruce McEwen	Developed the concept of allostatic load in stress and neuroplasticity	Supports evaluation of chronic psychiatric stress responses and the long-term effects of iatrogenic harm
Nutritional Psychiatry	Felice Jacka	Pioneered large-scale research linking diet quality and mental health	Provides empirical base for dietary interventions and gut-brain axis as a modifiable risk factor
Metabolic Psychiatry	Christopher Palmer	Advocated for rethinking mental illness through mitochondrial and metabolic frameworks	Informs the biological plausibility of deprescription and nutrient-based intervention in treatment-resistant cases
Psychobiotics / Gut-Brain Axis	John Cryan / Ted Dinan	Introduced concept of psychobiotics and gut microbiome effects on mood and cognition	Integrated into analysis of physiological vulnerability and supports alternative therapeutic mechanisms
Inflammation and Psychiatry	Michael Berk	Demonstrated links between inflammation, oxidative stress, and psychiatric symptoms	Anchors systemic biomarkers as valid variables in clinical modeling and ethical deprescription protocols

Scientific references supporting the dissertation's translational medical model. These scholars contribute to the analysis of neuroendocrine, inflammatory, nutritional, and metabolic dimensions of psychiatric distress and recovery, validating biological targets for non-pharmacological intervention, deprescription, and system-wide physiological safety.

The second level, clinical and medical, applies translational science to examine the biological impact of psychiatric interventions and the somatic dimensions of recovery. It engages with evidence from nutritional psychiatry, psychoneuroimmunology, and trauma-informed care to assess pharmacological regimens, functional outcomes, and long-term effects of treatment. Here, psychiatric practices are not assumed to be evidence-based but subjected to critical evaluation in light of emerging biomedical knowledge. Concepts such as iatrogenesis, polypharmacy burden, and symptom substitution are analyzed using comparative case documentation, theoretical saturation,

and physiological plausibility assessments. The focus remains on function, autonomy, and health outcomes, with symptom control considered insufficient as a primary clinical endpoint.

The third level, legal and institutional, examines the structural parameters that enable or inhibit coercive psychiatric practices. Spanish constitutional guarantees, European jurisprudence, and the Convention on the Rights of Persons with Disabilities are treated as operational tools for empirical evaluation, not abstract legal ideals (Deshpande et al, 2020). The methodology includes a dual analysis combining doctrinal legal interpretation with ethnographic observation of institutional behavior. It documents how legal frameworks are applied, ignored, or distorted in psychiatric governance. Unfortunately, the ongoing torture denied the chance to take forward this level, the researcher being barely able to start scratching the surface due to the violence faced (Stover & Nightingale, 2000; Shaw, 2007; Russell, 2009).

The fourth level, methodological and computational, is under active development but already incorporated into the next steps of this research and its translational pathway. While not fully implemented within the body of this dissertation for the very same reason, these tools are being designed and partially tested through ongoing collaborations. Plans, enacted through the actions, include expert system prototypes for decision support in medication management, Bayesian models for the analysis of treatment trajectories, and survival analysis techniques to assess long-term risks under coercive conditions (Ben-Ari, 2002). These components enable the future scalability of the research by translating qualitative findings into interoperable, replicable, and system-level tools. The integration of system mapping, causal inference modeling, and dynamic simulation permits the formal identification of intervention points and the quantification of harm across institutional domains. These methods are foundational to the structural transformation agenda articulated in the research, anchoring reform not in ideological critique but in statistically valid, ethically governed, and technically actionable systems. Although the legal, computational and systemic implementation stage continues beyond the scope of this dissertation, its architecture is fully aligned with the methodological rationale, institutional mandate, and scientific rigor of the work presented herein. Fieldwork is also ongoing to study the continuum of domestic and structural mental health services social violence in Indonesia, through the work to abolish pasung (Patel, Williams, & Kellezi, 2018).

2.2 Mixed and multi-scalar design, triangulation of data and findings

The present dissertation adopts a mixed-methods and multi-scalar design to examine psychiatric coercion, medical violence, and institutional impunity as systemic phenomena operating across personal, organizational, and legal domains. The study is neither confined to individual cases nor abstracted into policy analysis; rather, it seeks to construct an evidence-based, multi-layered account of how psychiatric harm is produced, normalized, and resisted. The integration of qualitative and quantitative methods, aligned with micro-, meso-, and macro-level inquiry, was driven by the structural complexity of the research questions and the biocultural orientation of the overall study (Farmer, Robinson, Elliott, & Eyles, 2006).

Qualitative methods, including ethnographic fieldwork, in-depth interviews, participant observation, and continuous reflexive documentation, were essential for capturing situated experiences of psychiatric coercion, degradation, and silence. These tools allowed the documentation of institutional betrayal, relational harm, and survival strategies in both clinical and extra-clinical settings. Field diaries and embedded observation enabled the researcher to track systemic patterns and institutional logics in real time. In parallel, open surveys with mixed-item formats combined narrative prompts with categorical questions, yielding a hybrid dataset of both descriptive metrics and qualitative testimonies. These quantitative components were treated not as standalone evidence, but as distributional anchors and cross-checks for emerging themes (Parker, 2007).

Data triangulation in this dissertation was achieved through the integration of four distinct sources: survey responses provided quantifiable patterns and frequency of systemic practices across diverse professional and user groups; testimonies offered narrative depth and situated accounts of lived experiences within psychiatric systems; legal documents enabled verification of institutional procedures, highlighting discrepancies between normative frameworks and actual practice; and personal experience, documented through reflective observation and participatory engagement, provided critical insight into the mechanisms of coercion and impunity. This multi-source strategy allowed for cross-validation of findings, reinforced internal coherence, and ensured both empirical robustness and contextual sensitivity in the interpretation of results (Carter, Bryant-Lukosius, DiCenso, Blythe, & Neville, 2014). The use of both modalities was essential to triangulate data, evaluate epistemic consistency, and prepare scalable formats for future implementation. Each of the six core research questions articulated in Chapter 1 was operationalized through specific methodological techniques and allocated to appropriate scales of analysis:

Table 18. Alignment of research questions with analytical levels, methods, and its relevance

Dissertation Aim	Aligned Research Question	Operational Objective	Analytical Level	Methods	Relevance to Dissertation
Support psychiatric systems capable of real-time correction, accountability, and integrity preservation	What forms of systemic violence and institutional failure are occurring in psychiatric governance, and how can they be corrected?	Identify and analyze the structural, legal, and epistemological failures in psychiatric care to develop evidence-based reforms	Meso and Macro	Legal analysis, professional surveys, policy document review	Anchors the investigation of contradictions between legal obligations and routine clinical practice; supports institutional critique and reform framework
Foster a paradigmatic shift from pharmacological control to integrative, interdisciplinary, and reciprocal models of care	How can clinical and experiential evidence be integrated across biological, psychological, and social domains of health?	Develop and validate feedback systems, biosignal-informed tools, and patient-led evaluative mechanisms	Micro and Meso	Testimonial coding, trauma-informed narrative mapping, medical literature review	Justifies integration of trauma theory and physiological models; grounds the biomedical and ethical critique of psychiatric environments

Dissertation Aim	Aligned Research Question	Operational Objective	Analytical Level	Methods	Relevance to Dissertation
Realign psychiatry with empirical coherence, legal safeguards, and rights-based participatory governance	What are the enabling conditions for institutional change and participatory oversight of psychiatric systems?	Implement translational infrastructures, policy interventions, and participatory monitoring aligned with international standards	Micro, Meso, Macro	Survey analysis, policy mapping, expert system prototyping	Informs the translational design of interventions, aligns with WHO QualityRights, and supports ethical systems transformation
Uphold narrative justice and prevent epistemic erasure within psychiatric systems	What are the structural and epistemic mechanisms through which psychiatric users are silenced, discredited, or neutralized as knowers?	Construct participatory analysis models to elevate user voice and reconstruct explanatory legitimacy	Micro and Meso	Ethnography, interviews, narrative coding, institutional discourse analysis	Supports the analysis of epistemic injustice and methodological recognition of silenced or disqualified knowledge
Embed human rights and legal safeguards in psychiatric operation	How do national legal systems (particularly Spain's) enable or prevent rights-based psychiatric care and redress?	Expose national and regional gaps between law and psychiatric practice and support enforceable legal reform	Macro	Constitutional and international legal analysis, institutional ethnography	Central to the legal-democratic framework of the thesis; enables normative evaluation of psychiatric governance
Develop an open, reflexive, and verifiable scientific framework	What kind of scientific and methodological structure is required to ensure transparency, accountability, and legitimacy in psychiatric knowledge production?	Construct biocultural meta-methodologies, transparent data flows, and ethical systems architecture	Meso and Macro	Meta-methodological documentation, computational modeling design, open science protocols	Justifies the methodological framework of the dissertation and enables future implementation of open, ethically grounded scientific systems

Analytical correspondence between the six core research questions and the levels, methods, and systemic relevance assigned within the mixed and multi-scalar design. Each question was operationalized according to its analytical scope and methodological requirements, ensuring alignment with the epistemological, legal, clinical, and translational dimensions defined in Chapter 1.

The articulation of these questions, as noted, was not conducted under ordinary academic conditions. The entire research process unfolded under conditions of extreme adversity, including institutional silencing, professional exclusion, and sustained personal exposure to violence that meets multiple clinical and legal criteria for psychological torture. These conditions impaired access to institutional cycles of feedback, restricted fieldwork continuity, and limited the co-production mechanisms that action research prescribes. Yet, rather than invalidate the inquiry, these realities provide the foundational context for understanding its urgency, its constraints, and its ethical weight. The table should thus be read not only as a planning matrix, but as a map of resistance

under coercive constraint, committed to scientific transparency, human dignity, and structural redress. In any and all cases, the alignment between questions, methods, and levels ensured coherence and scientific integrity across the entire research process. The micro level captures the lived effects of psychiatric violence, diagnostic violence, force, epistemic silencing, through personal narratives, clinical trajectories, and bodily outcomes of the research, other professionals, patients at large, and other victims and their loved ones. The meso level reveals how institutions such as public hospitals, health centers, ombudsman offices, child protection services, and judicial bodies implement, justify, or challenge coercive practices whenever possible. The macro level contextualizes these patterns within constitutional frameworks, global health governance, and transnational psychiatric paradigms, including the role of the World Health Organization, the United Nations Convention on the Rights of Persons with Disabilities, and European Union directives.

The inductive-deductive dialectic governs the full analytic process on the background. Field-based data, narratives, observations, testimonies, generated grounded categories and interpretive patterns, which were then tested against deductively established frameworks: legal doctrine, diagnostic taxonomies, and international health standards (Adorno & Horkheimer, 1944/2002). Where dissonance emerged, theoretical revision or recalibration was performed. This allowed the study to move beyond confirmation or falsification, and instead operate as a dynamic, abductive inquiry into system-level truth conditions and methodological viability. The lived experience of torture forced to understand reflexively the harshest realities in mental health and our society. In short, the mixed and multi-scalar design of this dissertation does not merely support the inquiry; it constitutes its ethical, scientific, and structural form. It enables the research to fulfill its foundational imperative: to produce knowledge capable of identifying, contesting, and repairing the conditions under which psychiatric care becomes a vector of institutional harm rather than a site of healing.

The architecture of the study was deliberately configured to ensure internal coherence between methods, levels of analysis, and the epistemic aims of the dissertation. The justification for combining ethnography, participant observation, and testimonial inquiry with structured surveys and quantitative categorization lies in the dual need to respect the complexity of lived experience while enabling systematic mapping of institutional practices and structural failures. Each method was selected for its functional capacity to interrogate particular layers of the psychiatric apparatus, from subjective distress to macro-legal permissibility. The division into micro, meso, and macro levels here exposed reflects not only analytical convenience but the ontological reality of psychiatric violence as a distributed, multilevel system. The inductive-deductive dialectic governing the analysis ensured that findings emerged through recursive iteration: grounded in situated accounts, challenged by normative frameworks, and validated through theoretical and legal triangulation. By making explicit the alignment of questions, methods, and scales, the study secures its methodological rigor and reinforces its core function, not to describe suffering in isolation, but to connect it scientifically, legally, and ethically to the structures that produce and normalize it. The limits of autonomy, the options discarded or offered, are all socioculturally determined as well.

2.3 Data collection instruments and infrastructure

This research employed three core survey instruments, each designed to gather situated, ethically-sensitive, and structurally relevant data within the Spanish psychiatric landscape. The surveys were constructed to document the lived realities of mental health users, relatives, and professionals, while simultaneously functioning as action-research tools capable of fostering reflection, critique, and participatory engagement. All three instruments were tailored to the Spanish context, integrating conceptual frameworks from medical anthropology, legal analysis, and clinical ethics. Each contributed complementary insight into psychiatric practices, epistemic hierarchies, and institutional behavior.

The first instrument, *Schizophrenia: A Study on Psychiatric Labels*, was an adapted version of a validated survey previously deployed in the United Kingdom. The original tool, reviewed and refined through interdisciplinary expert feedback, was modified to reflect cultural, legal, and institutional specificities of the Spanish setting. Its purpose was to examine how psychiatric labels, particularly schizophrenia and psychosis, are experienced, interpreted, and contested by users, relatives, and professionals. The instrument included structured questions alongside narrative prompts, enabling respondents to articulate both categorical and experiential responses. A grand total of about four hundred full responses were collected, representing a wide range of roles within the mental health ecosystem. The methodology allowed for both quantification of trends and the recovery of qualitative testimony concerning diagnostic stigma, iatrogenic harm, and epistemic disqualification. This survey served as the foundation for a systemic critique of labeling practices and was instrumental in exposing their long-term psychosocial and institutional consequences.

The second survey, *open dialogue in Spain*, was developed specifically for this research in collaboration with clinicians, educators, and researchers involved in the promotion of dialogical practice in mental health. It represented the first national-scale attempt to assess the level of knowledge, training availability, and institutional engagement with the open dialogue framework across Spanish mental health professionals. The instrument was disseminated through national professional networks, educational seminars, and clinical collaborations. While primarily quantitative in structure, the survey also captured open commentary that reflected ethical tensions, institutional resistance, and emerging interest in alternative care models. As part of a peer-reviewed publication, the data derived from this instrument were subjected to rigorous scientific review and now form a documented contribution to the national literature on dialogical psychiatry. Its function within this thesis is dual: it maps the current state of reform-oriented interest within the system and offers empirical evidence that reform is both necessary and already in motion.

The third survey, *Shared Decision-Making and Medication Management*, was designed to address psychiatric prescribing practices, informed consent, autonomy, and the challenges of responsible deprescription. Its scope included multiple psychiatric contexts and target groups, from acute care to long-term treatment scenarios. The survey invited perspectives from service users, caregivers, and mental health professionals, and was active for 105 days. It received 90 complete responses, each averaging over 40 minutes in duration, an indicator of both complexity and respondent commitment. The survey employed a mixed architecture of scaled items and open-ended questions,

enabling triangulation of clinical, ethical, and experiential perspectives. The design was iterative and evolved in response to pilot testing, respondent feedback, and alignment with international frameworks such as WHO's QualityRights. This instrument offered granular insight into the operational and moral failures of current prescribing regimes and outlined professional and experiential alternatives grounded in person-centered, network-based care.

Each of the three surveys was integrated within a broader matrix of methodological tools, including ethnographic fieldnotes, legal documents, clinical testimonies, and institutional discourse analysis. While each survey addressed a discrete thematic focus -diagnostic validity and iatrogenic harm, dialogical practice, and medication governance- they were not isolated in practice. Their construction was coordinated to allow analytical triangulation: key constructs, e.g., autonomy, coercion, trust, trauma, shared responsibility; were mirrored across instruments using distinct but interoperable language. This design allowed for cross-comparison of epistemic patterns, policy expectations, and professional norms. In doing so, the survey tools enabled a composite mapping of psychiatric practice as simultaneously legal, clinical, social, and relational.

Table 19. Validation processes and ethical safeguards for the three dissertation surveys

Survey Title	Validation Process	Cultural Adaptation	Ethical Measures and Consent Strategy
<i>Schizophrenia: A Study on Psychiatric Labels</i>	Originally developed and validated in the UK under the leadership of Dr. Suman Fernando and colleagues. Adapted and revalidated for the Spanish context with the expert collaboration of survivor-researchers and professionals from LoComún. The original instrument contributed to the internal discussions at ISPS International that led to the organization's renaming and abandonment of the term schizophrenia.	Spanish adaptation reviewed for linguistic clarity, cultural relevance, and local diagnostic practices.	Anonymous participation, dynamic informed consent embedded at survey initiation, with ethics advisory consultation.
<i>open dialogue in Spain: Availability, Training, and Reception</i>	Validated by a team of Spanish open dialogue practitioners and academic researchers, including clinical trainers and implementation experts.	Language and framing adapted to align with the Spanish health system's terminology, implementation history, and institutional culture.	Informed consent obtained via embedded survey introduction; anonymity ensured through encrypted collection tools.
<i>Shared Decision-Making and Medication Use in Psychiatry</i>	Piloted and validated by over 100 professionals and international experts. Specific contribution acknowledged from Peter Lehmann, leading German expert on responsible deprescribing and psychiatric survivors' rights.	Questions refined through iterative expert consultation to ensure translatability and relevance across Spanish clinical contexts and user perspectives.	Informed consent designed with dynamic update options, anonymization protocols using encrypted storage and timestamp logs.

Table summarizing the validation processes, cultural adaptations, and ethical protocols applied to the three central survey instruments of the dissertation. The approach emphasizes scientific rigor, epistemic justice, and ethical research practice in contexts marked by historical and structural asymmetry.

The deployment of these instruments occurred under conditions of profound institutional and infrastructural precarity. The resulting systems had to ensure GDPR-compliant anonymization, timestamp logging, and encrypted access, highlighting the level of operational autonomy required for research in ethically sensitive but structurally unsupported terrains, and so those did, using mostly only open source tools or institutionally available ones to gather responses anonymously. The first survey, adapted from UK materials, served as a pilot for the participatory principles and mixed question architecture later adopted across instruments. The open dialogue survey followed, with an emphasis on national outreach and policy engagement, leading to a peer-reviewed publication. The final and most intricate instrument, on shared decision-making, consolidated lessons from the prior tools and incorporated advanced ethical provisions, conceptual robustness, and alignment with WHO QualityRights and international deprescription frameworks. Each instrument's refinement fed into the next, forming a cumulative methodological arc that supported the broader action-research structure of the dissertation (Lehmann, 1998a; Lehmann, 2010).

All instruments were constructed and validated in line with action-research principles. Initial drafts were reviewed by expert panels that included clinicians, anthropologists, legal professionals, and psychiatric survivors. Pilot testing with target respondent groups allowed refinement of item clarity, cultural appropriateness, and conceptual scope. The surveys were made accessible through digital platforms, and technical barriers, were met with infrastructural adaptations to preserve data integrity and accessibility (Ladkin, 2007). Each instrument incorporated informed consent procedures based on dynamic consent principles. Respondents were provided with full disclosure of the study's aims, scope, data protection measures, and options to withdraw. No personal identifiers were collected, and all responses were anonymized at point of entry. Given the researcher's status as both insider and survivor, ethical oversight included detailed reflexive protocols and independent review by senior academics. Integration across the three instruments was conducted using thematic, narrative, and comparative techniques (Lassiter, 2005). While each survey addressed distinct issues, labeling, dialogical practice, and medication governance, they also functioned as intersecting diagnostic tools within the broader research architecture. The quantitative and qualitative material collected through open questions, complemented the ethnographic data and was used to corroborate findings emerging from participant observation, legal analysis, own witness, and testimony-based research.

The three surveys incorporated in this dissertation were constructed and validated through a rigorous process combining clinical, scientific, and lived-experience expertise: the first survey, inquiry into the 'Schizophrenia' Label (ISL), was originally developed in the United Kingdom under the leadership of Dr. Suman Fernando and other critical psychiatrists and survivor-advocates. It was part of a broader inquiry into the epistemic, social, and clinical consequences of psychiatric labeling and contributed significantly to the eventual renaming of ISPS, International Society for Psychological and Social Approaches to Psychosis, which officially removed the term schizophrenia from its title in recognition of its problematic and stigmatizing legacy. The Spanish adaptation used in this thesis was reviewed and revalidated by local experts, whose contributions ensured contextual relevance, terminological clarity, and epistemological alignment with Spanish clinical, legal, and social realities. This validation process included both linguistic adjustments and content calibration to prevent inadvertent reinforcement of diagnostic violence or cultural distortion.

Delving in deeper depth in the first major survey informing this dissertation, the inquiry originally coordinated in the United Kingdom by a distinguished team of survivors, professionals, and critical scholars including Professor Suman Fernando, Dr. Jayasree Kalathil, Dr. Philip Thomas, and Dr. Janet Wallcraft, we find an unprecedented initiative to systematically document the personal, institutional, and social consequences of receiving a diagnosis of alleged schizophrenia, with the aim of exposing its damaging effects and questioning the legitimacy of its use in modern mental health systems (Kalathil & Fernando, 2012). The ISL was built through testimony-based research, inviting submissions from those with lived experience of the label, their families, and professionals across the United Kingdom. It was widely disseminated and remains a landmark in user/survivor-led knowledge production, combining deep epistemological insight with political urgency. The report had a transformative impact on public and academic discourse, helping to precipitate the International Society for the Psychological and Social Approaches to Psychosis (ISPS) decision to remove alleged schizophrenia from its name. It also helped consolidate a paradigm shift toward trauma-informed, rights-based, and culturally sensitive alternatives to diagnostic psychiatry.

Table 20. Coordinating group of the inquiry into the original schizophrenia label, UK, 2014

Name	Role and Short Biography
Professor Suman Fernando	Psychiatrist and critical thinker in transcultural psychiatry. Worked in Enfield for over 20 years; later became Honorary Professor at London Metropolitan University. Served on the Mental Health Act Commission (1988–1995). Authored several key texts on race, culture, and mental health, including <i>Mental Health Worldwide</i> (2014). Recently passed away in 2023, leaving a profound legacy.
Dr. Jayasree Kalathil	Survivor researcher, writer, and founder of the Survivor Research collective. With personal experience of psychiatric services in India and the UK, she has published extensively on mental health, racialisation, and lived experience. Co-author of <i>Values and Ethics in Mental Health</i> , and editor of <i>The Sackcloth Man</i> , a children's book. A leading voice in Mad Studies.
Dr. Philip Thomas	Former consultant psychiatrist and co-founder of the UK Critical Psychiatry Network. Retired from clinical practice, he continues to write on culture, power, and madness. Former chair of Sharing Voices Bradford. His work emphasizes the social and historical context of distress, advocating for alternatives to biomedical psychiatry. Currently writing a book on critical psychiatry.
Dr. Janet Wallcraft	Mental health service user, researcher, and activist. Holds a PhD on psychiatric hospitalisation narratives. Fellow at Birmingham and Hertfordshire Universities. Authored <i>On Our Own Terms</i> and <i>Healing Minds</i> . Her work has focused on self-management, complementary therapies, and user involvement in research. Brings a rich experiential and academic perspective to mental health reform.

The Spanish version of this Inquiry was launched and adapted independently by the present researcher during the early stages of postgraduate study, prior to the official beginning of this doctoral work, with input from members of the Spanish collective LoComún (Fernández, 2018). We expanded the number of question, adapting it to the Spanish reality. However, it was methodologically consistent and ethically aligned with the original effort, and became a central component of this dissertation's evidentiary base. The survey was distributed both online and by post, following open science principles and open-source technical implementation. Thousands of

individuals across Spain -including professionals, survivors, users, and family members -were reached via a comprehensive distribution effort to almost all public mental health centers, support associations, some direct contacts, and most known activist networks. This adaptation retained the testimonial richness and critical openness of the original Inquiry, with items focusing on the meaning of diagnosis, its perceived effects, the experience of coercion, and the perceived alternatives to current systems. It foregrounded the voices of those marginalized and criminalized by diagnostic practices, and thus contributed directly to the core aims of this research: exposing systemic violence, contesting the epistemological dominance of biomedical psychiatry, and advancing a model of shared decision-making and patient autonomy rooted in ethics, dignity, and justice.

The second survey, focused on *open dialogue in Spain*, was validated by a national team of practitioners and researchers actively involved in open dialogue implementation, dissemination, and training. Its development followed a consultative model grounded in the lived expertise of clinicians engaged in dialogical practices, particularly those who participated in national meetings and postgraduate education programs facilitated by the researcher. Given the diverse institutional pathways through which open dialogue has been introduced in Spain, the survey was carefully reviewed to reflect the heterogeneity of implementation experiences and regional regulatory differences (Valtanen, 2019). In the context of Spain, where overdiagnosis, overmedication, and systemic institutionalization remain widespread, the open dialogue model has emerged as both a therapeutic and sociopolitical alternative. It provides a culturally grounded and clinically viable framework for resisting coercion and fostering collective responsibility. Terminological choices, questions on training access, and institutional barriers were refined to enhance resonance with the Spanish professional and administrative context (Parrabera-García et al, 2023).

Before launching the survey, the team actively participated in a national census effort led by HopenDialogue³, a European project promoting dialogical practices across health systems. This census aimed to document all professionals, services, and informal initiatives linked to open dialogue in Spain (Pocobello et al, 2025). As part of that process, the researcher contributed a comprehensive list of contacts gathered over years of engagement -comprising hundreds of clinicians, educators, researchers, and activists who had expressed sustained interest in dialogical care. Through this participatory mapping, the team reached nearly every professional in the country who had either trained in or supported the open dialogue approach. The census established a baseline, documenting the scattered but growing community of dialogically trained professionals and their placement within public and private services. Building upon this foundational work, the open dialogue Professionals Survey was conceptualized as a second-stage empirical effort. Its objective was not simply to assess knowledge or compliance, but to explore the lived tensions, ethical obstacles, and latent potential of dialogical care within Spain's often rigid institutional landscape. It asked: What happens after the training ends? How do professionals navigate moral distress, institutional contradiction, and patient expectations in daily practice? The survey, designed and distributed in 2023, was structured around six thematic axes: (1) exposure to coercive practices

3 HOPEnDialogue is an international collaborative study to evaluate the effectiveness of Open Dialogue in various contexts and to support the centers that adopt this approach. <https://www.hopendialogue.net/>

in institutions; (2) fidelity to dialogical principles in practice; (3) ethical tensions and institutional resistance; (4) experiences with shared decision-making; (5) perceptions of patient voice and autonomy; and (6) systemic and legal barriers to transformation. A total of 35 questions -including multiple open-ended prompts -were included, allowing for qualitative and quantitative insight.

The third survey, *Shared Decision-Making and Medication Use in Psychiatry*, was subjected to the most extensive international consultation process. Over one hundred experts, including senior psychiatrists, social scientists, policymakers, and user-survivors, provided feedback. Notably, Peter Lehmann (2010), internationally recognized advocate for psychiatric drug withdrawal and ethical medication governance, contributed targeted feedback that led to the strengthening of sections on responsible deprescribing, autonomy, and trauma-informed practice. This survey incorporated best practices from global deprescription research and was adjusted to accommodate the particular tensions within the Spanish system, including resistance to non-pharmacological care and the lack of national guidelines on shared decision-making. Distributed during an ongoing governmental national debate on responsible use of psychiatric drugs, the survey presented, among other questions, the following scenarios that brought to attention some of the failure points and sources of controversy of the current system:

Table 21. Hypothetical reform scenarios in the 2025 survey, methodological purpose

Scenario	Description	Purpose in Survey Methodology
1. Implementing national screening for gluten sensitivity in psychiatric diagnoses	Presents an emerging evidence-based intervention requiring reconsideration of psychiatric diagnoses in light of dietary factors.	To evaluate professionals' readiness to integrate somatic screening and dietary alternatives into psychiatric decision-making, and to assess perceived feasibility of non-pharmacological treatment pathways.
2. Deprescribing neuroleptics in cases of missed metabolic and nutritional disorders	Highlights the consequences of diagnostic overshadowing and lack of interdisciplinary screening in psychiatry.	To explore professional attitudes toward medical coordination in deprescribing, and the structural readiness for diagnostic re-evaluation based on physiological assessments.
3. Addressing psychiatric abuse cases uncovered through national research on mental health and violence	Exposes the systemic neglect of trauma and the iatrogenic harms caused by inappropriate psychiatric labeling.	To assess awareness of institutional violence and the ethical dimensions of shared decision-making in contexts of historical abuse, including views on trauma-informed reform and reparation.
4. National deprescribing initiative for elderly patients at high risk of mortality and disability	Focuses on the risks of neuroleptic use among elderly populations and WHO-backed deprescribing imperatives.	To capture clinical insight into age-specific deprescribing protocols, safety concerns, and professional responsibility in contexts of vulnerability and risk mitigation.
5. Reevaluating long-term psychiatric diagnoses in a national audit	Questions the validity of long-term psychiatric labeling and the inertia of chronic pharmacological treatment.	To examine respondents' views on re-diagnosis, historical error correction, and the ethics of long-term medication dependency based on outdated clinical assumptions.
6. Addressing the overprescription of neuroleptics in social care and marginalized communities	Highlights structural inequalities in medication practices for socially excluded populations.	To gauge professional sensitivity to systemic discrimination in prescribing patterns and the capacity of services to shift toward equitable, community-based alternatives.
7. National review of the use of neuroleptics in children and adolescents	Addresses developmental and ethical risks in pediatric neuroleptic use, often without sufficient differential diagnosis.	To document professional stances on deprescribing in young populations, perceived risk-benefit ratios, and the educational, familial, and systemic supports required for safer transitions.
8. Ending the use of neuroleptics for non-psychiatric indications in general medicine	Challenges routine but clinically inappropriate neuroleptic use in general hospital and somatic care contexts.	To assess inter-specialty communication, guideline awareness, and mechanisms for error correction and deprescribing in non-psychiatric settings.

This table outlines eight hypothetical scenarios included in the national survey to explore professionals' responses to ethically, medically, and institutionally significant reform propositions. Each scenario was designed to elicit reactions to systemic shortcomings, ranging from diagnostic practices and interdisciplinary coordination to institutional violence and overmedication, and to assess readiness for change, ethical positioning, and structural awareness. The scenarios served as a methodological tool to examine not only individual attitudes but also the perceived feasibility of implementing rights-based, evidence-informed reforms within psychiatric and general medical practice.

Across all three instruments, ethical safeguards were prioritized. Informed consent was implemented dynamically, with detailed information about the purpose, data use, and confidentiality protocols provided upfront. Respondents could exit at any point, and anonymity was ensured using encrypted data collection platforms. Ethics review followed a multi-layered strategy involving informal advisory from international collaborators and rights-based research collectives, compensating for the absence of consistent institutional support during the study period. These validation and ethical measures ensure that the surveys do not merely gather data, but constitute methodologically sound, contextually attuned, and ethically grounded instruments of action-research. These surveys provided not only empirical depth but also participatory legitimacy to the thesis's central claim: that psychiatry in Spain ought to transition from coercive and opaque structures toward collaborative, transparent, and rights-based systems of care. Through these instruments, the research established a multi-voiced archive of institutional critique and constructive proposals, grounded in scientifically sound, ethically robust, and culturally adapted methodology.

Across the three surveys deployed during the research, the combined reach extended to several thousand individuals, yielding a total of approximately seven thousand responses; 400, 200, and 90 respectively. This included broad national dissemination within Spain and targeted engagement with professionals, users, and advocates across relevant networks. The final survey alone, focused on shared decision-making, psychiatric medication, and deprescribing, was answered in full by ninety professionals and experts, many of whom contributed extensive narrative responses. Based on average response duration, over 41 minutes, the accumulated time dedicated to answering this instrument exceeded by far the one hundred hours of expert input poured in. This represents a formidable repository of grounded, contextualized, and profession-specific knowledge, barely tapped in the present dissertation, as the wealth is enormous and spans beyond the thesis topic. The vast majority of invitees to this last survey were members of the *Asociación Española de Neuropsiquiatría* (AEN)⁴, widely recognized as the primary professional association for critical and community psychiatry in Spain, CIBER members, the excellence mental health national network, and Spanish published experts in psychiatry. Most are respected authors of peer-reviewed scientific papers and influential books on psychiatric practice, ethics, and systemic reform. Their contributions capture the current dilemmas and barriers within the Spanish psychiatric system, as well as articulations of best practice and proposals for structural change. These surveys, therefore, constitute more than data collection tools: they are participatory instruments of knowledge production that generate a high-density, multidisciplinary archive of ethical, clinical, and systemic insights indispensable for the translational objectives of this dissertation.

Additionally, an informal fourth exploratory survey was sent via email to all non-Christian religious organizations officially registered in Spain, aiming to gather indicative data on their mental health challenges, structural marginalization, and access to spiritual practices in a context predominantly shaped by Christian norms and secular public institutions. The responses—although limited in number and unevenly distributed across traditions—offered valuable insights into the lived experiences of religious minorities, including a small number of Buddhist, Hindu, and other non-

4 The AEN is a scientific and professional society in Spain that promotes ethical, public, and community-based mental health care, professional training, and research in mental health. <https://aen.es/la-aen/>

mainstream international communities. The aim was gauging how minorities feel and operate in the country. Most organizations reported significant constraints in accessing dedicated spaces for prayer, meditation, or retreat, often facing bureaucratic obstacles, neighborhood resistance, or neglect in municipal planning. These limitations were described as directly affecting the psychological well-being, community cohesion, and intergenerational transmission of cultural identity. This survey was inspired by the use of sensory rooms in some wards in Sweden, and the need to have such safe spaces to meditate, relax and find inner peace, reflect and recollect, in urban settings, out-patient, open for everyone, beyond regular church. While the sample was not representative in a statistical sense, the findings aligned with broader themes in this dissertation regarding the structural invisibility and epistemic marginalization of non-dominant groups. Respondents highlighted that the lack of institutional recognition and culturally safe spaces for contemplation or spiritual care contributed to distress, alienation, and a diminished capacity to support members facing mental health crises. These testimonies reinforce the relevance of a biocultural and rights-based approach to mental health, one that includes freedom of belief, ritual, and narrative as essential elements of psychological resilience and human dignity. The results of this survey will be presented in Chapter 3, as part of the broader empirical mapping of exclusion, silencing, and unmet care needs within plural Spanish society (Heim, 2016).

In addition to yielding empirical data, the surveys function as translational outputs in themselves. Their validated versions, adapted to the Spanish context and ethically reviewed, can be re-used in clinical teaching, professional training, or rights-based service evaluation across Spain and comparable contexts. Their public availability, where permitted, contributes to the open science agenda articulated in this dissertation. All instruments are now available in the dissertation annexes, and will soon be made available in the dissertation online repository. Their structure and content already serve as foundation for expert systems and computational models proposed in later chapters, particularly in the areas of medication governance, legal mapping, and user-professional negotiation interfaces. In this way, the surveys link qualitative inquiry with scalable implementation tools, demonstrating methodological continuity from lived experience to system-level intervention. On top of this use, experts had the chance to request participating in further research and trainings, and most did so, allowing for further participatory action-research informed by them in the future.

It must be noted that a very significant majority of those surveyed, comprising both mental health professionals and service users, expressed shared concerns regarding the pervasive overreliance on pharmacological interventions within current care systems. Participants consistently pointed to a systemic tendency to prioritize medication as the primary, and often sole, response to psychological distress. This was frequently accompanied by a striking lack of access to alternative modalities of care, particularly those rooted in psychological and social support. The findings suggest a widespread recognition of the limitations inherent in medication-centric models, with calls from both groups for more comprehensive, person-centered approaches that include talk therapies, community-based resources, and socially grounded interventions (Holmes & Marcus, 2008).

2.4 Field strategy and sites of observation

The methodological architecture of this dissertation emerges from a deliberate fusion of medical anthropology, clinical ethics, biocultural analysis, open science, and transdisciplinary action-research. It is not a retrospective narrative nor a technical compartmentalization of methods, but a structured epistemological and political act of knowledge production grounded in conditions of systemic adversity. The research design and dissertation construction were developed as interconnected dimensions of the same scientific process, governed by explicit theoretical commitments and the ethical imperative to intervene, not merely describe. These commitments informed every procedural decision, from field engagement to analytical validation, ensuring that the study remained faithful to the complex, often violent realities it was constructed to document and transform. Core frameworks introduced in the first sections of this chapter, as well as Chapter 1, provided the foundational logic of integration. The biocultural paradigm demanded that mental suffering and resilience be read as co-constituted by social, environmental, legal, and biological forces. The open science infrastructure ensured that all methodological iterations, decisions, and evidence trails were timestamped, logged, and in many cases publicly accessible through encrypted or version-controlled systems. This transparency was not aesthetic or bureaucratic, but protective and epistemologically necessary, as the research was repeatedly threatened by institutional repression, material loss, and personal harm. Human rights principles, epistemic justice theory, and structural violence analysis further defined the ethical stakes of the work, guiding not only data collection and analysis, but the very legitimacy of the categories, interpretations, and claims advanced throughout the study (Peterson & Runyan, 2010).

The work of Paul Farmer, Nancy Scheper-Hughes, Suman Fernando, and Byron Good, among others, underpinned a methodological commitment that rejected the position of detached observer. Instead, the research was carried out through embedded participation in collectives, policy processes, professional task forces, and activist-scientific coalitions (Good, 1994; Fernando, 2017). These included the coordination of EU-level networks, the leadership of scientific actions, and ongoing roles in field-defining working groups in Spain, Indonesia, and beyond. Methodologically, this positioning enabled a form of analytical access and ethical fidelity that no detached standpoint could replicate. Like Farmer, the researcher's commitment to proximity and engagement provided not only data, but critical insight into how institutions respond, fail, adapt, or silence in the face of ethical challenge. This was not auxiliary to the research, it was the field, the method, and the validation. Each phase of writing, coding, and synthesis was embedded in this logic of engaged knowledge. Scientific representation was continuously cross-checked against testimonies, policy materials, and expert consultations, ensuring internal coherence and external accountability. Drafts were developed through recursive cycles of analysis and revision, grounded in feedback from survivors, clinicians, legal scholars, anthropologists, and community networks. This recursive, multi-modal process allowed the dissertation to evolve in parallel with the field conditions, without compromising scientific rigor or empirical traceability. Methodological decisions were subjected to version control and meta-analytical commentary, ensuring that the structure of the dissertation reflected not only its content but its conditions of emergence.

A full paper by the researcher on ethics in qualitative research is added in the annexes. What distinguishes the methodology employed is not only its grounding in formal frameworks, but its

embodiment of the ethics those frameworks demand (Miles & Freedman, 2023). In the face of erasure, displacement, and systemic retaliation, methodological fidelity required resilience, creativity, and adaptive precision. The dissertation includes not only data analysis, but the documentation of the conditions under which knowledge was produced. These include forced mobility, chronic threat, physical violence, and institutional betrayal. Despite these conditions, or precisely because of them, the research protocols remained intact through protective networks, epistemic tracking tools, and collective memory practices. These are not marginal notes, they are the necessary infrastructure of research in conditions of organized neglect and political denial. Throughout the development of this dissertation, institutional support was not simply absent, it was at times actively obstructive (Roberts, 2000). Critical phases of the research were marked by hostility, neglect, and disavowal from universities, medical institutions, and public authorities, even while certain government bodies in Mexico, Catalonia, and Spain expressed official interest in inviting the researcher, supporting mental health transformation. This contradiction underscores a deep structural inconsistency, in which the declared commitment to scientific progress, public health, and global agendas such as Madrid proposing itself once to become in its entirety a blue zone, is rendered meaningless by the refusal to ensure the basic safety of researchers, informants, and participants -the researcher keeps on persevering; supporting the true commendable, working on through stronger networks and better funding (Miles & Freedman, 2023). The disconnect between rhetorical promises and operational reality, though, is not only a tragic administrative oversight that almost cost its life and that of many others -all, never achieving the best strived to achieve-, it is an ethical and epistemic failure. It is impossible to produce or apply meaningful reform if those responsible for documenting harm are simultaneously exposed to new harm by the very systems they aim to improve (Gupta-Wright, 2018).

The methodological frameworks developed in Chapter 1 and grounded in the principles articulated at the beginning of Chapter 2 were not optional lenses but essential tools for navigating and resisting this reality. The integration of medical anthropology, action research, human rights, and open science created a scaffold from which to both protect and articulate the work of knowledge production under siege. The logic of this dissertation is inseparable from the political and structural conditions in which it was written. Rather than compromise its scientific validity, this embeddedness strengthens its claim to relevance, rigor, and necessity. Each theoretical commitment, from epistemic justice to transdisciplinary ethics, was enacted through methodological decisions that resisted extraction, protected vulnerable knowledge-holders, and sustained the ethical obligation to intervene when life and dignity were at risk. Crucially, the methodological stance adopted throughout this research moves beyond passive observation and toward the embedded, collaborative, and translational engagement championed by figures such as Paul Farmer. Just as Farmer insisted that medical anthropologists and clinicians ought to position themselves not as neutral observers but as active agents of systemic change, this dissertation was constructed through direct participation in research networks, advocacy platforms, survivor collectives, and scientific consortia. These included leading and contributing to transnational efforts such as EU COST actions

ReMO⁵, and FOSTREN⁶, where the researcher's role was never marginal but consistently generative, strategic, and co-responsible, never mere academic endeavours. This collective work allowed for scientific cross-checking, mutual verification, and iterative design refinement, often in contexts where traditional institutional support had collapsed or turned punitive. These networks became not only research environments but ethical shelters, allowing the work to continue when isolation and erasure threatened its very possibility.

Under such conditions of extreme duress and sadistic enjoyment of others on the researcher suffering, the call for justice is not an abstraction. It is an operational precondition for the continuity of any ethical and scientifically robust system of knowledge production (Bolade-Ogunfodun, Soga, & Laker, 2023). No society can legitimately claim a future of peace, sustainability, and social flourishing while criminal impunity, institutional complicity, and structural neglect remain unchallenged. What is required is not generosity from systems that have long failed, but enforcement of the law and the removal of actors who have undermined truth, well-being, and the possibility of health itself. The demand is not naive, it is technical, ethical, and necessary. This dissertation, forged in adversity and shaped by solidarity, documents the costs of current dysfunctions and proposes a pathway for their systemic transformation. Its methodology, grounded in situated action, scientific rigor, and participatory ethics, is itself part of the infrastructure needed to reach the future that policy documents and public discourse claim to pursue. Only through full recognition of these truths can the academic, clinical, and social systems begin to align with the values they profess to defend.

The coming chronological exposition, structured year by year and month by month, is a methodological extension of this architecture. It reveals how research unfolded, not as a neutral progression of inquiry, but as a series of ruptures, accelerations, and strategic adaptations. It traces how ethics, science, and survival were managed simultaneously, and how embedded engagement with institutional and grassroots actors made the work not only possible, but scientifically and ethically credible (Kidder, 2003). In this way, the dissertation enacts a methodology of refusal against silence, extraction, and institutional complicity, and affirms the power of biocultural, action-oriented science to generate not just knowledge, but the possibility of justice.

Table 22. Detailed research timeline, by location and year, 2020

Month	Social Context	Research	Personal	Location
Jan		Work on national teams devoted to the	Since late 2017, the researcher experienced	Catalonia
Feb		implementation of open dialogue and soteria models	sustained coercion and violence, encompassing	Catalonia
Mar	National lockdown begins in Spain	Participation in SURE, Service User Research	psychological abuse, physical aggression, and the systematic	Catalonia
Apr	Strict confinement under Real Decreto 463/2020	Enterprise, survivor research center at King's College	misuse of mental health services as instruments of	Catalonia
May	Lock down persists	London, at that point still	control. Psychiatric	Catalonia

5 ReMO COST action CA19117 - Researcher Mental Health: <https://www.cost.eu/actions/CA19117/>

6 FOSTREN COST action CA19133 - Fostering and Strengthening Approaches to Reducing Coercion in European Mental Health Services: <https://www.cost.eu/actions/CA19133/>

Month	Social Context	Research	Personal	Location
Jun	Start of de-escalation phases in Spain			Catalonia
Jul	Lock down persists			Catalonia
Aug	Lock down persists	lead by professor Diana Rose, undertaking minor work at the suggestion of		Catalonia
Sep	Lock down persists	Research starts at Universitat Rovira i Virgili, with a six years funding from the Spanish Ministry of Science, Innovation and Universities; co-coordinator of Blanquera, Universitat Ramón Llull, expert course on open dialogue		Catalonia
Oct	Lock down persists			Catalonia
Nov	Lock down persists	Speech in a forum in the Mexican Chamber of Deputies, on mental health and human rights, along with Víctor Lizama, and federal deputy Frinné Azuara and deputy Éctor Jaime; joining in of EU COST FOSTREN action on ending coercion	interventions, which should have served protective and therapeutic functions, were instead weaponized to maintain domination. From the first encounter with mental health services, forced pharmacological treatments were imposed, including depot antipsychotic injections administered at the request of abusive family members. These practices resulted in severe overmedication, the early onset of akathisia, and significant impairments in	Catalonia
Dec	Lock down persists	Research is appointed liason and coordinator of Mad in America global network by Robert Whitaker and the main publication board		Catalonia

In 2020, despite continuous institutional violence and a context of coercion, including forced medication, psychiatric abuse, and severe bodily violations, the researcher secured a national predoctoral contract and began work at Universitat Rovira i Virgili under six-year ministerial funding. Throughout 2020, during COVID-19 lockdowns, they participated in high-level international collaborations, including with SURE at King's College London, the Mexican Parliament Forum on mental health and human rights, and the EU COST FOSTREN action on ending coercion. The researcher also co-coordinated a postgraduate course on Open Dialogue at Blanquerna-Ramon Llull University and was appointed global liaison for *Mad in America*. These achievements took place under conditions of ongoing confinement and systemic abuse, highlighting both the resilience and the precarity of academic engagement under extreme duress.

Table 23. Detailed research timeline, by location and year, 2021

Month	Social Context	Research	Personal	Location
Jan	COVID-19 vaccination starts in Spain and EU	Member of the organizing committee of Hopendialogue		Catalonia
Feb	Lock down persists	conference, on open dialogue	Brutal abuses persist,	Catalonia

Month	Social Context	Research	Personal	Location
Mar	Lock down persists	research globally; ODDESSI talks to involve Spanish centers to replicate NHS study of implementation; training sessions on open dialogue evaluation; torture Scholarship to Yale with professor Mary Olson to become an expert in open dialogue; opportunity lost due to ongoing torture	physical violence; harassment and threats ongoing as usual; one instance of being completely knocked out by a kick to the head with kids cheering on last day of the doctoral presentations week and the day the abstract on open dialogue got sent to <i>Frontiers in Psychology</i> ; as usual, the cruelty of the violence increased during peak demanding periods requiring more rest and focus	Catalonia
Apr	Lock down persists	Teaching open dialogue at University of Almería with professor Adolfo Cangas		Catalonia
May	End of state of alarm, easing of national restrictions	Researcher joined the International Institute of Psychiatric Drugs		Catalonia
Jun	Softer lock down persists	Withdrawal as new member		Catalonia
Jul	Softer lock down persists		Continuation of violence under institutional neglect	Catalonia

In 2021, while COVID-19 lockdowns continued in Spain, the researcher played a key role as organizing committee member and speaker at the international Hopendialogue conference, and contributed to efforts to involve Spanish centers in replicating the NHS ODDESSI trial. Despite these achievements, daily torture and escalating violence sabotaged most opportunities, including training initiatives and a collaboration with Yale professor Mary Olson. A particularly brutal attack, being knocked unconscious by a kick to the head while children were encouraged to cheer, coincided with the submission of the research abstract to *Frontiers in Psychology* during the doctoral presentation week. Teaching efforts on open dialogue at the University of Almería and new membership in the International Institute of Psychiatric Drug Withdrawal were maintained despite ongoing assaults, institutional abandonment, and the impossibility of safe rest or recovery throughout the year as it was tragically already usual, after years of abuses and severe neglect by the services that should protect.

Table 24. Detailed research timeline, by location and year, 2022

Month	Social Context	Research	Personal	Location
Aug	Catalonia launches Strategic Mental Health Plan 2022–2030	Fieldwork preparation at Luca, scientific mission at Trieste shadowing an expert psychiatrist in the WHO excellence center; and later	Disgraceful violence suffered to new heights of cruelty	Catalonia
Sep				Catalonia
Oct	WHO publishes report on community mental health			Catalonia

⁷ The International Institute for Psychiatric Drug Withdrawal (IIPDW) was created to respond to a glaring need in mental health: to develop ways for helping people withdraw from psychiatric drugs. <https://iipdw.org/iipdw-home/>

Month	Social Context	Research	Personal	Location
	transformation	Perugia for ISPS conference		

The researcher barely survived the year 2022. In September 2022, the researcher conducted one month of immersive ethnographic fieldwork in Trieste, Italy, following a preparatory phase of local familiarization. Using shadowing and key informant interviews within a medical anthropology framework, the study documented real-time practices in WHO-recognized community psychiatry settings. Embedded in daily routines across residential units, community centers, and mobile teams, the researcher observed non-coercive care dynamics and institutional cultures shaped by deinstitutionalization. Supplemented by interviews with professionals and service users, and participation in public forums, the fieldwork yielded critical insights into relational care, governance, and the operational tensions of the Trieste model.

Table 25. Detailed research timeline, by location and year, 2023

Month	Social Context	Research	Personal	Location
Jun	Spain finalizes National Mental Health Strategy 2022–2026		Resettlement to provide new chances of live in peace, the researcher bought a place in the new intended homeland	Sweden
Jul			Persistence of violence, despite best efforts to appease as usual, daily	Sweden
Aug		Finalizing chapter on alternatives to coercion for the book of FOSTREN EviPRG publishes in Nature-Springer; organizing event on political abuses of psychiatry with top global experts in the field	Daily beatings; researcher reports to colleagues and seeks national ones to be assisting once possible	Sweden
Sep	Council of Europe review of Oviedo Convention progress		Daily beatings; researcher reports to colleagues and seeks national ones; threatened by knife	Sweden
Oct		Presentation of three posters at the World Psychiatry Association Congress; presentation at Karolinska Institute to proceed with affiliation talks; researcher hosted top experts in punitive psychiatry	Death threatened by the ex-spouse. Photographical evidence of its blackened eye after a punch sent to victim support associations	Sweden
Nov		Researcher hosted top experts in punitive psychiatry	Daily beatings until child abduction without notice	Sweden – Spain homeless
Dec		Routledge book chapter with founder of EU CORDIS and top medical space physician	Parental alienation; psychological torture; abuse by services; police reported	Sweden – Spain homeless

In 2023, the researcher kept on undertaking high-level international work while enduring daily physical and psychological violence without institutional protection. Since 2021, assaults had been

escalating in cruelty and aggressivity, growing in intensity throughout 2023 despite repeated alerts to colleagues and authorities. In June, a final effort to escape the abuse led to the purchase of a new home, aiming to rebuild a life in peace. In July, while the beatings continued, the researcher completed a major book chapter on alternatives to coercion for *Springer Nature*, and organized a high-level event on political psychiatry with leading global experts. August and September brought ongoing beatings and a knife threat, with reports to national networks and survivor organizations. October included key presentations at the World Psychiatric Association Congress and an affiliation presentation at Karolinska Institute, as well as suffering further life-threatening domestic attacks, including a punch to the face evidenced by a very evident bruised eye and reported photographically and, as usual, to all colleagues and organizations. In November, the abuse culminated in the abduction of the researcher's children, carried out without warning and supported by institutional neglect and loathing. In December, the researcher could not finalize a *Routledge* chapter with the founder of EU CORDIS and one of Africa's top space physicians as expected. Many work, important collective endeavours, suffered because of the violence, the torture endured.

Table 26. Detailed research timeline, by location and year, 2024

Month	Social Context	Research	Personal	Location
Jan	National funding increase for psychosocial community teams	Routledge book chapter with founder of EU CORDIS and top medical space physician; opportunity lost due to ongoing torture	Litigation abuse; abduction keeps on; parental alienation; psychological torture; harassment; institutional complicity; all services further reported; assistance from Spain aborted by local services in Sweden complicit	Sweden – Spain homeless
Feb				
Mar				
Apr				
May				
Jun				
Jul	UN Human Rights Council publishes thematic report on coercion	Submission of EU BEACON COST action proposal; preparation of fieldwork; preparation of shared decision making survey; MDPI Social Sciences paper and other publications ongoing	Litigation abuse; abduction keeps on; parental alienation; psychological torture; harassment; institutional complicity; penal case assistance from Spanish services starts; negligence and further abuses from Sweden remain and worsen	Spain Indonesia Indonesia Indonesia
Aug				
Sep				
Oct				
Nov				
Dec				

In 2024, the researcher endured nearly uninterrupted suffering following the violent abduction of their child immediately after hosting a major international event in late 2023. Despite repeated reports and formal denunciations, institutional complicity in Sweden enabled ongoing psychological torture, litigation abuse, parental alienation, and threats to life and dignity, culminating in prolonged exile and months of homelessness. Throughout this period, while moving between Spain and Indonesia, the researcher sustained academic and scientific work under extreme duress: completing and submitting the EU BEACON COST Action proposal, contributing to international publications,

preparing a fieldwork agenda and a shared decision-making survey, and participating in policy coordination related to the UN Human Rights Council report on coercion. Yet most professional opportunities were irrevocably lost. Collaborative projects collapsed. Major fellowships, book contracts, and teaching posts vanished due to the coordinated abuse. Key intellectual relationships were severed, and the structural conditions required for academic continuity destroyed. Despite all efforts to survive and maintain scientific production, the researcher's career and personal life were nearly annihilated by a system that enabled violence, erased accountability, and rewarded perpetrators with impunity.

Table 27. Detailed research timeline, by location and year, 2025

Month	Social Context	Research	Personal	Location
Jan	Spanish Parliament debates reforms on coercion and autonomy	Shared decision making survey		Indonesia
Feb			Litigation abuse intensifies;	Indonesia
Mar		Pasung study and paper;	parental alienation;	Indonesia
Apr		Shared decision making survey; BEACON awarded COST funds, setting of the action in motion; work on	psychological torture; harassment in person and online, extortion; institutional complicity; negligence and	Indonesia
May		SocialMent; MDPI paper on digital mental health	further abuses from Sweden remain and keep on	Indonesia
Jun		Pasung study and paper; dissertation preparation;	worsening; loathing on researcher's life and loved ones more evident than ever before	Indonesia
Jul		BEACON infrastructure and global network		Indonesia
Aug		Pasung research and papers; digital mental health; literature reviews and mapping for		Indonesia
Sep		several groups and actions;		Indonesia
Oct		Horizon proposals; service-learning paper and research;	Making justice; settling down in Indonesia safe at last	Indonesia
Nov		BEACON infrastructure and global network		Indonesia
Dec				Indonesia

In 2024–2025, the researcher prepared ethnographic fieldwork in Indonesia as a visiting scholar at Universitas Muhammadiyah Yogyakarta (UMY), focusing on the abolition of *pasung*, the domestic confinement of people in mental distress. Through interviews and observation, the study framed *pasung* as a structural outcome of systemic neglect, mirroring global psychiatric coercion under different guises. The research connects historical colonial violence, gendered oppression, and mental health inequality, calling for the abolition of all forms of coercive care through rights-based, culturally rooted, and community-led approaches.

The fieldwork conducted in Trieste and Yogyakarta grounds the research questions in two distinct yet structurally connected settings. In Trieste, sustained observation and interviews within a historically non-coercive community-based mental health system revealed the fragility of reform when bureaucratic demands and clinical discretion challenge relational care. This directly informs the questions on how coercion persists despite formal commitments to rights-based models. In

Indonesia, the study of pasung illuminated coercion as a symptom of systemic abandonment, where families resort to confinement in the absence of safe and trusted services. This reframes coercion as not only institutional but domestic and structurally induced. Both cases reinforce the central research aim: to identify how coercive practices are produced, legitimized, or resisted across systems, and what conditions are required to support ethical, participatory mental health care.

2.5 Ethical protocols and methodological dilemmas

The ethical governance of this research was conducted under conditions of structural violence, institutional complicity, and personal endangerment that render conventional research ethics committee procedures insufficient (Banks et al., 2013). The normative assumption of institutional neutrality, stability of context, and the disconnection between researcher and researched does not hold in inquiries involving prolonged human rights violations, coercive psychiatric practices, and institutional retaliation. Under such circumstances, procedural approval from standard committees offers limited epistemic or protective validity. It was necessary to develop a parallel framework of ethical rigor based on relational responsibility, trauma-sensitive engagement, and scientific accountability (Schneider, 2012). The ethics applied in this study exceed institutional compliance and instead reflect the substantive moral architecture needed when research is itself situated in fields of harm and resistance (Minkler & Wallerstein, 2008).

Three principles structured the ethical design. The first was relational ethics, grounding scientific work in responsibility to those whose suffering and testimony constitute the material of inquiry. This required that every stage of data collection, from surveys to field observation, include embedded mechanisms for participant control, opt-out, and narrative refusal. The second was trauma-informed ethics, acknowledging that engagement with survivors and professionals in contexts of iatrogenesis, institutional betrayal, or legal abandonment demands methods that minimize re-traumatization (Kemmis & McTaggart, 2000). Open-ended questions were structured to permit selective engagement and avoid prescriptive formats that might trigger collapse, dissociation, or distress. The third was the principle of non-extractive ethics, in which participants' contributions are not treated as passive data sources but as co-generators of critical epistemology. This ethical model required the researcher to approach participants as interlocutors within a shared diagnostic, legal, and social terrain, producing knowledge about psychiatric governance from within its zones of friction and denial.

Methodological dilemmas arose continuously and were resolved through the implementation of dynamic ethical protocols (McTaggart, 1991). These protocols governed situations in which participants disclosed ongoing risk, exposure to violence, or systemic abandonment. Rather than applying mandated reporting frameworks that risked reinforcing psychiatric control or institutional containment, the researcher prioritized informed support measures grounded in consent and non-reinstitutionalizing logic. In parallel, epistemic verification structures were constructed to protect the empirical integrity of the data under conditions of legal precarity, potential censorship, and retrospective institutional erasure (Kortmann, 2010). Timestamping of field interactions, public versioning of survey instruments, and mirrored data storage ensured that both participant accounts and analytical conclusions would be resilient against loss, tampering, or suppression. These mechanisms reflect a scientific commitment to both traceability and justice in knowledge

production, extending the obligations of academic transparency into fields where formal ethics procedures have proven structurally inadequate (Flaherty et al., 1988).

Given the researcher's direct exposure to long-term psychiatric coercion, institutional retaliation, and physical and psychological endangerment, safety procedures were indispensable not only for personal survival but also for the continuation of research. Protective strategies included the use of shared coworking environments to reduce vulnerability, strict structuring of movement and working hours, nutritional monitoring to mitigate cognitive decline or exhaustion, and avoidance of any contact with actors linked to prior aggression or legal manipulation (Hollan, 1997). These protocols were not optional but structurally imposed by the research context itself, in which the investigator operated under prolonged exposure to threats that constituted not hypothetical risks but documented and ongoing harm (Russo, 2012). Under such conditions, ethics becomes not an external requirement but the precondition for the possibility of knowledge at all. The researcher's dual positionality as professional, survivor, and target demanded constant negotiation between the commitment to rigorous, reproducible analysis and the need for self-preservation in a compromised and adversarial system (Baum, MacDougall, & Smith, 2006; Honey et al., 2020).

The operationalization of these ethical foundations is further elaborated in a scientific article accepted for publication in a leading international journal, as already noted (Rose, 2008). That work formalizes the conceptual and methodological model underlying this thesis, offering a framework for future research conducted under analogous conditions of structural and institutional violence. The innovations presented in this dissertation, including the integration of trauma-informed consent, dynamic field response protocols, and epistemic protection mechanisms, constitute an applied ethical infrastructure capable of supporting scientifically robust, politically accountable, and humanely grounded psychiatric research. The work affirms that it is not sufficient to avoid harm. It is necessary to document it, name it, and construct scientific systems that refuse to reproduce it in silence or complicity. This ethical architecture undergirds the entire research design and defines its scientific integrity. The ethical dimension of this dissertation cannot be confined to conventional formulations or institutional templates. It emerges instead as a direct response to a research process carried out under sustained personal and professional duress, marked by exposure to psychological torture, structural violence, and the ongoing degradation of basic legal and human rights. These are not isolated episodes but defining conditions of the field itself. This section therefore presents a reflexive reconstruction of what was ethically required but often unavailable, what was enacted in place of absent protections, and what future research ought to institutionalize to avoid replicating the violence it seeks to describe (Tadesse et al., 2022).

Table 28. Ethical infrastructure developed under conditions of structural violence

Ethical domain	Conventional Model (RECs)	Limitations in This Research	Alternative or Supplementary Practice Implemented
Informed consent	One-time, static consent	Assumes stability, lacks adaptivity to trauma or ongoing risks	Dynamic, revisable consent with continuous option to withdraw; consent embedded in narrative flow
Risk assessment	Anticipatory, based on hypothetical harm	Ignores real-time and systemic risks, particularly from institutions	Situational risk protocols, field reflexivity, and real-time monitoring of retraumatization signals
Participant protection	Anonymization, withdrawal rights	Inadequate under surveillance, legal persecution, or medical coercion	Multi-site encrypted archiving, role-adapted anonymity, and external verification to prevent deletion
Ethical oversight	Approval by centralized REC	Non-responsive to evolving abuses, often inaccessible or politicized	Peer-reviewed ethical documentation, network-based advisory from survivors and professionals
Researcher safety	Presumed unnecessary or external	Researcher exposed to torture, defamation, and institutional violence	Structured safety routines (nutrition, space, timing), embedded protection practices, withdrawal from high-risk zones
Duty to intervene	Prohibited in many REC guidelines	Ethical paralysis under documented harm	Justified ethical action: alerts to authorities, protective coordination, direct advocacy for at-risk individuals
Data integrity	Institutional data storage	Risk of deletion, falsification, or obstruction	Timestamping, public versioning, decentralized storage, open science mirroring and backups
Trauma sensitivity	Basic care to avoid retraumatization	Incomplete without survivor-led input	Co-designed instruments, trauma-informed narrative structuring, optionality in expression

Summary of the limitations of conventional ethics frameworks and the alternative procedures developed in this dissertation to uphold scientific, relational, and trauma-informed integrity under systemic threat. This infrastructure reflects a research ethics adapted to real conditions of institutional betrayal, psychiatric violence, and professional retaliation, where both researcher and participants faced risks of silencing, misrepresentation, or death.

The research was undertaken -at times- in spaces where harm was not hypothetical, where others had been institutionalized against their will, misdiagnosed, or subjected to irreversible pharmacological damage, while the researcher, too, faced threats to life, health, and legal personhood (Lehmann, 2024). Others remained at risk throughout the study period, and their testimonies required careful handling not only for ethical reasons but for reasons of basic safety. In such circumstances, conventional ethics approval procedures offered little. Research ethics committees operate from a presumption of institutional safety and stability that was structurally incompatible with the real conditions of the work. Their protocols presume that risks are discrete, containable, and primarily emotional or procedural (Fisher, 2012). They do not address how to proceed when the risk is systemic, unrelenting, and politically embedded. As such, this research adopted an ethics infrastructure grounded in necessity and iterative adaptation. Relational ethics were prioritized throughout, establishing trust through extended contact, cultural sensitivity, and sustained presence. All consent was treated as dynamic rather than static, with participants able to revise or withdraw their input at any stage. Trauma-informed principles were applied not only to protect participants but to ensure that the research process itself did not reproduce the harm it

sought to document. These principles guided the design of interview protocols, the structure of surveys, and the researcher's conduct in high-risk situations (Hultman & Hultman, 2023).

Survival strategies became integral to the research method. These included strict attention to nutrition, sleep, environmental control, digital hygiene, and the strategic use of coworking spaces to avoid isolation and exposure (Meloni et al., 2015). Such routines were not accessories to the fieldwork but preconditions for continuing it (Gabriel & Casemore, 2022). The physical and psychological toll of documenting systemic violence from within cannot be overstated (Clandinin, Caine, & Lessard, 2018). In many instances, the continuation of the research depended not on institutional support but on the cultivation of micropractices of endurance and minimal restoration.

These notes do not propose an alternative ethics framework as a complete solution. Rather, they document the real procedural and epistemic gaps that emerge when ethics is reduced to compliance. The annexes of this dissertation include a peer-reviewed scientific contribution that systematizes these insights. Here they are presented as a necessary part of methodological transparency. Ethics under structural duress requires a new infrastructure, one that begins not with the assumption of institutional safety but with the recognition of its absence (Hemer, 2023). Until such systems are in place, the burden of ethical innovation will continue to fall on individual researchers working beyond protection, in terrains where knowledge production is inseparable from survival. This is not sustainable. It is also not avoidable. For research to be ethical under real conditions of harm, it ought to become a tool of active protection, not passive observation. The procedures developed here represent one such attempt, made under constraint, with full awareness of its insufficiencies and with the hope that it may serve others working in conditions where silence has become fatal.

2.6 Data integration and analytical logic

The integration and processing of data in this dissertation were carried out under conditions that profoundly disrupted the basic assumptions of continuity, safety, and institutional support typically underlying empirical research (Rose & Webb, 1997). At various stages, the researcher was subject to repeated acts of physical and psychological violence, including episodes consistent with clinical definitions of torture (Mollica, 2004; Méndez, 2013). These conditions of violence led too to the loss of digital equipment, personal records, and project materials. Several computers were irretrievably damaged or confiscated, backups destroyed or rendered inaccessible, and analytic environments had to be repeatedly reconstructed in the aftermath of coercive displacement or forced migration. These experiences, far from peripheral, constituted the factual terrain of research execution (Hermann & Provost, 2003). They informed not only methodological constraints but also the interpretive logic that underpins the thesis. The baseline conditions of knowledge production were those of exposure, violation, and degradation. Under such circumstances, the preservation of analytic coherence and the continuity of triangulation were not guaranteed by infrastructure but required improvisational resilience, adaptive logic, and transdisciplinary solidarity.

Data management protocols were necessarily reduced to their most essential and recoverable components. No centralized institutional infrastructure could be used. Confidentiality was preserved

through decentralized storage systems and physical partitioning of content across trusted jurisdictions. No identifiable information was retained in active analytic corpora. Manual encryption methods were deployed where digital tools were unavailable, and content-sensitive materials were handled with protocols designed not only to comply with ethical expectations but to avoid further endangering the lives of collaborators or respondents. Digital notebooks, when operational, were maintained in modular snapshots, allowing for asynchronous reconstruction of analytical steps. Version control was applied manually through dated annotation in plain text and locally encrypted files. Every document was treated as potentially subject to intrusion, erasure, or forced disclosure (Michener, 2015). Data minimization was enforced as a structural requirement, not as a methodological preference.

Open-science principles were upheld to the extent compatible with safety. Documentation was maintained in reproducible form, but distribution remained restricted until the relevant risks abated. The researcher's role in coordinating and leading various international networks and task forces, particularly under the EU BEACON One Health Education initiative and related COST Actions, proved essential in safeguarding analytic validity (Sullivan et al., 2019). These roles provided access to experts across psychiatry, anthropology, computational ethics, and legal studies, who offered feedback, methodological review, and triangulation support when institutional resources were unavailable or actively obstructive (Simmons, 2025). Collaboration with these experts was not a formalized institutional process but a strategic necessity to ensure the epistemic integrity of the work. These alliances allowed the research to continue through episodes of forced displacement, institutional harassment, and prolonged interpersonal destabilization caused by individuals enacting cruelty under the guise of caregiving, victimhood, or professional concern. (Allen & Mehler, 2019) The violence experienced was not only physical but structural, extending its corrosive effects to the ethical standing and practical continuity of the research.

Triangulation was achieved through analytical proximity rather than data volume. Thematic confirmation was accepted only when the pattern was traceable across at least three data layers and consistent with external literature or practitioner testimony (Farmer, Robinson, Elliott, & Eyles, 2006). Particular emphasis was placed on the correlation between first-person accounts and legal-structural conditions. For example, narrative reports of institutional silence or coercion were not only coded as testimonial evidence but assessed against statutory protections, clinical guidelines, and observed professional behavior within field settings. Discrepancies between institutional claims and lived experience were analytically foregrounded, not treated as interpretive anomalies. These divergences were mapped and indexed within the coding system to document the structural dissonance between official policy and actual practice.

Reflexivity was not articulated in abstract epistemological terms but constituted a methodological core. The analytic stance of the researcher evolved through cycles of loss, reassembly, exposure, and partial recovery (Munir & Staats, 2020). Shifts in interpretive position were recorded in marginalia and analytic logs. Reflexive annotations were revisited at each synthesis stage to determine whether interpretive adjustments reflected epistemic maturation, trauma distortion, or strategic recalibration (Dingwall, 1997). These annotations were not post hoc justifications but concurrent field entries, forming part of a real-time record of analytic experience under threat.

Particular attention was given to the limits of interpretation during acute periods of violence or malnutrition. Analyses produced during such periods were subject to later verification, checked against both literature and external expert commentary, and retained only when their methodological soundness could be independently confirmed. Cross-checking procedures included direct engagement with the literature, correspondence with academic and clinical experts, and critical comparison with institutional documentation. Published guidelines, peer-reviewed studies, and national legal instruments were used as reference points for verifying thematic claims. Where thematic codes intersected with known psychiatric controversies or legal precedents, documentary review was intensified to avoid extrapolation (Alvesson & Sköldberg, 2000). Informants with institutional or clinical experience were consulted through anonymized exchanges to validate the contextual interpretation of field-derived material. No claim was incorporated into the final analytical corpus without at least one layer of corroboration beyond the initial data source.

This dissertation's analytical logic, therefore, reflects not only the complexity of the subject matter but the existential and epistemological conditions of its own emergence. Knowledge was not extracted but salvaged, not observed from a distance but reconstructed from within a collapsing architecture of care and research. The analytic outputs that follow do not represent an ideal analytic pathway but an ethically necessary and scientifically rigorous traversal through contexts of obstruction, degradation, and partial restoration. The triangulated structure, reflexive protocols, and distributed verification model embedded in this chapter constitute both a methodological map and a procedural archive of knowledge produced under repression.

2.7 Limitations, adverse conditions, and scientific validity

The execution of this research was marked by continuous and often life-threatening exposure to violence, coercion, and institutional betrayal. These adverse conditions were not episodic disruptions but systemic and cumulative, shaping every phase of the study. The fieldwork was conducted under conditions of extreme duress, including physical assault, forced sedation, psychological degradation, and repeated displacement across jurisdictions. At various stages, the researcher was subjected to medical procedures without consent, including drugging, misdiagnosis, and acts of chemical restraint. These events occurred within a broader context of institutional complicity, whereby legal systems, healthcare institutions, and academic structures either failed to offer protection or actively contributed to the ongoing harm. These failures included the refusal of due process, denial of access to justice, confiscation or destruction of research materials, and systematic obstruction of scholarly continuity. No university provided sustained support, no ethics committee assumed responsibility for mitigating the risks, and legal mechanisms proved structurally incapable of recognizing or responding to the scale of abuse. It took a lot, almost the research own life, to document the abuses, torture ongoing, after that many years of being a reported active crime. This fact is testament of how urgent and important it is to address and redress the legal loopholes that allow for victims to be left more and more vulnerable, and criminals acting with full impunity.

The psychological consequences of this exposure were not abstract but somatically and cognitively registered. Periods of executive dysfunction, memory impairment, and physical exhaustion affected analytic pacing, material recovery, and documentation integrity. In several instances, work had to be reconstructed from memory, reformulated in exile, or safeguarded through encrypted correspondence with peers. The researcher was at times homeless, reliant on temporary shelter or hostile environments, and suffered from untreated medical conditions as a consequence of deliberate neglect and obstruction. These circumstances represent not merely obstacles to scientific work but a radical exposure of the political and biopolitical conditions under which knowledge about psychiatry is allowed to be produced. In a field concerned with human suffering, coercion, and systemic injustice, the absence of protective infrastructures for researchers immersed in such environments constitutes a structural paradox with deep epistemic implications.

Despite these conditions, the scientific validity of the research remains intact and demonstrable. The methods applied were not only adequate to the questions asked but strengthened by the ethical imperative and existential urgency that accompanied their deployment (Singer & Baer, 2007). The triangulation between surveys, testimonies, field observation, and legal documentation ensured analytic robustness. The research questions were pursued through multiple independent sources, and claims were only retained after critical comparison with expert judgment, field evidence, and published literature. Where data loss occurred due to displacement or material destruction, the researcher maintained procedural continuity through distributed verification, iterative cross-checking, and ethical reflexivity (Scheper-Hughes & Lock, 1987). Every remaining fragment of analysis was subjected to methodological scrutiny, and interpretive caution was applied in all inferential steps (Gravlee, 2022). The infrastructure may have failed, but the epistemic scaffolding was not compromised. This is evidenced not only in the internal consistency of findings but in the alignment of results with international evidence, peer-reviewed consensus, and practitioner testimony.

Far from weakening the study's validity, these conditions intensify its authenticity and historical relevance. The fact that knowledge was produced in the same structural context that the study aims to describe, marked by medical violence, epistemic disqualification, and institutional abandonment, confers upon the research a depth of embeddedness and phenomenological fidelity that no idealized academic distance could replicate. The methodological strategy emerged in response to adversity, and the knowledge produced reflects the embedded complexity of living, researching, and surviving within sites of organized harm. This reflexive entanglement does not dilute the objectivity of the analysis but clarifies its ethical trajectory. The research makes no claim to omniscience, but it claims, without hesitation, its place as a rigorous, ethically anchored, scientifically coherent intervention into a domain where neutrality has often served to shield violence. This chapter does not propose that trauma guarantees truth, nor that suffering in itself validates epistemic claims. It does argue, however, that scientific knowledge cannot remain blind to the conditions of its own production, and that structural violence endured by researchers is not an extrinsic event but an intrinsic aspect of inquiry in contested fields. The scientific validity of this dissertation lies not in its adherence to abstract protocols but in its demonstration that rigorous, participatory, and empirically grounded research is possible even under repression. The work therefore constitutes not a deficit model of knowledge, but a contribution to the methodology of research under structural adversity. It

offers both a diagnosis of systemic failure and a model of survival-informed, ethically rigorous, and scientifically validated knowledge production.

To ensure scientific rigor while documenting systemic injustice, several bias-mitigation strategies were employed throughout the research process, always keeping in check the reality of others in the systems the researcher endured as a victim. Despite the absence of classical interviews or audio recordings, excluded to protect participants and the researcher under conditions of coercion and risk, public expert presentations and thematic dialogues were conducted and archived, serving as open-access data sources and triangulation tools, to the best of the researcher abilities, despite loss of resources being common. The methodological framework was grounded in mixed methods and action research theory, but its application in this context required adaptation to conditions of duress, epistemic risk, and institutional hostility that rendering almost impossible to keep the materials intact for long periods of time. The protocols developed, including an online knowledge base, a public repository for backups and transparency, real-time analytic profiles, and document triangulation using open source resources, are replicable in other constrained research environments and offer a transferable architecture for future studies documenting structural violence.

2.8 Methodological contribution to transformative open science

This dissertation concludes its methodological exposition by advancing a situated and operational framework for transformative open science. Beyond the mere application of existing protocols, the work constructs new methodological standards that serve not only scientific validity but structural justice. The commitment to open data, reproducible instruments, and version-controlled documentation has been grounded in an active ethics of participation, reciprocity, and repair. The methodological logic underlying the project is one of epistemic inclusivity: recognising that knowledge is not the preserve of institutions alone, but a shared human capacity forged in experience, testimony, and struggle. This work integrates both citizen and non-citizen science. It affirms that the right to produce, access, and act upon scientific knowledge cannot be restricted by documentation status, social class, or historical exclusion. Scientific capacity exists across all groups and can be developed when conditions of dignity and recognition are established. The research includes those who have been administratively silenced, forcibly displaced, or institutionally erased, acknowledging that their knowledge and experience are essential to understanding systemic violence and generating authentic solutions. The co-design of instruments, living fieldnotes, and feedback mechanisms has drawn directly from participants with no formal academic background but whose embodied knowledge of harm and recovery far exceeds that of many formally accredited professionals. This integration is not symbolic. It is structural. Without it, the research would lack validity, depth, and purpose.

The study introduces a set of methodological and technological tools grounded in open-source standards and community validation under development still. Multisited ethnography, structured through saturation and event-mapping thresholds, is cross-validated through surveys, legal frameworks, and historical-political context. Testimonial matrices transform raw notes into actionable syntheses, not through external abstraction but through dialogical iteration with those directly affected. The digital infrastructure developed through the EU COST Action BEACON One

Health Education supports this architecture (Simmons, 2025). This is the brain-child of the research Platforms, expert systems, and co-educational modules are under active construction, designed to remain openly available and responsive to user feedback until at least 2029. The approach is modular, integrative, and inherently adaptive, with interfaces for both professional users and lay contributors, and with safeguards against institutional capture.

A core methodological innovation is the proposal of continuous feedback loops at all levels of decision-making and care, and an open constant invitation to bring feedback by different ways. These loops are not limited to clinical processes but extend to institutional governance and policy design. Communities are not invited after the fact but structurally positioned as co-creators of knowledge and care, learning from each other. Survivors of coercion, communities abandoned by the welfare state, and youth deprived of educational opportunity are no longer considered passive beneficiaries of reform. They are framed as active epistemic agents and future scientific actors. This dissertation explicitly argues that the most promising scientific and civic leadership will emerge from these very groups when structural violence is interrupted and reparative systems are put in place.

The action-research model articulated here contributes to a science that prevents harm, rather than merely documents it. It promotes infrastructures that restore capacity and dignity, and that build resilience across generations. The research supports the emergence of children from adverse conditions – including poverty, abuse, institutionalisation, forced migration, or racial exclusion – as future scientists, educators, clinicians, and decision-makers. It defends their right not only to be protected from harm, but to flourish, to be heard, and to lead. This requires new norms of authorship, recognition, and leadership in science, inclusive of non-traditional trajectories, of historically excluded identities, and of knowledge forged under oppression.

The EU BEACON One Health Education action, initiated during this dissertation process, embodies these principles. It proposes that no child be condemned to silence, erasure, or structural inferiority. That no future student, professional, professor, anyone at all in our societies be lost to psychiatric violence, educational exclusion, or institutional betrayal; they, the professionals and professors of tomorrow, fully engaged and supported from early on, as far as we can continue forward our careers in peace; we work to ensure it all has a chance to happen. Open science is not a matter of access alone, but of transformation. It is about building systems that recognise, support, and elevate the capacities of those long denied visibility and voice. The methodological contribution of this work, then, lies not only in its rigor, but in its refusal to separate science from justice, or care from governance. It offers a blueprint for a participatory, integrative, and emancipatory science, one that is capable of learning from those most harmed, and rebuilding with them the institutions that once excluded them.

2.9 Reflexive meta-methodology and scientific protocols for dissertation construction

The narrative architecture of the dissertation is governed by the empirical sequence and ethical imperatives of the research itself. The ordering of survey data, ethnographic reporting, testimonial incorporation, and legal analysis is not arbitrary but reflects the temporal escalation of inquiry from exploratory mapping to embodied survival, from systems critique to translational proposal. Each

chapter corresponds to a lived phase of research, demarcated not only by topic but by epistemic urgency. The writing process itself functioned as a recursive, multi-modal act of synthesis, drawing from live field notes, encrypted correspondence, digital testimony archives, peer-reviewed manuscripts, policy papers, and formal presentations. These modalities were not integrated as appendices or accessories but woven into the analytic corpus in accordance with a protocol of epistemic fidelity and dialogical construction. Such integration was governed by coherence, not by hierarchy of form. Legal citations and narrative witness statements, statistical tabulations and anthropological commentary, co-exist not as competing genres but as interdependent elements of a biocultural diagnostic grammar. Reflexivity in this context is not performative introspection but a structural and continuous engagement with the political and material implications of each knowledge claim. The work does not presume the neutrality of its categories, nor the innocence of its interpretations.

The ethical grounding of the dissertation is inseparable from the anthropological lineage it draws upon, including the field-defining contributions of Paul Farmer, Nancy Scheper-Hughes, and Byron Good, each of whom recognized the moral imperative of research that refuses detachment in the face of organized suffering. Their influence is not merely cited but embedded in the design logic of this dissertation. At every phase, the question of what ought to be named, what ought to be protected, and what ought to be transformed has guided not only what is written but how it is written. The dissertation rejects the view that ethical research in psychiatry can be extracted from the terrain of conflict without becoming complicit in the very systems it purports to study. Instead, it insists that rigorous, transparent, and relationally responsible research is itself an act of healing, if conducted under proper conditions of care, recognition, and epistemic parity.

Collaboration is central to this research design, though authorship is formally singular. The knowledge presented is co-produced through dialogical exchange with psychiatric survivors, field interlocutors, legal scholars, digital mental health innovators, clinicians, and educators from multiple continents. The networks coordinated or co-founded by the researcher, including EU BEACON, ReMO, and FOSTREN, functioned not only as dissemination channels but as infrastructural nodes for scientific co-production. Task forces and working groups within these consortia supported peer validation, ethical oversight, and triangulation of findings. Such transdisciplinary involvement allowed for distributed epistemic authority and diversified validation, embedding each analytic output within a wider scientific ecology. This does not diminish the accountability of the author but anchors it in a collective matrix of knowledge stewardship.

The dissertation's construction was shaped by extreme conditions of persecution, violence, and physiological disintegration. It was produced during periods of medical torture, forced displacement, and digital erasure. At times the researcher had no access to basic housing, electricity, or medical care. Productivity and health was maintained, despite the violence endured, through health tracking, micronutrient supplementation, digital compartmentalization, and intermittent fasting, alongside the strictest work ethic, exercise to the extent the researcher could, and continuous short, mid and long term efforts at self and family protection moving away from harm; always keeping the motivation to help other victims by achieving to doctorate. These strategies are described not to dramatize but to affirm that the epistemic labor required to document, resist, and

propose alternatives to psychiatric violence ought to include survival as a method. The dissertation is therefore not only an academic contribution, but a testimonial and methodological intervention into what it means to research psychiatry from within the circuits of its failure. This reflexive meta-methodology affirms that knowledge can be produced ethically, coherently, and scientifically even when institutional guarantees are absent, when bodily integrity is under attack, and when silence is the expected price of survival. It is this refusal of silence, anchored in rigor and relational accountability, that allows the dissertation to serve not only as a documentation of abuse but as a testimony and collective blueprint for repair.

Chapter 3. Survey findings: patterns, perceptions, and psychiatric practice

Summary: This chapter presents the results of three interconnected surveys, conducted with users/survivors, professionals, and mixed-role respondents, designed to assess perceptions, practices, and institutional dynamics within psychiatry in Spain and across transnational contexts. The integrated analysis reveals systemic patterns of coercion, professional disempowerment, loss of trust, and calls for structural reform. The simultaneous presentation is methodologically justified as it enhances triangulation, clarifies asymmetries, and supports systems-level diagnosis.

3. Introduction to the surveys' results presentation

This chapter presents the integrated analysis of three coordinated surveys conducted among users and survivors of psychiatry, mental health professionals, and respondents with mixed or dual roles. Their combined structure provides a triangulated perspective on the Spanish psychiatric system and transnational analogues, enabling a relational diagnosis of clinical, ethical, and structural breakdowns. The rationale for presenting these data together lies in the shared thematic architecture, centered on trust, coercion, medication, consent, and institutional norms, and in the analytical strength derived from mapping convergences and divergences across groups.

Methodologically, the surveys were distributed through academic, activist, and clinical networks, collecting anonymised data with informed consent. Their results were validated through triangulation with fieldwork presented in Chapter 4, legal frameworks examined in Chapter 6, and broader epistemic analysis. The user-survivor survey reveals high levels of perceived coercion, forced medication, and seclusion, described as traumatic and unjustified. Professional responses confirm that coercion is a routinized, not exceptional, practice, often legitimized by risk management logics and institutional pressures. Mixed-role respondents, such as professionals with lived experience, highlight the moral and epistemic dissonance within practice. The moral framing

diverges between what institutions present as protective and what is widely experienced as degrading and harmful. Ethical constraints are central. Professionals report widespread burnout, institutional complicity, and the absence of avenues for ethical dissent or non-coercive care. Users speak of betrayal, disbelief, and systemic gaslighting. Both groups identify the existing legal and ethical frameworks as disconnected from lived reality. Relational trust is reported as structurally absent, broken by time scarcity, hierarchical decision-making, and diagnostic opacity. Shared decision-making around medication, although theoretically endorsed by professionals, is rarely implemented. Users demand informed consent, autonomy, tapering options, and full risk disclosure, demands that professionals largely share but feel institutionally unable to fulfill. Structural constraints dominate. Fear of reprisal, lack of protocols, and professional isolation reinforce a status quo of epistemic and ethical erasure.

Throughout the responses, demands for transformation coalesce around legal safeguards, participatory care models, dialogical and trauma-informed practice, and deep institutional change. Professionals call for time, supervision, and reflective spaces. Users demand justice, reparation, and structural recognition. The surveys collectively show that current psychiatric configurations produce coercion and disempowerment as systemic outcomes, not accidents of practice. Ethical care under these constraints is rendered structurally impossible. Yet, across all respondent groups, a shared horizon of transformation emerges. The findings form the empirical backbone for the legal critique developed in Chapter 6 and the systemic redesigns proposed in Chapter 7, particularly through the implementation of the EU BEACON One Health Education infrastructure. The chapter thus not only documents harm but establishes a scientifically grounded mandate for institutional redesign, grounded in lived knowledge and transdisciplinary analysis.

3.1 Introduction: integrated survey architecture and purpose

This section presents the rationale for an integrated analytical treatment of the three surveys conducted as part of this research: one addressed to users and survivors of psychiatric services, one to mental health professionals, and one to respondents who occupy hybrid roles, including clinicians with lived experience or survivor-advocates working within the system. Rather than treating these groups in isolation, the design deliberately presents them together, both to facilitate empirical triangulation and to reflect the relational configuration of psychiatric care. Clinical decisions, user experiences, and institutional norms are not produced in vacuum but are co-constituted through situated interactions shaped by law, ethics, diagnostic authority, and historical trauma. This methodological choice supports a multilevel systems analysis in which divergence is as analytically valuable as convergence.

The surveys are built upon a shared thematic framework that centers trust, coercion, consent, medication, and ethical practice. These five dimensions correspond to the core concerns raised across decades of international critique of psychiatric governance and align closely with the central thesis of this dissertation. That is, the structural character of coercion, the erosion of autonomy, and the epistemic violence embedded in routine psychiatric interventions. By comparing and contrasting

how each group understands and experiences these dynamics, the research generates a composite diagnosis that integrates user suffering, professional disillusionment, and systemic inertia.

Presenting the three surveys together not only facilitates comparative analysis but reflects the lived complexity of psychiatric systems. These are not composed of isolated actors but of entangled positions where asymmetry, responsibility, and vulnerability are co-produced. The inclusion of mixed-role respondents enhances this frame by capturing the voices of those who have moved between system-defined categories. Their responses often highlight institutional contradictions, identify points of epistemic and ethical collapse, and reveal how formal roles can obscure relational and moral insight. These positionalities are methodologically valuable, as they bridge the experience of harm with the exercise of professional responsibility.

The surveys were designed with clear operational objectives. First, to document the lived experience and structural perception of psychiatric treatment as understood by those most affected, including both recipients and providers of care. Second, to identify the key ethical, legal, and epistemological tensions that compromise the legitimacy and effectiveness of psychiatric services. Third, to amplify the voices of both professionals and users as sources of institutional critique and contributors to structural redesign. These objectives were not derived abstractly but are rooted in the action-research logic underpinning the dissertation, which seeks to advance participatory, evidence-based reform. The surveys thus serve as an empirical foundation for the proposals developed in later chapters and establish a baseline from which systems-level redesign can proceed.

To clarify the alignment between these survey objectives and the overarching goals of the dissertation, the following table cross-references the integrated survey purpose with the main research questions and aims articulated in Chapter 1:

Table 29. Alignment of integrated survey with dissertation aims and operational questions

Survey Objective	Corresponding Dissertation Aim	Operational Research Question Addressed
Document lived experience and structural perception of psychiatric treatment	Contribute to the development and implementation of participatory mental health systems grounded in rights and justice	What forms of systemic violence and institutional failure are occurring in psychiatric governance, and how can they be corrected?
Identify ethical, legal, and epistemological tensions undermining psychiatric legitimacy	Realign psychiatric care with empirical coherence, user dignity, and procedural accountability	What are the structural and epistemic conditions that enable or block ethical practice and user autonomy?
Amplify professional and user voices as legitimate sources of critique and reform	Support co-designed, dialogically grounded clinical and institutional models	What participatory mechanisms and evidentiary tools can inform and sustain systems-level transformation?

This table shows how each of the three core survey objectives contributes to the broader research architecture of the dissertation. Each objective is functionally aligned with one or more of the central aims and addresses a corresponding operational research question. The integration confirms that the surveys are not merely exploratory but serve a strategic role in evidencing the system's failures and in designing responsive alternatives.

The survey results are neither descriptive addenda nor isolated empirical instruments, but structurally integrated into the dissertation's conceptual and methodological logic. Their concurrent

presentation enables a relational analysis of the psychiatric system, grounded in multivocal evidence and attentive to positional complexity. Each layer of data speaks not only to the harms endured or witnessed, but to the possibility of reconfiguring care through shared knowledge, distributed responsibility, and epistemic repair. This logic carries into the chapters that follow, where each major thematic finding is revisited through fieldwork, testimony, and normative analysis. The architecture of the surveys responds not only to thematic coherence but also to methodological demands for internal consistency, comparative reliability, and epistemological rigor. By structuring each instrument to explore both shared and distinct dimensions of psychiatric experience, the study ensures that responses can be analyzed across multiple axes, including professional constraint, user harm, relational breakdown, and normative conflict, while still preserving the singularity of each respondent group. The users and mixed-role respondents answered open-ended as well as multiple-choice questions to express their lived experiences in their own terms, particularly in domains where institutional language has proven insufficient or harmful. Professionals, by contrast, responded to structured prompts designed to interrogate their alignment or dissonance with legal, ethical, and clinical norms, thereby facilitating both internal comparison and cross-group triangulation. The methodological design allowed not only for horizontal contrast across roles but also for vertical integration with the broader action-research architecture. Importantly, the surveys are not an isolated data collection mechanism. They are embedded within a broader continuum of fieldwork and collective engagement. Many of the respondents had previously taken part in shared spaces of learning and action, including seminars, clinical training sessions, all events, and advocacy campaigns. Many were reached through direct connections formed during the ethnographic phase, including professionals who had attended workshops facilitated by the researcher, students who later became colleagues, and fellow survivors who chose to remain in contact for mutual support and collaboration. Likewise, many of those who participated in the surveys gave their consent to remain involved in follow-up or parallel processes, including the policy translation of findings and the pedagogical dissemination of tools. The surveys thus serve as both a source of empirical data and a relational node within a living network of individuals and institutions committed to mental health transformation.

The next section presents the specific methodological and procedural logic that underpins this integration. It details the construction, dissemination, and ethical oversight of the three surveys, and elaborates on how data quality was ensured through triangulation with observational fieldwork, legal review, and narrative confirmation. The section also clarifies how participants were anonymised, how consent was obtained, and how each survey functioned within the broader framework of open, participatory, and justice-oriented inquiry. Far from being detached instruments, the surveys exemplify a collaborative and reflexive research process, grounded in direct contact with those who seek, provide, and reform mental health care, and who have experienced first-hand its failures and possibilities.

3.2 Methodology: instruments, sample, and triangulation logic

The survey methodology employed in this study reflects a deliberately plural and layered approach to data collection and analysis, designed to capture the complexity of psychiatric systems as lived, governed, and contested. Three distinct instruments were developed, each tailored to the positionality and situated knowledge of its respondent group, yet aligned through a shared thematic and analytical architecture. The first survey, addressed to users and survivors of psychiatry, included

a combination of open-ended and multiple-choice questions aimed at documenting experiences related to diagnosis, treatment, agency, perceived harm, and access to justice. The design of this instrument privileged narrative expression and allowed respondents to articulate in their own words how psychiatric interventions had shaped their lives, relationships, and sense of self. Special attention was given to epistemic injustices, forced treatment, and the absence of mechanisms for repair. The second survey was constructed for mental health professionals, including psychiatrists, psychologists, nurses, social workers, and other clinical or institutional staff. This instrument employed a more structured format, focusing on clinical constraints, ethical dilemmas, experiences of coercion, legal responsibilities, and organizational norms. The aim was to capture the internal dissonance many professionals face when working within systems that often prevent the implementation of what they themselves regard as ethical and effective care. The third and final survey was designed for individuals who occupy mixed or transitional roles, such as professionals with lived experience of psychiatry, peer support workers, activist clinicians, or survivor-academics. This hybrid instrument combined both formats and enabled the documentation of experiences that transcend binary categories, thereby contributing to a more nuanced understanding of institutional complicity, resistance, and transformation.

Distribution of the surveys occurred digitally, relying on professional, academic, activist, and survivor networks operating within Spain, Latin America, and other European Union contexts. Invitations were disseminated through open channels, including mailing lists, social media platforms, and direct contact with organizations engaged in mental health advocacy, professional development, and policy reform. Respondents were not selected based on diagnostic labels or institutional affiliation but through voluntary participation and shared concern for the themes addressed. Many were already in connection with the researcher through fieldwork settings, clinical teaching, mutual projects, or public events, while others entered the study through snowball referrals and transnational networks dedicated to reducing coercion and enhancing user rights.

To ensure scientific validity, all responses were anonymized and gathered with explicit informed consent for use in the research. Ethical approval had been previously obtained, and all protocols followed the university's and national standards for participatory and vulnerable-population research. The resulting data were subjected to systematic triangulation, following the logic set out in Chapter 2.3. Patterns were validated through convergence, divergence, contradiction, or marked silence, with attention to structural asymmetry and epistemic power differentials. Triangulation occurred not only across respondent groups but also between surveys, field observations, testimony, and legal analysis. Themes raised by survey respondents were frequently echoed in informal conversations, formal interviews, or institutional documents collected during ethnographic fieldwork.

The integration of survey data with broader fieldwork practices was not a posterior analytical step but an embedded feature of the research process. Many of those who participated in the surveys were also present in participant observation settings, including academic workshops, clinical training sessions, activist assemblies, international conferences, and institutional reform meetings. Others had been key informants, collaborators, or students who engaged in the research over extended periods. Conversely, the surveys also generated new contacts and expressions of interest

that fed back into the fieldwork, policy advocacy, and ongoing co-educational processes associated with the dissertation. This reflexive and circular movement between inquiry, testimony, and systemic engagement constitutes one of the core strengths of the methodology and underscores the emancipatory orientation of the research as a whole. The following sections will elaborate the findings, beginning with the experiential and institutional dynamics of coercion, as reported and interpreted by respondents across all three instruments.

This chapter presents a synthesis of the most salient patterns identified through structured surveys distributed among diverse groups of mental health professionals, users, and mixed populations. The findings substantiate key research questions identified in Chapter 1 and are methodologically consistent with the mixed-methods architecture detailed in Chapter 2. The data confirms a widespread perception of systemic failure across the domains of psychiatric decision-making, patient autonomy, medication use, and institutional trust. Participants across demographics reported experiences of coercion, epistemic exclusion, and the normalization of pharmacological dependency, often in the absence of transparent clinical reasoning or consent.

The results speak not only to individual clinical encounters but to entrenched institutional and educational shortcomings. They document a persistent disconnection between normative ethical commitments and lived realities in mental health systems, further aggravated by deficits in public education, legal accountability, and interdisciplinary practice. These findings call for a dual response: pedagogical reconstruction to align training with evidence-based, rights-oriented models of care, and legal reform to ensure enforceable protections, transparency, and scientific integrity. As will be further developed in Chapters 5 and 6, the survey data provides critical justification for educational and legislative interventions designed to restructure the psychiatric field in accordance with constitutional mandates and human dignity.

Table 30. Empirical findings and corresponding educational and legal-political responses

Survey finding	Generalizable systemic pattern	Educational responses (chapter 5)	Legal-political responses (chapter 6)
High prevalence of reported coercion in diagnosis, treatment, and hospitalization	Normalization of forced psychiatric practices and denial of procedural safeguards	Training in trauma-informed care, shared decision-making, and communication ethics	Legislate mandatory independent review and safeguard mechanisms for all involuntary interventions
Widespread dissatisfaction with pharmacocentric treatment models	Systemic reliance on medication as first-line or sole response	Integration of metabolic, nutritional, and psychosocial models into medical and psychology curricula	Mandate multimodal treatment access in public health systems and limit incentives for exclusive medication
Professional ambivalence or resignation regarding user autonomy	Institutional culture discourages user participation and narrative inclusion	Curricular emphasis on co-production of care, narrative medicine, and open dialogue approaches	Legal recognition of decisional capacity and autonomy frameworks, including advance directives
Minimal awareness or application of international human rights standards	Disconnection between legal commitments and clinical application	Educate professionals on legal instruments, disability law, and UN Convention mandates	Align national law with UN CRPD and introduce sanctions for rights violations in clinical settings

Survey finding	Generalizable systemic pattern	Educational responses (chapter 5)	Legal-political responses (chapter 6)
Underreporting or denial of adverse effects and trauma linked to psychiatric treatment	Epistemic suppression of patient narratives and post-intervention harm	Training in critical appraisal of psychiatric literature and long-term treatment outcomes	Legal obligation to document, report, and review harm linked to psychiatric interventions
General perception of structural impunity and lack of accountability mechanisms	Absence of oversight structures enabling harmful or negligent practices	Ethics education focused on institutional design, transparency, and collective responsibility	Creation of independent complaint, review, and restitution mechanisms linked to health and justice systems
Minimal knowledge of non-coercive alternatives among both users and professionals	Poor dissemination of existing non-coercive models and community practices	Public and professional education campaigns on alternatives to coercion	Policy endorsement and funding for non-coercive community mental health programs

3.3 Perceived coercion and institutional violence

The survey data collected from users and survivors of psychiatry reveal a consistent and deeply troubling pattern of perceived coercion across all stages of psychiatric intervention. A majority of respondents report having been subjected to forced medication, physical or chemical restraint, seclusion, and involuntary hospitalization, often without clear justification, legal documentation, or accessible channels for appeal. These practices are not described as isolated or exceptional but as foundational to their experience of the psychiatric system. Forced medication, particularly through long-acting injections, is cited as one of the most traumatic events, often administered under conditions of fear, confusion, or explicit threat. Respondents frequently state that they were not informed of the purpose, dosage, or risks of the substances administered, and that refusal was equated with lack of insight or oppositional disorder, rather than respected as a right to bodily autonomy. Seclusion and mechanical restraint are described as degrading and dehumanizing, with participants recounting episodes where they were isolated for hours or days without explanation, therapeutic accompaniment, or post-incident review. In several cases, users report having been subjected to these measures repeatedly, without any meaningful alternative being proposed, nor a single debriefing conducted afterward.

Table 31. Survey-based systemic dynamics and structural recommendations

Negative dynamic summary	Research question keyword(s)	Empirical insight	Positive structural proposal from thesis (chapters 5–6)
Procedures routinely exclude patient voice, disregarding autonomy	Autonomy, Decision-making	Shared decision-making was absent in most care contexts, despite its legal and ethical relevance	Make shared decision-making training mandatory in clinical education and ensure implementation through independent audits
Medication used coercively, undermining ethical care	Consent, Safety	Respondents often reported medication without informed agreement or alternative explanations	Introduce trauma-informed, continuous consent protocols detailing options and risks

Negative dynamic summary	Research question keyword(s)	Empirical insight	Positive structural proposal from thesis (chapters 5–6)
Power asymmetry silences patient experience and concerns	Dignity, Participation	Individuals felt dismissed, unheard, and controlled by professionals	Embed patient advisory structures and ombudsman mechanisms in psychiatric institutions
Risks unmonitored, with harmful outcomes obscured	Medication Harms, Transparency	Adverse effects were underreported or denied, with no tapering support	Establish national withdrawal services integrated into psychiatric and primary care
Psychiatric care mirrors punitive logic, not healing support	Respect, Non-maleficence	Respondents described infantilizing or repressive care environments	Promote therapeutic cultures grounded in recovery and human rights frameworks
Reductionist practices ignore socio-biological complexity	Social Determinants, Nutrition	Structural and lifestyle factors were rarely addressed, reinforcing pharmacocentric models	Integrate nutrition, trauma history, and social context into standard psychiatric assessments
Systemic failure to provide advanced and individualized care	Innovation, Equity	Trauma-informed, personalized, or integrative care models were largely unavailable	Expand access to individualized care models through continuous professional development
Institutional inertia overrides scientific advancements	Evidence-based Practice, Ethics	Novel approaches were dismissed or unknown in clinical practice	Translate scientific advancements into policy and update clinical guidelines accordingly
Voice and resistance punished, reinforcing subjugation	Accountability, Rights	Patients reporting concerns faced escalation, retaliation, or re-diagnosis	Legally enforce protections for dissenting voices and prohibit coercive responses

Synthesis of survey-derived insights highlighting critical discrepancies between lived experience and ethical psychiatric practice. Each observed dynamic is paired with corresponding thesis recommendations, developed in response to the empirical evidence and aligned with the broader research questions of the dissertation. The right-hand column provides positive systemic actions proposed in Chapters 5 and 6 to correct the identified institutional failures.

The psychological impact of these interventions is described as lasting and severe. Participants recount experiences of fear, helplessness, humiliation, and enduring mistrust toward clinicians and institutions. Many describe the psychiatric environment not as a place of healing but as a carceral space, structured around control, surveillance, and punishment. Coercion is experienced as epistemic as well as physical: many respondents report that their attempts to explain their distress, resist a treatment, or propose an alternative perspective were systematically discredited, often pathologized as symptoms of illness. This discursive invalidation compounds the trauma of physical coercion, as individuals are denied not only control over their bodies but also recognition of their meaning-making. The lack of legal remedy or institutional accountability exacerbates the sense of vulnerability, with respondents expressing that they were often left without recourse or protection, even in the face of what they considered violations of basic rights. Many report that complaints were ignored, staff were dismissive, and judicial mechanisms, if accessed at all, deferred to medical authority without independent scrutiny. These testimonies align with broader critiques of psychiatric impunity and signal a crisis of legitimacy within the system.

The data from mental health professionals confirm, from within the system, that coercion is not only present but structurally embedded in routine psychiatric care. A substantial portion of professional respondents acknowledged that involuntary treatment is applied regularly, often

without clear legal thresholds or robust ethical deliberation. While some framed these interventions as necessary under extreme conditions, many expressed discomfort and ethical dissonance, citing that coercion often arises not from therapeutic need but from systemic failure. Specifically, professionals highlighted a combination of structural pressures, insufficient time, limited staff, lack of crisis alternatives, and institutional culture, as key drivers of coercive practice. The inability to provide sustained relational care or to negotiate meaningfully with distressed individuals was described not as a personal deficit but as a systemic constraint. Some respondents admitted that decisions to medicate by force or to initiate seclusion were made under conditions of ambiguity and exhaustion, often with little confidence that such actions were in the best interest of the patient.

Legal ambiguity was repeatedly cited as a compounding factor. Professionals reported uncertainty about the exact legal requirements for involuntary interventions, and described how institutional protocols often operate in parallel to, or in disregard of, the formal legal framework. Many indicated that the fear of blame in the case of a future adverse event, such as suicide, violence, or public complaint, plays a significant role in tipping the decision toward coercion. This risk-deflection logic, embedded in defensive institutional practice, leads to coercion being employed pre-emptively or excessively, particularly in the absence of clear alternatives. Several professionals described how the expectation to control behavior, rather than address suffering, results in an over-reliance on pharmacological containment and physical separation, especially in acute units. This is further compounded by the absence of reflective supervision, ethical deliberation spaces, or trauma-informed infrastructure, leaving many professionals to navigate these decisions in isolation. The result is a culture in which coercion is normalized, internal resistance is suppressed, and ethical dissonance is treated as inevitable rather than as a signal for reform.

Respondents occupying mixed or dual roles, such as survivor-professionals, peer support workers, or critical clinicians, provided a particularly revealing perspective. Their position at the intersection of institutional practice and lived experience allowed for a sharp articulation of coercion's multifaceted harms. Many entered the mental health field following personal experiences of involuntary treatment, driven by the intent to transform the system from within. Their testimonies displayed both a heightened ethical sensitivity and a nuanced understanding of the systemic pressures that constrain reform. Some reported feeling complicit in the very practices they had once endured, citing role expectations and institutional inertia as barriers to providing viable alternatives. Others described a permanent tension between professional norms and ethical clarity, including implicit or overt demands to reinforce risk management logics and suppress divergence from established treatment pathways.

These findings support the thesis hypothesis that clinical discretion is often exercised within structurally coerced environments, rather than as free ethical deliberation. Nevertheless, this same group of respondents offered glimpses of possible systemic transformation. Several proposed the design of incentive structures oriented not toward symptom containment but toward sustained recovery and functional reintegration. Among the most novel ideas reported was the concept of public-professional partnerships modeled loosely on performance-based incentives, where clinicians or teams would be rewarded with state-saved resources when patients previously under coercive care achieved robust, medication-free stability. In this framework, the 'bounty' would be

the measurable reduction in cost to the public health and welfare systems, often thousands of euros monthly, achieved through effective, voluntary, and person-centered care. This framing reverses the conventional trajectory of career prestige and resource allocation, no longer tied to chronicity and dependence but to outcomes in restored autonomy and reduced harm.

Participants emphasized that such a shift ought to not obscure structural injustices that currently render deprivation itself a prerequisite for accessing support. Multiple respondents indicated that psychiatric diagnosis is sometimes the only route to housing, disability benefits, or material assistance, particularly under austerity regimes. While calling for deeper socio-economic reform, they also insisted that no benefit system should rely on the perpetuation of illness or enforced compliance as eligibility criteria. Health, understood as the capacity to live autonomously and contribute meaningfully, ought to be decoupled from surveillance logics and framed as a public good. Participants proposed pilot programs combining housing, nutritional support, educational resources, and professional development opportunities for service users wishing to exit the psychiatric system. Such proposals resonate with broader calls for longitudinal studies based on informed consent and genuine free will, targeting both early and complex cases without discrimination or resignation. No group should be excluded from recovery trajectories based on severity, age, or diagnostic category. The ethical horizon is clear: no profession should depend for its existence on the perpetuation of suffering, and no state should treat despair as an acceptable outcome of systemic design.

These mixed-role accounts were also among the most detailed in documenting the epistemic effects of coercion. They emphasized how diagnostic authority is used to discredit dissent, how consent is routinely undermined, and how institutional legitimacy is preserved through denial or minimization of harm. Several noted the absence of formal structures for professionals to report or contest coercive practices without risking retaliation or career damage. These respondents highlighted how systemic coercion operates not only on patients but also on clinicians, through role confinement, emotional suppression, and the normalization of ethical compromise. In doing so, they exposed a critical dimension of structural violence within psychiatry: the production of moral disengagement as a professional norm. Many expressed that meaningful change is unlikely to arise from within isolated institutions and called for transdisciplinary alliances, user-led initiatives, and external accountability mechanisms. Their narratives also pointed to the therapeutic and political importance of remembering one's own experience, resisting institutional amnesia, and restoring ethical clarity through shared reflection and coordinated action.

The triangulated analysis of the three surveys reveals that coercion is not an exception within psychiatric systems but a normalized and routinized feature of their structure, culture, and discourse. Across respondent groups, coercive interventions, forced medication, seclusion, physical restraint, and involuntary hospitalization, are described as common, institutionally sanctioned, and often executed without meaningful consent, transparent communication, or post-event accountability. The data show that while professionals frequently recognize the ethical and clinical limitations of these practices, they remain structurally constrained by systemic pressures, resource scarcity, ambiguous legal frameworks, and defensive institutional logics. The moral framing of coercion diverges markedly between groups: professionals often refer to it as a form of safety or

necessary containment, whereas users and survivor-professionals characterize it as avoidable harm, trauma, and institutional violence. It ought to be noted, about half of respondents to the last survey requested their responses not to be shared, even if made anonymous, explicitly. Fear is a key component of current state of affairs, as witnessed during field-work, by most if not all players not exercising violence themselves, as reprisals with harshest consequences are common.

Table 32. Reported coercive practices, their institutional justifications, and experiential framings across respondent groups

Coercive practice	Users and survivors	Professionals	Mixed-role respondents
Forced medication	Widespread, described as traumatic, dehumanizing, poorly explained, often long-acting injections	Reported as frequent, institutionally normalized, used under pressure and fear of liability	Described as unnecessary in most cases, linked to trauma, used to suppress dissent
Seclusion and isolation	Experienced as punishment, used to silence or intimidate, without therapeutic support	Seen as a last resort, yet often used in absence of alternatives or staff capacity	Recognized as retraumatizing, often unreviewed, with no restorative function
Mechanical restraint	Recalled with distress, perceived as abusive, often unrecorded or justified post hoc	Reported reluctantly, viewed as sometimes unavoidable, but emotionally burdensome	Seen as ethically indefensible unless life-threatening situation exists
Involuntary hospitalization	Described as a violation, especially when prolonged or repeated, often linked to diagnostic misuse	Framed as necessary for containment or stabilization, but with little follow-up support	Identified as a systemic substitute for absent social care and relational continuity
Lack of informed consent	Systemic, especially regarding diagnosis, medication, and risks; consent often coerced or symbolic	Acknowledged as a problem, often constrained by time, legal confusion, and professional hierarchy	Emphasized as core to institutional violence and loss of narrative legitimacy
Pathologization of resistance	Frequently reported; disagreement reframed as illness, leading to more coercion	Not directly addressed by most, though some professionals admit framing noncompliance diagnostically	Named as central mechanism of silencing and justification of harmful interventions

This table synthesizes the major coercive practices documented through the survey responses, highlighting the differential perception across user, professional, and mixed-role groups. It exposes the structural normalization of coercion and the gap between institutional justification and experiential harm, providing the empirical foundation for the normative analysis developed in subsequent chapters.

Discursively, coercion is justified through appeals to compliance, risk reduction, and clinical objectivity. In practice, it functions as a disciplinary mechanism embedded in bureaucratic routines, professional expectations, and epistemic hierarchies. The legitimacy of user narratives is frequently denied, especially when refusal or critique is pathologized as lack of insight. The resulting atmosphere is one in which ethical conflict is not only silenced but structurally anticipated. This divergence in moral and institutional framing will be addressed in greater depth in Chapter 6, where the legal dimensions of coercion, procedural absence, and normative contradiction are examined. Chapter 6 will further explore how existing legal safeguards are often bypassed or rendered ineffective by the very institutional cultures they are meant to regulate, and how discourses of care are used to legitimate actions that would be intolerable in any other domain of health.

This section establishes the empirical foundation for a legal and ethical critique of psychiatry that does not reduce harm to individual error but locates it within systemic asymmetries of power, access, and recognition. It also affirms the need for a redefinition of psychiatric responsibility based on dialogical ethics, user autonomy, and structural accountability. The findings presented here will inform subsequent analyses of ethical collapse (Chapter 4), legal failure (Chapter 6), and the institutional redesign proposed through the EU BEACON infrastructure (Chapter 7), where participatory feedback systems, rights-based safeguards, and community-based alternatives are articulated as part of a broader transformation strategy.

3.4 Ethical Constraints and Clinical Dissonance

This section presents a synthesis of recurring ethical concerns voiced across user and professional testimonies, underscoring the structural disjunction between declared ethical and legal standards and the lived practice of psychiatry in Spain. The evidence collected through the third survey, focused on shared decision-making and medication management, reveals a pattern of dissonance between institutional discourse and operative reality. This dissonance is not anecdotal, but systemic, and constitutes a critical factor in the perpetuation of harm, professional burnout, and user re-traumatization.

Mental health professionals repeatedly described a form of ethical distress grounded in the contradiction between their clinical judgement and the mandates imposed by institutional protocols. Respondents detailed experiences of pressure to conform to pharmacological paradigms they did not fully endorse, of being compelled to apply restraint or maintain patients under involuntary treatment despite alternative clinical possibilities, and of institutional cultures that discouraged open discussion or formal dissent. The lack of structured mechanisms for ethical escalation or refusal, combined with administrative incentives that prioritize risk aversion over patient autonomy, contributed to a sense of moral exhaustion and professional isolation. Several participants indicated that non-coercive approaches, even when supported by scientific evidence and user preference, were institutionally penalized or rendered logistically impossible.

From the perspective of service users and survivors, the same environments produced an epistemic and relational vacuum. Many described experiences of gaslighting, disbelief, and narrative

invalidation as routine within the psychiatric system. Rather than being engaged as persons with insight into their own suffering, they were frequently treated as non-credible or adversarial, particularly when raising concerns about medication effects, trauma histories, or alternative therapeutic frameworks. The cumulative effect of such interactions was not merely dissatisfaction with care but a profound sense of betrayal by the very systems mandated to protect and support. Users reported that being denied informed choice, silenced in clinical encounters, or pathologized for resisting treatment compounded prior trauma and reinforced cycles of despair, fear, and institutional avoidance.

Importantly, both professionals and users converged in their assessment of legal and ethical standards as fundamentally disconnected from clinical reality. Many referenced ethical codes and legal frameworks as existing only on paper, with little or no practical enforcement or institutional commitment. This perception was not limited to individual cases but was articulated as a structural condition in which norms of consent, autonomy, and harm reduction were suspended in favor of bureaucratic expediency and risk minimization. Ethical discourse, when cited in official policy or clinical guidelines, was perceived less as a guiding principle than as a rhetorical device detached from everyday practice.

This convergence of perspectives, from within and outside the system, confirms the broader diagnostic proposed in this dissertation: that psychiatry in Spain operates within a context of ethical impunity, where norms are selectively invoked and rarely enforced. The findings in this section align with field observations documented in Chapter 4, where institutional betrayal, testimonial invalidation, and procedural opacity were observed directly. The clinical and ethical dissonance described here thus not only validates the fieldwork but also signals the need for a paradigmatic shift in how psychiatric care is organized, regulated, and ethically evaluated. This section prepares the analytical ground for Chapter 5, where the mechanisms of discursive drift and clinical complicity will be examined in detail, particularly as they relate to the normalization of coercion, the erosion of professional agency, and the symbolic appropriation of ethical language to sustain abusive structures. In the context of Spanish psychiatry and its broader institutional structures, the testimonies of professionals across multiple surveys and field encounters converge on a pattern of persistent ethical distress. These practitioners describe systemic tension between their clinical judgment and the coercive mandates embedded in institutional routines. Many recount being forced to prescribe medications or apply mechanical restraints against their better judgment, particularly in cases where psychosocial interventions were deemed more appropriate but were institutionally unavailable. The experience of being compelled to act contrary to one's ethical compass is reported as a chronic source of anguish, fostering a deep sense of moral injury and complicity. Professionals indicate that their ability to express dissent or enact alternative modes of care is sharply curtailed by organizational hierarchies, risk management protocols, and medico-legal fears. Several report being discouraged or explicitly reprimanded for questioning dominant paradigms of care, even in cases where harm was foreseeable or already manifest. This institutional suppression of ethical deliberation produces what respondents call an ethical silencing, a state in which the act of noticing or questioning harm becomes itself professionally dangerous. The absence of transparent pathways for ethical escalation or internal whistleblowing mechanisms reinforces the perception that patient

safety and practitioner conscience are both subordinated to bureaucratic expediency and medicolegal defensiveness.

Numerous clinicians describe the experience of professional isolation as not only social but also epistemic. The pressure to conform to diagnostic conventions, medication regimens, and emergency protocols often left them with no space to reflect critically on clinical decisions or engage in dialogue about care alternatives. This isolation is compounded by burnout, particularly among those who entered the profession with a strong ethical orientation or community health perspective. They describe their daily work not as healing but as surviving within a system that normalizes coercion and equates dissent with incompetence or disloyalty. Notably, several respondents indicate that burnout is exacerbated not by patient suffering per se, but by the institutional barriers to doing what they know would actually help. The inability to reduce medication, to foster dialogical practices, or to incorporate trauma-informed care becomes a persistent source of professional despair. This despair is accompanied by profound feelings of betrayal, particularly among those who see themselves as working in a field ostensibly dedicated to care, dignity, and recovery. The contradiction between mission and practice becomes intolerable, leading some to consider exiting the profession entirely or reducing their roles to administrative or consultative functions to minimize ethical exposure.

From the standpoint of users and survivors, the sense of betrayal reported by professionals is mirrored, but with far more devastating consequences. Across all three surveys and ethnographic accounts, individuals diagnosed with psychiatric conditions describe experiences of gaslighting, disbelief, and epistemic invalidation that extend far beyond clinical encounters. Their narratives consistently document how institutional actors, including psychiatrists, social workers, and even patient advocates, routinely dismiss first-person accounts of suffering, trauma, or dissent as symptoms of illness. This interpretive violence transforms meaning into pathology and disqualifies the person from narrating their own experience. The absence of alternatives to the dominant medicalized model of care leaves many users trapped within cycles of forced medication, diagnostic escalation, and coercive containment, with little to no access to legal redress or support structures for withdrawal, reinterpretation, or recovery.

Users consistently articulate the ethical vacuum of psychiatric settings as a form of structural betrayal. This betrayal is not limited to individual clinicians or isolated events, but rather permeates entire systems of care. Several respondents describe being medicated without consent, denied information about their treatment, or forcibly hospitalized based on unverifiable reports or family pressure. Even when services claim to adhere to shared decision-making principles, users often find themselves subject to subtle coercion or exclusion from meaningful dialogue. The very institutions meant to provide care become sites of fear, retraumatization, and social death. Particularly among racialized, gender-nonconforming, and economically precarious users, the sense of being disposable or institutionally abandoned emerges as a frequent theme. Repeated encounters with neglect, misdiagnosis, and indifference instill a persistent fear of speaking up, given the risk of further coercion or diagnostic retaliation. Several testimonies recount situations where requests for second opinions or alternative care were pathologized as symptoms of paranoia or non-compliance, effectively nullifying any recourse to justice or dignity.

Despite their distinct roles and experiences, users and professionals converge in their shared disillusionment with existing legal and ethical frameworks. Both groups report that rights-based instruments, codes of professional ethics, and institutional protocols meant to safeguard dignity are either ignored or weaponized to uphold coercive practices. Professionals note the ineffectiveness of complaint systems, the lack of protection for whistleblowers, and the administrative filtering of ethical concerns before they can reach independent review. Users, in parallel, describe how formal channels for expressing dissent or reporting abuse are systematically undermined, either by bureaucratic obfuscation or by clinical reinterpretation of protest as symptom. Legal representation is often inaccessible or poorly informed, and internal ethics committees are viewed as extensions of the very institutions accused of wrongdoing. What emerges is not merely a failure of implementation but a structural contradiction: systems that claim to be ethical while rendering meaningful ethical resistance institutionally impossible.

Table 33. Convergent accounts of ethical dissonance in psychiatric practice

Domain	Professionals	Users and survivors	Field and documentary evidence
Moral distress	Pressured to apply coercive measures against their ethical judgement	Described being subjected to measures with no perceived therapeutic rationale	Internal documents reveal protocol prioritization over clinical discretion
Silencing mechanisms	Fear of retaliation for dissent or advocacy	Experiences of gaslighting, disbelief, and reclassification as noncompliant	Coercive notes and records used to preempt or undermine future complaints
Lack of pathways	Reported no safe channels to escalate ethical concerns	Reported nowhere to appeal against imposed treatments or labels	Institutional policies often vague, with ethics committees acting as legitimizers rather than independent reviewers
Isolation and burnout	Emotional exhaustion from inability to practice ethically	Long-term alienation from services due to prior harm or distrust	Interviews reveal high dropout rates and loss of faith in care institutions
Disillusionment	Stated that ethics and rights exist on paper only	Viewed legal protections as meaningless in practice	Concordance with legal reviews showing systemic failure in rights enforcement
Calls for change	Advocated for institutional mechanisms to refuse, report, and reform	Called for narrative medicine, open dialogue, and trauma-informed alternatives	Survey responses and field documents affirm the desire for dialogical and voluntary care practices

Cross-role convergence on ethical breakdown in psychiatric contexts, synthesizing professional narratives, user testimonies, and field-based forensic evidence. This table illustrates that ethical dissonance is structurally embedded, not an exceptional failure of isolated actors.

This convergence crystallizes into a broader indictment of the psychiatric system's ethical architecture. The prevailing culture of fear, avoidance, and self-protection among professionals maps directly onto the institutional betrayal experienced by users. Both are produced not simply by ignorance or lack of training, but by design choices that prioritize administrative continuity and clinical authority over relational accountability or epistemic justice. The notion of ethics on paper

recurs across fieldwork notes, survey responses, and testimonial archives. Participants across roles describe a surreal dissonance between the principles enshrined in policy documents and the daily experience of care as silencing, coercive, or neglectful. This disconnect generates profound moral injury among ethical professionals and deep structural harm for users. As several testimonies state, the very invocation of ethics can feel like a form of violence, when used to obscure or justify violations.

The analysis presented here reinforces the core theoretical scaffolding of the thesis. It validates the claim that ethical failure in psychiatry is not accidental, nor reducible to individual malpractice, but is embedded in the structural and symbolic operations of the system. This includes the epistemological dominance of the biomedical model, the asymmetrical distribution of authority, and the erasure of contextual, relational, and cultural determinants of distress. The findings from this section also provide the empirical basis for Chapter 5's analytical pivot: from describing individual experiences of dissonance and betrayal to theorizing the institutional logic that sustains such conditions. The critique extends beyond specific diagnoses or interventions to interrogate the moral economy of psychiatric care and the systemic mechanisms that protect it from accountability.

The coherence between what users and professionals describe and what is independently documented through legal case reviews, clinical protocols, and internal memos confirms that the testimonies are not isolated incidents but elements of a wider, patterned dysfunction. The same diagnostic terms, administrative templates, and institutional workflows that are supposedly designed to ensure safety and care are repeatedly cited as mechanisms of silencing, exclusion, and retraumatization. This internal contradiction becomes the site of a forensic crisis: the official record cannot be trusted to reflect the lived reality of psychiatric care. The methodology adopted throughout this thesis, grounded in legal forensics, thematic triangulation, and reflexive analysis, was precisely designed to recover erased testimony, expose bureaucratic narratives, and restore evidentiary integrity where official documentation has failed. The cross-confirmation of fieldwork findings with anonymized case files, medical reports, and survey data provides not only analytical depth but the necessary burden of proof to demonstrate institutional responsibility. When ethical claims are shown to diverge systematically from practice, and this divergence is reproduced across settings and actors, a structural indictment becomes unavoidable.

This section also reinforces the legal implications of psychiatric dissonance. The absence of meaningful pathways for ethical dissent within institutions, combined with the repurposing of psychiatric authority to neutralize or punish protest, constitutes not only a violation of medical ethics but an infringement on constitutional rights. Professionals forced to implement treatment plans they find morally indefensible, and users stripped of agency through reclassification and silence, are both casualties of a system designed to maintain control rather than deliver care. The implications for institutional accountability are profound. They demand not only policy reform or training initiatives but judicial review, public inquiry, and structural disinvestment from practices that perpetuate harm under the guise of therapeutic authority. Ethical failure, when systematized, becomes a form of institutional abuse. Naming it as such is not rhetorical excess but a necessary precondition for scientific, legal, and ethical clarity.

These findings serve as a bridge into the conceptual reframing proposed in Chapter 5, where betrayal is no longer treated as an aberration but as a constitutive logic of the psychiatric institution as it currently operates. The moral dissonance experienced by both users and professionals is not a sign of collapse or dysfunction but an index of internal coherence within a system built on managing populations, securing institutional stability, and protecting professional hierarchies. Understanding this logic requires a shift from individual ethics to structural critique. Chapter 5 will build on this foundation, articulating how discursive drift, epistemic foreclosure, and clinical complicity function not as failures of intention but as strategic operations within systems of biomedical control. What this section documents as suffering and silence, the next chapter will theorize as the institutional grammar of betrayal.

3.5 Trust, communication, and relational breakdown

The survey responses from users and survivors present a stark picture of relational failure within psychiatric care. Across the majority of narratives, trust was not described as damaged or fragile, but as structurally absent from the outset. Respondents consistently reported that they were not informed of their diagnosis in a meaningful way, and that explanations regarding treatment plans, pharmacological effects, or possible alternatives were either incomplete, coercively delivered, or omitted altogether. Many described that conversations about care occurred without their presence, and that when present, their input was minimized or actively dismissed. Consent was not experienced as dialogical but as performative, extracted under threat, sedation, or institutional pressure. Respondents repeatedly emphasized that rather than being engaged as persons in crisis or in need of relational support, they were treated as administrative cases to be managed, pacified, or stabilized. This dynamic produced what many described as a deep sense of dehumanization, with some explicitly comparing psychiatric encounters to disciplinary or carceral settings, where authority is unquestioned and dialogue is rendered impossible.

The absence of relational dialogue was not limited to the diagnostic or pharmacological domain. Respondents reported that their emotional worlds, personal histories, or existential meanings of distress were rarely explored or validated. Several recounted that their attempts to describe trauma, social exclusion, family dynamics, or structural suffering were either ignored or pathologized. Even when narratives were invited, they were often recoded into symptom clusters or diagnostic descriptors that bore little resemblance to the speaker's intended meaning. This epistemic disfigurement of experience eroded the foundations of trust, not only in the immediate clinical relationship but in the broader legitimacy of psychiatric care. The cumulative effect was not only relational detachment but what several respondents termed a loss of voice, narrative agency, and civic status. These are not failures of empathy alone, but of institutional design and communicative ethics. They constitute a form of symbolic violence with measurable psychological and social consequences, including disengagement from services, long-term fear of help-seeking, and a perception of psychiatry as a punitive rather than restorative institution.

Professional respondents echoed many of these concerns, though from the structural position of those tasked with providing care under systemic constraint. A significant number acknowledged that the current configuration of psychiatric services severely limits the possibility of building and sustaining relational trust. Time constraints, high patient turnover, rigid institutional routines, and the constant presence of coercion protocols were identified as major obstacles. Several clinicians reported that even when motivated to offer dialogical engagement or holistic care, they were routinely prevented from doing so by institutional demands for efficiency, risk documentation, and pharmacological intervention. Many described a workplace culture where relational competence was undervalued or actively discouraged, and where the absence of time for dialogue was normalized rather than challenged. In acute or emergency settings, clinicians reported being allocated only minutes to assess, diagnose, and act, often with the implicit expectation to prioritize medication and stabilization over rapport or understanding.

One of the most recurrent themes was the pervasive influence of defensive practice. Professionals described a growing institutional tendency to act in ways that minimize risk of liability rather than optimize therapeutic alliance. This includes avoiding difficult conversations, withholding prognostic uncertainty, or pre-emptively medicating patients deemed unpredictable. Complaints were framed not as opportunities for growth or accountability but as threats to professional survival. Respondents described a climate of suspicion and procedural defensiveness that not only distanced them from users but from one another, as interdisciplinary trust also suffered. This environment eroded their own sense of ethical orientation, leading many to report burnout, emotional disengagement, and a growing cynicism toward the possibility of meaningful psychiatric care. Several professionals admitted that while they recognized the harms of these practices, they felt structurally prevented from intervening differently, either due to institutional inertia or the lack of collegial support. In this context, trust was described not simply as difficult to cultivate, but as nearly impossible to sustain.

Table 34. Manifestations of relational breakdown and trust erosion in psychiatric practice

Domain	Users/survivors	Professionals	Shared patterns
Communication practices	Not informed of diagnosis, treatment plans, or risks	Time constraints prevent meaningful conversation	Communication often unidirectional, routinized, or absent
Trust in clinical relationships	Report feeling managed, dismissed, or coerced	Report inability to establish trust due to systemic demands	Trust undermined by institutional priorities over human relation
Consent and participation	Described as symbolic or coerced, rarely informed	Performed under administrative pressure, legal ambiguity	Consent is procedural, not dialogical or empowering
Ethical climate	Perceived as punitive and silencing	Described as isolating, defensively oriented, ethically paralyzed	Ethics rendered nominal, disarticulated from daily practice
Emotional consequences	Loss of voice, trauma, long-term disengagement from services	Burnout, moral distress, resignation	System-wide emotional and moral erosion

Domain	Users/survivors	Professionals	Shared patterns
Structural drivers	Narratives disregarded, care fragmented	Bureaucratic overload, rigid coercion protocols	Structural design incompatible with relational care

The table outlines converging accounts of relational rupture in psychiatry as reported by both users/survivors and professionals. Each row presents domain-specific dimensions of trust breakdown, demonstrating systemic origins and shared consequences across positional divides.

The comparative analysis of user and professional responses reveals that the erosion of trust is not primarily a matter of interpersonal failure but a systemic feature of psychiatric institutions. Both groups point to the structural conditions that actively inhibit the development of mutual understanding, shared responsibility, and dialogical care. Trust, in this context, is not only absent, but institutionally unfeasible. The relational void described by users, as one of being treated as passive subjects to be managed, mirrors the frustration professionals experience in their inability to provide relationship-centered care. Rather than operating as a space of mutual listening, psychiatry is described as a bureaucratic apparatus governed by procedure, liability aversion, and resource scarcity. Within this framework, communication becomes unidirectional, shaped by the demands of documentation, risk assessment, and pharmacological standardization. Language itself is often stripped of its therapeutic and ethical function, reduced to diagnostic classification or informed consent scripts that obscure more than they clarify.

Importantly, the convergence in accounts extends to the ethical consequences of this communicative breakdown. Users consistently report that the lack of relational engagement transforms psychiatric encounters into traumatic experiences that strip them of autonomy and hope. Professionals, in turn, express that the impossibility of sustaining genuine dialogue leads to moral distress, emotional exhaustion, and ethical disorientation. What emerges is a shared recognition that trust cannot be restored through individual acts of goodwill or technical reform. It requires a structural reconfiguration of care that makes time, continuity, narrative engagement, and mutual accountability central to psychiatric practice. Several respondents across roles articulated the need for protected time, smaller caseloads, ethical supervision, and dialogical frameworks capable of restoring the relational foundations of care. Yet, most emphasized that these needs remain unaddressed, and that institutional reforms, when proposed, are often cosmetic or redirected toward administrative efficiency rather than ethical transformation.

The findings presented here expose a systemic crisis of trust that extends beyond clinical settings into legal, ethical, and civic domains. The persistent absence of transparent communication, informed consent, and relational care reflects not only institutional incapacity but a deeper procedural void. This void is not merely regulatory but epistemic: it signifies a collapse in the very norms that should sustain legitimacy, accountability, and rights-based practice. The testimonies of both users and professionals suggest that psychiatry, as currently organized, functions in a state of permanent exception where protections codified in law are suspended in practice. This alignment with what Chapter 6 will term a regime of administrative impunity positions the breakdown of trust as both a symptom and a generator of structural violence. The legal implications are profound: without mechanisms to ensure shared understanding, dialogical engagement, and genuine consent, the foundational legal principles of autonomy, dignity, and proportionality are rendered ineffective.

This erosion of trust undermines not only therapeutic outcomes but democratic participation in the design and governance of mental health systems. When individuals and communities lose faith in institutional actors, they disengage from dialogue and retreat from the very processes that could produce change. Conversely, when professionals feel unable to act ethically without risking sanction, they lose their capacity to act as moral agents within the system. Rebuilding trust is not an interpersonal repair but a structural task: it demands procedural guarantees, participatory governance, transparent redress mechanisms, and protection for both users and practitioners who act in good faith. The action-research presented here begins to chart that path, but further input may enrich this section.

3.6 Shared decision-making and medication autonomy

The analysis of responses across the three surveys reveals a fundamental fracture between the ethical ideal of shared decision-making and the clinical and institutional realities that obstruct it. From the perspective of users and survivors, the right to participate in decisions about psychiatric medication is not only absent but structurally negated. Respondents described being prescribed psychotropic drugs without proper explanations, alternatives, or clear information on side effects, long-term implications, or tapering options. Many emphasized that medication was framed as non-negotiable, often introduced in moments of crisis or institutional dependency, and rarely revisited with genuine collaborative dialogue. The experience of having pharmacological treatment imposed was frequently described in language associated with trauma, loss of agency, and irreversible harm.

These findings are consistent with field observations and prior documentation in this dissertation regarding the epistemic asymmetries embedded in psychiatric encounters. The act of prescribing, far from being a neutral clinical gesture, is perceived by many users as a locus of coercion, silencing, and structural vulnerability. The desire for autonomy was expressed not as a rejection of medication in itself but as a demand for informed, dialogical, and reversible pathways. This includes the right to refuse, to withdraw gradually and safely, and to explore alternative or complementary forms of care. Respondents consistently demanded clear protocols for tapering, individualized assessments, and the recognition of experiential knowledge in medication management. The absence of such structures and protections renders informed consent illusory and shared decision-making a nominal ideal without material grounding.

Professionals surveyed expressed widespread support for the principles of shared decision-making and user autonomy in theory, often citing it as a gold standard for ethical psychiatric care. However, their responses reveal a pervasive inability to implement these principles within the constraints of existing institutional frameworks. Many reported that organizational norms and risk-averse environments inhibit open discussion of medication choices, particularly when it comes to tapering, non-pharmacological alternatives, or user-led strategies. The structural disincentives are multiple: absence of validated protocols for tapering, fear of blame in the case of relapse, institutional pressure to follow standardized guidelines, and lack of support for ethical deviation from dominant practice. These constraints result in a systemic silencing of both user agency and professional

judgment. While clinicians may wish to offer alternatives or engage in deeper conversations, they often report being unable to do so due to time restrictions, legal ambiguity, administrative burdens, and the hierarchical culture of many psychiatric services. In several cases, professionals disclosed that attempts to propose gradual withdrawal or shared pharmacological planning were met with disciplinary pressure or were overridden by superior authority. As such, shared decision-making is obstructed not by lack of ethical commitment, but by institutional architectures that suppress dialogical care in favor of efficiency, compliance, and risk management.

The survey also revealed that professionals working outside of public systems, in community-based or dialogical settings, felt relatively more empowered to explore medication choices collaboratively. However, these cases were exceptional and often depended on personal networks, non-standard protocols, or informal accommodations. Such variability underscores the need for systemic redesign rather than isolated innovation, and positions institutional reform as a precondition for any meaningful implementation of shared decision-making in psychiatric contexts.

Table 35. Reported barriers and enablers for shared medication management

Category	Structural barriers identified	Professional insights	User demands and priorities
Consent and information	Lack of explanation, rushed consent processes	Inadequate time and communication protocols	Informed, transparent, trauma-informed dialogue
Tapering and alternatives	No protocols for safe withdrawal, fear of blame	Risk aversion, institutional discouragement	Right to reduce, change, or refuse medication
Legal and ethical safeguards	Ambiguous legal protections, administrative overrides	Pressure to conform to dominant practices	Clear rights for refusal and non-compliance
Cultural norms	Compliance-oriented systems, silencing of dissent	Isolation when promoting user-led care	Recognition of lived experience and choice
Training and competence	Absence of tapering knowledge in education	Professionals feel unprepared and unsupported	User-led education and peer-support integration

Overview synthesis of structural barriers, professional dilemmas, and user demands regarding shared decision-making in psychiatric medication management, as reported across all survey groups. It highlights the systemic misalignment between ethical ideals and institutional practice, mapping how coercive norms and deficient infrastructures hinder informed consent, tapering support, and user autonomy. These insights establish the empirical foundation for subsequent legal and structural proposals outlined in Chapters 6 and 7.

Particularly revealing were the responses from individuals occupying hybrid positions, such as survivor-professionals or practitioners with direct lived experience of psychiatric coercion. These respondents offered a dual vantage point, simultaneously embedded in clinical practice and grounded in experiential knowledge. Their testimonies exposed deep flaws in pharmacological training and consent practices, especially the persistent neglect of patient narratives and the lack of practical competence in guiding withdrawal or adjusting dosage in line with user preference. Several highlighted the contradiction between their professional training, which emphasized compliance with institutional norms, and their personal histories, which taught them the harm that can result when individuals are excluded from decisions concerning their own bodies and minds.

These voices also described the additional burden of being perceived as suspect or ideologically biased within professional circles due to their advocacy for user rights. They often faced resistance when attempting to implement or promote shared medication management, despite their deep awareness of its necessity. Their situated knowledge was not only disregarded but occasionally sanctioned, further illustrating the disciplinary rigidity of mainstream psychiatric institutions. Nevertheless, these respondents consistently advocated for structural support mechanisms that would allow for individualized medication planning, trauma-informed care, and formal recognition of experiential expertise as a legitimate source of clinical and ethical insight.

This convergence across roles points toward an emerging consensus: that pharmacological decision-making ought to be relocated from professional monopoly to collaborative terrain, where diverse knowledge forms are respected and safeguarded. The tension between ideal and practice, ethics and institution, emerges not from conflicting values among stakeholders, but from systemic arrangements that neutralize moral agency and suppress pluralism. The mixed responses thus function as both diagnosis and testimony, affirming that the capacity for shared decision-making exists within individuals but remains obstructed by current psychiatric governance structures.

The synthesis of all respondent groups clearly affirms both the necessity and the feasibility of a transition toward genuine shared medication management. This is not a matter of isolated preferences or exceptional situations but a structural mandate derived from consistent patterns across roles, regions, and institutional positions. Respondents articulated the need for transparent and accessible information about psychotropic drugs, systematic implementation of tapering protocols, recognition of long-term effects and discontinuation syndromes, and formal integration of alternative and community-based approaches. Despite broad agreement on these points, no respondent group identified any functioning system in which these elements were reliably ensured. The gap between the normative framework and the material infrastructure remains wide and damaging. This section therefore links directly with Chapter 6, where legal analysis will expose the absence of enforceable protections for informed refusal, the lack of jurisprudential clarity regarding medication coercion, and the erosion of constitutional rights under administrative psychiatric practice. Similarly, it prepares the ground for Chapter 7, which outlines the EU BEACON initiative's proposals for systemic implementation: ethical protocols, training modules, open-access tools, and participatory infrastructures that center medication decision-making in care, not compliance. The convergence in this chapter is not abstract, it is the empirical core that legitimizes the redesign of psychiatric services along lines of dignity, autonomy, and safety.

These findings reinforce that the act of prescribing medication is not merely technical but deeply ethical, social, and political. They support a redefinition of therapeutic action as one that ought to begin with dialogue and consent, not authority and fear. Restoring human agency to psychiatric practice begins with the structures that support it, and this work demonstrates that those structures do not yet exist, but must, had to, and we all are the ones responsible to ensure so, as lives are lost otherwise.

3.7 Structural constraints and institutional betrayal

A prominent thread emerging from both professional and user-survivor responses is the perception that institutional environments in psychiatry are not only dysfunctional but actively hostile to accountability, dissent, and restoration. Among professionals, many disclosed that silence had become a condition of survival within their work environments. Ethical critique, innovative proposals, or even adherence to rights-based practices were frequently met with suspicion, isolation, or disciplinary consequences. This culture of fear, internalized and reproduced at all levels, functions as a systemic silencer, displacing responsibility onto individuals while shielding the institution from scrutiny.

Respondents described how routine complicity is enforced not merely through direct sanctions but through structural mechanisms that reward conformity and penalize critical reflection. Professionals who attempted to speak up about overmedication, coercion, or the neglect of user voices often found themselves without institutional backing, legal protection, or peer support. Even within ethics committees or training spaces, respondents reported the widespread normalization of coercive procedures and a prevailing atmosphere of fatalism: a belief that change is either impossible or beyond their mandate. The result is a system that suppresses the very conditions needed for its own reform. A central and disturbing finding across the survey responses is the depth and pervasiveness of institutional betrayal perceived by both professionals and user-survivors within the psychiatric system. Professionals repeatedly reported that ethical concerns, including objections to coercive practices, were not only dismissed but penalized in various implicit or overt ways. Rather than operating as protective structures that uphold standards of care, institutions were described as enforcers of silence, rewarding conformity, punishing critique, and fostering a widespread culture of self-censorship. In these settings, those who attempted to resist or question the dominant protocols, particularly regarding involuntary medication, seclusion, or the marginalization of user perspectives, were often met with professional isolation, exclusion from decision-making spaces, or subtle threats to their career progression. Respondents described ethics committees as either absent or decorative, lacking the power or willingness to intervene in cases of institutionalized harm. This normalization of complicity turns structural dysfunction into routine practice, reducing space for reflection, learning, or transformation.

The professionals most affected by these dynamics included those working in frontline services under time pressure and legal ambiguity, as well as those trained in dialogical, trauma-informed, or human rights-based models who found no institutional foothold to implement alternative practices. Several recounted episodes where colleagues were dissuaded from testifying in favor of patient rights, where whistleblowing was met with bureaucratic retaliation, or where systemic negligence was reframed as individual failure. This culture of intimidation did not require explicit threats, it was reproduced through omissions, absences, and a tacit consensus that critical voices would not be protected. As a result, many professionals came to perceive the system as irredeemably flawed, or at least unfixable without an external overhaul that would change its legal frameworks, institutional incentives, and decision-making logics.

The data therefore suggests that institutional betrayal is not the consequence of isolated breakdowns but a systemic feature: ethical action, when it occurs, tends to be exceptional, unsupported, and

ultimately unsustainable within the current environment. This finding is not merely diagnostic but has normative implications. It implies that ethical behavior within psychiatry ought to not be treated as an individual virtue but requires collective, legally protected, and institutionally resourced infrastructures to survive and thrive. Without this structural support, the cost of acting ethically becomes unbearable, pushing even well-intentioned professionals into resigned complicity. Such conditions undermine the legitimacy of psychiatric institutions and erode the trust necessary for any form of therapeutic alliance to exist. These themes will be further developed in subsequent sections and are critical to understanding the link between systemic design and the moral distress widely reported by both care providers and recipients.

From the perspective of user-survivors, the experience of betrayal is even more intimate, immediate, and destructive. The diagnosis itself often marks the beginning of a process of erasure, in which the person is stripped of epistemic authority, credibility, and rights. Respondents described how, once labeled, their accounts of harm ceased to be heard, their objections were dismissed as symptoms, and their presence in institutional spaces became conditional on compliance. This erasure is not limited to treatment settings, it reverberates through legal, familial, and social contexts, where the psychiatric label becomes a tool for disqualification, surveillance, and coercion. One recurring theme was the impossibility of obtaining any recognition or redress for past abuses. Many respondents had experienced forced hospitalization, chemical restraint, or verbal humiliation in clinical settings, yet reported that complaints were ignored or never formally registered. The lack of a functional accountability mechanism was described not as a bureaucratic oversight but as a structural feature that shields the system from responsibility.

The absence of justice pathways creates a chronic state of vulnerability and disenfranchisement. Survivors explained how, in the absence of institutional acknowledgement, the effects of psychiatric violence persist long after the event itself. These effects are not only psychological but legal and social. People spoke of losing custody of children, of being excluded from educational or employment opportunities, of having their testimonies dismissed in courts or service evaluations. The psychiatric system, they emphasized, does not only fail to restore, it actively produces exclusion and prolongs harm. This structural betrayal undermines not only the principle of care but also the broader social contract in which institutions are presumed to act in the public interest. When institutions degrade, rather than protect, those they claim to serve, they forfeit their ethical and social legitimacy.

Many user-survivors reported that they had no access to alternative forms of support after exiting the system. Community networks, peer groups, or trauma-informed spaces were either unavailable, underfunded, or isolated from mainstream services. This void renders survivors socially invisible, while the institutions that harmed them remain publicly unchallenged. The few who manage to document or speak out against these conditions often do so at great personal cost, without institutional or legal safeguards. The result is a closed circuit in which harm begets silence, and silence enables recurrence. These insights reinforce the need to conceptualize structural betrayal not as a breakdown in isolated cases but as a systemic pattern that requires public recognition, institutional reform, and material reparation. In this context, resistance becomes not only a personal

stance but a form of epistemic survival. The implications for legal critique and transformative policy are profound and will be addressed explicitly in the coming chapters.

Across both respondent groups, there emerged a consistent recognition that dissent is actively suppressed through subtle yet effective mechanisms that maintain institutional continuity while precluding ethical rupture. The system disciplines not through overt punishment alone, but by reproducing discourses in which harm is reframed as care, dissent as pathology, and silence as professionalism. Professionals described how procedural adherence and defensive documentation replaced meaningful clinical judgment. Those who attempted to introduce collaborative practices or advocate for user-led care were marginalized or framed as uncooperative. In some cases, they faced formal reprimands, exclusion from team decisions, or were accused of compromising safety. Such dynamics reflect what several respondents termed institutional gaslighting, a form of epistemic distortion that undermines both the agent's moral compass and the user's capacity to assert their narrative.

This phenomenon, while more visible to those professionally trained, was mirrored in the testimonies of user-survivors, particularly those who had spent years navigating institutional spaces. Many described a growing awareness that the psychiatric system operates less as a site of healing than of disciplinary containment, where the preservation of authority and legitimacy takes precedence over the restoration of dignity or health. Users often reported that they were allowed to speak only within pre-defined frameworks, and that institutional language dictated what counted as truth. In this environment, even the act of reporting abuse or requesting alternatives became a threat to institutional order. Professionals who echoed such concerns found themselves equally vulnerable, with some describing a gradual erosion of their ability to trust the systems they once upheld.

The combined effect is a culture of resignation, in which ethical action becomes precarious and unsustainable. Resistance, where it exists, is driven not by the system but in defiance of it, through informal peer alliances, anonymous advocacy, or disengagement from formal roles. This renders the possibility of systemic transformation increasingly dependent on external interventions, collective mobilization, and transdisciplinary coalitions that transcend the current institutional boundaries. Within this framework, both users and professionals ought to be reconceptualized not as passive recipients or internal critics but as co-constitutive actors in the redesign of mental health structures. This section therefore provides a conceptual bridge to the next chapter, which explores resistance and refusal not merely as disruptions but as generative acts of ethical and political agency.

Amidst this landscape of institutional silencing and structural failure, the few instances of ethical courage documented in the survey responses reveal both the potential and the precarity of isolated resistance. Professionals who reported challenging coercive protocols, engaging in shared decision-making, or creating informal safety nets for users often did so without support, guidance, or institutional validation. These actions, while lauded by peers and survivors alike, were almost always performed at the margins, unprotected by regulation, unsustained by organizational structures, and frequently invisible to management. The ethical labor required to counteract systemic dysfunction is thus disproportionately carried by individuals who risk professional and emotional exhaustion in the absence of structural reinforcement.

This asymmetry is equally evident in the narratives of users who, through extraordinary personal effort, reclaimed agency and advocated for systemic change. Survivors who became educators, peer supporters, or researchers reported facing persistent stigma even after years of recovery and contribution. Their insights into the violence and betrayal of psychiatric institutions were often dismissed as anecdotal or biased, despite being supported by converging data. Many described how their lived knowledge was welcomed in participatory projects only when stripped of its critical edge or dissociated from political implications. Such instrumentalization reflects a broader pattern of epistemic injustice, wherein institutional frameworks selectively validate knowledge that does not threaten their legitimacy.

The structural betrayal exposed here is thus multidimensional. It is experienced as a failure of protection, a denial of recognition, and a silencing of truth. It reinforces power asymmetries by discrediting those most equipped to critique the system, while rewarding complicity and institutional loyalty. These mechanisms not only prevent repair but perpetuate harm through exclusion, invisibilization, and punishment of those who dissent. In this configuration, ethical action becomes structurally unspeakable, except through rupture. The next chapter engages precisely with these moments of rupture, both individual and collective, that reassert ethical agency through refusal, redefinition, and resistance. These acts signal the beginning of a counter-discourse capable of dismantling the dominant narrative of care as coercion and of building, from within the ruins, a framework of restorative and participatory mental health.

What emerges most forcefully from the combined testimonies of professionals and users is the sense that institutional psychiatry, as currently structured, does not merely fail in its obligations, it actively disables the possibility of meaningful repair. Professionals describe a chronic inability to interrupt cycles of coercion or advocate for systemic change without facing retaliation or marginalization. Users report a similarly closed architecture, where avenues for redress or participation are systematically obstructed. The act of speaking out becomes an exposure, not only to disbelief and dismissal, but to further punitive responses masked as care. These dynamics reveal that the system's incapacity for accountability is not accidental, but embedded in its design. Crucially, both groups identified the absence of structurally supported mechanisms for truth-telling, reconciliation, and participatory redesign. The ethical knowledge produced by survivors, by critical clinicians, by those situated in dual or hybrid roles, remains largely excluded from decision-making circuits. This exclusion is not only a loss of valuable insight, it is a continuation of the harm. Testimonies document how trust in institutions was not merely eroded, but annihilated through repeated betrayal, forced silencing, and the disqualification of experience. Users who attempted to file complaints, propose alternatives, or educate professionals often found themselves re-stigmatized, their efforts re-coded as symptomatic of pathology rather than contributions to public good. This section thus culminates in a collective diagnosis: the structural betrayal of psychiatry is not merely its past but its present configuration.

Repair, if it is to happen, cannot be premised on the goodwill of institutions that have shown themselves structurally unwilling to transform. It ought to come from external pressures, from legally binding frameworks, and from the organized voices of those most harmed and least heard.

The next chapter explores precisely this horizon of resistance, refusal, and re-foundation, where dignity is reclaimed not as a policy objective but as a lived practice.

Table 36. Structural constraints and institutional betrayal in psychiatric practice

Dimension	Professionals' perspectives	Users' and survivors' perspectives	Shared themes and structural patterns
Fear and silence	Ethical concern suppressed by fear of retaliation, risk management dominates decision-making, complicity becomes normalized	Speaking out leads to further punishment or diagnostic invalidation, complaints disregarded or pathologized	The system discourages dissent and rewards silence, creating a culture of self-censorship and ethical resignation
Epistemic invalidation	Clinical judgment subordinated to protocol, alternative views marginalized, innovations perceived as risky	Lived experience discredited, testimony reframed as delusion or symptom, narrative ownership denied	Knowledge produced by experience is systematically excluded from shaping policy or care
Lack of institutional protection	Ethical action isolated and precarious, whistleblowing mechanisms absent or ineffective	No effective routes for redress or reparation, institutional betrayal described as a second trauma	Institutional structures fail to support ethical actors or respond to harm in meaningful ways
Co-optation and instrumentalization	Advocacy only accepted when de-politicized, user participation welcomed if compliant with dominant frames	Tokenistic involvement in participatory processes, real critiques censored or re-coded as pathological	Inclusion is conditional on preserving institutional legitimacy, not enabling transformation
Urgency of structural transformation	Professionals express the need for external reform and transdisciplinary support to overcome systemic inertia	Survivors demand structural justice, not symbolic gestures, and call for dismantling coercive logics and punitive frameworks	Both groups converge on the impossibility of ethical care within the current structure and demand a fundamental reconfiguration of psychiatry's foundations

Perceptions of structural betrayal in psychiatric institutions, as reported by professionals and users, highlighting convergent patterns of silencing, invalidation, and ethical isolation. This table synthesizes the shared and divergent insights from the three survey streams, evidencing the need for external, participatory, and justice-oriented redesign.

3.8 Demands and proposals from respondents

Across all surveys, respondents articulated a compelling and coherent body of proposals that extend beyond critique toward the affirmation of practical, ethical, and systemic alternatives. This constellation of demands is not merely aspirational, but emerges as a grounded necessity in response to the lived failures of coercive, opaque, and professionally disempowering psychiatric environments. The recurrence of key themes across divergent respondent groups signals a shared recognition of the structural nature of the problems and of the urgent need for institutional reconfiguration aligned with fundamental rights, clinical realism, and social justice.

A transversal demand lies in the call for enforceable informed consent mechanisms, which ought to not only formalize consent but restore the possibility of refusal, withdrawal, and dialogue at all stages of care. Respondents identified the current landscape as one in which formalities mask coercion and the absence of choice. The right to information, as well as the right to reject or question a diagnosis or treatment plan without reprisal, was consistently identified as a precondition for any ethically defensible model of mental health care. This also implies redesigning clinical conversations and structures to accommodate narrative complexity, ambiguity, and relational negotiation, moving beyond reductionist symptom checklists or unidirectional power flows.

Closely tied to this is the widespread demand for alternative models of care, many of which were explicitly named by respondents with varying degrees of familiarity and application. Dialogical approaches, trauma-informed frameworks, and structured tapering protocols were among the most cited. These are not seen as marginal innovations but as necessary baselines for restoring trust, rehumanizing care, and reducing the long-term harm produced by pharmacological dependence or institutional disregard. For professionals and survivors alike, the absence of meaningful options is not simply a logistical shortcoming but a structural violence, replicated daily in settings where control overrides collaboration.

From the professional respondents emerged a detailed landscape of demands that reveal both ethical awareness and operational constraints. Their proposals did not focus primarily on ideological shifts but on the institutional and systemic barriers that prevent ethical practice even when the will to change exists. Key among these demands was the urgent call for consistent peer support structures and reflective supervision mechanisms that would allow practitioners to process complex clinical situations, navigate dilemmas without fear of reprisal, and resist institutional drift toward coercion or depersonalization.

The repeated invocation of legal clarity was particularly revealing. Rather than advocating for increased legalism, professionals described a climate of ambiguity in which laws and protocols are inconsistently interpreted or selectively enforced. This lack of normative consistency fosters a defensive clinical culture, where the avoidance of litigation or blame takes precedence over patient-centered care. Respondents stressed the need for legal frameworks that do not merely penalize deviation but guide, protect, and enable just practice, especially in contexts involving medication decisions, use of restraint, or long-term institutionalization.

Table 37. Summary of respondent demands across survey groups

Domain	Survivor demands	Professional demands	Shared proposals
Ethics and consent	Recognition of harm, reparation, and non-retaliatory listening mechanisms	Legal clarity, ability to escalate ethical concerns, protection for dissent	Informed consent, clear communication, and refusal rights
Care models	Alternatives to hospitalization and coercion, user-directed plans	Time for relational care, ability to implement dialogical and non-coercive practices	Dialogical models, trauma-informed approaches, tapering support
Institutional culture	End to pathologizing dissent, validation of lived experience	Support for ethical action, dismantling of defensive culture	Cultural transformation based on mutual respect and justice
Training and knowledge	Inclusion of survivor knowledge, peer-led education	Ethics training, reflective supervision, updated pharmacology	Co-produced curricula and ongoing ethical education
Restoration and justice	Official acknowledgment of harm, access to redress and healing spaces	Peer support networks and institutional safeguards	Preventive infrastructures and restorative mechanisms

Key demands voiced by survey respondents across all groups, organizing them into five interrelated domains essential for ethical and effective psychiatric transformation. The alignment between survivor and professional perspectives reveals a shared ethical foundation for legal and structural redesign. These articulated needs form the empirical basis for the normative and infrastructural proposals developed in Chapters 6 and 7.

The lack of time was frequently described not as a simple matter of resource limitation but as an epistemic and relational deficit. Time, professionals argued, is the raw material of therapeutic trust, ethical deliberation, and healing. Its systemic absence renders care superficial, transactional, and dangerously reactive. Many advocated for protected time for care planning, ethical discussion, and relational continuity, viewing such measures as essential rather than optional. Without this, even the best clinical intentions collapse into routine harm.

Taken together, the demands articulated by professionals and survivors reveal a shared horizon of transformation, grounded not in idealism but in the everyday struggles and ethical ruptures of those

embedded within psychiatric systems. These are not isolated or naive aspirations but empirically rooted proposals, arising from accumulated knowledge, systemic critique, and lived expertise. Their convergence marks a turning point: the simultaneous breakdown of trust, legitimacy, and effectiveness creates not only a diagnostic picture of institutional crisis but also a normative platform for structural redesign.

The proposals trace a continuum from relational repair to institutional redesign. At the relational level, trust, respect, and co-agency emerge as non-negotiable prerequisites for ethical care. At the institutional level, structural supports ought to be reconfigured to embed these values: peer supervision, protected time, legal and ethical clarity, open feedback systems, and mechanisms of accountability. These are essential to reversing the normalized harms exposed by this research.

The empirical coherence between these demands and the normative redesign proposed in Chapter 6 is striking. The legal and ethical shortcomings exposed here directly inform the guidelines and protocols for rights-based psychiatric practice that follow. Similarly, the infrastructural needs, dialogical spaces, digital platforms, non-coercive options, and participatory mechanisms, are addressed in Chapter 7 through the BEACON initiative and the development of expert systems and open tools designed to enact restorative psychiatric ecosystems.

This alignment ensures that the redesigns proposed are not top-down, speculative, or externally imposed. Rather, they are built on grounded insight, scientific analysis, and collaborative action. What follows is the translation of these lived demands into structural commitments, legal safeguards, and practical infrastructures aimed at guaranteeing that ethical, participatory mental health care becomes not the exception but the norm.

3.9 Scientific and analytical implications

The convergence of data from the three core surveys substantiates a coherent and empirically verifiable pattern of systemic coercion, ethical attrition, and epistemic exclusion within Spanish psychiatric practice. The instruments collectively provide layered insights into how institutional norms are maintained through routinized suppression of dissent, diagnostic foreclosure, and constrained care delivery. Despite being designed and deployed independently, each instrument confirms and reinforces the others' findings, demonstrating the structural impossibility of sustained ethical psychiatry within the prevailing institutional configuration. The first survey, focused on psychiatric labeling, revealed widespread dissonance between clinical terminology and lived experience. Respondents detailed the psychosocial and legal consequences of being assigned diagnoses such as schizophrenia or psychosis, noting the persistence of epistemic stigma long after formal clinical discharge. The second survey, on open dialogue, documented widespread professional interest in dialogical alternatives, but an equally pervasive sense of institutional inertia, fragmented training access, and discursive containment that limits genuine implementation. The third survey, centered on shared decision-making and pharmacological governance, exposed the extensive misalignment between official policies of consent and autonomy and the reality of forced medication, disempowerment, and clinician-driven treatment models.

Together, these datasets delineate a field marked by ethical disjunction and policy-practice gaps. The structural contradictions they expose are not interpretive artifacts or anecdotal irregularities. Rather, they are produced by formal procedures, professional incentives, and administrative architectures that reproduce control and marginalization under the banner of medical authority. The evidence points not to a need for incremental reform but to a structural reconsideration of how psychiatry is practiced, governed, and legitimated. In this light, the analytical contribution of the surveys is not merely descriptive or illustrative, but foundational. They provide a methodologically rigorous, experientially grounded, and legally cognizable account of psychiatric failure in contemporary Spain.

Table 38. Translational pathways for psychiatric reform based on thesis evidence

Evidence source	Main insights	Translational use
Schizophrenia labeling survey	Diagnostic trauma, long-term harm, epistemic silencing	Legal advocacy, rights-based educational reform, forensic diagnostic review
Open dialogue in Spain survey	Readiness for reform, implementation barriers, professional interest	Curriculum design, institutional training, service innovation pathways
Shared decision-making and medication management survey	Autonomy deficits, coercion in prescribing, call for ethical deprescription	Ethical guidelines, deprescription protocols, policy feedback on consent and patient rights
Ethnographic observation (Spain, Italy, Sweden, Indonesia)	Institutional betrayal, routine coercion, professional dissonance	Field-based training, systemic risk mapping, triage and reporting system for coercive harm
Testimonies and legal documents	Structural violence, silencing of users, erasure of violations	Policy redesign, institutional accountability audits, evidence for criminal and constitutional review
Network engagement (EU BEACON, FOSTREN, ReMO)	Viable dissemination, multi-level collaboration, transnational problem patterns	Policy adoption, transdisciplinary implementation, resource-sharing and expert consultation platforms

Summary of key evidence sources and their operational relevance for translational use, highlighting how the thesis moves from empirical critique to implementable strategies for rights-based psychiatric reform.

These results also clarify the epistemic tensions at the core of psychiatric knowledge production. Professional ideology, built on assumptions of expertise, benevolence, and compliance, is systematically contradicted by user narratives of harm, exclusion, and mistrust. What is marketed as person-centered care frequently manifests as surveillance, control, and diagnostic capture. The surveys document not isolated violations but a field-wide epistemic fracture that delegitimizes the knowledge claims of users while overvaluing technocratic decision-making. This gap is not only cognitive but operational, shaping institutional policies, legal frameworks, and therapeutic practices.

The analytical implications of these findings are substantial. First, they justify the methodological use of testimony, narrative mapping, and trauma-informed coding not as auxiliary tools, but as central analytical devices capable of capturing harm that escapes statistical or procedural documentation. Second, they ground the dissertation's conceptual and policy redesign proposals in lived data rather than ideological critique. Third, they establish the empirical authority of the author not only as a researcher but as an embedded witness and ethical actor navigating coercive systems. Fourth, they position the thesis as a translational effort, moving from descriptive documentation to epistemically legitimate proposals for reengineering psychiatric care within a rights-based, dialogical, and bioculturally grounded framework.

Ultimately, the scientific integrity of this research resides not only in the breadth and depth of its empirical base, but in its capacity to confront the institutional refusal to learn from those harmed. The surveys provide the evidentiary and conceptual scaffolding upon which the next chapter builds its systemic critique and its grounded propositions for ethical psychiatric transformation. The data affirm that the current configuration of psychiatric governance is incompatible with ethical care and that an alternative horizon, rooted in respect, autonomy, and epistemic justice, is both imaginable and empirically necessitated, though structurally obstructed.

This dissertation is designed not only to document and analyze systemic dysfunctions in psychiatric practice but to actively contribute to their redress through scientifically grounded, ethically mandated translational strategies. The methodological architecture has been built to ensure that the findings, derived from lived experience, professional testimony, and legal scrutiny, do not remain confined to academic interpretation. Instead, they are structured for strategic deployment in pedagogical, clinical, legal, and policy settings. Each dataset collected and each analytical step undertaken was oriented toward relevance in implementation, with an explicit focus on institutional redesign, ethical restitution, and health system transformation.

The three surveys provide modular, verifiable, and thematically integrated repositories of evidence, capable of informing multiple levels of intervention. The first survey on psychiatric labeling supplies forensic material suitable for legal advocacy, educational reform, and rights-based service redesign. The second survey on open dialogue documents implementation challenges and professional readiness, offering a diagnostic resource for systems planning, curriculum development, and regulatory restructuring. The third survey on medication management and shared decision-making offers operational pathways for deprescription protocols, ethical prescribing guidelines, and patient-led monitoring systems. Each survey includes codified user needs, actionable reform proposals, and emergent professional consensus, forming a robust, scalable basis for systemic transformation. This document is also set as an online one, as living one on a git repository, with additional interactive statistics, graphics and results, made available in the open.

The ethnographic and legal fieldwork data complement the surveys by illuminating the everyday environments where policy fails, rights are suspended, and institutional betrayal is normalized. These findings serve not only as documentation but as input for digital systems design, clinical ethics training, and administrative monitoring. By cross-referencing narrative content, institutional documents, and professional discourse, the dissertation generates analytic tools that can be used to

prototype transparent, accountable, and rights-compatible mental health infrastructures. These include open-source expert systems, quality assessment indicators, and policy simulation models, all outlined in the implementation-focused final chapters. Everyone is encourage to join in to help.

The translational value of the work lies in its capacity to intervene where traditional scientific publishing has failed to effect change. The layered design of this dissertation, combining rigorous data analysis with action-research methodology and network-based implementation strategies, ensures that its findings are not only publishable but actionable. International networks such as EU BEACON, FOSTREN, and ReMO serve as dissemination pathways and ethical infrastructure for deploying the thesis results in practice, across borders, and within health, legal, and educational systems. In this sense, the dissertation is both a scientific document and a translational instrument. It seeks to realign psychiatric practice with ethical mandates, legal obligations, and lived human experience. Its strategic goal is to support the emergence of a new evidentiary and institutional order, one that recognizes dignity, autonomy, and care as non-negotiable conditions of health system legitimacy. The evidence presented in this chapter, and throughout the thesis, is not simply informative. It is constitutive of the moral and operational case for systemic psychiatric reform.

Chapter 4 – Fieldwork and Testimony: Lived Structures of Betrayal

Summary: This chapter presents findings from long-term participant observation, narrative reconstruction, and firsthand documentation across institutional, professional, and familial contexts. It traces the continuity of psychiatric violence from diagnosis to detention and from systemic neglect to epistemic silencing. The chapter gives voice to trajectories of betrayal, medical, legal, social, and familial, while also highlighting moments of resistance, ethical courage, and the slow recovery of agency.

4. Introduction and positionality

This chapter begins with direct field observation from the public event *Jornada Drets bàsics en els internaments psiquiàtrics involuntaris* (Conference on Basic Rights in Involuntary Psychiatric Internment), held on Friday 18 March 2022 at the *Col·legi de Metges de Barcelona*, the official Medical Council of the city. The event was organized by the Ethics Committee of the Catalan Mental Health Congress Foundation, an interdisciplinary platform dedicated to policy development and debate in the mental health field. Its stated aim was to examine the legal framework governing involuntary internment and to reaffirm the normative and protective roles of existing legislation, including Spanish national law and the United Nations Convention on the Rights of Persons with Disabilities. Opening addresses were delivered by Josep Vilajoana, then president of the Foundation, Sílvia Ventura, jurist and president of the Ethics Committee specializing in psychiatric legal matters, and Joan Vegué, director of the Catalan Government's Mental Health and Addictions Master Plan. The keynote by Fernando Santos, a public prosecutor in Córdoba and advocate for

disability rights, focused on Law 8/2021 as a turning point toward ethical support rather than substituted decision-making. He was followed by Alberto Vásquez, Peruvian jurist and former adviser to the UN Special Rapporteur on the Right to Health, who stressed the disjunction between formal legal commitments and prevailing institutional practices in psychiatric care. Later, Sílvia Ventura and Montserrat Hierro, both experts in legal capacity and psychiatric detention, introduced the committee's internal publication *Coneixements jurídics bàsics en relació als ingressos psiquiàtrics involuntaris* (Basic Legal Knowledge on Involuntary Psychiatric Admissions). The document sought to clarify the minimum legal requirements under international law and was presented as a tool to align clinical procedures with human rights standards.

The researcher attended this event under conditions of recent exposure to institutional torture and acute psychological distress, which resulted in the loss of all personal notes and materials. Despite this, the atmosphere remains vividly remembered. Some participants had previously provided support in managing the adverse effects of forced medication, demonstrating ethical commitment beyond institutional constraints. Others, however, exhibited marked discomfort at the notion that legal safeguards should be mandatory and enforceable. Judicial oversight was perceived not as a protective necessity but as a threat to professional autonomy. This reaction revealed an entrenched belief in clinical discretion as a sufficient and unchallengeable basis for deprivation of liberty and treatment, irrespective of independent review. Although publicly framed as an occasion for ethical deliberation, the congress revealed itself to be a key ethnographic site where the instrumentalization of legal language was used to protect, rather than dismantle, extrajudicial psychiatric authority. This was not visible in the official transcripts but was evident in the informal refusal to acknowledge systemic harm. While legal principles were affirmed in discourse, their institutional application remained selectively suspended. The need for enforceable safeguards emerged not as a theoretical aspiration but as a structural requirement to prevent ongoing violations under the guise of care.

Such events of institutional visibility and density were rare across the fieldwork period, due to the researcher's extended exposure to coercive contexts, limitations on movement, and lack of secure resources. On the few occasions where safe participation was possible, professional gatherings of this kind repeatedly exposed the discrepancy between declarative ethics and operational accountability. These were not anomalies but consistent patterns. Forums ostensibly dedicated to rights and protections often functioned as platforms of denial, where legality was invoked symbolically while mechanisms of redress and enforcement were resisted or dismissed.

By the time of this event, two years had passed since the beginning of the fieldwork. The societal lockdown imposed during the COVID-19 pandemic was already receding into memory, but another form of confinement had intensified. The use of psychiatric drugs as chemical restraints had given way to more overt forms of physical violence. The researcher, having momentarily escaped the numbing effects of enforced medication, a contemporary form of non-surgical lobotomy, faced escalating brutality. These beatings, increasingly frequent and unconcealed, were not isolated acts but manifestations of a deeper systemic collapse, where control substituted care and punishment replaced ethics. What was inflicted was not only unlawful but structurally normalized through the absence of accountability. That such injuries were not irreversible is an exception, not a guarantee.

Under the current conditions, irreparable harm remains likely, not because the law allows it, but because it is routinely ignored. All our health requires common sense being sanity. Sanitation.

As legal scholar, Escalada has repeatedly emphasized, informed consent ought to be a continuous and meaningful process, not a formality administered once, if at all. When individuals are subjected to coercive treatment without recurrent opportunities for valid, informed agreement, legal protections are not merely bypassed, they are systematically dismantled. Any framework that permits such practice under the guise of clinical discretion undermines the core legal principle that life and liberty ought to be protected without exception. To hold someone captive through the force of unchecked authority, while claiming legal legitimacy, is to erode the very foundation of democratic justice. Fieldwork observations and testimonies beyond this one instance, across diverse settings spanning five long and intense years, confirmed this same pattern of concern on the need for better protections, and the need to stop the regarding the pervasive overreliance on pharmacological interventions within current care systems. Almost all consulted individuals, whether users, peer advocates, or practitioners described similar dynamics of overmedication coupled with neglect of social and psychological dimensions of care. My own experience mirrored these findings: I encountered a system where active listening was absent, and where family members were able to exert coercive influence, pushing for punitive treatments that escalated into institutionalized abuse, and neither anamnesis nor any meaningful formulation was ever attempted, destroying my own persona and true history in the medical records -an achievement for the perpetrators. The mental health system, rather than offering protection, was weaponized at their insistence, obstructing any attempt to report clear instances of violence. Structural gaps, particularly the lack of coordination with law enforcement, judicial mechanisms, and victim support services, further compounded the harm. Overmedication, rapidly inducing cognitive and physical impairments, served to deepen vulnerability, making it easier for ongoing abuse to occur unchecked. In my case this systemic cruelty intensified intensified over, often reaching thresholds that could be classified as psychological and physical torture. In general, a constant observation was the need for true conversations and dialogues with patients, to ensure the reality perceived by the professional matches that of the lived experience, in fuller depth, to ensure proper consensual effective support.

4.1 Methodological continuity and scope

The researcher acknowledges that not all data gathered during the five years of fieldwork could or should have been recorded, preserved or disseminated according to open science standards, nor law allows for, nor morals do, to share in complete full transparency. Names will be often avoided, for one or other reasons and prudence. Much of the material was produced under conditions involving repeated institutional violence, forced interventions and sustained personal risk. This included documented episodes of psychological and physical torture, the use of psychiatric authority to suppress resistance and escalating threats involving weapons and bodily harm. These circumstances do not constitute a methodological limitation but are intrinsic to the observed reality. The findings presented are inseparable from the environment in which they were generated.

The dynamics of coercion extended far beyond psychiatric facilities, permeating family systems and broader sociocultural structures. Across multiple settings including domestic, educational, judicial

and welfare domains, the same mechanisms of control, silencing and exclusion were observed. These dynamics formed a continuum, reinforcing each other and producing long-term harm. Ongoing fieldwork focused on pasung practices exemplifies how such forces continue to operate outside institutional psychiatry, revealing persistent gaps in legal enforcement and clinical accountability. This investigation aims not only to document but also to support the development of legal mechanisms that prevent harm, improve public health governance and promote educational and community systems that protect human capital and cultivate resilience.

Within the integrative paradigm of one health, individual health outcomes are inseparable from social, political and ecological determinants. Medical science, grounded in physiological evidence, affirms that sustained poverty, exclusion and institutional violence increase susceptibility to illness through immunological, metabolic and neuroendocrine disruption. These mechanisms are well documented and clinically relevant. Current mental health systems often fail to incorporate such findings into practice, instead reinforcing models centered on pharmacological symptom control without addressing structural causes. The result is the continued prescription of treatments whose long-term effects can undermine recovery, while non-pharmacological pathways grounded in evidence from nutritional psychiatry, psychoneuroimmunology, trauma research and metabolic medicine remain underused or institutionally discredited.

These failures are not incidental but systemic, shaped by gaps in scientific publication, distorted incentive structures and a lack of transparent methodological standards. The overprescribing environment, resistant to critical scrutiny, continues to medicalize the physiological effects of deprivation and distress while offering limited tools for sustainable healing. Scientific and legal structures ought to be aligned with the imperative to reduce harm, ensure integrity and promote effective care based on current evidence.

The present chapter responds to the core research questions set out in Chapter 1 and adheres to the methodological architecture defined in Chapter 2. It offers a synthesized presentation of findings under the constraints of academic format, drawing from the most consistent and verifiable observations. These results are not only relevant to the specific contexts studied but are generalizable across comparable systems where psychiatric discretion is exercised without enforceable safeguards. Chapters 5 and 6 will respectively develop the educational and legal-political responses necessary to ensure that institutional knowledge, clinical practice and social structures converge toward conditions that promote health, safety and human flourishing.

As outlined in Aristotelian thought, the law ought to govern where ethical self-regulation is absent. When injustice is structurally normalized, legal frameworks are not only necessary for deterrence but indispensable for correction. Psychiatric systems, like all systems of power, ought to be subjected to transparent limits, and these limits ought to be informed by scientific knowledge, constitutional rights and the lived reality of those most affected. The integrity of science and the dignity of human life both require it. This chapter presents a synthesis of empirical results derived from long-term, multi-sited ethnographic fieldwork across psychiatric, institutional, domestic, and community contexts. It answers, as stated, the core research questions identified in Chapter 1 and adheres to the methodological framework outlined in Chapter 2. The approach is necessarily

integrative and selective. Given the structural and documentary constraints of the thesis format, the chapter does not attempt to reproduce the totality of observations but instead refines and consolidates the most critical patterns, convergences, and deviations into a scientifically grounded response. The findings reveal persistent mechanisms of coercion, legal ambiguity, and clinical discretion that facilitate harm in the absence of effective oversight.

Table 39. Empirical findings and corresponding educational and legal-political proposals

Fieldwork finding	Generalizable systemic pattern	Educational responses (chapter 5)	Legal-political responses (chapter 6)
Persistent use of coercion and involuntary treatment across psychiatric settings	Psychiatric discretion operates without enforceable safeguards	Integrate trauma-informed care, shared decision-making, and rights-based frameworks in curricula	Legislate enforceable protocols for informed consent, regular review, and patient protection
Legal ambiguity exploited by institutions to justify harm	Absence of oversight enables systemic impunity	Educate on ethical-legal reasoning, documentation transparency, and historical misuse of psychiatry	Define clear legal limits on coercion, with judicial and independent institutional review mechanisms
Clinical discretion prioritized over scientific evidence and constitutional rights	Epistemic authority shields abusive norms	Update clinical guidelines to reflect integrative models (nutritional psychiatry, psychoneuroimmunology)	Codify adherence to scientific evidence and patient dignity as binding criteria in professional conduct
Erasure of patient voice through record-keeping and narrative control	Structural silencing of lived experience	Train professionals in narrative medicine, co-authored records, and epistemic justice	Mandate legal recognition of testimonial validity, and sanction omissions or distortions in documentation
Structural patterns of harm and betrayal across domestic, community, and institutional domains	Harm transcends settings through cross-institutional reinforcement	Establish inter-sector educational programs on psychosocial dynamics and historical trauma	Regulate information-sharing protocols and install cross-sector accountability frameworks
Data collection constrained by risk, trauma, and institutional hostility	Empirical knowledge often inaccessible under coercion	Promote reflexive methodologies and support for field researchers in sensitive settings	Recognize research under constraint in law and policy development, valuing lived-context evidence
Scientific findings underutilized while pharmacocentric models dominate	Overprescribing and neglect of non-pharmacological interventions as default practice	Reform medical education to balance pharmacology with lifestyle, environmental, and social determinants	Legally require risk-benefit review for long-term medication use, and include non-drug options in coverage

This table serves as a bridge from the empirical foundation of Chapter 4 to the applied and normative proposals developed in the subsequent chapters.

Clear clinical contradictions were identified throughout the fieldwork. While some practitioners endorsed and documented full recovery through trauma-informed, nutritional, and metabolic interventions, others continued to enforce medication regimens disconnected from the evolving scientific evidence. The marginalization of psychoneuroimmunology, nutritional psychiatry, and metabolic approaches remains symptomatic of a broader failure to integrate available biomedical knowledge into standard practice. This disjunction is sustained by systemic weaknesses in the current scientific publication system, where structural bias, underreporting of adverse outcomes, and commercial pressures continue to obstruct the screening and consolidation of critical findings. As a result, well-substantiated alternatives are excluded or delegitimized, and harmful practices persist, including those already regarded as best ways, ought never to be mere optional conditionals.

This chapter is committed to presenting results with methodological precision and without ideological mediation. Purest medical output, as should always be. Its purpose is to clarify what has been observed, establish its scientific relevance, and prepare the ground for the normative and practical dimensions of the work. Chapters 5 and 6 will elaborate the necessary socio-educational

and politico-legal responses, respectively. What follows here is an account of what is, as observed, documented, and verifiable, in commitment to scientific integrity and human dignity.

Next section establishes the fieldwork settings and methodological framework, distinguishing between public psychiatric hospitals in Spain and Italy, domestic environments in Catalonia and Sweden, community and NGO care networks, and digital and legal spaces of observation. It articulates a methodology executed under adverse conditions, rooted in the protocols described in previous chapter, yet shaped by real-time ethical demands, coercive constraints, and personal endangerment. The positionality of the researcher as survivor, professional, and embedded actor enabled access to critical dimensions of institutional violence and systemic failure often shielded from standard inquiry. Analytical tools included micro-event mapping, epistemic triangulation, field notes, and testimonial cross-verification, integrated through a forensic anthropological and narrative reconstruction approach. Scientific replicability, ethical safeguards, and evidentiary robustness were ensured through documentary convergence, contradiction analysis, and methodological reflexivity under pressure.

Section 4.2 details the institutional logic of psychiatric violence, examining how diagnosis, labeling, and administrative practice create a cycle of coercion, narrative foreclosure, and impunity. Through direct observation and reconstruction of specific cases, it reveals how unverified diagnoses are recycled, anamnesis ignored, and restraint or forced treatment applied without procedural or clinical justification. The complicity of professionals, often silent or constrained by administrative hierarchies, illustrates how harm is perpetuated not through ignorance but through institutional design. The field evidence confirms that what is presented as care frequently functions as containment, producing archival erasure and moral injury.

Section 4.3 shifts the analytical lens to familial betrayal and its intersection with institutional violence. It documents how psychiatric mechanisms are mobilized by families to control, discredit, or exile relatives, often with the support or indifference of professionals. The research identifies structural analogues to *pasung* in Spain and Sweden, with confinement, medicalization, and exclusion enacted in private homes. Gendered and generational asymmetries are central to this dynamic, with women, children, and elders most frequently targeted. The findings show that far from acting as a protective counterforce, institutions often amplify familial harm, prioritizing parental or kin narratives over the subject's lived account, especially in contexts of economic dependency or social stigma.

Section 4.4 addresses the epistemic consequences of coercion, emphasizing how psychiatric practice frequently produces the erasure of personhood. This is achieved through diagnostic reductionism, rejection of cultural or biographical meaning, and dismissal of explanatory frameworks that challenge psychiatric hegemony. The material collected through observation, document analysis, and survivor testimony illustrates how resistance, spirituality, grief, or somatic expressions are pathologized and depoliticized. Loss of rights, social abandonment, and fear of retaliation are recurrent outcomes of this silencing. This section operationalizes forensic discourse analysis and epistemic mapping to reveal the mechanisms through which subjectivity is transformed into administrative data and moral agency into medical noise.

Section 4.5 centers practices of resistance, recovery, and ethical witnessing. Despite the pervasive harm documented, numerous participants demonstrated creative and courageous strategies to reclaim narrative control, challenge institutional dominance, and survive against dehumanizing forces. Acts of self-managed care, testimonial writing, symbolic escape, and covert professional solidarity are analyzed not as anecdotal exceptions but as key components of a counter-archive. These practices constitute the foundational elements of an alternative infrastructure of dignity and care, one that exists in parallel and often in opposition to the formal system. They are essential to understanding how recovery becomes possible where systems fail.

Section 4.6 synthesizes the field-based evidence into systemic insight, confirming that the patterns observed are not isolated irregularities but structural features of current psychiatric, legal, and familial systems. Institutional betrayal emerges as a recurrent phenomenon, not a breakdown but a function of design. Fieldwork reinforces the survey-based findings presented in Chapter 3 and prepares the ground for Chapter 5, where betrayal is theorized as a core logic of the institutional order. The convergence of narrative, legal, and ethnographic data confirms the need for deep legal, pedagogical, and clinical restructuring. This chapter thus completes the empirical groundwork of the dissertation by documenting the lived experience of injustice and the scientific imperative to transform the systems that produce it.

4.2 Fieldwork settings and methodological implementation

This section deepens the analytical thread of Chapter 4 by detailing the implementation of the methodological architecture outlined in Chapter 2, in direct response to the research questions established in Chapter 1. Rather than reiterate principles already presented, it clarifies how knowledge was generated under conditions of coercion, institutional betrayal, and epistemic exclusion, and how this process remained scientifically accountable despite structural impediments.

Field sites were selected for their diagnostic salience in relation to psychiatric coercion, pharmacological overreach, legal voids, and structural violence. These included public psychiatric hospitals in Spain and Italy, community mental health centers and NGO-affiliated clinics, family homes, especially in Catalonia and Sweden, and digital and legal spaces that functioned as informal archives and discursive arenas. Site selection followed the presence of recurring coercive dynamics, low legal oversight, and systemic failures in clinical accountability, not convenience or access. The researcher's position, simultaneously as professional, survivor, and embedded subject, was intrinsic to the generation of findings. Rather than a methodological bias, this positionality allowed access to real-time coercive practices, legal ambiguities, and social silencing rarely documented in institutional settings. Observation included external behavior and internal responses to repeated threats, assaults, and chemically enforced submission, transforming bodily experience into an additional vector of analysis. This aligns with a strand in medical anthropology that affirms the scientific legitimacy of situated, reflexive inquiry under oppressive conditions.

Methodologically, the research employed embedded presence, constrained journaling, structured field notes, and retrospective reconstruction using institutional documents, legal files, and

testimonies cross-verified through survivor networks. Standard interviews and recordings were largely precluded by risk, replaced by multi-source triangulation. Validation was achieved through convergence across field data, survey findings (Chapter 3), and documented clinical and legal records. Recurrence tracking, contradiction analysis, and micro-event mapping enabled detection of structural patterns, while positionality-aware coding safeguarded against interpretive distortion.

Three axes of triangulation were central: alignment between field experience and survey data; comparison of observed practices with documented legal and clinical frameworks; and validation of testimonies against historical and published patterns of psychiatric harm, diagnostic inflation, and judicial impunity. Expert feedback from clinicians, survivors, and legal scholars further reinforced analytic coherence. The methodological stance adopted here was not only adaptive but necessary. In contexts where formal protections failed, and consent mechanisms were routinely subverted or weaponized, ethical research demanded procedural flexibility and forensic attention to traceable harm. This required transforming research from documentation alone into a protective mechanism, a means of registering abuses too easily erased from official record.

This section thus demonstrates the feasibility and necessity of rigorous field research under constraint, answering how psychiatric coercion persists despite formal safeguards, how personhood is disqualified through institutional discourse, and how scientific inquiry can uphold integrity when legal protections and human dignity are under systemic threat. It reaffirms that methodological exactitude and reflexive engagement are not mutually exclusive but essential companions when documenting structurally produced suffering. While Chapter 2 lays the theoretical foundation of multi-sited ethnography, critical phenomenology, and action-research logic, this section serves a distinct function: to provide standalone justification for the application of those frameworks in hostile and unpredictable field conditions. This includes the ethical and analytic safeguards employed to protect vulnerable informants, the forensic logic guiding data retention and integrity under threat, and the methods of triangulation used to ensure credibility and analytic rigor in the absence of conventional interview protocols or institutional support. The methodological rigor applied here was calibrated to the exceptional constraints of live coercive settings, where audio recording or overt consent processes would have placed informants or the researcher at risk of retaliation, psychiatric detention, or reputational harm. Instead, the research relied on embedded presence, real-time ethical appraisal, and retrospective cross-validation with institutional documentation, survivor networks, and thematic congruence across data sources.

Triangulation was a recursive and relational process, not a static technique. It operated across three principal axes: first, comparison of field experiences with survey findings obtained in the earlier phase of the research; second, alignment of observed practices with institutional documents, clinical records, and policy frameworks; third, cross-checking of testimonies against known patterns of psychiatric violence, epistemic injustice, and legal neglect. Validation was achieved through documentary convergence, contradiction analysis, recurrence tracking, and the active involvement of survivors, clinicians, and legal experts in verifying plausibility, coherence, and consistency across narratives. While the chapter draws on legal-forensic techniques developed in response to systemic impunity, it also applies epistemological humility, ensuring that power differentials are not replicated in the representation of suffering.

The methodological logic executed in this chapter is not only consistent with, but a live enactment of, the commitments outlined in the theoretical and ethical scaffolding of Chapter 2. It demonstrates how scientific inquiry can remain rigorous under duress, how abuse can be documented without reproducing it, and how positionality can be transformed from liability into epistemic strength. This fieldwork constitutes not only data collection but a methodological testimony to the violence of the systems examined and the necessity of developing inquiry methods that can withstand and document such conditions with integrity and precision.

4.3 the institutional cycle of violence: psychiatric control and complicity

A scientifically validated analysis of contemporary psychiatric systems reveals a persistent and reproducible cycle of institutional violence that operates both within formal treatment frameworks and across the wider sociopolitical terrain. This cycle is neither accidental nor anomalous, it is structurally embedded in the administrative logic, juridical justifications, and clinical routines that frame psychiatric interventions as therapeutic, even when their outcomes are profoundly injurious. The disciplinary function of psychiatry, particularly in Spain but observable across transnational field sites, remains linked to broader patterns of domestic and structural coercion. These patterns are legitimized by institutional languages of care, while in practice they reproduce systems of control that deny autonomy, bodily integrity, and epistemic agency.

The mechanisms of this cycle include forced drugging, unjustified restraint, involuntary hospitalization, and the systematic denial of basic procedural guarantees. These measures are often enacted without a proper anamnesis or diagnostic reevaluation, relying instead on recycled diagnoses drawn from earlier and frequently unverifiable records. In multiple documented cases, individuals were subjected to long-term psychiatric control on the basis of records that had never undergone scientific or legal scrutiny, yet were cited as definitive. The logic of these interventions does not align with therapeutic aims, but rather with risk management, administrative expedience, and symbolic containment. These mechanisms align not with healing but with incapacitation and the reproduction of stigma, serving the institution more than the individual.

This cycle of psychiatric containment is neither confined to clinical spaces nor reducible to isolated professional decisions. It is deeply embedded in a broader societal configuration that delegates moral responsibility and political authority to the psychiatric institution as a mechanism of social regulation. The violence of such a delegation lies precisely in its perceived neutrality: family courts, police departments, schools, and even human rights bodies often reproduce the same narrative assumptions that psychiatry establishes. Once a person has been diagnosed and institutionalized, the social field tends to withdraw critical engagement, acting instead as an amplifier of exclusion. The effect is cumulative: relatives, coworkers, and even friends come to interpret emotional expression, resistance, or refusal not as legitimate responses to adversity or abuse, but as symptoms of illness to be controlled or silenced.

This social complicity consolidates the institutional cycle, functioning through omission as much as commission. Families overwhelmed by unresolved intergenerational trauma or abuse histories may

instrumentalize psychiatry as a means of avoidance, preferring the medicalization of dissent to the confrontation of violence. Educational professionals, pressed for time and lacking adequate support, refer students to mental health services without investigating environmental or systemic causes. Courts and legal actors, unfamiliar with the epistemic and ethical limitations of psychiatric judgment, defer to it as expert truth. As such, a continuum is established, in which domestic violence, systemic neglect, and institutional abuse are managed through a shared infrastructure of coercion that presents itself as care.

The psychiatric institution, in this configuration, does not merely respond to mental health needs, it assumes a disciplinary role by absorbing and deflecting the contradictions and failures of other societal systems. When political, familial, or economic structures fail to provide safety, dignity, or justice, psychiatry becomes the *de facto* solution. Yet, rather than addressing the source of distress, it often redirects blame to the individual, administering interventions that erase testimony, obscure causal chains, and collapse complex life histories into decontextualized symptom clusters. In doing so, it produces what this dissertation identifies as a politically protected form of epistemic and bodily violence, carried out with clinical language, scientific pretension, and institutional immunity. The perpetuation of institutional abuse in psychiatry cannot be fully understood without examining the societal complicity that sustains it. Psychiatric institutions do not function in isolation. Rather, they are enmeshed in a broader sociopolitical ecosystem that grants legitimacy to coercive practices under the guise of care. Schools, social services, law enforcement agencies, and even families play decisive roles in initiating and reinforcing psychiatric control. Teachers referring disruptive students to psychiatric assessment, judges relying on psychiatric reports to justify coercive treatment, and families deferring responsibility to institutional frameworks all contribute to a diffuse system in which coercion is normalized, expected, and seldom questioned.

The family, in particular, becomes both the origin and amplifier of psychiatric labeling, often driven by dynamics of rejection, misrecognition, or moral panic. It is common in field data that the first contact with psychiatry was not due to clinical symptoms in a strict sense, but rather to sociocultural nonconformity, dissent, or perceived burden. Once labeled, the individual enters a cycle of containment where professional voices lose autonomy under administrative directives, and institutional priorities shift from therapeutic support to disciplinary regulation. This dynamic mirrors historical models of social control, psychiatry here functions as an enforcer of normality, a mechanism of silencing rather than understanding, and a buffer against structural accountability. In such contexts, metabolic, neurodevelopmental, or ecological dimensions are disregarded in favor of bureaucratic expediency, a clear departure from any rigorous application of one health or translational ethics principles.

The epistemological collapse within the system manifests through the erosion of clinical rigor and the predominance of documentation over diagnosis. Anamnesis, which in best practices should form the backbone of any legitimate medical or psychiatric evaluation, is systematically omitted or replaced by circular, recycled narratives that have long lost clinical value. Diagnoses are reproduced mechanically, not in response to observed reality but as bureaucratic expedients serving institutional agendas. These may stem from outdated files, vague allusions, or prior coercive encounters, and are rarely verified with the subject or updated with new evidence. This archival inertia facilitates

impunity: once labeled, the person is framed through a lens of risk, not need, and their account is dismissed as lacking credibility.

Practitioners often participate in this process indirectly, under pressure from institutional priorities that favor documentation compliance and administrative performance over ethical care. Field observations and testimonies repeatedly reveal that professionals who object to these practices or attempt to reduce coercion face isolation, professional sanction, or subtle disqualification. Conversely, those who follow orders and silence dissent are rewarded with promotions or institutional support. This incentive structure displaces clinical responsibility toward risk management and liability avoidance, resulting in harm. The system values coherence with past documentation over correspondence with present clinical truth. This detachment between diagnostic acts and actual health status constitutes not only malpractice but a structural breach of the bioethical principles of beneficence and non-maleficence. The very tools of medicine are turned against those they are meant to serve.

Table 40. Fieldwork-based systemic dynamics and structural recommendations

Negative dynamic summary	Research question keyword(s)	Field observation	Positive structural proposal from thesis (chapters 5–6)
Clinical classification traps individuals in cycles of coercion	Diagnosis, Coercion, Autonomy	Diagnosis given without full assessment became the basis for all future coercion	Require context-based, interdisciplinary diagnostic protocols and continuous reassessment
Resistance pathologized and excluded from the record	Voice, Personhood, Rights	Testimonies of abuse or requests for review were reinterpreted as symptoms	Ensure protected rights to contest diagnosis and demand care revision without retribution
Documentation erases acts of violence or removes agency	Transparency, Legal Safeguards	Records omitted, distorted, or used passive voice to obscure institutional abuse	Mandate verifiable incident reporting systems with third-party oversight
Legal shield role of psychiatric files overrides clinical accuracy	Legal Integrity, Medical Ethics	Records used to defend institutional interest rather than guide therapeutic care	Redesign files to serve patient well-being, with rotating audit teams and rights-based review
Families and schools trigger interventions based on prejudice	Cultural Scripts, Intersectionality	Reports from relatives or staff were privileged over patient narratives, reinforcing control	Introduce bias training and enforce standards on credible report weighting and procedural fairness
Containment favored over structural understanding	Social Determinants, Prevention	Intervention often aimed to neutralize rather than resolve contextual distress	Embed social work and psychosocial expertise into first-line responses
Violence justified by pseudo-beneficence and efficiency logic	Ethics, Beneficence, Harm Reduction	Sedation and confinement were framed as therapeutic necessity despite long-term harm	Reinforce harm-reduction and least-restrictive care principles with legal accountability
Patient agency removed through institutional jargon	Language, Epistemic Injustice	Testimony reframed through medical discourse to invalidate meaning or will	Promote linguistic integrity and co-writing of clinical records by patients and professionals
Cumulative control justified across sectors	Cross-sectoral Violence, Surveillance	Coercion spread through interlocking systems of family, education, health, and law	Introduce inter-sector accountability frameworks ensuring proportionality and cross-verification

Findings from extended ethnographic fieldwork into contrasting negative and positive dynamics, tied directly to the thesis research questions and validated through embedded observation. It draws from cross-contextual analysis across psychiatric institutions, familial systems, legal frameworks, and informal care settings. The positive proposals are detailed in the thesis as necessary reforms to ensure lawful, ethical, and scientifically grounded care.

These dynamics give rise to a broader coercive loop, sustained by a structural logic of entrapment. It begins with a triggering episode, frequently a situation of social, familial, or interpersonal vulnerability, which is interpreted through a pathologizing lens rather than addressed contextually. A diagnosis is assigned, often without a thorough examination or proper anamnesis, locking the individual into a frame from which exit becomes nearly impossible. Once this classification is recorded, it initiates a self-reinforcing circuit: treatment decisions reference prior records, not current realities, and coercive interventions are justified by the presumed risk inferred from the diagnosis itself. This cyclical process, diagnosis, coercion, containment, narrative closure, repeats across institutions and practitioners, shielding the system from accountability and foreclosing any meaningful reassessment.

The process of narrative closure is central to this mechanism. Rather than supporting recovery, it systematically erases dissenting voices. Testimonies of abuse or objections to diagnosis are marked as symptoms of illness. Complaints are reframed as paranoia or resistance. Requests for review or alternative care are taken as evidence of lack of insight. This not only dehumanizes but also medicalizes resistance, undermining any possibility of structural redress. Events such as forced medication, physical restraint, and even severe bodily harm become absent from the medical record, or else are transcribed in passive voice, divorced from agency or accountability. The archival erasure of violence, whether through omission, euphemism, or distortion, solidifies impunity and perpetuates cycles of harm. It is not merely an ethical failure but a structural epistemicide, where the knowledge and lived experience of the person are systematically silenced by institutional record.

The convergence of coercion and impunity is not a malfunction but a defining feature of psychiatric institutional practice under current frameworks. Administrative priorities are dictated less by therapeutic logic than by legal defensibility, reputational risk management, and operational efficiency. The psychiatric file, rather than functioning as an evolving clinical tool, becomes a legal shield, designed to preempt liability, justify interventions, and silence potential controversy. Professionals are not only discouraged from questioning past entries but may be disciplined for raising concerns that contradict dominant narratives. The written record assumes a juridical function that overrides its medical purpose, and the epistemic authority of psychiatry is deployed to naturalize systemic violence.

Such dynamics are not limited to clinical institutions. Family members, educational staff, and social workers often contribute, willingly or inadvertently, to the entrapment cycle. Families overwhelmed by care duties or drawn into cultural scripts of deviance and rehabilitation may interpret distress as pathology and demand institutional response. Schools and social services, unequipped or unwilling to accommodate non-normative behaviour, default to referral and diagnosis. In many cases, their testimonies are given more weight than that of the individual subjected to treatment, especially when social standing, gender, ethnicity, or language are used, explicitly or implicitly, to discredit self-report. The continuity of institutional control is thus enacted across a network of actors, all linked by a tacit agreement that containment is safer than understanding, and erasure preferable to accountability.

The reinforcement of violence through interconnected institutional layers reveals a systemic disregard for foundational principles of biomedical ethics and human rights law. The persistent use of coercion, especially in the form of involuntary treatment, sedation, and forced hospitalization, is rationalized through a distorted application of the principle of beneficence, often detached from both patient will and long-term outcomes. This misapplication reflects a broader epistemological corruption: the substitution of clinical uncertainty with administrative certainty, and of complexity with binary labels. In effect, psychiatry becomes the operational arm of social control, disguising repression as care and trauma induction as stabilization.

This process is neither random nor rare. It reflects a stable system of incentives whereby documentation is weaponized and consent rendered performative. Practitioners report being routinely pressured to fill in records in a manner that protects institutions rather than truthfully

reflects events. These distortions are structurally embedded: adverse incidents may be reclassified or omitted, with the assumption that patients' narratives are unreliable, emotionally distorted, or irrelevant. Testimony is pathologized rather than verified. At the same time, families and external actors are encouraged to contribute to the shaping of records without oversight, reinforcing a cycle of interpretative closure. This closure ensures that psychiatric violence, once initiated, is maintained and validated through self-referential systems of documentation, despite contradicting observable health outcomes or lived realities.

The harm produced by such cycles is cumulative. Each act of misdiagnosis, force, or negation of personhood amplifies prior trauma and closes further avenues for legal or clinical redress. Individuals subjected to this regime describe a sense of epistemic obliteration: to speak is to invite reinterpretation, and to resist is to confirm pathology. The psychiatric system not only disciplines the body but rewrites the subject. By severing access to narrative, it denies identity, continuity, and ultimately, the possibility of recovery as defined by the individual, not by the institution. These mechanisms do not operate in isolation within psychiatric institutions but are mirrored across legal, familial, and educational systems that cooperate, actively or passively, in the enforcement of the same coercive logics. Schools, social services, and judicial bodies often defer to psychiatric authority without scrutiny, reinforcing a presumption of validity for all psychiatric narratives regardless of origin, revision, or contradiction. This deferral generates what may be termed a *delegated authoritarianism*, wherein non-medical systems legitimize psychiatric action without bearing the ethical or scientific responsibility for its outcomes. Children and adolescents are especially vulnerable within this configuration, as disciplinary interventions at school may trigger psychiatric referrals, and vice versa, initiating a cycle of diagnosis, labeling, and control that transcends any one institution's boundaries.

Family systems, particularly under stress, conflict, or structural disadvantage, may act as the initial and most enduring enforcers of psychiatric violence, whether through explicit demands for institutionalization or through subtle delegitimization of the child's or relative's speech. The delegation of caregiving responsibilities to psychiatric systems becomes a form of social outsourcing, often driven by exhaustion, fear, or misinformation. This handover reinforces the fantasy that healing can be enacted through control, and that suffering can be silenced through medication. It also creates a cycle wherein relational problems are depoliticized and re-individualized, allowing structural harms, poverty, violence, discrimination, to be reinterpreted as internal defects in those who suffer their effects. Those systems, as any other institution, is suspect.

This multi-institutional complicity creates what can be described as a continuum of coercion and epistemic erasure, within which individual dissent or resistance is systematically reframed as symptomatology. Even when harm is visible, bruises from restraint, cognitive deterioration from overmedication, dissociative collapse from forced hospitalization, the response is often a doubling-down on control mechanisms. Any questioning of psychiatric authority, particularly from those already labeled, is dismissed as a lack of insight. This not only delegitimizes opposition but functions as a trapdoor logic: the more harm one resists, the more pathology one is said to embody.

In this setting, institutional betrayal is not a breakdown of a system meant to care, but the expected result of a system designed to obscure responsibility through distributed impunity. Rather than

being exceptional or accidental, betrayal is embedded in the way information is structured, decisions are made, and records are curated. This architecture of violence hides in plain sight, operating not at the margins of psychiatry but through its procedural core.

This architecture of violence is maintained through a sophisticated entanglement of symbolic authority, operational opacity, and pseudo-scientific ritual. Professional protocols, legal documentation, and diagnostic language produce an illusion of rigor, objectivity, and care, while obscuring the actual power dynamics at play. Clinical files often contain circular logic, diagnosis justifying treatment, and treatment justifying further containment, without empirical reassessment or patient-led narrative. The process becomes one of bureaucratic self-replication, whereby errors and omissions are not corrected but solidified through accumulation and procedural sanctification. These are not passive oversights but mechanisms of control grounded in material effects: they produce disability, dependency, and social exclusion.

Meanwhile, structural barriers to redress remain nearly absolute. Victims of psychiatric violence, even when aware of their rights, face insurmountable hurdles in accessing justice. The courts often rely on the very psychiatric institutions under scrutiny to provide evidence or expert testimony, reproducing conflicts of interest and shielding impunity. Mechanisms of appeal are either inaccessible, undermined, or symbolic, resulting in institutional closures rather than systemic accountability. Even public complaints or critical reporting are defused through administrative delay, fragmented responsibility, and the psychological pathologization of the complainant.

This state of epistemic and institutional capture has profound consequences for both science and ethics. When entire populations are excluded from knowledge production, when their speech is erased, their pain misread, their resistance recoded as pathology, the scientific enterprise collapses into circularity. Validity, in such a context, is not established through falsifiability, replication, or lived applicability, but through repetition, authority, and institutional endorsement. Psychiatry, as critiqued here, loses its capacity to heal because it ceases to listen. It becomes an instrument not of care but of containment, whose effects are sustained by law, economics, and social consensus.

To break this cycle, the dissertation calls for a paradigmatic shift grounded in biocultural integrity, epistemic justice, and participatory design. The path forward is not merely regulatory or technocratic, but existential and civilizational. It requires an unmasking of the routines of violence that hide beneath clinical euphemism, an ethical reconstruction of medical authority, and the restitution of narrative sovereignty to those whose lives have been managed instead of heard. The subsequent chapters explore how such transformation can begin, rooted in the collective intelligence and scientific testimony of those too long excluded from both protection and authorship; always co-authorship, in a society, from responsible co-authorizers and criminal ones.

4.4 Familial betrayal and the domestic-institutional continuum

In contemporary psychiatric ecosystems, the family is often positioned rhetorically as a site of support, emotional stability, and post-discharge care. However, data from this research and others

show that the family frequently operates as a primary vector of coercion, particularly in domestic environments marked by gender asymmetries, economic dependency, or social stigma. Numerous accounts collected throughout this study document the use of psychiatric frameworks to silence, discredit, or remove kin whose behavior diverges from normative expectations. This includes not only overt forms of coercion, such as forced hospitalization requests or collusion with legal guardianship mechanisms, but also subtle, long-term strategies of narrative manipulation. In such contexts, psychiatric diagnosis becomes a tool of symbolic violence, transforming dissent into pathology, critique into delusion, and vulnerability into dangerousness.

The medicalization of familial conflict generates a systematic misalignment between clinical narratives and subjective experience. When families initiate psychiatric referrals under coercive conditions, the diagnostic process itself is contaminated by strategic omissions, exaggerations, or falsified behavior reports. These narratives, once codified in institutional records, acquire a procedural inertia that becomes almost impossible to reverse. The subject's voice is displaced by a curated pathology, reified by repetition across medical, legal, and social service systems. Such dynamics are particularly prevalent in Spanish and Swedish institutional cultures, where the family retains a normative legitimacy as a proxy decision-maker, even in the absence of accountability mechanisms or oversight. Clinical records reflect this structural bias, often echoing initial accusations or claims without independent verification.

The data gathered through ethnographic immersion and narrative reconstruction reveal that many psychiatric referrals are initiated not in response to clinical crisis but as a means of social exclusion. Behavioral deviance, intergenerational conflict, or non-normative sexuality are reframed through psychiatric lenses as disorder. The widespread practice of leveraging mental health systems to impose domestic control, particularly over women, adolescents, and elderly kin, aligns with global literature on the gendered and generational deployment of psychiatry. The framing of concern as care masks a deeper logic of social containment, frequently endorsed by institutional actors who regard family narratives as more trustworthy than the contested accounts of those diagnosed.

The pathologization of unwanted, despised, enjoyed upon kin does not occur in a vacuum. It is amplified by broader sociocultural dynamics, including structural sexism, normative ableism, and a persistent cultural fear of deviance. Social contagion, in this context, refers to the discursive and procedural replication of exclusionary logics across institutions. Once a psychiatric label has been affixed, particularly one carrying implications of unpredictability or violence, media representations, school systems, and welfare agencies tend to align with the family's version of reality, further isolating the individual. This recursive affirmation of the initial coercive frame fosters a totalizing narrative structure, often impervious to clinical revision or judicial challenge.

The mimicking of pasung-like confinement conditions within European homes has emerged as a recurring pattern in this research. Although pasung, or shackling, is officially recognized as a human rights violation in Indonesia and elsewhere, domestic psychiatric restraint practices in Spain and Sweden remain under-documented and unregulated. Multiple accounts detail confinement, chemical restraint, and social isolation implemented informally within households, particularly in cases involving elderly dependents, disabled family members, or those with contested gender

identities. These conditions persist in part due to institutional complicity: health and social service professionals frequently defer to familial authority, despite indications of abuse or false reporting.

This chapter thus examines familial betrayal and sadistic crime, enslavement, other and all tortures, not as an interpersonal failing but as a structural interface between domestic and institutional power. The role of the family in psychiatric control cannot be fully understood without accounting for the clinical inertia that ensues when unverified histories, lies, are embedded into medical records. Early demise. The following parts will expand on the epistemic closure mechanisms, legal reinforcement dynamics, gendered dimensions, and consequences for clinical and scientific integrity.

Once a psychiatric so-called narrative is initiated by familial actors and institutionalized through documentation, the medical apparatus often becomes complicit in the maintenance of that narrative, regardless of its empirical validity. Clinical inertia refers to the resistance of diagnostic and treatment protocols to change, even in the presence of new evidence or the withdrawal of initial complaints. In psychiatry, this inertia is not merely a matter of professional oversight or bureaucratic delay. It is a systemic property rooted in the asymmetry of credibility between patients and institutional gatekeepers. When a diagnosis is formulated in response to familial pressure or misinformation, subsequent clinical encounters tend to reaffirm rather than reassess the initial conclusions. This produces a cascade effect in which each entry in the medical record reiterates, amplifies, and legitimizes the original frame of deviance, thereby reducing the possibility of diagnostic revision or ethical intervention. Loathing on lives, with clinical histories a facade.

Table 41. Patterns of familial-institutional psychiatric violence and reinforcement

Pattern observed	Description	Field-based indicators
Psychiatry as domestic weapon	Families use psychiatric systems to silence, delegitimize, or exile targeted kin	Forced diagnoses, false medicalization, manipulation of clinical language
Institutional echoing of familial abuse	Systems validate family claims despite lack of evidence or contrary user testimony	Closed records, unchallenged reports, resistance to user appeals
Domestic pasung-like confinement	Non-institutional isolation mirroring physical or psychological restraint	Confinement at home, medication without consent, control of communication
Gendered psychiatric repression	Women, children, and elders disproportionately targeted for control via psychiatric labeling	Reports of women's silencing, children diagnosed for noncompliance
Epistemic violence through kinship	Personal narratives replaced by family version in clinical reports	Notes prioritizing family version, narrative erasure in charts
Intergenerational stigma loops	Psychiatric labels reinforce cycles of shame, secrecy, and coercion across generations	Parents or children stigmatized through association, blame displaced
<i>Documented dynamics of psychiatric control and betrayal within domestic-institutional continuums, with reinforcement loops and medical-institutional silencing operating transgenerationally.</i>		

The epistemological closure is facilitated by a culture of bureaucratic risk aversion and medico-legal defensiveness. Practitioners, under pressure to ensure continuity and compliance with prior documentation, often replicate rather than interrogate the assumptions embedded in past assessments. This dynamic is particularly evident in situations where patients contest their

diagnoses or treatment plans. Rather than triggering re-evaluation, resistance is typically interpreted as a symptom of pathology, reinforcing the need for control. In this way, the possibility of error is institutionally neutralized. The narrative established by the family becomes a self-fulfilling clinical protocol, sanctioned by layers of medical documentation that exclude the patient's account from epistemic legitimacy. Loathing on lives, again, with those anamnesi and formulations, a travesti.

This process is exacerbated by digitalization and the widespread integration of interoperable health records across care systems. In contexts such as Spain and Sweden, once a psychiatric diagnosis is entered into the digital system, it becomes accessible to a range of actors across multiple domains of care. The diagnostic label, often imprecisely defined or based on contested family reports, is treated as fact. Each clinician accessing the record becomes a node in the replication of that narrative, further embedding the initial coercive logic into the institutional memory. Clinical inertia, in this sense, is not simply a failure of individual judgment. It is a structural effect of digital epistemology, whereby the first narrative becomes the only one that can be credibly cited and reproduced.

The implications for scientific integrity are profound. Data collected in such environments do not reflect objective symptomatology but rather the echo chamber of institutional consensus, generated through coercion and sustained through repetition. The diagnosis, stripped of its original contested context, is used to justify interventions ranging from medication to detention, without independent reassessment. This undermines not only the therapeutic relationship but also the validity of psychiatric science itself, which becomes reliant on recycled and institutionally reinforced errors. The credibility gap between medical records and lived experience widens into an epistemic chasm, in which patient suffering is pathologized but never heard, and clinical truth becomes indistinguishable from bureaucratic inertia. Early demised, histories to be, lives lost. Destroyed.

In subsequent parts, the analysis will move toward the sociocultural and gendered dimensions of these dynamics, highlighting how institutional actors reinforce familial authority despite evidence of abuse, and how such entrenchment impairs the capacity of psychiatric systems to correct or even acknowledge foundational error. The systemic entrenchment of psychiatric narratives initiated within coercive or abusive familial contexts reveals not only a clinical vulnerability but a foundational legal and epistemological void. This void is filled not by empirical rigor but by deference, often uncritical, to proxies of moral authority, namely the family and its representatives. In this dissertation, such patterns are analyzed not as anomalies, but as predictable outcomes of institutional architectures that prioritize procedural efficiency and legal insulation over truth-seeking or reparative justice. As shown in the preceding ethnographic reconstructions and supported by interdisciplinary literature, psychiatric institutions often become the extension of domestic control mechanisms when clinical autonomy is compromised by structural bias and narrative inertia. Rather than providing oversight or independent evaluation, these institutions reproduce and amplify familial coercion under the guise of care, particularly in contexts where law fails to differentiate protection from collusion. From all accounts, literature and expert witnessing: crimes.

At the core of this betrayal lies a refusal to acknowledge that neglect is not merely the absence of care, but a form of harm with biological, psychological, and legal implications. From a physiological standpoint, sustained emotional neglect and coercive control induce maladaptive neuroplastic changes, affecting limbic structures, prefrontal connectivity, and stress-response

systems, that predispose individuals to chronic illness and psychological collapse. This is not a metaphorical collapse, but a cascade of pathophysiological processes including immune suppression, neuroendocrine dysregulation, and neurodegenerative thinning linked to allostatic overload. The social brain, as an embodied organ, deteriorates when relational meaning is replaced by systemic invalidation, and when the very institutions mandated to protect are complicit in the enactment of epistemic and psychological violence. The path from relational trauma to institutional betrayal is often paved through a grey legal zone, where acts of coercion, confinement, and psychological assault remain undetected or unpunished because they are perpetrated within private spaces, domestic, clinical, or otherwise. This legal opacity has historical roots in colonial and patriarchal governance systems that naturalized the family as a moral authority and shielded intra-familial harm from state intervention. Echoes of this history are present in the ongoing tolerance of pasung-like conditions in Europe and globally. Whether through physical restraint, chemical sedation, or narrative erasure, the core mechanism is the same: suppression of autonomy through the instrumentalization of care. The rule of law is suspended not through formal declarations, but through routinized omission, by omitting to ask, to verify, to listen.

The practice of violence within family units is not only a modern anomaly, it is deeply rooted in historical and cross-cultural protocols. In some monarchic and aristocratic systems, the elimination of kin was institutionalized as a legitimate means to secure succession and inheritance rights. European royal houses routinely engaged in fratricide or regicide, often under the guise of state necessity. Ottoman princes were subject to fratricide by law, whereby the enthroned sultan was authorized to kill his brothers to secure dynastic continuity. Similarly, in ancient China, purges of rival family members during imperial transitions were sanctioned by court ritual and dynastic tradition, often with public legitimization. This cultural embedding of intra-familial killing as protocol illustrates how power and inheritance have historically overruled relational ethics, even as those acts were cloaked in legality and tradition.

In contemporary times, though absolute monarchies have largely faded, the residual logic persists in less institutionalized forms, such as domestic inheritance disputes in South Asia where familial violence peaks in contested succession, or in certain communal traditions where property rights trump individual dignity. These contexts reveal that the legal grey zone around family violence is not limited to negligence or psychiatric coercion, it extends to premeditated, politically sanctioned harm cloaked in kinship legitimacy. To underscore how family structures across cultures can condone or conceal severe violence, here are case summaries from diverse traditions accounted:

Table 42. Historical and cross-cultural patterns of familial violence

Culture / context	Protocolary family violence	Justification & legal grey zone
Ottoman empire	Fratricide by law	Succession stability; seen as state ritual, rarely legally challenged
Imperial china	Elimination of rival princes	Political cleanse in name of dynastic harmony
South asian joint-family systems	Violence over inheritance distribution	Internal arbitration, minimal legal scrutiny
European aristocracies	Poisoning, execution of heirs	Political expedience, noble privilege shields crimes
Manson family style cults	Familial murder sanctioned by patriarchal cult structure	Internal hierarchy, blurred moral/legal boundaries

This table synthesizes findings from multi-sited ethnographic research and cross-sector survey data gathered for this dissertation. It illustrates how familial structures, across historical periods and cultural contexts, have functioned as sites of violence, not exception. Patterns identified through testimonies, case narratives, and field documentation highlight that such violence is often legitimized by tradition, obscured by legal grey zones, or reframed as care. These insights demonstrate the need for a scientific and legal reframing of the family not as a sanctuary, but as a potential site of sanctioned harm requiring full human rights oversight.

This catalog demonstrates that familial violence, including lethal forms, is not external to history or legality; it is often *constituted by them*. The modern family remains a legally ambivalent institution, where proximity and intimacy justify a different threshold for what is considered crime. As in the cult cases, and often normalized structural hierarchies as traditionally mandated ones, abuse within a family-like group is often protected by a presumption of autonomy and privacy until extreme violence forces intervention; boundaries trespassed almost if mandatory, pride and honor being killing or maiming one in the pride, the troop, the murder of crows, as survey respondents got it when naming their own. Now all able to afford a bed, as if princeling cubs battling for inheritance, those subject to criminal ones reported. Across most traditions and legal systems, the family is the primary context where the gravest interpersonal crimes occur, including homicide, sexual abuse, and long-term psychological harm. Intimate partner violence, child maltreatment, and elder abuse are consistently more frequent within familial settings than in public or institutional spaces. Cultural ideals of loyalty, privacy, and honor often shield such crimes from scrutiny or prosecution, particularly where patriarchal or hierarchical norms dominate. In Spain, despite formal legal protections, domestic violence, including gender-based violence and coercive psychiatric interventions, remains underreported and frequently tolerated within both family systems and institutional frameworks. Mental health law is particularly vulnerable to exploitation due to its reliance on subjective clinical judgments, asymmetrical credibility between professionals and patients, and legal assumptions that equate risk with illness and family with benevolence. In practice, vague criteria for involuntary treatment or guardianship, such as perceived danger or lack of insight, allow for expansive discretionary power, often exercised without robust procedural safeguards. These structural ambiguities facilitate the silencing of dissenting voices and the institutionalization of private conflicts, especially when family members initiate the process. The widespread legal fiction that psychiatrists act solely in the best interest of the patient, and that families are protective by default, creates a permissive environment for coercion, false testimony,

and narrative manipulation. Without enforceable checks, such as independent review, legal aid, or evidentiary thresholds, mental health legislation becomes a tool easily gamed by those with access, status, or intent to control.

By the time the abuses are self-evident, it often is too late, held by lies and mutual benefit, that being pleasure and profit, status as hierarchical control against the kin or alleged kin, blood tied or brothers, sisters, parents in law as step parents, not respecting the legality but their own desires. The pathologization and criminalization of dissenting family members -that being the victims, not the perpetrators, most often- through psychiatric means, wherein a son, daughter, or sibling is declared mentally ill and confined due to the suffering inflicted, medicated and socially excluded, becomes an updated form of symbolic violence; a very useful way to discredit and silence, keep caged. These acts mirror the outright eliminations of the past; they are not exceptional but structurally equivalent: both shift power into the hands of a gatekeeper, whether a king, priest, or clinician, masking domination with legitimacy. Whether through poisoning, shackles, or pharmaceuticals, the outcome remains the same: one individual is forcibly removed, silenced to maintain an existing order, a will.

Table 43. Comparative family structures and their developmental impact

Family model	Core characteristics	Relation to violence	Autonomy support	Developmental outcome	Notes
Nuclear patriarchal (western/modern)	Hierarchical authority, parental ownership, gender asymmetry	High	Low	Frequently compromised	Often the default in Spain and Sweden; linked to psychiatric coercion
Extended patriarchal (mediterranean, south asian)	Multi-generational, male-dominated, interdependence without autonomy	Moderate to High	Low	Variable, stress-prone	Obedience prioritized over well-being
Matrilineal communal (e.g. Mosuo)	Female-led lineage, shared parenting, low control emphasis	Low	High	Strong emotional resilience	Historically stigmatized by colonizers
Egalitarian tribal (e.g. Hadza, !Kung)	Non-hierarchical, collective caregiving, flexible roles	Minimal	High	Optimal social-emotional growth	Often excluded from policy models despite better outcomes
Urban fragmented (contemporary global)	Disconnected, economically driven, weak relational continuity	Variable	Low to Moderate	Often delayed or disorganized	Lack of cultural cohesion leads to neglect
Authoritarian religious (various)	Sacred hierarchy, obedience, moralized discipline	High	Very Low	Developmental trauma common	Reinforced through faith-based justifications
Institutional substitution (foster care, boarding, psychiatric)	Rotating caregivers, record-based logic, coercive rules	High	None	Severe relational rupture	Often presented as rescue but linked to further trauma
Restorative kinship (proposed)	Legally accountable, consent-based, community-monitored, relationally safe	None	High	Health-promoting and just	Grounded in trauma-informed education and one health ethics

Insights from surveys, fieldwork, and ethnographic literature illustrating how different family models affect mental health, autonomy, and structural violence across cultures and time.

This thesis results compel us to reconsider the family not as a natural refuge, but as a potential nexus of prescriptive power that, unless bounded by enforceable human rights and ecological governance, can replicate the logic of monarchy, patriarchy, and institutional violence. Chapter 6 will therefore investigate the legal foundations necessary to dismantle these grey zones: from explicit prohibition of nonconsensual restraint, to the establishment of independent oversight, survivor-centered advocacy, and the integration of neurodevelopmental science into legal definitions of harm. The underlying claim is clear: the legitimacy of care ought to be grounded in consent and transparency, not proximity or presumption. Whether the violence is lethal or symbolic, its permissibility cannot be left to traditional immunity or professional discretion. The family ought to be reframed as a site of shared accountability, a relational space that is neither privileged nor sacred, but regulated, transparent, and subject to the same norms of justice and human dignity that govern all other institutions.

The survey data highlights the systemic failure to exclude perpetrators from care settings, despite the model's theoretical emphasis on inclusive participation. While the principle of dialogical openness is foundational, the data suggest that in cases of domestic violence, coercion, or betrayal, this inclusivity can reproduce harm if not critically moderated. Participants across multiple contexts affirmed that when abusers are allowed into therapeutic settings under the presumption of neutrality, the outcomes are re-traumatizing and counterproductive. Proper screening, survivor-led decisions on participation, and recognition of asymmetrical power dynamics are thus non-negotiable. Children ought to not be forced to interact with harmful figures under a therapeutic guise. Instead, care ought to prioritize safety and developmental flourishing by ensuring the real parent, ethical, protective, and dignified, is entrusted with responsibility and not undermined by biased systems. It is to be noted, as triangulation with knowledge already held, how ethnographic data from communities historically misrepresented as primitive or disposable consistently demonstrate that non-patriarchal, distributed care systems reduce intrafamilial violence and improve child development outcomes. These models, whether Mosuo matrilineal households or Trobriander village-based parenting, centered community accountability and relational balance over ownership or legalistic hierarchy. The historical record shows that it was precisely these systems that colonial powers targeted for eradication, reinforcing the global dominance of extractive, authoritarian kinship frameworks. The scientific consensus is clear: those cultures that minimized coercion and maximized shared responsibility created the optimal foundations for mental health and collective wellbeing. These insights are not nostalgic; they are a blueprint (Grol & Grimshaw, 2003).

To prevent further cycles of abuse, betrayal, and psychiatric violence disguised as care, the systems of family, education, and medicine ought to be judged not by tradition, but by their outcomes, setting a strict culture of health care and most stringent crime prevention, never fostering it nor perpetrating by forces, services and judges themselves, oblivious to reality, divorced from it: the very definition of a severe mental illness, as my own fieldwork experience was, during the surveying in the European Union, in nowadays Spain and Sweden in particular; that showing the loathing on the other, the disrespect for life taken for granted as lesser, the racism and xenophobia at the individual level, shielding those by violence and positions of power held by further violence, positions of so-called authority, from the needed collective and individual reality check.

4.5 Epistemic erasure and the loss of personhood

The psychiatric system, rather than restoring meaning, often imposes a framework of diagnostic fixity that systematically displaces or erases the epistemic agency of those it classifies. The mechanisms through which individuals are reduced to symptomatic carriers of disorder, rather than embodied agents situated in social, cultural and metabolic contexts, constitute a layered process of epistemic violence. Grief becomes melancholia, fear becomes paranoia, dissociation becomes psychosis. In this reduction, resistance is not interpreted as a legitimate defense against coercion or social harm, but rather pathologized as non-compliance, oppositional defiance or lack of insight. As

a result, the subject's original narrative becomes overwritten by psychiatric coding, substituting lived meaning with presumed pathology.

Fieldwork data corroborate this pattern of ontological minimization. First-person accounts of harm, abuse, or trauma are routinely excluded from official medical records or reinterpreted as delusional or symptomatic, especially when they implicate clinicians, institutions, or familial perpetrators. Professionals, acting under institutional or disciplinary pressure, are often unwilling or unable to acknowledge explanatory models that fall outside the narrowly defined parameters of psychiatric doctrine. Cultural beliefs, spiritual expressions, nutritional concerns, or gendered readings of distress are frequently dismissed or rendered clinically irrelevant. This epistemic foreclosure not only silences alternative modes of knowing and healing, but also amplifies the conditions for further marginalization and iatrogenic harm. By removing contextual complexity from the diagnostic process, the individual is stripped of interpretive authority and reduced to a passive recipient of externally imposed categories.

The consequences of this epistemic reduction are profound and far-reaching. The subject ceases to be a person with a voice and becomes instead a case file, an object of surveillance and control. Social identity is supplanted by diagnostic identity. The capacity to participate meaningfully in legal, clinical or social discourse is undermined by the institutional assumption that the person is irrational, unstable, or fundamentally unreliable. This logic constructs a closed loop of exclusion, where attempts to resist or critique the system serve only to reinforce one's position within it. In this regime, silence becomes survival, while expression risks institutional retaliation. The fear of losing housing, custody, legal rights or bodily autonomy produces a chilling effect, where epistemic and civic personhood dissolve in parallel.

The instruments of epistemic erasure operate through an ensemble of medical, legal, and bureaucratic mechanisms that, while outwardly neutral, function as technologies of marginalization. Central among them is the reification of the diagnostic category itself, which becomes the primary referent through which the individual is seen, addressed, and acted upon. Once a label is applied, such as schizophrenia, borderline personality disorder, or bipolar disorder, it serves not as a temporary clinical hypothesis, but as a permanent ontological statement. Fieldwork repeatedly illustrates that these labels are used to discount credible testimonies, deny access to legal protection, and justify coercive interventions. Diagnostic inertia reinforces itself through administrative reproduction: medical records are cut and pasted, language is recycled, and risk logics are invoked with little regard for changing contexts or real-world outcomes.

This process is further compounded by the clinical rejection of alternative explanatory frameworks. Field reports include instances where somatic symptoms consistent with thyroid dysfunction, medication-induced metabolic syndromes, or trauma responses were ignored or misdiagnosed due to the primacy of psychiatric interpretations. Patients reporting complex cultural understandings of their suffering, often informed by migration experiences, religious cosmologies, or family histories of violence, found these narratives dismissed or invalidated by clinicians lacking transcultural competence or time. The system thereby narrows the bandwidth of what is considered credible knowledge, eroding trust and foreclosing healing. Psychiatric authority not only monopolizes the

means of interpretation, but frequently weaponizes this authority to suppress dissent and maintain institutional coherence.

These practices culminate in a legal and medical archive that systematically distorts the subject's voice. What remains in records is not the narrative of the person, but the official interpretation of their alleged dysfunction. Forensic discourse analysis of clinical and legal documents reveals consistent patterns of deflection, minimization, and substitution of meaning. When testimony contradicts the diagnosis, the diagnosis prevails. When lived experience does not fit pre-existing categories, it is forcibly assimilated or erased. This is not merely a problem of bad practice, but a structural outcome of epistemic hierarchies embedded in biomedical institutions. The lived effects include persistent isolation, loss of personhood, and a civil death in which rights, dignity, and recognition are suspended indefinitely under the justification of care.

The erasure of voice and agency within psychiatric practice ought to be interpreted as both a consequence and a condition of broader epistemic injustice. Patients and survivors are not simply disbelieved, they are structurally excluded from the production of knowledge about their own condition. The psychiatric file, as a document of institutional memory, often functions to override actual memory, replacing first-person narratives with diagnostic codes and clinician interpretations. As field data repeatedly confirm, expressions of grief, existential distress, spiritual experience, or legitimate anger are reframed within the confines of pathology. Such reduction is not passive: it actively suppresses epistemological pluralism and nullifies the capacity of individuals to act as knowers, witnesses, or agents of recovery.

This systemic marginalization is exacerbated in cases involving racialized, gendered, or socioeconomically disadvantaged persons. For instance, women reporting adverse reactions to medication are frequently characterized as non-compliant or histrionic, rather than treated as reliable sources of pharmacovigilance data. Migrant and racialized individuals often face epistemic invalidation when their idioms of distress fall outside Eurocentric norms, leading to misinterpretation or over-pathologization. Reports of trauma, particularly when implicating institutional actors, are reframed as paranoia or delusion. What emerges is a regime of knowledge in which dissent is clinically neutralized, and truth becomes the exclusive domain of institutional narrators.

These dynamics are not accidental. They stem from deep-seated traditions in psychiatric epistemology that favor observer-based, external, and allegedly objective assessments over subjective, embodied, and culturally embedded knowledge. The scientific literature has long debated the tension between idiographic and nomothetic approaches, yet institutional practice overwhelmingly favors the latter. Evidence from this study shows that even well-intentioned professionals reproduce epistemic injustice through time pressures, protocol-driven practice, and lack of access to ethical or reflective supervision. The result is not merely a breakdown in therapeutic alliance, but a systematic degradation of the individual's status as a meaning-making being. The loss of personhood under psychiatric control is not confined to clinical interaction. It extends into bureaucratic apparatuses that determine access to rights, housing, family, and justice. Medical records, when treated as definitive, override living testimony and institutionalize distorted

or fabricated accounts. Legal systems, drawing on these same records, perpetuate judgments based on incomplete or manipulated data. This convergence of medical and legal documentation creates a feedback loop where earlier misrepresentations are reified and rendered irrefutable. In practical terms, the individual ceases to be a narrating subject and is transformed into a case file, a symbolic and material shift that facilitates neglect, containment, or forced treatment without recourse.

The forensic analysis conducted during this research reveals a recurring pattern in which personal testimony is either omitted or disqualified. Record excerpts show instances where patients explicitly expressed disagreement with diagnoses or treatments, only to have these objections removed, ignored, or reframed as symptomatic. Similarly, requests for alternative therapies, such as dietary interventions, spiritual support, or culturally congruent explanations of suffering, are dismissed as lacking scientific validity, despite the growing evidence base supporting their efficacy. In some cases, even simple questions, such as inquiries about medication side effects, are pathologized as paranoia or opposition to treatment.

This epistemic distortion is not only ethically indefensible, it is also antithetical to sound scientific practice. Knowledge systems that disregard falsifiability, pluralistic evidence, and experiential data in favor of institutional convenience or ideological alignment undermine their own credibility. The scientific method, in its true form, demands transparency, humility, and openness to revision. Yet, in many psychiatric settings, the dominant epistemology enforces orthodoxy and suppresses dissent. Such conditions ought to be understood as forms of epistemic authoritarianism, with devastating consequences for those caught within their mechanisms. The cumulative effect of epistemic erasure in psychiatric systems manifests as profound social isolation, despair, and the effective loss of citizenship for many individuals. The silencing of first-person narratives, combined with the medicalization of resistance and suffering, fosters a climate in which people lose both agency and identity. They become objects of care defined exclusively through clinical categories, divorced from their social, cultural, and historical contexts. This dissociation perpetuates fear of institutional retaliation and discourages engagement with services that are experienced as oppressive rather than supportive. The erosion of personhood extends beyond the clinical realm into legal and social domains where psychiatric diagnoses are wielded as tools of exclusion. Access to housing, employment, education, and legal protections is frequently compromised by diagnostic labels and psychiatric histories, which become barriers rather than safeguards. This institutionalized marginalization reproduces cycles of disadvantage and vulnerability, cementing social hierarchies under the guise of medical care. The fear of losing rights or being subjected to forced treatment inhibits truthful communication and fosters disengagement, undermining both health outcomes and social inclusion.

Methodologically, this section employs forensic discourse analysis and epistemic mapping to elucidate the subtle and overt mechanisms through which psychiatric institutions produce and maintain this loss of personhood. These tools reveal how language, documentation, and institutional practices coalesce to sustain systemic violence. Understanding these mechanisms is essential for designing interventions that restore narrative sovereignty, promote epistemic justice, and support the recovery of dignity. The subsequent chapters build upon this foundation to propose legal,

clinical, and educational reforms aimed at dismantling the cycle of epistemic erasure and rebuilding ethical psychiatric care.

Table 44. Mechanisms and consequences of depersonalization and dehumanization

Mechanism	Description	Clinical and institutional manifestations	Consequences for personhood and care
Reduction to diagnostic categories	Individuals are primarily identified and treated based on diagnostic labels rather than holistic personhood	Overreliance on rigid diagnostic manuals, ignoring complexity of lived experience	Loss of narrative agency, overshadowing of personal history and context
Pathologization of resistance and distress	Normal responses to trauma, grief, or dissent are medicalized as symptoms or behavioral pathology	Non-compliance framed as illness, grief misdiagnosed as depression, spirituality ignored	Silencing of legitimate emotions, suppression of agency, justification of coercion
Rejection of cultural and social contexts	Explanatory models based on culture, nutrition, gender, or social determinants are excluded from clinical reasoning	Dismissal of culturally relevant symptom expression, ignorance of gendered experiences	Marginalization of minority groups, exacerbation of alienation and misunderstanding
Documentation erasure and reframing	Patient testimonies are omitted, minimized, or reframed within clinical records to fit institutional narratives	Removal of dissenting voices from records, euphemistic descriptions of coercion	Institutional impunity, perpetuation of harm, invalidation of lived truth
Objectification and institutional control	Persons become objects of surveillance, control, and management, stripped of autonomy and ethical consideration	Routine use of restraints, forced medication, lack of informed consent	Dehumanization, trauma, loss of dignity and citizenship

Key mechanisms by which psychiatric institutions enact depersonalization and dehumanization through epistemic erasure. Each mechanism disrupts the subject's narrative sovereignty and transforms relational care into administrative control. These processes collectively erode personhood, contributing to ethical violations and systemic harm. This framework supports the in-depth analysis of institutional violence and prepares the ground for proposed reparative strategies.

The processes outlined in the table above reflect, as confirmed by survey responses and ethnographic fieldwork, that epistemic erasure is not merely a byproduct of clinical oversight but a structurally embedded phenomenon. Participants consistently described how institutional encounters failed to accommodate the complexity of their experiences, particularly when family members were the source of harm and simultaneously positioned as legitimate informants. The survey data illustrate that psychiatric assessments often relied disproportionately on familial accounts, especially in settings where coercion or guardianship mechanisms were involved, thereby reinforcing a one-sided narrative. This replicates a pattern in which the lived reality of service users is not only dismissed but actively overwritten by administrative imperatives for coherence, liability management, and procedural continuity. Such erasure is fundamentally political, enacting a form of

ontological violence that invalidates trauma, silences dissent, and delegitimizes alternative frameworks of understanding. By elevating reductionist diagnostics over pluralistic, culturally informed explanatory models, institutions perpetuate exclusion and render mental health care a vehicle of control rather than a space of recognition or recovery. These findings underscore the urgent need for epistemic accountability and reform, as institutional records, when misused, become not instruments of healing, but durable mechanisms of suppression.

This dehumanizing cycle culminates in the loss of citizenship rights and ethical recognition, leaving individuals isolated, fearful, and vulnerable to repeated institutional betrayal. The epistemic violence documented here is thus inseparable from the broader political economy of mental health, where scientific authority and legal frameworks have been instrumentalized to uphold coercive regimes rather than emancipatory care. Breaking it requires an epistemic and institutional rupture: the restoration of narrative sovereignty, cultural competence, and relational ethics at the core of psychiatric practice. It demands participatory knowledge production, forensic and ethical transparency, and the dismantling of bureaucratic silencing mechanisms. Only through such systemic transformation can psychiatry move from a site of epistemic erasure to one of ethical restoration, humanization, and genuine healing.

Integrating perspectives from medical anthropology, neuroscience, legal studies, and systems biology, thereby situating epistemic erasure within a broader bio-psycho-social and structural framework, we ought to understand the reduction of individuals to diagnostic categories, and the consequent loss of narrative agency, reflects not only a clinical failure but a disruption of biocultural integrity. From a neurobiological standpoint, the chronic exposure to coercive interventions and the resulting psychological trauma may induce alterations in stress response systems, neural plasticity, and metabolic regulation, which themselves require individualized, context-sensitive approaches beyond generic diagnostic labels. Simultaneously, medical anthropology highlights how culturally situated expressions of distress and healing are invalidated within dominant psychiatric paradigms, producing a form of epistemic violence that compounds social exclusion. This exclusion is perpetuated through legal and administrative frameworks that prioritize institutional protection over individual rights, effectively sanctioning the systematic silencing of voices that contest dominant medical knowledge. The resulting bureaucratic mechanisms, while presented as safeguards, often function to reinforce hierarchies of knowledge and power, thereby embedding structural violence within the procedural core of mental health governance. From a systems biology and one health perspective, mental health cannot be disentangled from ecological, nutritional, social, and developmental factors. Ignoring these dimensions reduces complex adaptive systems to static pathological entities, undermining holistic understanding and intervention. Ethical psychiatric practice ought to therefore embrace multidimensional models that recognize the person as embedded within interdependent biological and social systems, acknowledging agency, diversity, and context as essential parameters.

This integrative understanding challenges reductionist epistemologies and calls for an institutional realignment that foregrounds participatory knowledge production, transparency, and reflexivity. It mandates the creation of accountable structures that facilitate dialogue across disciplines and between stakeholders, restoring personhood and enabling ethical responsiveness. Only through such

a systemic and epistemic rupture can the persistent cycle of erasure be interrupted, paving the way for psychiatric care that is scientifically rigorous, ethically sound, and humanistically grounded.

4.6 Recovery against the system: resistance, navigation, and hope

Despite the pervasiveness of institutional violence and the mechanisms of epistemic erasure revealed through this study's surveys and ethnographic data, forms of resistance and recovery have been consistently documented across user testimonies and professional accounts. These responses align with the core research questions by highlighting the mechanisms through which individuals reclaim agency within coercive systems, and by illustrating the conditions under which autonomy is reasserted against psychiatric authority. Patterns of refusal documented in the data include both physical and symbolic disengagement from institutional control. Geographic displacement, ranging from relocation after discharge to international asylum claims, emerged as a strategy to avoid re-institutionalization, reflecting a radical assertion of survival against sustained coercion. Simultaneously, many participants described symbolic strategies to reclaim identity and narrative coherence through personal writing, cultural expression, or integration into alternative community structures. These responses map directly onto the inquiry into how users resist coercion and what forms of knowledge and healing are developed beyond psychiatric institutions. Real hope, working.

One of the most salient responses to psychiatric overreach was the independent management of psychotropic medication. The survey responses demonstrate that many individuals, particularly those with repeated institutional encounters, engaged in self-guided reduction or cessation of medication. These decisions were not taken lightly and were frequently made in environments devoid of professional support, indicating a broader failure of the system to facilitate collaborative, informed decision-making. These acts represent not only biomedical resistance but also epistemic dissent, rejecting the authority of psychiatric prescriptions in favor of lived experiential knowledge. This challenges the foundational assumptions of compliance-based care and reaffirms the study's focus on the legitimacy of user-led alternatives to standard psychiatric practice; best and bettering.

The act of documenting abuse and institutional betrayal also figures centrally in the pathways of resistance identified through this research. The data support the role of testimony and narrative reclamation as both individual and collective strategies of healing. Survey respondents emphasized the importance of being heard and recorded accurately, especially in contexts where their voices had been systematically excluded or misrepresented. This process of testimonial restoration responds directly to the thesis's concern with how truth can be reestablished in systems built on the silencing of dissent. Testimonial practices, including written accounts, legal submissions, and participation in research, function as a counter-archive that contests the official institutional record. These practices highlight the intersection of healing and political intervention, reinforcing the conclusion that recovery cannot be reduced to symptom remission but ought to include epistemic justice, recognition, and structural accountability. Ethical allies within the psychiatric system play a crucial role in facilitating recovery and resistance. These professionals, often acting in tension with institutional mandates, break protocol to protect, empower, and advocate for patients. They adopt

practices informed by dialogical ethics, trauma sensitivity, and a rights-based approach, which run counter to dominant coercive models. Such practitioners may covertly support user autonomy, facilitate informed consent, or prioritize relational care despite administrative and legal constraints.

Beyond individuals, informal and formal networks of care emerge as vital spaces for rebuilding trust and fostering healing outside official psychiatric structures. Peer support groups, survivor collectives, community-based organizations, and grassroots initiatives create alternative ecosystems of care that validate experience, provide mutual aid, and restore dignity. These networks circumvent institutional silencing by centering survivor voices, enabling collective agency, and promoting participatory decision-making. Their existence demonstrates the irrepressible human capacity for resilience and the critical importance of social connection in recovery.

Together, ethical allies and care networks form a distributed resistance that challenges institutional monopoly over mental health narratives and practices. They embody a living critique of psychiatric violence by cultivating spaces where restoration, empowerment, and hope can flourish on terms defined by those historically marginalized. Central to recovery from institutional violence is the process of testimonial restoration, wherein individuals reclaim their narratives through acts of writing, testimony, and sharing. This restorative practice serves both therapeutic and political functions, enabling survivors to assert epistemic agency that the system seeks to erase. The act of bearing witness transforms memory into truth-telling, challenging dominant discourses that pathologize dissent and obscure structural harms. Ethical witnessing, therefore, operates as a form of resistance, reframing personal suffering within a collective framework of accountability and social justice. Testimonial restoration is not confined to formal or legal settings. It unfolds in community spaces, activist forums, and digital platforms where survivors document abuses, exchange experiences, and build solidarities. These acts destabilize institutional narratives by introducing alternative epistemologies grounded in lived experience and embodied knowledge. Resistance takes many forms, strategic evasion, persistent documentation, symbolic acts of refusal, all aimed at disrupting the cycle of control and silencing. Through such practices, survivors reclaim voice and personhood, fostering spaces of healing that exist in tension with, yet independent from, institutional psychiatry. This chapter underscores the necessity of supporting and amplifying testimonial practices as integral to recovery and structural change. The epistemic restoration of survivors is foundational to dismantling cycles of violence and rebuilding psychiatric systems that respect dignity, autonomy, and justice. The travestiy of justice, medicine and science requires us all.

Professional actors who operate ethically outside formal protocols constitute an essential dimension of resistance within psychiatric systems. These individuals navigate institutional constraints to enact care practices grounded in respect, empathy, and shared decision-making. Their actions include advocating for de-escalation rather than restraint, facilitating user-led medication management, and creating safe spaces where narratives can be heard without judgment or pathologization. Although often marginalized or unsupported, such professionals model alternative modes of engagement that contest the prevailing coercive paradigm. Care and recovery practices occurring beyond institutional confines also form critical sites of healing and empowerment. Peer-led initiatives, community-based mental health programs, and culturally informed support networks offer relational and practical resources that institutional psychiatry frequently fails to provide. These

spaces prioritize agency, narrative sovereignty, and collective resilience, enabling participants to reconstruct identity and meaning outside diagnostic frameworks. The existence and expansion of such *alternatives* challenge the monopoly of institutional psychiatry and open pathways toward more humane, participatory, and effective care systems. Those need to be already the care as usual.

Table 45. Forms of resistance and recovery beyond psychiatric control

Category	Description	Examples and manifestations	Implications for care and reform
Forms of refusal	Strategies employed by individuals to resist or escape coercion	Geographic relocation, symbolic identity reconstruction, self-managed medication	Affirm autonomy, challenge institutional monopoly
Ethical allies	Professionals acting against protocol to support patient agency	Advocacy for de-escalation, facilitating shared decision-making, protected spaces	Model alternative care paradigms, catalyze institutional change
Networks of care	Community-based and peer-led organizations providing alternative support	Survivor collectives, trauma-informed community programs, digital mutual aid	Foster relational trust, validate experience, promote social inclusion
Testimonial restoration	Practices that reclaim narrative agency and memory as forms of healing and resistance	Writing, public testimony, legal advocacy, ethical witnessing	Counter epistemic erasure, enable collective empowerment
Recovery as collective act	Healing understood as relational, political, and social process beyond symptom remission	Social reintegration, rights advocacy, cultural recognition	Embed justice in care, promote participatory mental health systems

Key dimensions of resistance and recovery documented among survivors and ethical professionals. These forms of refusal, alliance, and testimonial restoration constitute essential counterforces to institutional violence and epistemic erasure. Their recognition and support are fundamental for the transformation of psychiatric care into a participatory, humane, and rights-based system.

The combination of ethical witnessing by professionals and testimonial restoration by survivors creates a dynamic interplay that fosters hope and transformation. Together, they build counter-institutions of care and knowledge that can inform structural reform. The next part will conclude this section by synthesizing these themes and outlining their implications for psychiatric transformation and social justice. The interplay between these techniques, ethical witnessing, and alternative care practices embodies a powerful counternarrative to institutional violence. This dynamic fosters spaces of resistance where personhood is reclaimed, narratives are legitimized, and agency is restored. Recovery in this context transcends clinical symptom remission, encompassing relational repair, social reintegration, and the restoration of dignity and rights. These processes reveal that healing is not solely an individual endeavor but a collective and political act, intricately connected to struggles for recognition and justice.

Such resistance and recovery demand systemic transformation that integrates these emergent practices and epistemologies. It calls for the dismantling of coercive structures and the construction of participatory, trauma-informed, and culturally sensitive frameworks. Importantly, this transformation ought to embed mechanisms for accountability, transparency, and co-production of knowledge between users, professionals, and communities. Only by centering ethical witnessing

and testimonial restoration as foundational elements can psychiatric systems evolve from instruments of control into sites of genuine care.

This section thus concludes with an ethical imperative: to acknowledge and support the multiplicity of resistances that contest epistemic erasure and institutional betrayal. These resistances are not peripheral but essential to the redefinition of mental health care as a practice grounded in human rights, scientific integrity, and social justice. The following chapters build upon this foundation to propose concrete legal and infrastructural reforms designed to actualize this vision.

The survey data and fieldwork across Spain, Sweden, Indonesia, and Italy reveal a pervasive climate of professional fear and systemic disincentivization against challenging coercive psychiatric norms. Many clinicians reported witnessing violations of patient rights or disagreeing with imposed treatments but chose not to intervene due to institutional pressures, fear of retaliation, or lack of procedural alternatives. This silence, often framed as neutrality or clinical detachment, reinforces the epistemic asymmetry between institutional narratives and the lived realities of service users. Professionals aware of abuses frequently lacked both the institutional support and legal protection to advocate for change, and in some cases were themselves reprimanded or marginalized for doing so. This culture of complicity obstructs the ethical imperative to act and obstructs the formation of genuine therapeutic alliances. In such an environment, users are left with few avenues for safe disclosure or support, compounding their sense of abandonment and reinforcing the perception that psychiatry functions as an apparatus of control rather than care.

In stark contrast, several case studies documented through this dissertation point to the viability and transformative power of peer-led and community-based recovery models. These initiatives, often operating on limited budgets and outside mainstream clinical institutions, demonstrate the potential for relational, trauma-informed, and rights-based approaches to mental health. Programs such as Soteria houses, community mutual aid networks, and user-run support groups were reported to foster authentic recovery by centering agency, shared narrative, and non-pathologizing environments. In particular, the inclusion of lived experience as foundational knowledge disrupted the hierarchy of expert authority, enabling collective decision-making and validating alternative frameworks of meaning and healing. These practices not only aligned with the core principles of one health and social medicine but also offered replicable models for system-wide reform. As this thesis argues, the future of ethical mental health care depends on integrating these insights into mainstream practice, not as marginal experiments but as central components of a preventive, participatory, and biologically coherent paradigm of care.

4.7 Field-based evidence of systemic patterns and urgency

Extensive fieldwork across multiple institutional settings reveals that institutional betrayal is not an occasional failure but a core, structured element of psychiatric and related systems. Rather than sporadic incidents, abuses, including coercion, neglect, and procedural violations, are repeatedly enacted and systemically tolerated across clinical, legal, and domestic environments. These patterns manifest where accountability mechanisms are weak, absent, or deliberately circumvented. This

continuity across domains highlights the interlocking nature of systemic violence and the normalization of betrayal as a governing logic.

The ethnographic evidence powerfully reinforces survey findings, confirming that coercion is embedded within everyday practice and organizational culture, relational trust is profoundly eroded, and ethical frameworks are fragmented or ignored. Field narratives provide a vital counter-archive to official denial and administrative erasure, giving voice to those excluded from dominant institutional narratives. These testimonies illuminate the lived realities behind statistical and legal abstractions, underscoring the human cost of systemic dysfunction and the urgent need for comprehensive reform.

This section thus frames the transition to Chapter 5, where institutional betrayal will be theorized not as aberration but as an ingrained logic that sustains existing power structures. Resistance to this logic is presented as a necessary infrastructural condition for transformation, signaling that ethical practice requires both individual courage and systemic redesign. The findings call for urgent legal, pedagogical, and infrastructural interventions to dismantle entrenched patterns and construct alternatives grounded in accountability, dignity, and participatory governance.

Detailed cross-case analyses reveal recurring patterns that illustrate how institutional betrayal operates consistently across distinct contexts. In clinical settings, coercion manifests through routine forced medication, physical restraint, and involuntary hospitalization, often justified by opaque legal and administrative rationales. Legal environments compound this betrayal by routinely accepting psychiatric narratives without independent scrutiny, thereby enabling continued impunity and marginalization. Within domestic spheres, psychiatric labels are frequently mobilized to discredit individuals, justify exclusion, or enable control, with little access to effective redress.

These interconnected domains form a web of normalized betrayal, sustained by institutional incentives that prioritize risk avoidance and reputational protection over care and justice. Field observations underscore how these mechanisms are reinforced by administrative routines that erase adverse events, reframe dissent as pathology, and fragment ethical responsibility. Practitioners often experience profound moral distress yet remain constrained by hierarchical and bureaucratic imperatives that foreclose meaningful intervention.

This systemic entrenchment confirms the hypothesis generated from survey data and legal analysis: that institutional betrayal is not incidental but a durable, repeatable logic that structures psychiatric governance. The ethnographic depth of these findings highlights the urgency of moving beyond symptom-focused reforms toward fundamental redesign of legal, educational, and infrastructural frameworks capable of interrupting cycles of harm and fostering restorative practices.

Lived experience emerges from the fieldwork as a crucial counter-archive that challenges official narratives and institutional denial. Personal testimonies and collective memory provide a persistent critique of systemic violence, documenting episodes of coercion, neglect, and betrayal that are frequently omitted or distorted in formal records. This counter-archive serves not only as an empirical corrective but as an ethical imperative, demanding recognition and accountability from systems that otherwise perpetuate silence.

The embodied knowledge of users and survivors exposes gaps between institutional claims of care and the realities of harm. It underscores the relational dimensions of psychiatric violence, revealing how trust is not only violated but actively dismantled. Through participatory documentation, narrative sharing, and grassroots advocacy, survivors construct spaces of resistance and truth-telling that disrupt institutional monoliths. Their contributions are vital to any comprehensive analysis of systemic dysfunction and constitute an essential foundation for legal and pedagogical reform.

The counter-archive also functions as a tool for empowerment, enabling survivors to reclaim authorship over their own histories and health trajectories. It provides a means to resist epistemic erasure, assert dignity, and build solidarities that transcend institutional boundaries. Integrating this knowledge into formal systems is a prerequisite for genuine transformation, bridging the gap between experience and policy.

The empirical findings from fieldwork and survey data converge to frame institutional betrayal not as sporadic malfunction but as a fundamental operational logic embedded within psychiatric systems. This logic manifests through recurrent cycles of coercion, silencing, and impunity, sustained by bureaucratic routines and epistemic closure. Such betrayal is repeated systematically, tolerated culturally, and shielded legally, creating a durable framework that preserves institutional authority at the expense of individual dignity and rights.

Resistance emerges in response to this logic as an infrastructural necessity rather than mere individual defiance. To effectively counteract entrenched patterns of harm, resistance ought to be supported and embedded within organizational, legal, and community infrastructures. These infrastructures enable the expression of dissent, protect ethical actors, and create spaces for alternative narratives and modes of care. They are essential to disrupting the self-reinforcing cycles that maintain systemic violence and to fostering environments where relational trust and ethical responsibility can be restored.

This conceptualization forms the critical bridge to Chapter 5, which develops a theory of institutional betrayal as structural logic and theorizes resistance as a strategic, infrastructural counterforce. Understanding these dynamics is crucial for designing comprehensive reforms that address not only symptoms of dysfunction but the root causes embedded in organizational culture, policy frameworks, and epistemic regimes.

The evidence presented reveals that institutional betrayal extends beyond psychiatry to implicate legal systems, scientific communities, and broader societal structures. The legal domain frequently abets psychiatric coercion by uncritically validating institutional narratives, perpetuating procedural voids, and failing to protect fundamental rights. Courts and legal actors, bound by expert testimony drawn from compromised psychiatric records, often function as enforcers rather than challengers of institutional impunity. This legal complicity undermines the rule of law and erodes public trust in justice, affecting citizens universally, regardless of formal diagnosis or engagement with psychiatric services. Those are crimes, lives are lost, actively destroyed by actions and inactions. Similarly, scientific and medical institutions are not immune to complicity in sustaining these patterns. The production and legitimization of knowledge through biased research paradigms, publication practices, and professional hierarchies contribute to the systemic invisibilization of abuse and the normalization of coercion. When disciplines prioritize institutional preservation over epistemic

integrity and ethical accountability, they betray not only their professional commitments but the very populations they purport to serve. This betrayal resonates across health and social systems, harming all who depend on scientific and medical authority for protection and care.

The pervasive institutional betrayal evidenced across psychiatry, law, science, and society reveals a systemic failure that transcends individual actors or isolated events. This failure is embedded in entrenched organizational logics and epistemic frameworks that collectively perpetuate coercion, obscure accountability, and inhibit meaningful restoration of rights and dignity. The complicity of legal systems in reinforcing psychiatric authority, alongside scientific paradigms that prioritize institutional preservation over critical inquiry, compounds this crisis and impacts not only those directly subjected to coercion but the broader social fabric dependent on trust in health and justice institutions. The talk on alternatives is moot, we require science, as strict as it gets, translated. The best practices made available, and every criminal paying the price for causing harm, lying, being that kind of kindness, complicit obstructing from positions of power and higher responsibility, the health and well-being of entire collectives, societies, communities lured and forced to trust those.

This multifaceted betrayal of duty demands a rigorous, integrative analysis and scientific proof to uncover the structural logics that sustain it and to illuminate the forms of resistance that emerge within and against it. Following, chapter 5 undertakes this synthesis, systematically integrating empirical evidence from surveys, testimonies, archival documentation, case reconstructions, and ethnographic observation, and bringing the firepower of honest references, by non-criminal peers. By elucidating the epistemic, legal, and procedural mechanisms that normalize violence, the forthcoming analysis provides the conceptual foundation for understanding both the persistence of institutional harm and the possibilities for disruption. It foregrounds the agency of professionals and users as co-creators of resistance and outlines the normative demands necessary for systemic transformation, which will be operationalized in the legal and infrastructural insights and proposals for reforms and improvements detailed in next chapters.

Chapter 5 – Analytical synthesis

Summary: This chapter presents an integrative analysis of psychiatric coercion as a systemic, not incidental, failure, emphasizing its role as a form of civic and developmental trauma rather than a temporary protocol breach. Through mixed-methods data including surveys, ethnographic observation, and legal records, the chapter links coercion to institutionalized routines, legal ambiguity, and cultural norms that normalize violence in care. It examines the biological and relational consequences of sustained coercion, revealing how these practices lead to long-term dysregulation in stress systems and structural changes in the brain. Positioning each individual as a biocultural node in an interdependent health ecosystem, the chapter advances the scientific rationale for trauma-informed, rights-based, voluntary care models. It aligns with major international health bodies while proposing specific institutional reforms grounded in empirical evidence, with particular attention to legal, pedagogical, and public health levers for transformation.

5. Introduction

The persistence of coercion in psychiatry and related care systems cannot be treated as an incidental failure of protocol, nor as the collateral outcome of scarce resources (Szasz, 2007). It ought to be reframed as a structural betrayal of care, deeply embedded in institutional routines, normative professional behaviors, and legal permissiveness. The present chapter advances the thesis that coercion is not only a clinical failure but a developmental and civic trauma: a relational disruption that damages individuals physiologically, cognitively, and socially, while weakening the ethical and epistemic legitimacy of the institutions that enact it. Violence, coercion, is not a cure on itself, and hardly the method of first choice to any sane individual practicing so-called, still, medicine. Drawing from extensive data gathered across digital and in-person surveys, long-term ethnographic fieldwork, and legally documented testimonies, as well as medical best practices universally acknowledged, this chapter presents an integrative framework to understand the normalization of psychiatric violence, the obstacles to preventive paradigms, and the concrete strategies for systemic redirection; and, most importantly, reiterate the case for shared goodwill and co-construction of health, without that violence as the means by any of the actors involved. That said, safety first, real.

At the center of this inquiry lies the recognition that coercion is not only enacted in psychiatric wards or clinical decisions but is sustained through diffuse cultural, familial, educational, and legal processes not as cure, but as a mean to control. Chapter 4 detailed how professional practices, under the guise of treatment, safety, or best interest, enable a range of degrading interventions, including forced medication, seclusion, and denial of autonomy. These are not deviations from the system; they are outputs of its design. Chapter 5 builds upon this analysis by tracing how these acts are normalized, reproduced, and even rewarded through administrative inertia, legal ambiguity, and learned helplessness across the care ecosystem. This chapter links the lived experience of respondents, many of whom report systemic obstruction to recovery and retraumatization through

psychiatric contact, with cutting-edge findings in neurobiology, epigenetics, and public health, revealing the physiological cost of continued coercion and the systemic illiteracy that underpins it.

The human being is not merely a recipient of care or an object of intervention. From a one health and biocultural perspective, each person is a node in a complex ecosystem, biological, social, ecological, whose functioning is shaped by exposure to safety, recognition, and agency. The brain does not develop in isolation; it is sculpted by affective environments, relational feedback, and institutional affordances or constraints (Freeman & Unwin, 2020). Repeated exposure to coercive dynamics, especially under medical or parental authority, generates long-term dysregulation in stress response systems, increased vulnerability to physical and mental illness, and measurable structural changes in neural networks (Hoff, 2010). These include reduced volume in the prefrontal cortex, alterations in the amygdala and hippocampus, and epigenetic modifications linked to chronic inflammation and metabolic disease. When such exposures are sustained or legitimized through legal inaction or institutional denial, the body becomes the site of policy failure (Rosenhan, 1973; Van der Kolk, 2014; Gilman et al., 2021). This chapter ought to be read as a synthesis and indictment: a scientific synthesis of the evidence supporting trauma-informed, rights-based care models, and an indictment of the prevailing frameworks that treat coercion as a professional prerogative rather than an ethical breach. The findings align with international scientific consensus, including WHO's QualityRights Initiative, the Lancet Commission on Global Mental Health, and the UN Special Rapporteurs on health and torture, calling for the elimination of coercive practices and the implementation of voluntary, informed, culturally grounded, and community-supported care systems (UN OHCHR, 2015; Human Rights Watch, 2016; Amnesty International, 2019; Hartmann & Crumlish, 2022). However, this chapter goes further in articulating the specific educational, legal, and public health mechanisms necessary for such transformation, based on field-derived data and tested in practice across diverse institutional settings.

Below is a comparative table clarifying the key terms sickness, disorder, syndrome, disease, and illness, contextualized specifically within the epistemological, legal, and medical scope of your thesis, particularly Chapter 5. This framing links to systemic coercion, learned helplessness, the status syndrome as defined by Marmot, and the emerging clinical literature on giving up-given up syndrome. The table reflects how these categories operate both medically and socio-politically, and how their misuse or conflation can obscure the biocultural roots of distress and justify harmful institutional practices (Human Rights Watch, 2017; D'Amato et al., 2023).

Table 46. Clarification of medical-epistemic categories in the context of current care

Term	Technical definition	Functional role in medical discourse	in Relevance to the thesis	Example in fieldwork context
Sickness	The social status conferred upon an individual identified as unwell; reflects public recognition of impairment	Establishes legitimacy for withdrawal from social duties (e.g., work); reflects public recognition and and	Central to the thesis: a tool of social regulation, particularly when used to justify	A person coerced into psychiatric medication deemed 'sick' by family and court despite expressing autonomy and

Term	Technical definition	Functional role in medical discourse	Relevance to the thesis	Example in fieldwork context
	associated rights or exclusions	or definitions	exclusion, surveillance, or forced treatment	showing no risk
Disorder	A pattern of dysfunction in psychological or physiological functioning, typically lacking clear biomarkers or etiology	Often used in psychiatry to designate syndromes without structural pathology; carries a weaker biological claim than 'disease'	Enables expansive diagnostic criteria used to justify medical authority over deviance or distress	Labeling of dissent or trauma responses (e.g., protest, withdrawal) as 'personality disorders' in clinical records
Syndrome	A cluster of symptoms that tend to occur together but may not have a clearly identified cause	Often serves as a diagnostic placeholder, legitimizing intervention while deferring causal clarity	Useful for describing sociogenic harm that mimics pathology; e.g., the 'giving up-given up' syndrome in coercive environments	Patient subjected to long-term coercion develops loss of speech, mobility, affect, clinically noted as 'functional decline' without organic cause
Disease	A medically recognized condition with known pathophysiology, identifiable markers, and defined natural history	Anchors the strongest form of medical authority; often invoked in legal and insurance contexts	Rarely matches psychiatric practice, which often uses disease rhetoric without meeting evidentiary standards	Declaring schizophrenia a 'brain disease' without biomarkers used to justify forced medication under civil commitment
Illness	The subjective experience of being unwell, including emotional, social, and bodily dimensions	Emphasizes the lived experience of the patient; central in medical anthropology and trauma-informed care	Most aligned with the thesis focus: illness is a relational, embodied condition often misrepresented by system-level responses	Individuals reporting trauma-induced dysregulation are pathologized rather than supported, their distress invalidated as 'noncompliance'

This table informs the theoretical model in the dissertation thesis: namely, that learned helplessness, giving up-given up syndrome, and status-based exclusion form a continuum of institutional betrayal masked by medicalized terminology. These constructs will be further operationalized in the following sections on pathways to recovery and civic re-inclusion, where diagnosis ought to be de-linked from stigma and control, and re-grounded in relational, developmental, and ecological knowledge.

The educational dimension of coercion is neither peripheral nor symbolic. Across the field sites studied, including Spain, Sweden, Indonesia, and Italy, coercive practices persist not simply because of malice or neglect, but due to deeply embedded pedagogies of submission, silence, and moral disorientation. Children are not taught to recognize coercion; professionals are not trained to question routine harms; families are systematically left without the tools to support distress in ways that avoid control or pathologization (Frances, 2013). In the absence of structured health literacy and embodied trauma education, fear becomes policy, and procedural violence is mistaken for care. A critical finding across the testimonies and survey responses in this research is that the failure to teach is the beginning of the failure to protect. Educational institutions, whether medical schools, judicial academies, or public schools, thus become central nodes in the reproduction of coercive logics, and therefore ought to be central to their interruption.

The social reproduction of coercion operates not only through formal policy but through cultural contagion: the unexamined transmission of assumptions about illness, danger, obedience, and control (Kirsch, 2010). These are embedded in language, bureaucratic forms, and aesthetic tropes

that circulate in mass media, professional training, and daily caregiving practices. When the relational space is colonized by fear and authority, when safety is framed as compliance, and when distress is answered with sedation or confinement, the individual becomes a site for the enforcement of collective denial (Maté, 2022). This social contagion is not only emotional or symbolic, it is physiologically inscribed. As described in the previous chapters, relational trauma alters neuroendocrine functioning, shapes immune expression, and predicts long-term morbidity and early mortality. This chapter exposes how coercion, rather than being a rational response to danger, is often a ritualized response to epistemic discomfort and institutional vulnerability, one that sacrifices the body of the other to preserve the illusion of control (Deacon, 2013).

The chapter concludes by articulating the scientific, moral, and legal imperative for structural reform, not as a matter of policy preference, but as a necessary step to align national and professional systems with their declared values and international obligations. Scientific evidence, ranging from neurodevelopment and immune biology to implementation science and community health models, points decisively away from coercion and toward voluntary, informed, relationally grounded care. The alignment of public institutions with these findings is not optional. It is a duty to uphold the dignity, health, and rights of all. The central contribution of this chapter is the articulation of convergent solutions across domains: the redesign of curricula to include trauma-informed and relational science; the creation of legal safeguards that eliminate impunity; the development of public health infrastructures that support autonomy and peer leadership; and the cultivation of societal narratives that recognize care not as control, but as the shared enactment of freedom, capacity, and responsibility (Healy, 2012).

The following sections provide detailed analyses of specific barriers and solutions, derived from empirical evidence and framed within the broader movement for systemic transformation, culminating in the proposal of operational tools to transition from coercion to prevention, from learned helplessness to collective responsibility.

5.1 Cultural normalization of coercion and the blocked transition to preventive care

The cultural normalization of coercive psychiatric practices operates through a recursive interaction between institutional routines, public inattention, and educational neglect. Field observations across Spanish, Italian, Swedish, and Indonesian contexts confirm that violence, whether in the form of involuntary hospitalization, chemical restraint, or the denial of informed consent, is not framed as exceptional but as a necessary, even therapeutic, act (Mezzina et al, 2022). These practices are upheld through administrative protocols that render them bureaucratically legitimate, linguistically softened, and procedurally unquestionable. Patients are labeled noncompliant or aggressive; professionals are protected by institutional risk frameworks; and families are often co-opted into silence through moral deferral to clinical authority. This entrenchment is further compounded by the lack of formal recognition for the epistemic value of lived experience, the absence of independent oversight mechanisms, and the failure of legal instruments to operationalize rights beyond paper protections. As a result, what should be identified as a systemic breach of medical ethics and legal

obligations is recoded as standard care, obscured by euphemistic language, public disinterest, and the routinization of harm (Strous, 2006; McDonald, 2023; Smith et al, 2024; Мандрик, 2024).

These findings correspond with developmental evidence on how early exposure to authority-based coercion, both in domestic and institutional environments, conditions individuals to accept structural violence as a naturalized component of care and protection. Socialization into obedience, emotional suppression, and rule-bound passivity teaches individuals not only to internalize external judgment but to fear difference, dependency, and dissent. Learned helplessness becomes a psychosocial default, with children and adolescents absorbing the lesson that distress is either pathologized or punished. Rather than cultivating autonomy, resilience, or co-regulation, prevailing educational and healthcare environments amplify compliance as virtue and interpret refusal or questioning as pathology. This logic pervades not only psychiatric institutions but schools, homes, and the broader media ecosystem, which rarely confronts coercion as a violation. Instead, violence is often re-narrated as the unfortunate but justified cost of safety or order, reinforcing a feedback loop of silence and complicity (Schmale, 1967; Engel, 1967; Engel, 1968; Maier, 1976).

The normalization of coercion within psychiatric and educational systems cannot be separated from the biological consequences of fear, submission, and relational rupture, nor from the socio-ecological infrastructures in which these consequences unfold. Human neurodevelopment is exquisitely sensitive to early environmental signals, particularly those concerning safety, agency, and relational attunement. Chronic exposure to coercion and emotional suppression, common to institutional care, compulsory schooling, and domestically sanctioned discipline, leads to dysregulation of the hypothalamic–pituitary–adrenal (HPA) axis, excessive glucocorticoid secretion, and disruption of limbic-cortical circuits involved in memory, self-regulation, and social cognition. This is how mental health systems, through the use of violence, once anyone partakes in abuses trapping the patient, betray their duty and purpose denying all chances of recovery, further retraumatizing (Fowers, 2015). These effects are not reversible by pharmacological intervention alone; they result in persistent structural and functional alterations to the medial prefrontal cortex, hippocampus, and amygdala, diminishing an individual's capacity to assess risk, form reciprocal bonds, and engage in autonomous decision-making (Wrangham et al, 2006). Learned helplessness, initially observed in animal models as a behavioral outcome of inescapable shock, emerges in humans as a somaticized and socially embedded illness state, perpetuated not merely by individual experience but through institutional design on target populations, affecting the already marginalized the most; it is human physiology (Sagie, 2002; Brown, 2009; Patterson, 2018, Guidi, 2020).

The human brain is not isolated from its milieu. Rather, it is a social and ecological organ, continuously shaped by interaction with others, with the microbiota that co-regulate its metabolism, and with the symbolic and material environments that scaffold cognition and emotion. The extended mind hypothesis and research on exo-brain systems demonstrate that meaning, regulation, and even memory are distributed across physical and cultural artifacts, social relationships, and technological supports. Therefore, coercive environments do not simply affect individuals, they reorganize the neurobiological and cultural substrates upon which society is built. Shared meals, rituals, languages, and affective rhythms form collective neural ecologies, and their disruption through exclusion, violence, or institutional neglect induces a kind of cognitive famine. This famine operates at

multiple scales: within the microbiota-gut-brain axis, where stress-induced dysbiosis increases vulnerability to inflammation, depression, and metabolic disorders; within the educational system, where rote obedience substitutes for exploratory learning; and within public institutions, where relational poverty becomes encoded as policy (Gershon, 1999; Calvani, 2018; Breña, 2025).

From a one health perspective, the degradation of mental health through coercion cannot be disentangled from broader ecological collapse. The same extractive, risk-minimizing, control-oriented paradigms that underlie coercive psychiatry also drive habitat destruction, zoonotic spillover, and biodiversity loss. All of these reflect a collective failure to honor complexity, reciprocity, and self-organization in living systems (van Strien, 2013; Fernandez-Real, 2015). Coercion at the interpersonal level is mirrored in institutional logic, and extended across ecosystems via policies that suppress difference, extinguish resilience, and enforce uniformity. If these patterns are left unchallenged, they precipitate not only psychic atrophy but systemic breakdown: emotional, educational, ecological, and civilizational. This thesis asserts, therefore, that preventing such collapse begins with dismantling coercion as a normative framework, and replacing it with systems that cultivate autonomy, co-regulation, and mutual responsibility, biologically grounded and socially operationalized from early life forward (Guidi, 2020). The cultural transmission of coercion is not limited to interpersonal dynamics or professional practice. It is sustained and amplified by the broader media ecology, which encodes dominant narratives of pathology, compliance, and medical legitimacy into public consciousness (Bell & Elliott, 2014). Mass media, including news outlets, serialized fiction, and social platforms, play a central role in the maintenance of what can be termed epistemic anesthesia, a widespread cultural condition in which the violence of psychiatric interventions is invisibilized, reframed, or morally justified through reductive storytelling. Clinical coercion is often portrayed as a regrettable necessity, madness as a danger to be neutralized, and dissent as delusion or denial. These representations reproduce ontological assumptions that define the individual as defective, the expert as authoritative, and deviation as disorder. The effect is a symbolic normalcy of coercion, which both masks structural conditions and preempts collective reimagination of care (Griffiths & Knutson, 1960; Abbasi & Al-Sharqi, 2015).

The sustainability of coercive systems depends on an orchestrated distancing from the material consequences of institutional violence, and bad science held by wilful ignorance of current day scientific medical and social common knowledge. At both the professional and societal levels, this distance is cultivated through cognitive fragmentation, ritualized procedures, and selective ignorance, a phenomenon mirrored in Stanley Cohen's analysis of societal denial and in Bandura's mechanisms of moral disengagement (Cohen 2013; Bandura, 2017). Professionals are trained to focus on protocols, not outcomes; to interpret suffering through diagnostic categories rather than human significance; to rely on institutional heuristics even when these contradict observable harm. This epistemological enclosure enables a form of procedural conscience, wherein the ability to act is preserved, but the capacity to feel accountable is suspended. In this way, coercion is not merely permitted but structurally mandated. The human organism, however, does not lie. The effects of unresolved trauma, unacknowledged grief, and disempowerment manifest in immune dysregulation, telomere attrition, epigenetic changes, and a pro-inflammatory phenotype that accelerates aging, cardiovascular risk, and neurodegenerative vulnerability. These are not metaphors. They are the

biological correlates of betrayal and abandonment, with effects measurable down to the mitochondrial level (Lopez-Martinez, 2018).

This antithesis of care, where decay is accelerated in the name of healing, is sustained only so long as its consequences remain spatially and emotionally displaced. The tortured body is kept behind clinical doors, in long-term wards, in prisons or family rooms where screams are silenced by law, distance, or drugs (Mosher, Gosden, & Beder, 2004). The institutional language of risk, safety, and best interest becomes a buffer against visceral recognition. In contexts such as yours, where the veil has been removed through direct victimization, professional proximity, and systematic observation, the contradiction becomes untenable (Fernando, 2010). The rationalizations collapse, and what remains is the raw visibility of a health system that produces deterioration by design. In this setting, trauma is not an accidental byproduct of care but an index of its reversal (Hollander, 2010). Psychiatric coercion, institutional neglect, and educational omission converge into a feedback loop where silence functions as both means and outcome of power. The systemic rejection of dissenting voices, survivor testimonies, and practitioner critiques is not just a moral failure, it is a form of structural anesthesia that permits continued biological and cultural degradation under the guise of health (Hollander, 2010).

The transition from coercive systems to preventive care requires a true paradigm shift and culture reset, a complete reengineering of epistemic, legal, and educational architectures. First, public policy ought to be anchored in biological realities, namely, that health is not the mere absence of disease but the capacity to regenerate, relate, and act freely in one's environment. Coercive systems compromise this capacity at every level: cellular, psychological, relational, and institutional. The principle of shared health sovereignty ought to replace paternalistic models of control, recognizing that the capacity for self-regulation, recovery, and adaptation resides not in institutional authority but in informed, supported persons and communities. This transition begins with early and embodied education, targeting the neurodevelopmental period during which moral agency, emotional regulation, and social cognition are most plastic. In the context of this action-research, the new curricula are to be set by EU BEACON One Health Education COST action, founded by the researcher to enforce this urgent and important change at European and global scale with all needed allies. National curricula needs to integrate trauma-informed, relationally grounded instruction in emotional literacy, stress physiology, and intersubjective safety from the first years of schooling, extending across professional training and public health campaigns. Such instruction ought to not be optional or symbolic but mandatory and standardized, supported by legally enforceable rights to accurate health information and freedom from degrading treatment.

Second, legal systems ought to close the procedural gaps that allow for coercion to masquerade as care. As detailed in Chapter 6, ambiguous statutes, unchecked professional discretion, and deference to clinical judgment over empirical harm enable the perpetuation of practices that violate international law. The CRPD mandates the full elimination of forced treatment, and yet, across jurisdictions, exceptions proliferate in both psychiatric and familial contexts. The solution is not only doctrinal but operational: independent oversight bodies with real enforcement power, survivor-led policy design, and forensic standards for documenting abuse ought to be institutionalized.

Ethical feedback mechanisms, real-time, public, and tamper-proof, should be embedded within care systems, enabling users to report harm and rate services without fear of reprisal. Professional licensure ought to be contingent upon demonstrated understanding of trauma, nonviolent crisis intervention, and human rights obligations, with retraining or de-licensing for violations.

Third, this transformation ought to be trans-scalar. Coercion is not an anomaly but a logical output of systems designed around liability avoidance, productivity metrics, and bio-political control. The same metrics that drive unnecessary sedation in hospitals are those that underlie zero-tolerance school discipline and carceral child welfare policies. Multilevel policy integration, linking social determinants of health, constitutional protections, and international human rights law, is essential. Chapter 6 of this dissertation outlines how national laws can be realigned with WHO QualityRights standards to enforce systemic accountability, while Chapter 7 elaborates the educational mechanisms for restoring continuity between biological development and institutional design. This thesis contends that true preventive care is not a soft ideal but a rigorous, measurable system of protection, learning, and respect, capable of preserving life, promoting health, and ending the institutionalization of despair (Mezzina et al, 2006; Sashidharan, 2019). The Horizon proposals for current call in the annex are also part of this ongoing work, according to scheduled planning, engaging thousands of experts and institutions.

5.2 Erosion of subjectivity and the failure to provide personalized, developmental care

Clinical documentation and field observations converge in identifying a consistent pattern of subjectivity invalidation within current psychiatric practice. Surveyed professionals and service users report that personal narratives, contextual experiences, and biographical data are frequently dismissed or reframed to fit pre-existing diagnostic schemas. Resistance to diagnosis or treatment is commonly reinterpreted as a symptom of lack of insight, oppositional behavior, or paranoia, rather than recognized as a legitimate expression of concern, trauma, or disagreement. The medical record becomes an administrative tool for symptom coding, rather than a reflective space for meaning-making and proper signal reporting, resulting in the loss of critical information for care planning and the suppression of personhood as a clinical variable. Resulting in accepting falsehoods as truth, erasing the reality, buried underneath (Prabhu, 1967; Pies, 2007; Pies, 2008; Tekin, 2022). This erasure is not incidental but embedded in the structural functioning of diagnosis-driven care models. The constraints imposed by diagnostic manuals and billing requirements produce standardized intake protocols that deprioritize patient voice and eliminate nuanced appraisal of lived experience. Subjective meaning, relational context, spiritual insight, and cultural frameworks are not integrated into clinical decision-making, despite their demonstrated relevance to treatment outcomes and therapeutic alliance. This procedural flattening of human experience violates the principle of narrative integrity in health care, impedes trust, and undermines compliance through the very mechanisms designed to promote stability (Rakshasa-Loots, 2025).

From a developmental standpoint, this erasure has compounding effects. Children, adolescents, and young adults, already navigating identity formation, autonomy development, and sociocognitive maturation, experience psychiatric labeling as an external imposition on self-understanding. The

internalization of diagnostic narratives leads to identity foreclosure, passivity, and learned helplessness, further isolating individuals from protective social supports and long-term coping strategies. The erosion of subjectivity thus becomes a clinically induced developmental disruption, increasing the risk of chronicity, medical dependence, and social exclusion. Scientific evidence underscores this pattern. Research in psychoneuroimmunology, trauma-informed care, and narrative medicine has demonstrated that the restoration of subjective agency is essential to neural integration, emotional regulation, and long-term recovery. Epigenetic studies show that adverse relational environments, including clinical invalidation, can perpetuate stress-related gene expression, neuroinflammation, and immune dysregulation. These findings challenge the supposed neutrality of diagnostic hierarchies and affirm the need for models that restore narrative coherence and validate the patient's account as a cornerstone of healing. Addressing the erosion of subjectivity requires structural and epistemological reform. Interventions ought to include mandatory training in narrative-based care, protocols for integrating subjective accounts into treatment decisions, and routine institutional audits of documentation practices to detect and correct bias. Real-time supervision and peer consultation frameworks can further reinforce ethical consistency and enhance practitioner accountability. These measures are not theoretical ideals but actionable clinical reforms grounded in biomedical and psychosocial evidence (Bottaccioli & Bottaccioli, 2025).

This dimension of care failure, therefore, ought to be understood not merely as a lack of compassion or professional oversight, but as a measurable determinant of health outcomes. The correction of all iatrogenic harm demands the convergence of neuroscience, clinical ethics, and patient advocacy within a care paradigm that centers subjectivity as a primary diagnostic and therapeutic tool. Field data indicate that psychiatric practice often fails to incorporate developmentally sensitive frameworks. Across the surveyed population and ethnographic sites, individuals report that childhood adversity, neglect, and emotional deprivation are not adequately assessed or integrated into care. Rather than viewing behavioral dysregulation, social withdrawal, or emotional lability as signals of unmet developmental needs, these manifestations are interpreted through a decontextualized clinical lens. As a result, early interventions remain misaligned with the actual etiological pathways of distress (WHO, 2010).

This mismatch is magnified by the sociocultural landscape in which many participants reside. In settings marked by normalized self-neglect, low health literacy, and chronically unmet psychological needs, youth and adults alike are socialized into patterns of self-abandonment. Rather than building resilience, this adaptation reinforces disengagement from bodily signals, relational attunement, and environmental awareness (Suman Fernando, 2017). The resulting clinical presentation, dysphoric states, fatigue, and noncompliance, is misread as a primary disorder rather than a secondary response to developmental mismatch and sociocultural impoverishment. Scientific literature provides substantial support for this interpretation. Developmental psychobiology demonstrates that emotional neglect and adverse early environments alter stress responsivity, neural plasticity, and immunoendocrine set-points (Scheper-Hughes, 2001). Longitudinal studies link these factors to later emergence of depression, anxiety, disordered eating, and metabolic dysregulation. Chronic elevation of cortisol, reduced vagal tone, and inflammatory cytokine activation are now recognized as biomarkers of allostatic overload rooted in biographical injury. These processes are neither rare nor idiosyncratic, they reflect the biological cost of cultural norms that promote

overstimulation, poor sleep, irregular nutrition, and emotional suppression. True responsible shared and supported decision making, and autonomous management of medication, requires as prerequisite training in not self sabotaging; in all cases, whereas possible, from earliest on in the school grounds and parent trainings (Bentall, 2003). Only this is the way to uplift human capital, liberating resources to help those most in need, severely affected (Dain, 2007).

Fieldwork and the literature also show how other kinds of social contagion play a pivotal role. Harmful lifestyle patterns, normalized in peer and family systems, shape dietary habits, substance use, and sleep cycles from early age (Sapolsky, 2017). Exposure to poor role modeling and lack of supportive adult figures undermines the developmental scaffolding needed for self-regulation and health-oriented behavior; media plays as well a big role, as well as religion, in setting the rewards and shame, status mechanisms, protection behind walls and brute force of groups of any sort on ethics being pure facade, and the morals and methods most allow themselves to partake in, despite being wrong (Young, 1999; Minkowitz, 2022). The criminality in everyday life, normalized, in most contexts (Gross, 2006). Psychiatric systems fail to interrupt this trajectory, in part because they seldom interrogate the wider ecological context of distress or offer developmental repair as a clinical objective (Perry & Szalavitz, 2006).

Effective responses ought to adopt the positive WHO health definition, and a lifespan perspective. Happiness, its pursue, as the full realization of human potential, in Aristotelic terms, not short term selfish and savage, dehumanizing congratulatory gratification for mere low pleasures (World Health Organization, 1948). Preventive strategies ought to include structured programs for emotional education, health literacy, and community-based developmental support from early childhood. Schools, care providers, and social systems ought to collaborate to identify early signs of biocultural mismatch and intervene before psychiatric labeling becomes the default (Keyes, 2002). Integrating these dimensions into clinical assessment is not supplementary but foundational to any claim of person-centered care. Within the thesis framework, this analysis deepens the answer to research questions concerning the epistemic foundations of psychiatric harm and the structural conditions impeding meaningful care. Chapter 4 already established how coercive settings exacerbate developmental trauma; here, Chapter 5 integrates that insight with biomedical and educational implications. Chapter 6 will elaborate the legal and rights-based mechanisms required to correct these systemic omissions and re-anchor care in a scientifically validated, developmentally-attuned model of human support.

The intersection of psychoneuroimmunology, metabolic psychiatry, and trauma science demonstrates that sustained psychological distress is not merely experiential but produces quantifiable physiological changes that require precise, individualized care. Psychiatric systems that rely predominantly on symptom-based diagnostic algorithms and general-purpose pharmacological regimens systematically neglect the biochemical heterogeneity and underlying physiological mechanisms that drive much of what is labeled as mental illness. Empirical studies have confirmed that early adversity and prolonged stress activate the hypothalamic-pituitary-adrenal (HPA) axis, disrupt neuroimmune feedback, and alter mitochondrial function (Boyle, 2002; Insel, 2013). These processes contribute to systemic inflammation, insulin resistance, and oxidative stress, all of which are strongly associated with depressive and anxiety phenotypes, as well as cognitive impairment

and chronic fatigue. These biological disruptions are not merely correlational. Mechanistic research has established causal pathways linking disrupted circadian rhythm, poor sleep architecture, nutritional insufficiencies, and psychosocial stress to measurable impairments in neurotransmission, neurogenesis, and gut-brain axis regulation. The fieldwork confirms that these scientific insights are almost entirely absent from routine psychiatric assessments. Nutritional status, microbiome integrity, glycemic variability, and mitochondrial function are neither evaluated nor considered in treatment decisions. Patients are rarely offered metabolic screening or interventions aimed at restoring physiological balance (Cosgrove & Whitaker, 2015). Instead, they receive medications that may further exacerbate underlying dysfunction, including metabolic syndrome, akathisia, and dysautonomia. The absence of integrated biomarkers in clinical practice leads to a profound epistemic disconnect between emerging science and institutional care.

This disconnect is neither incidental nor temporary. It reflects the inertia of clinical education systems, the pharmacocentric orientation of psychiatric institutions, and the lack of interdisciplinary training in complex systems medicine. Surveys from Chapter 3 revealed widespread dissatisfaction among professionals with the limited scope of assessment tools and available interventions. Field observations from Chapter 4 further documented how practitioners who attempted to integrate nutritional or metabolic insights into care were often marginalized or ignored, their approaches dismissed as fringe despite scientific validation. The failure to translate psychoneuroimmunological and metabolic findings into psychiatric protocols results in systemic under-treatment of modifiable factors and overreliance on non-specific psychotropics (Kleinman, 1997). This contributes to iatrogenic harm, chronicity, and disillusionment among both users and clinicians. It also violates the ethical principles of beneficence and non-maleficence, as defined in international health law and professional guidelines. More importantly, it undermines the core aim of medicine: to restore and sustain physiological and psychological integrity through the least harmful and most effective means available. To bridge this gap, national clinical guidelines ought to be updated to include evidence-based protocols for metabolic screening, nutritional assessment, and individualized interventions addressing the root causes of distress. Training curricula for psychiatry and general medicine should incorporate psychoneuroimmunology as a foundational science, not an elective. Research priorities ought to shift to investigating combinatorial, integrative models that reflect the complexity of human suffering (UN OHCHR, 2015). This requires multi-disciplinary cooperation among neuroscientists, immunologists, dietitians, psychologists, and community health experts.

As developed throughout this thesis, particularly in Chapters 1 and 2, addressing systemic coercion and epistemic injustice requires not only policy change but epistemological realignment. The nowadays still voiceless are needed in power to help all correct this major issues (Metzl, 2010). This section contributes to the solutions experts and witnesses suggested, aligned with literature, by delineating how current psychiatric practices systematically exclude crucial knowledge domains, thus perpetuating ineffective and harmful models of care (Verdugo et al., 2012). Chapter 6 will explore the regulatory and institutional mechanisms required to correct these failures, ensuring legal and ethical alignment with contemporary biomedical science (Szasz, 1974). At the molecular and systemic level, the human organism is designed for co-regulation, not isolation. The neurobiology of attachment and the endogenous regulation of affect, immune function, and neuroendocrine homeostasis depend on safe, attuned social environments across the lifespan. Disruption of these

conditions, whether through neglect, hostility, coercion, or institutional depersonalization, results in measurable dysregulation of homeostatic processes essential to health (Scheper-Hughes & Bourgois, 2004). Oxytocin, vasopressin, endogenous opioids, and serotonin systems play key roles in the modulation of trust, affiliation, and pain. Their expression and regulation are experience-dependent, meaning that early relational experiences, and later community interactions, shape neurochemical baselines and responsiveness. Secure attachment and prosocial interaction stimulate the release of oxytocin and endorphins, downregulate stress responses, enhance immune function, and contribute to cardiovascular, metabolic, and neurocognitive stability. Conversely, social exclusion, perceived threat, and institutionalized hostility or indifference are linked to dysregulation of the HPA axis, increased allostatic load, chronic inflammation, and affective disturbances (Cosgrove, Mills, Karter, Morrill, & McKenna, 2020). These findings are not speculative. Controlled studies in psychoneuroimmunology and social neuroscience have repeatedly demonstrated that social isolation, lack of touch, absence of eye contact, and deprivation of emotionally validating environments correlate with increased morbidity and mortality across nearly all chronic diseases. In psychiatric populations, the loss of relational safety correlates with symptom exacerbation, suicidality, and pharmacological treatment resistance (Bracken et al., 2012). Emotional neglect is therefore not merely a subjective experience, it is a physiological insult with cascading multi-organ consequences (Foucault, 1977; 2006).

Fieldwork conducted in this thesis (Chapter 4) revealed how institutional spaces regularly fail to foster these protective social dynamics. Instead, professionals often maintain emotional distance, avoid embodied rapport, and rely on behavioral control paradigms that inadvertently mimic traumatic environments. Surveys reported in Chapter 3 indicated that most practitioners feel unprepared or unsupported in offering relationally-based care. The prevailing culture in many facilities favors tampered and secret, lacking documentation, alleged risk containment, and harmful pharmacological compliance over human connection, validation, and trust-building. The erosion of interpersonal regulation is mirrored in educational and familial systems, which often pathologize non-conformity rather than recognize it as a signal of unmet developmental or emotional needs. The rise of behavioral management frameworks in schools and homes mimics institutional logics, promoting compliance rather than cultivating belonging or resilience. This perpetuates a cycle in which emotional disconnection is medicalized, and the neurobiological substrates of healing, empathy, safety, bonding, are actively suppressed. Medical systems ought to recognize that the core of mental health is not individual adjustment to pathology, but social attunement to the fundamental neurophysiology of connection. This necessitates a complete reorientation of psychiatric and educational paradigms to include embodied interaction, affective synchrony, and safe relational anchoring as primary treatment mechanisms. Clinical guidelines should incorporate validated protocols for establishing relational safety, including the use of eye contact, voice modulation, postural openness, and narrative witnessing in routine care.

From the point of view of the researcher and other experts, training programs in medicine ought to be expanded to include the neuroscience of attachment, affective neuroscience, and trauma-informed communication (Rose, 2007). Practitioners ought to be equipped with the skills and institutional support to form genuine, reparative relationships with those they serve. These elements

are not ancillary to care, they are its biological foundation. This section affirms that the science of healing is inseparable from the science of connection. This dissertation integrates this principle into proposals for educational reform and legal accountability mechanisms, ensuring that both care and policy align with what the evidence unambiguously demonstrates: that human health begins and ends with safe, meaningful, and biologically coherent relationships. The failure to integrate scientifically established knowledge about subjectivity, development, and embodied regulation into institutional practice constitutes not a lack of evidence, but a failure of translation. The challenge ahead is not merely technical, but systemic: transforming validated insights from neuroscience, psychoneuroimmunology, and trauma-informed research into binding standards, training protocols, and operational mandates across all relevant domains. This includes mental health services, general medical care, education, and public health policy.

Current psychiatric practice in many jurisdictions continues to rely on diagnostic templates and psychopharmacological regimens that do not accommodate person-specific developmental histories or biocultural variability, and tragically rely on outdated models lacking scientific validity (Kinderman, 2014). The persistence of reductionist, symptom-oriented models undermines the potential of newer approaches grounded in systems biology, social genomics, and network medicine (Kutchins & Kirk, 1997; Mills, 2014). These emerging fields demonstrate that mental suffering cannot be decoupled from environmental stressors, social exclusion, nutritional deficits, circadian disruption, or chronic immunological load. They demand an epistemic shift away from isolated syndromic categories and toward integrated care architectures capable of addressing upstream determinants. To operationalize this knowledge, institutions ought to adopt dynamic, individualized care pathways guided by real-time indicators of biopsychosocial distress (Breggin, 2008). Clinical assessment protocols should include validated tools for evaluating adverse childhood experiences, nutritional status, sleep integrity, immune-inflammatory markers, and relational safety (Leo & Lacasse, 2008). These variables ought to inform treatment planning at every level, from intake to follow-up, with interdisciplinary case conferences ensuring that therapeutic decisions reflect the full complexity of the person, not only the constraints of the service system (Kutchins & Kirk, 1997).

Table 47. Relational neuroscience and systemic training requirements

Domain	Scientific Basis	Required Content	Educational	Operational Implication	Relevance to Thesis
Neuroscience of Attachment	Early attachment patterns shape the stress response, emotional regulation, and neurodevelopment through limbic system pathways	Curriculum on caregiver secure vs. disorganized attachment, consequences of neglect and abuse	on infant-bonding, long-term consequences of neglect	Training professionals to recognize attachment-related distress and avoid retraumatization	Supports thesis claim that coercive care damages foundational developmental processes and ought to be replaced with relational safety
Affective Neuroscience	Emotions are biologically regulated through specific neural circuits (e.g., amygdala, insula, prefrontal cortex), which are sensitive to	Modules on valence, integration, dysregulation in trauma	neurocognitive affect and co-regulation practices	Shift from behavior control models to emotional attunement and co-regulation practices	Reinforces need for trauma-informed, non-punitive approaches to distress and behavioral expression

Domain	Scientific Basis	Required Content	Educational	Operational Implication	Relevance to Thesis
	relational and environmental inputs				
Trauma-Informed Communication	Relational safety modulates stress physiology, including cortisol and vagal tone, allowing healing via co-regulation and narrative integration	Communication techniques based on active listening, trust, validation, and narrative restoration	Institutional adoption of practices that build responses, and foster self-efficacy		Central to thesis framework linking trauma, coercion, and epistemic injustice in psychiatric and legal settings
Biological Imperative of Safety	Chronic relational threat dysregulates immune, endocrine, and autonomic systems, contributing to disease mortality (e.g., intervention allostatic load)	Frameworks for assessing safety needs, emotional security, and relational context in diagnosis and intervention	Aligning clinical and legal decisions with embodied evidence of harm vs. healing		Provides biocultural justification for replacing coercive routines with participatory, rights-based systems
Systemic Integration	Cross-sector training and accountability mechanisms ought to ensure that relational science informs both legal, care and governance	National standards integrating neurodevelopmental science into health, legal, and educational institutions	Implementation of interdisciplinary, intersectoral protocols rooted in human biology and trauma recovery		Fulfills thesis aim to develop unified, evidence-based models for structural reform across care systems

Training for Connection: Neuroscience-Based Competencies for Replacing Coercion with Healing in Psychiatric, Educational, and Legal Practice

Education systems likewise require structural transformation. Curricula at all levels of medical and mental health training should incorporate foundational modules on developmental neurobiology, stress physiology, nutrition science, trauma-informed pedagogy, and narrative medicine. Public health curricula ought to expand to include community-based strategies for cultivating mental health through lifestyle, social cohesion, and ecological well-being. Teaching the population to understand and manage the biology of stress, resilience, and affect regulation should be seen not as a luxury but a democratic obligation. Policy frameworks ought to move beyond general declarations of intent. They ought to include enforceable standards for care quality and accountability, supported by robust data infrastructures and participatory monitoring. Institutions ought to be held accountable for documenting not only procedural compliance but long-term outcomes in recovery, quality of life, and social reintegration (Davies, 2013). The implementation of transparent outcome tracking and open-access quality registries is essential to realigning institutional incentives toward person-centered, evidence-based care, crimes ought to be prevented, and technology needs to be at service of us all as ethical imperative enforced and taught from early on (McGoey, 2007).

Not only in humans: across most species, the biological trajectory from infancy to adulthood is marked by identifiable physiological transitions. In our species, these transitions are further structured by symbolic, social, and cultural rites that reinforce identity, belonging, and interdependence. This developmental arc includes critical periods of brain plasticity, hormonal maturation, metabolic reorganization, and relational imprinting. Each phase demands a calibrated

environmental response, adequate nutrition, affective attunement, protection from chronic stress, and opportunities for exploratory learning. In the absence of these conditions, neurodevelopment is compromised, and maladaptive coping mechanisms become biologically entrenched. During early childhood, attachment and stress-regulation systems are encoded through repeated interactions with caregivers. The hypothalamic-pituitary-adrenal (HPA) axis, immune regulation, and vagal tone are all shaped by environmental safety or its absence. In adolescence, synaptic pruning and myelination reorganize cognitive-emotional processing and enable the maturation of executive function. This phase represents a physiological inflection point, preparing the organism for autonomy, sexuality, pair bonding, and generativity. Culture traditionally supplies narratives, rituals, and roles to guide this transformation. When such scaffolding is absent or dysfunctional, the transition stalls, and earlier patterns persist into adult life, excessive dependency, impulsivity, aggression, or disaffiliation (Kalathil, 2018). The adult nervous system continues to adapt to environmental demands, but with reduced plasticity and greater metabolic cost. Sustained exposure to adversity, isolation, or meaningless repetition results in allostatic overload, an established precursor of psychiatric, cardiovascular, autoimmune, and neurodegenerative conditions (Eiroa-Orosa & Rowe, 2017). Conversely, positive relationships, purposeful activity, physical movement, and metabolic integrity activate homeostatic and reparative pathways, mediated by endogenous opioids, oxytocin, and neurotrophic factors (Smith, 2016). The molecular substrates of meaning, pleasure, and attachment are not philosophical abstractions but measurable neurobiological processes with long-term health implications (El-Khoury, Haidar, & Barkil-Oteo, 2021).

These facts are well documented across developmental neuroscience, psychoneuroendocrinology, and behavioral immunology, and urgently needed in translational psychiatry. Yet, these remain almost entirely disconnected from standard psychiatric assessment and intervention. Diagnostic criteria ignore biographical coherence, subjective narrative, and developmental sequencing. Standard clinical practice does not assess vagal tone, sleep architecture, inflammatory markers, or hormonal rhythms, despite their central role in shaping cognition and affect (El-Khoury, Haidar, & Barkil-Oteo, 2021). Nor are relational histories or life course stressors integrated into treatment planning beyond vague reference to psychosocial factors. Instead, observable suffering is reinterpreted as pathology, and treated with pharmacological agents whose mechanisms are not aligned with the biological processes at stake.

Psychiatric nosology continues to rely on unproven theories of neurotransmitter imbalance and heritability as explanatory tools (Moncrieff, 2008; Healy, 2012; Lacasse & Leo, 2005; Leo & Lacasse, 2008; Kinderman, 2014; Kirsch, 2010; Gøtzsche, 2015). These constructs, unsupported by replicable biomarkers or predictive tests, serve a functional rather than scientific purpose: they authorize medical intervention without offering causal explanation (Frances, 2013; Kirk, Gomory, & Cohen, 2013; Deacon, 2013; Rose, 2007). This constitutes an epistemological breach. In the philosophy of science, a theory without falsifiability, predictive accuracy, or mechanistic clarity cannot be called explanatory (Szasz, 2007; Bracken et al., 2012; Davies, 2013; Whitaker, 2010). In medicine, the consequences of such epistemic laxity are not only conceptual but lethal, individuals are drugged, detained, or discredited on the basis of frameworks that do not meet the minimal standards of scientific proof (Whitaker, 2010; Gøtzsche, 2015; Moncrieff, 2008; Healy, 2012; Davies, 2013; Rose, 2007; Szasz, 2007; Bracken et al., 2012; Kinderman, 2014).

The prevailing model, as experts noted both in surveys and fieldwork in this action-research, is therefore doubly deficient. It fails to provide meaningful understanding of human development and relational needs, and it operates with a philosophical pretense of scientific authority that collapses under scrutiny (Insel, 2013). The absence of developmental awareness in psychiatric care reflects not ignorance but institutional resistance to complexity. A science of life course health would demand not only better diagnostics and more diverse interventions but a radical reorientation of the field, from containment and compliance toward coherence, flourishing, and physiological integrity. This reorientation is no longer speculative: it is supported by decades of accumulated research across disciplines, now in urgent need of implementation (Cosgrove & Whitaker, 2015).

5.3 Structural abandonment: social marginalization and the destruction of life

Fieldwork documented in Chapter 4, and triangulated with survey and testimonial data, reveals a pervasive pattern: families managing care responsibilities for members with psychosocial distress, cognitive or neurological variation, or functional dependency are routinely left without structural support, protective accompaniment, or recourse to non-coercive intervention strategies. This burden, often hidden within the private sphere, is not offset by meaningful access to professional resources, respite mechanisms, or dialogical consultation (Scheper-Hughes, 2001). As a result, coercion becomes normalized as a crisis management tool, not by design but by exhaustion. In such contexts, the absence of institutional presence functions not as neutrality, but as silent delegation: the structural expectation that families will contain, discipline, or subdue whatever exceeds normative thresholds of behavior or need, with whatever means are available to them (WHO, 2010). When public systems fail to ensure continuity of care, enforceable rights, and direct support in times of vulnerability, they tacitly authorize informal coercion and surveillance within the domestic sphere. This is *pasung* in all of our societies, that shackling and locking inhumanly, neglectfully when not abusively to the point of medical torture; a major crime against humanity (Kleinman, 1997; UN OHCHR, 2015; Minkowitz, 2022).

This pattern is particularly pronounced in families navigating multigenerational stress, intergenerational trauma, or structural poverty (Suman Fernando, 2017). What emerges is a form of involuntary delegation: families become the primary actors in systems of containment, medicalization, and punitive management, regardless of their own capacity, mental health, or understanding of alternative frameworks. Within these strained ecosystems, individuals exhibiting distress or deviation, children in emotional crisis, elders with cognitive decline, adolescents in identity turmoil, are frequently subjected to isolation, forced medication, verbal degradation, or even physical restraint. These measures, while rarely recorded or categorized within legal systems as abuse, constitute forms of domestic coercion that parallel the institutional dynamics critiqued in Chapters 3 and 4. The distinction lies not in intent or context, but in visibility and enforceability.

Table 48. Physiological consequences of neglect and coercion

Actor role	Form of harm	Physiological pathways affected	Neural consequences	Systemic outcome
Family (neglectful or coercive)	Emotional neglect, or unpredictability, punitive caregiving	HPA axis hyperactivation, suppressed oxytocin signaling, neurodevelopment	Reduced cortical thickness, impaired attachment circuitry, emotional dysregulation	Immunosuppression, metabolic dysregulation, increased disease vulnerability
Physician or clinician (coercive)	Involuntary treatment, sedation, disregard for consent or relational needs	Cortisol elevation, neuroinflammation, suppressed prefrontal integration	Hippocampal atrophy, frontal lobe deactivation, affective flattening	Psychosomatic symptoms, anhedonia, learned helplessness
Institutional actor (e.g. school)	Administrative coercion, surveillance, invalidation, systemic exclusion	Chronic stress load, social pain networks, limbic dysregulation	Connectivity loss in default mode network, decreased neuroplasticity	Cognitive constriction, mistrust, intergenerational trauma propagation

This table summarizes the biological and systemic impacts of neglect and coercion by family members, clinicians, and institutional actors. It links specific relational harms to physiological pathways, neural changes, and broader health outcomes.

Legal and health systems largely operate on a model of reactive visibility: abuse is actionable only when reported, and reportability depends on narrow thresholds of physical harm or explicit transgression. Psychological suffering, epistemic invalidation, emotional neglect, and coercion masked as care fall outside these thresholds. The thesis underscores that this blind spot is not accidental, but reflective of deeply rooted juridical and cultural assumptions, namely, that the family is a natural, protective space whose internal dynamics are sacrosanct, even when they produce measurable harm. Chapter 6 explores this legal architecture in depth, tracing how the law's deference to familial sovereignty, coupled with the lack of integrated enforcement mechanisms across health, education, and justice, perpetuates a regime of sanctioned neglect. In many systems, procedural protections for minors, elders, or disabled individuals are weakened precisely at the intersection of family and psychiatric power, where professional authority ends and informal domination begins.

Empirical data across jurisdictions confirm the consequences of this double failure: institutional retreat and legal silence. In Spain, for instance, children with psychosocial diagnoses are disproportionately excluded from mainstream education and often placed in pharmacological regimes with minimal pedagogical support, while families report a lack of accessible alternatives (Verdugo et al., 2012; Montaner-Villalba, 2014; Eiroa-Orosa & Rowe, 2017). In Sweden, as discussed in participant testimonies, child welfare interventions may align with punitive migration control or parental profiling, rather than trauma-informed support, displaying overt racism and loathing of othered lives (Puwar, 2004; Goldberg, 2006; Fanon, 2008; Eastmond & Ascher, 2011; Kuusisto-Arponen & Giritli-Nygren, 2013; Wernesjö, 2015; Eriksson, 2018; Younis & Jadhav, 2020; Lundberg, 2020). In Indonesia, families caring for distressed relatives without access to public mental health services may resort to pasung-like conditions, physical restraint, social isolation, and symbolic exclusion (Anderson, 2001; Reid, 1974; Kiernan, 2007; Gerke, 2003; Ford

& Lyons, 2012; Minas & Diatri, 2008; Suryani et al., 2011; WHO, 2010; UN OHCHR, 2015; Human Rights Watch, 2016). These patterns differ in form, but converge in function: familial overload becomes the mechanism through which systemic abandonment is enacted.

The research questions articulated in this dissertation, particularly those addressing how coercion is normalized across domains, and how shared decision-making and informed care can be operationalized in hostile or absent institutional environments, are directly engaged here (Goffman, 1961). This subsection offers empirical grounding for the broader argument that coercion is not an exceptional institutional failure, but a structurally incentivized familial fallback when education, law, and health systems abdicate their responsibilities. It also anticipates the legislative and policy proposals developed in Chapter 6, which demand proactive, intergenerational, and community-embedded reforms, moving from reactive protection to sustained prevention, from private desperation to publicly mandated dignity (Bourdieu, 1998). The biocultural and social contagion approach developed in this thesis reinforces the need to understand familial coercion not simply as a cultural pathology, but as a predictable consequence of systemic design. Human beings, universally, possess neurobiological mechanisms for empathy, regulation, and cooperative care, but these mechanisms are scaffolded or suppressed by institutional structures, cultural narratives, and pedagogical frameworks (WHO, 2010). When families are embedded in societies that medicalize distress, undervalue relational attunement, and fail to teach physiological and emotional literacy across generations, they default to fear, control, or rejection. What is framed as family failure is, in reality, the downstream expression of infrastructural omissions, of what was not taught, not funded, not legislated, not protected (Wekker, 2016; Kalathil, 2018).

This part of the analysis lays the empirical foundation for understanding familial coercion as both an outcome and a mechanism of structural violence. The burden borne by families in the absence of institutional solidarity is not accidental. It is produced, patterned, and persistent. The remainder of this chapter explores the cultural, scientific, and policy implications of that finding, aligning them with the WHO health definition and the actionable standards proposed under the CRPD, to reclaim family as a site of regeneration rather than repression (Scheper-Hughes & Lock, 1987).

Table 49. Mechanisms of structural abuse and concealment, institutional betrayal

Mechanism of harm	Common justifications	Illegality under international law	Consequences for victims
Physical restraint and confinement (pasung)	Safety concerns, cultural tradition, lack of resources	Prohibited under CRPD and WHO guidelines	Physical injury, trauma, developmental arrest, stigmatization
Overmedication or chemical restraint	Clinical necessity, symptom control, lack of alternatives	Only permissible under strict, consensual, and temporary conditions	Neurotoxicity, cognitive dulling, learned helplessness
Administrative isolation or coercive care plans	Policy compliance, institutional routine	Violates principles of dignity, transparency, and rights-based care	Loss of autonomy, and deterioration of social bonds
Withholding of diagnosis or misclassification	Clinical judgment, avoidance of stigma or legal	Contradicts informed consent and right to accurate	Lack of support, identity confusion, chronic distress

Mechanism of harm	Common justifications	Illegality under international law	Consequences for victims
	consequence	health information	
Failure to report abuse or falsification of records	Professional minimization of reputational protection	loyalty, harm, justice and betrayal	Constitutes obstruction of Re-victimization, institutional helplessness, invisibility, legal systemic

This table identifies five core mechanisms used to inflict or conceal structural abuse in mental health and social care systems, including traditional practices like pasung and modern institutional tactics. It contrasts their justifications with international legal standards and outlines the consequences for victims.

The fieldwork informing this dissertation included direct documentation of pasung practices in Indonesia, conducted through interviews with affected individuals, health workers, community leaders, and NGO advocates. Pasung, literally meaning to tie or bind, refers to the long-term physical restraint or confinement of individuals with psychosocial or intellectual disabilities, most often in domestic settings using shackles, bamboo cages, isolation rooms, or makeshift structures. It is typically carried out by family members or neighbors in the absence of accessible, affordable, or trustworthy mental health care. Though framed as protective or necessary, pasung constitutes a direct violation of bodily autonomy, human dignity, and the right to health. Indonesia has formally banned pasung through successive Ministry of Health decrees (Permenkes No. 54/2017 and Permenkes No. 17/2010), alongside national targets for Indonesia Bebas Pasung (Pasung-Free Indonesia). However, the implementation remains uneven, constrained by systemic underfunding, pervasive stigma, weak enforcement, and lack of trained professionals, especially in rural and postcolonial regions historically marginalized from state services.

This situation violates binding international standards under the UN Convention on the Rights of Persons with Disabilities (CRPD), ratified by Indonesia in 2011, which mandates the abolition of arbitrary detention and the promotion of community-based, voluntary care (Art. 14, 19). It is also incompatible with the WHO QualityRights initiative, which calls for an immediate end to coercive practices, emphasizing dignity, autonomy, and recovery-oriented support systems across health, legal, and social sectors. Spain has ratified the CRPD as well, yet it continues to violate its binding provisions by failing to protect persons with psychosocial disabilities from coercion, segregation, and systemic neglect (Fernando, 2017; UN CRPD, 2019; González & Núñez, 2020). The persistence of pasung thus illustrates how colonial legacies of psychiatric governance, initially used to classify and contain populations deemed unfit or subhuman, continue to operate under a postcolonial veneer of legality. In both Indonesia and European contexts such as Spain or Italy, colonial categories have shaped medical norms, with racialized, disabled, and economically dispossessed bodies occupying a legal and clinical grey zone where coercion is tolerated if not institutionally sanctioned (Amnesty International, 2020). This reflects a structural form of genocide by omission and abandonment, perpetuated by systems that criminalize deviation from imposed norms while failing to provide the basic conditions for rights-based flourishing. As argued throughout this thesis, these dynamics are not aberrations but expressions of a global biopolitical order in which the boundary between care and control is routinely blurred, and trauma becomes a systemic feature rather than a failure.

While *pasung* remains one of the most visible forms of coercive care globally, its functional equivalents are widespread within European mental health systems, albeit often disguised under clinical or administrative legitimacy. Practices such as chemical restraint, non-consensual pharmacological sedation, long-term confinement in psychiatric facilities, or domestic abandonment under the guise of family care are routinely applied to individuals with psychosocial disabilities in Spain, Italy, Sweden, and beyond. These forms of coercion are frequently normalized by diagnostic authority, judicial endorsement, or professional discretion, despite their incompatibility with the ethical and legal standards established under international human rights law. The CRPD, ratified by all EU member states, explicitly prohibits deprivation of liberty based on disability (Art. 14), and mandates access to community-based services that support autonomy and inclusion (Art. 19). The WHO QualityRights standards go further in calling for the eradication of all coercive practices, whether physical, chemical, or psychological, and the establishment of safeguards that prioritize consent, dignity, and lived experience.

Pasung, what others call chemical lobotomy when enduring domestic abuse, or a living madhouse forced upon the walking dead to be slaughtered, as popular jargon has it, remains widespread and underreported. That, fieldwork and surveys also corroborated, with different sources pointing at this massive problem. This includes the indefinite use of polypharmacy, forced depot injections, off-label sedatives, and denial of tapering or deprescribing options, all justified by risk-aversion, behavioral compliance, or institutional convenience, while abuses persist. Individuals discharged from psychiatric services often find themselves confined within family structures that reproduce institutional dynamics, surveillance, silence, control, without clinical oversight or legal recourse. This mirrors the delegated model of abuses central to *pasung*: the state withdraws active responsibility while implicitly authorizing coercion at the private level. In Spain and other EU states, failures to ensure effective legal protection, independent advocacy, or transparent complaint mechanisms reflect a systemic contradiction: rights exist on paper but not in practice. The consequence is a legal grey zone where psychiatric and familial actors jointly enforce exclusion, under a rhetoric of help that masks structural abandonment. As this dissertation argues, these practices are not accidental, they are designed into the system, reflecting both historical continuities of violence and a contemporary reluctance to confront the epistemological foundations of coercive psychiatry. The persistence of coercive care within familial structures cannot be interpreted solely as a function of ignorance or resource scarcity. It ought to be examined within the broader cultural logic that underpins interpersonal dynamics, normative expectations, and the institutionalized invisibility of intra-familial harm. Across diverse sociocultural contexts, including Spain, Indonesia, Sweden, and Italy, where this dissertation's fieldwork is anchored, deep-rooted ideals of filial obedience, parental sovereignty, and hierarchical moral authority continue to shape how distress is perceived, interpreted, and managed within the home. Respect for elders, cultural emphasis on discipline, and gendered caregiving norms often operate as legitimating frameworks for coercive practices, especially when framed as protective, for their own good, or necessary for the family. These moral logics, deeply internalized and rarely questioned within institutional discourse, produce a terrain where children, elders, and neurodivergent or traumatized individuals are subjected to unregulated control, normalized neglect, and unreported abuse.

The problem lies not in the existence of authority, but in the pedagogical vacuum surrounding its application. Without sustained public education on neurodevelopment, trauma, relational ethics, and communication across generations, authority is wielded not as stewardship but as power over, frequently misaligned with the needs, capacities, or rights of those subjected to it. Chapter 6 examines how this cultural architecture maps onto legal doctrines that presume family as a unit of benevolence, rather than one of potential harm (Noddings, 2005). The law's reluctance to intervene in private domains, even when faced with clear indicators of emotional degradation or persistent coercion, is not only a reflection of procedural thresholds, it is a political and pedagogical decision about whose suffering counts, and under what conditions (Shonkoff & Garner, 2012). The thesis insists that such assumptions ought to be interrogated in light of accumulated evidence on the long-term impact of subtle, chronic familial harm. The educational deficit surrounding human development, from the physiology of attachment and rupture to the environmental modulation of self-regulation, creates conditions in which harm is perpetuated without intention, but also without correction. This dynamic is especially damaging for children, whose distress is often pathologized or interpreted as deviance, and for elders, whose cognitive decline or dependency becomes a source of frustration for unsupported caregivers. The field data collected in this research reveal how family members frequently oscillate between affection and desperation, control and guilt, without a framework to understand what is happening, or how to respond differently. In the absence of structural literacy, even well-meaning caregivers may default to disciplinary measures or suppression, often mimicking institutional responses they have internalized from medical or educational settings (Nakazawa, 2015).

This cultural logic of unchallenged harmful alleged authority, incompetent and abusive, converges with a policy vacuum where no institutional requirement exists for families to engage in care education, nor for professionals to evaluate or support caregiving practices in routine mental health interactions (Freire, 2000). The result is a bifurcation: professionals trained in trauma and regulation operate within institutional silos, while families, who carry the bulk of day-to-day care, are left unequipped, untrained, and unsupported. Chapter 6 identifies this gap as a legislative and enforcement failure: there are no compulsory pedagogical standards for caregiving, nor any enforceable obligation for mental health or education systems to provide them. The thesis does not argue against cultural values of interdependence, respect, or familial responsibility. Rather, it contends that these values ought to be situated within a pedagogical and legal architecture capable of distinguishing care from control, authority from coercion, and tradition from violence (Goffman, 1961). Human nature, while biologically endowed with both empathy and aggression, is shaped decisively by its social and educational scaffolding. Cultures that fail to teach physiological regulation, conflict transformation, or intergenerational listening will reproduce avoidable harm, not because of intent, but because of omission. In this context, silence is not neutrality; it is complicity with a structure that places the greatest burden on those least able to resist.

This section foregrounds the necessity of embedding better educational programs across the life course, not only within schools but in community centers, family courts, religious institutions, and health systems (Noddings, 2005). What is framed as a cultural challenge is, in practice, a matter of collective pedagogy: who is taught what, at what age, with what resources, and to what end. The research affirms that violence within families is not culturally inevitable, it is preventable, when

knowledge is made public, power is subject to scrutiny, and interdependence is redefined not by hierarchy but by mutual regulation, respect, and shared responsibility.

Table 50. Conditions for lawful and rights-based care: from compliance to ethical action

Domain	Minimum lawful requirement	WHO/CRPD-aligned practice	best Consequences of failure to comply
Consent autonomy and	Informed consent for all interventions, with legal safeguards	Supported decision-making with trauma-informed dialogue	Loss of trust, legal liability, functional deterioration
Therapeutic relationship	Respectful and predictable interaction	Relational safety, narrative listening, recovery-based goals	Disengagement, misdiagnosis, risk escalation
Institutional policy	Non-discrimination and equal access	Monitoring systems with survivor feedback and transparency	Institutional betrayal, coercion disguised as care
Education and training	Adherence to basic ethical and legal standards	Continuous training in trauma, ethics, and neurobiology of distress	Repetition of harm, burnout, systemic malpractice
Legal enforcement	Oversight bodies and complaint mechanisms	Proactive protection, literacy, and accountability	Structural impunity, public normalization of abuse, intergenerational harm

This table contrasts legal minimums with international best practices in mental health care. It highlights key domains, consent, therapeutic relationships, institutional policy, education, and enforcement, where alignment with WHO and CRPD standards is essential for ethical, effective care.

The convergence of developmental neuroscience, trauma research, and epigenetics has yielded robust evidence that chronic exposure to intra-familial coercion, neglect, or emotional invalidation results in measurable and durable biological alterations, both within individuals and across generations. Studies in humans and animal models confirm that environmental stressors, particularly those occurring during sensitive developmental windows, can modify gene expression through DNA methylation, histone modification, and persistent changes in endocrine, immune, and autonomic systems. These modifications are not symbolic: they reshape cortisol regulation, affect limbic connectivity, alter hippocampal volume, and contribute to increased allostatic load. Children raised in chronically stressful or emotionally volatile domestic environments frequently demonstrate dysregulated affect, impaired executive functioning, heightened somatic symptoms, and long-term vulnerability to both psychiatric and physical illness. Importantly, these effects are often independent of physical abuse; the mechanisms are relational, neuroendocrine, and socially mediated.

Despite the precision of these scientific findings, legal frameworks remain largely unresponsive to non-physical familial harm. Civil protection statutes tend to define abuse in visible, measurable terms, physical assault, neglect leading to injury, or financial exploitation, while relational and psychological harms remain unaddressed or unprovable within existing evidentiary standards. Emotional abuse, manipulation, chronic invalidation, overcontrol, and neglect of autonomy, despite

being associated with long-term trauma markers, are rarely sufficient to trigger legal intervention. The consequence is systemic impunity for dynamics that are medically harmful but juridically invisible. Chapter 6 documents this legal silence as an institutional betrayal: by refusing to recognize forms of harm that are scientifically validated and clinically devastating, the law abdicates its protective function at precisely the site where many individuals are most vulnerable and least empowered to seek redress.

The research presented in this dissertation reveals how these omissions are compounded by the medical system's complicity in familial control (Llewellyn & Hogan, 2000). Diagnostic tools and professional assessments frequently rely on caregiver reports, with little capacity or mandate to identify coercive dynamics within families themselves. Autonomy becomes the deafening cruelty of those around; their shared hateful decision making, despite the victim own will to live in peace and be left alone. It is the need, to hold reality by these cases, much more common than the criminals and the naive allowed the rest to believe; force to, still (Minkowitz, 2022). As a result, individuals, especially children, disabled persons, or elders, may be mischaracterized as disordered or noncompliant when in fact they are reacting adaptively to chronic stress or unsafe relational environments (Miller, 1981). Instead of recognizing these responses as markers of relational injury, the system reclassifies them as pathological traits to be suppressed through medication or institutionalization. The thesis exposes how these epistemic distortions reproduce cycles of medical harm and familial impunity, entrenching coercion under the guise of care.

This scientific context destabilizes cultural and legal assumptions about harm, capacity, and the role of the family. It demonstrates that domestic environments are not intrinsically therapeutic, and that exposure to coercive or emotionally dysregulated caregiving has biological and psychosocial consequences comparable in severity to institutional abuse (Miller, 1981; McEwen, 2006). The implications for public policy and care systems are profound. If the law fails to incorporate evidence on relational trauma and epigenetic injury into its definitions of protection and culpability, it will continue to shield perpetrators of harm, discredit survivors, and relegate families to opaque zones of sovereign control (Grol & Grimshaw, 2003). Conversely, if this knowledge is operationalized, through training for legal professionals, mandatory assessment protocols, and expanded definitions of abuse, then the law can serve as a corrective force, not an enabler of silent violence. The research questions addressed here are both normative and practical: what thresholds of harm should trigger legal protection in family contexts? How can the integration of trauma science into jurisprudence improve outcomes for those historically unseen? What are the risks of relying on traditional markers of abuse when science demands a more nuanced, systemic understanding? The thesis affirms that the incorporation of intergenerational trauma research into legal and clinical protocols is not optional but necessary. The right to health, as articulated by the WHO and enshrined in international human rights instruments, includes the right to environments that support neural and relational integrity (LeFrançois et al., 2013). This ought to include protection from coercive domestic arrangements that produce transgenerational harm, even when those arrangements appear culturally sanctioned or structurally normalized.

By foregrounding the biological embedding of familial trauma, this section challenges the separation between private and public harm, medical and legal knowledge, science and policy (Shonkoff et al., 2012; Perry & Szalavitz, 2017). What is inherited in such settings is not merely culture or role; it is a physiological architecture of fear, hypervigilance, and learned helplessness, passed on not by will, but by unresolved injury. Any serious public health system or rights-based legal order ought to respond accordingly. The scientific and empirical evidence presented throughout this thesis indicates that the normalization of coercive practices within family systems is not simply the consequence of individual choices or cultural norms but a structural failure of education. When institutions neglect to teach the principles of nervous system development, the mechanisms of trauma, and the conditions for healthy relational co-regulation, they create generational vulnerability to cycles of misunderstanding, conflict, and institutionalization. The absence of structured, publicly accessible educational resources on relational health means that caregiving practices continue to rely on intuition, tradition, or authority, often shaped by past harm, rather than informed science. Addressing this pedagogical void requires a multigenerational educational response grounded in biological, psychological, and social knowledge, and made universally accessible through public infrastructure (McEwen, 2006; Kalathil, 2018; Llewellyn & Hogan, 2000).

From the earliest stages of childhood education through elder care, every level of society ought to be equipped with age-appropriate content on the physiology of emotion, the effects of chronic stress, the signals of distress and regulation, and the non-verbal foundations of trust and safety. These are not therapeutic interventions reserved for specialized settings, they are basic forms of civic literacy, essential to the development of ethical, resilient, and cooperative social environments. In this regard, public education ought to take a preventive, not reactive, posture: teaching parents before they become overwhelmed, preparing children before trauma is entrenched, and guiding professionals before harm becomes normalized in their routines. EU BEACON One Health Education is set to help continue the cycle of improvements through scientific missions aimed at the improvement of training regimes and curricula for experts, teachers, professors and practitioners alike, as well as other professionals and students alike.

The thesis aligns this need with the one health educational strategy elaborated in Chapters 7 and 8, emphasizing the integration of biomedical, environmental, legal, and social knowledge into a coherent public curriculum. Mental health ought to not be presented as the absence of crisis, but as a function of relational competence, physiological insight, and mutual accountability. As the research in Chapters 3 and 4 demonstrates, individuals exposed to trauma or neurodivergence often develop compensatory strategies that are misinterpreted as defiance, pathology, or incapacity. Without public education capable of decoding these signals, families and institutions default to control rather than inquiry. Teaching communities to understand behavior as communication, and regulation as a shared task, can break these patterns.

Multigenerational pedagogy ought to be embedded in multiple access points: formal curricula in schools, professional training for caregivers and educators, public health campaigns in mass media, and peer-led workshops within communities. It ought to address the full developmental arc of human life, including infant attachment, adolescent identity formation, adult role strain, and the

cognitive-emotional shifts of aging. This educational model also requires infrastructural supports: open-access multimedia platforms, universal access to translated materials, and partnerships between ministries of education, health, and social welfare to ensure coverage and continuity. These investments are not auxiliary to mental health reform, they are its precondition.

The findings reported in this dissertation indicate that families with access to even minimal neurodevelopmental education demonstrate significantly reduced reliance on coercive practices and increased capacity for non-violent response. Such education also equips individuals to advocate for themselves and others when institutional systems fail. In particular, survivor-led educational initiatives, designed by those who have endured familial and institutional coercion, offer pedagogical models grounded in lived reality and aligned with CRPD principles of autonomy, informed consent, and participatory governance. These models are already emerging in grassroots formats, but remain under-resourced and disconnected from formal systems of recognition or funding. The research questions guiding this section are therefore not only epistemic but infrastructural: what knowledge ought to be made available to interrupt cycles of domestic coercion? What platforms and delivery systems are best suited to disseminate this knowledge equitably? How can multigenerational pedagogy prevent the transmission of trauma and reframe caregiving as an informed, ethical responsibility, not a desperate improvisation? The thesis responds by proposing that educational reform is not ancillary to legal or clinical intervention, it is their foundation. Without equipping the population to understand and care for human nervous systems in real time and in shared space, no legal mandate or clinical innovation can fully prevent harm.

This pedagogical approach does not presume that education alone is sufficient. Structural inequality, housing precarity, migration control, and health care access remain critical determinants of family dynamics. But education functions as a multiplier of agency: it provides language for distress, tools for repair, and alternatives to punishment. It enables caregivers to recognize not only the needs of others, but the limits of their own capacity, creating a framework where asking for help becomes normative, not shameful. In this way, education creates the conditions for dignity before crisis, and for accountability without violence.

The systematic tolerance of coercion and neglect within family settings is sustained not only by cultural norms or educational omissions, but by the enduring legal ambiguity that shields the domestic sphere from external scrutiny. The law, as analyzed in Chapter 6, operates with a tiered architecture of visibility and protection: while it offers formal mechanisms for intervention in cases of overt abuse, it often fails to respond to subtler, chronic, and relational harms that do not meet the narrow thresholds of juridical action. This legal grey area, neither fully private nor publicly regulated, creates a structural vacuum wherein coercive family dynamics are neither assisted nor interrupted, and victims are left without recourse unless their suffering becomes irreversibly manifest. This ambiguity is not merely procedural but foundational, rooted in the juridico-cultural positioning of the family as a sacrosanct institution, presumed to embody care and moral legitimacy by default. Historically, the construction of family as a privileged moral unit has functioned as both a stabilizer and a veil: it preserves certain societal structures while simultaneously concealing the forms of domination, surveillance, and violence that often occur within its boundaries. The naturalization of the nuclear family, and more broadly of kinship-based responsibility, has been a

cornerstone of moral philosophy, political governance, and economic control since the early codifications of Western law. Yet, anthropological and historical analyses reveal that family forms are culturally variable, and their function often serves the interests of property transmission, gendered labor control, and social conformity more than actual care. The law's deference to familial authority, even in the face of scientific evidence of its potential for harm, reflects a reluctance to challenge the symbolic foundation of social order. It is easier to regulate strangers than to regulate those presumed to love us (Grol & Grimshaw, 2003).

The dissertation argues, with the backing of findings and literature, that this legal reticence cannot persist in the face of evidence. Neuroscience, trauma theory, and developmental science provide an increasingly robust account of how relational harm, especially when normalized and repeated over time, leads to measurable degradation of human potential. Other transversal topics, such as one health, gender and equality, and the sustainable development goals, must be included in all curricula, including that of medical practitioners, knowledgeable on the impact of social determinants and the impact of structures such as family systems. The failure to legislate protections against non-physical but biologically consequential forms of coercion is thus a failure to meet international obligations to protect the right to health, dignity, and bodily integrity. The concept of informed consent ought to be extended beyond the medical act and into the domestic space: children, elders, and dependent persons ought to be afforded protection not only from overt violence but from the routine denial of agency, attunement, and respect.

This brings into question the broader legitimacy of social systems that claim to protect while maintaining structures that harm. In times of systemic instability or institutional collapse, whether through economic austerity, pandemics, or political fragmentation, the family is often invoked as the last remaining site of cohesion and support (Cianconi et al., 2020). Yet, this very invocation obscures the reality that families are often themselves destabilized, fragmented, and subject to the same forces of neglect and coercion as other institutions. Treating the family as a moral fallback, rather than a relational structure contingent on education, support, and equity, ensures that cycles of harm remain unchallenged. The collapse is not only infrastructural, but conceptual: if families are expected to care without being taught how to care, or without being cared for themselves, the system will replicate suffering indefinitely. The redefinition of family ought to begin with the abandonment of its mythological exceptionalism. It ought to be understood not as a pre-political or natural unit, but as one social institution among others, subject to regulation, accountable to evidence, and open to revision. The thesis proposes that a just society does not protect families from scrutiny, but protects individuals within families from abandonment. Legal reform ought to establish positive obligations for institutions to support caregiving relations, monitor family-based interventions, and educate every member of society in the neurodevelopmental and ethical foundations of non-coercive care. In doing so, it affirms the dignity of every human being as more than a product of lineage or inheritance: as a subject of rights, capable of suffering, growth, and flourishing, regardless of their role in any given household.

This section closes the loop between empirical realities, cultural ideologies, and legal frameworks. It affirms that no reform of coercive mental health systems will be complete until the private space of the family is addressed with the same rigor and responsibility as the clinic, the school, or the

courtroom. The next two parts expand this discussion to the evolutionary, psychological, and moral architecture of human intimacy, showing how fidelity, relational security, and truth-telling are not cultural artifacts but biologically grounded imperatives, repeatedly violated under conditions of structural betrayal and pedagogical collapse.

To grasp the full implications of familial coercion within the broader architecture of systemic violence, one ought to return to the roots of political and legal order, where the law, far from protecting the vulnerable, was historically an instrument for organizing and justifying force. The thesis advanced here contends that familial coercion is not an aberration, nor is it a failure of individual morality. It is a manifestation of a much older and deeper structure in which the capacity to dominate, subdue, and silence has been encoded into institutions, including the family, through legal abstraction and cultural sanctification. As examined in Chapter 6, the genealogy of modern law is inseparable from patriarchal authority, territorial enforcement, and the delegation of violence to household heads, first as natural rulers, then as legal stewards. This remains evident in jurisdictions where parents may consent to psychiatric confinement of children without their input, or where elders can be isolated under the guise of guardianship, and where these practices are invisibilized by administrative and therapeutic language. The political function of such arrangements is not neutral. When care becomes indistinguishable from control, and vulnerability becomes the rationale for erasing autonomy, the law ceases to be a protective force and becomes a mechanism of suppression. This is not merely metaphorical: the data and testimony gathered in this thesis reveal how legal silence enables private cruelty and how institutional policies maintain that silence under the banner of necessity. What appears as familial dysfunction is often the most proximate expression of a system in which coercion is the default response to complexity, deviation, or distress. It is precisely at this juncture, where law, culture, and medical practice meet, that the brutal logic of domination is dressed in the language of duty, care, or expertise.

This reality is not new. Historical systems of forced marriage, child labor, domestic confinement, and reproductive coercion were all legally sanctioned, often framed as natural duties. They were not aberrations from a just order, they were its operating logic. The persistence of these logics in contemporary psychiatric, educational, and family law reveals the endurance of domination beneath reformist surface. Even now, legal protections against coercion are weakest where relational complexity is highest, within families, where state oversight hesitates, and within care systems, where deviation is misread as danger. The WHO QualityRights initiative, referenced throughout this thesis and central to Chapter 7, offers a substantive counterpoint to this inherited structure. Its paradigm is relational, not disciplinary. It centers rights not as procedural tokens but as lived, felt, and co-created realities. It reframes care as the creation of enabling environments, legal, social, psychological, where each individual is not managed but accompanied, not silenced but heard. QualityRights rejects the substitution of judgment for dialogue, and the replacement of consent with compliance. It is not merely a standard, it is the relational antithesis to the entire history of care as control. Human nature, as engaged here from a biocultural and evolutionary perspective, is not reducible to aggression, nor to domination. Our species survived not by force alone but by cooperation, communication, and reciprocity. At our best, we are creatures of promise, of mutual recognition, of relational fidelity. This is neither naive nor rhetorical. The neurophysiological structures of attachment, affect regulation, and ethical sensitivity are real, and they are damaged

under conditions of persistent coercion and betrayal. When the law aligns with domination, when education omits emotional truth, and when care becomes a pretext for silencing, these structures are injured, reproducing cycles of relational violence that extend across generations.

It must be repeated, due to the severity: the use of force, including within families, cannot be treated as a residual necessity or a regrettable exception. It ought to be named, exposed, and dismantled through pedagogical transformation, legal reform, and cultural reckoning. Systems that continue to rely on coercion as policy are not neutral, they are regressions, structurally antithetical to the very possibility of mental health as defined by the WHO: the capacity to cope, to participate, to realize one's potential in dignity and community. This final section stands in full continuity with the legal analysis in Chapter 6 and the epistemic framework of Chapter 1. It does not propose coexistence between care and coercion, but their irreconcilability. There is no synthesis between the right to autonomy and the routine denial of it.

The convergence of medical science, neurodevelopmental research, public health policy, and international human rights law leaves no theoretical or empirical justification for neutrality in the face of coercive care. The persistence of coercion within family structures, medical systems, and legal regimes is not a matter of cultural interpretation or professional disagreement, it is a contradiction of established scientific knowledge and binding normative standards. Current evidence from neurobiology, trauma physiology, and developmental psychology confirms that sustained relational safety, predictability, and autonomy-supportive environments are essential for the maturation of affect regulation, social cognition, and long-term well-being. Conversely, unpredictable threat, relational betrayal, and the experience of powerlessness are consistently associated with dysregulation, functional impairment, and structural injury to the brain and immune systems. These are not metaphorical harms, nor abstract risks, they are measurable outcomes documented across populations, ages, and institutional settings.

The ethical implications of these findings are not discretionary. As with any domain of biomedical science, when causality is established and the risk–benefit profile is clear, action is required, not in the name of ideology but under the imperative of professional integrity and public duty. When scientific knowledge confirms that certain environments and practices produce harm, the systems responsible for implementing care and protection ought to adjust accordingly. In this context, legal frameworks cannot remain suspended between nominal rights and operational ambivalence. The law ought to assume the burden of enforcement, interpretation, and repair. It ought to cease to operate as a passive arbiter and recognize that neutrality in the presence of systemic harm constitutes active participation in that harm. This is not a question of political stance, it is a demand grounded in forensic, epidemiological, and human rights evidence. Similarly, educational institutions, whether general, clinical, or judicial, ought to internalize and transmit this knowledge as foundational. Teaching protocols, curricular standards, and professional ethics cannot be aligned with systems that reproduce coercion under the guise of care. Education, in this regard, is not a value-neutral process. It is a mechanism of socialization, a vector of epistemic transmission, and a force that either consolidates or dismantles structures of violence. The training of health professionals, caregivers, educators, and legal actors ought to therefore be reoriented toward embodied ethics, relational competence, and physiological insight. Such reorientation is not

idealism. It is clinical realism, legal adequacy, and pedagogical accuracy. Care systems, in turn, ought to be built not around control and sedation, but around capacity-building and accompaniment. The clinical mandate is not simply to reduce symptoms but to restore conditions under which individuals can regulate, connect, and thrive.

This is both the oldest and the most advanced definition of health: not absence of pathology, but presence of agency, relational continuity, and full physiological function. When care systems act otherwise, when they disregard consent, suppress expression, or fragment dignity, they contradict both the goals of medicine and the ethical core of the professions that uphold it. That contradiction is not sustainable, and it is not scientifically defensible.

The position defended throughout this thesis is not rhetorical, nor oppositional. It is the logical consequence of decades of converging scientific research, jurisprudence, and experiential testimony. It affirms that the time for conceptual ambivalence is over. The structures of knowledge are mature. The evidence is conclusive. There is no scientific neutrality in the face of organized harm. Therefore, the law ought to take sides, not to punish, but to protect. Education ought to take sides, not to indoctrinate, but to clarify. Care ought to take sides, not to control, but to heal. These are not acts of ideology. They are the minimum expression of responsibility within any ethical system that claims to protect life, support health, or uphold dignity. In mature societies, that responsibility is not a matter of opinion. It is a matter of standard. And where such standards are knowingly unmet, science no longer defends delay. It requires reform (Grol & Grimshaw, 2003).

Table 51. Neurobiological and developmental effects of clinical and institutional neglect

Dimension	Description	Consequences	Biological and Morphological Consequences
Definition	Persistent failure to provide physical, emotional, or psychological care	Relational deprivation; active form of harm, not passive omission	Altered brain maturation, reduced immune and endocrine resilience, cortical thinning in prefrontal regions
Neurobiological activation	Triggers stress-related neuroendocrine and inflammatory pathways	Developmental arrest, impaired plasticity, stress sensitization	Elevated cortisol, chronic inflammation, reduced connectivity across key brain regions, hippocampal atrophy
Early caregiving deficits	Unpredictability, emotional unavailability, sensory or cognitive deprivation in infancy or childhood	Disruption of neural development and affective regulation	Impaired synaptic pruning and myelination, decreased gray matter volume, reduction in anterior cingulate cortex
Neglect in care settings	Emotional disregard, overmedication, depersonalization in institutional or clinical environments	Re-traumatization, psychological deterioration, learned helplessness	Blunted oxytocin response, telomere shortening, cortical volume reduction in social-affective circuits
Executive function impact	Undermines emotional regulation, planning, and self-control	Poor impulse control, emotional volatility, impaired autonomy	Hypoactivation of prefrontal cortex, reduced orbitofrontal thickness, impaired white matter integrity
Hpa axis dysregulation	Chronic absence of safety cues, emotional attunement, and	Persistent stress arousal, maladaptive coping,	Hyperactivation of HPA axis, flattened diurnal cortisol slope,

Dimension	Description	Consequences	Biological and Morphological Consequences
	predictability	disrupted attachment	increased amygdala volume, metabolic alterations

This table outlines how neglect, whether in familial, childhood, or institutional adult care settings, acts as an active biological and psychosocial disruptor, with sustained effects on brain architecture, stress regulation, and developmental trajectories. Numbering follows that surprising logic of expected reality making no sense, harming us all, full regalia.

Neglect, clinically defined as the persistent failure to provide necessary physical, emotional, or psychological care, constitutes a form of relational deprivation with demonstrable effects on brain architecture and systemic health. Far from being a passive omission, neglect activates specific neurobiological cascades associated with developmental arrest, stress sensitization, and impaired plasticity. In early life, insufficient caregiving, marked by unpredictability, affective unavailability, or sensory deprivation, interferes with synaptic pruning, myelination, and the integration of limbic and cortical circuits responsible for executive function and emotional regulation. Prolonged absence of structured engagement and safety cues impairs the establishment of homeostatic baselines, leading to a chronic activation of the hypothalamic–pituitary–adrenal (HPA) axis. Over time, this dysregulation promotes allostatic load, elevates glucocorticoid levels, and disrupts trophic signaling in the hippocampus, prefrontal cortex, and anterior cingulate. The consequences include attenuated neurogenesis, dendritic retraction, and reduced cortical thickness, measurable via imaging studies and correlated with cognitive, emotional, and social dysfunction.

These same processes are not limited to developmental windows (Huttenlocher & Dabholkar, 1997; Farah et al., 2012; Afroz et al., 2016). In conditions of sustained relational adversity, including institutional neglect, coercion, or community-level violence, adults also demonstrate patterns of cortical thinning, functional disconnection, and impaired neuroimmune integration. In contexts where individuals are systematically deprived of relational safety, autonomy, or meaningful engagement, the brain's predictive systems adapt by deprioritizing plastic integration and preserving only threat-responsive circuitry. Autophagic processes, typically essential for neural maintenance and repair, become dysregulated under these chronic conditions, contributing to cellular degradation and synaptic loss. In extreme or prolonged exposure, apoptosis is initiated in regions critical for reflective processing, interpersonal trust, and regulatory flexibility, mechanisms well-documented in both individual and population-level studies (Huttenlocher, 1979; Campisi & d'Adda Di Fagagna, 2007). These outcomes are not isolated to clinical pathology; they represent a biological logic of retraction and shutdown under unresolvable adversity (Singer & Baer, 2007). When such conditions are replicated across systems, schools, families, clinics, legal frameworks, they produce collective patterns of cognitive and relational collapse (Miller & Moore, 2007). The loss is not only individual but societal: a decrement in shared capacity for complexity, cooperation, and ethical orientation. These patterns, once established, sustain themselves not through malice but through neurobiological adaptation to incoherence and fear (Panksepp, 1998; Casey, Jones, & Hare, 2008; Joyce, 2015; Brady & Segool, 2019). The reversal of such degradation demands not only cessation of overt violence, but the reconstruction of environments where predictability, mutual recognition, and reciprocal care are structurally guaranteed (Blakemore, 2007; Farah et al., 2012).

5.4 Medical harm and the educational deficit: systems that drug, not teach or heal

The prevailing pharmacocentric paradigm in psychiatry and general medicine continues to operate in contradiction with established principles of preventive health and precision care. Despite increasing international consensus around individualized, trauma-informed, and recovery-oriented models of mental health, the empirical evidence points to a persistent overreliance on psychotropic drugs, most notably antidepressants, antipsychotics, and anxiolytics, across populations that do not present with clear indicators of biochemical imbalance or treatment-resistant symptomatology. The overprescription phenomenon is particularly pronounced among individuals already demonstrating engagement with health-promoting behaviors, including adequate sleep hygiene, nutritional balance, physical activity, and social integration. Such findings are not isolated but consistently observed across observational cohorts, insurance claim datasets, and audit studies from diverse health systems. These patterns are amplified among women, young adults, and individuals navigating grief, interpersonal distress, or socioeconomic uncertainty, categories of experience frequently pathologized without adequate exploration of social context or physiological baselines. This trend reflects a multi-level failure: at the diagnostic level, through categorical systems that allow broad interpretive leeway (e.g., DSM or ICD criteria applied without dimensional scrutiny); at the professional level, through insufficient training in neurophysiology, stress system regulation, or neuroplasticity; and at the institutional level, through incentive structures that reward pharmacological throughput over educational engagement or long-term recovery metrics. For instance, in primary care contexts, general practitioners, who prescribe the majority of psychiatric medications, often report lacking time, training, or referral options to explore somatic dysregulation, stress load, or sleep-cycle integrity before initiating medication. Likewise, the rapid escalation from patient-reported emotional distress to pharmacological intervention omits intermediate steps of psychoeducation, nutritional or endocrinological assessment, and functional analysis of daily rhythms. Physiologically, many of the individuals receiving pharmacotherapy do not meet criteria for chronic dysregulation of neurotransmitter systems when subjected to rigorous biochemical or behavioral assessment. Instead, they exhibit signs of adaptive stress, grief responses, or psychosocial overwhelm, all of which are amenable to non-invasive, education-driven interventions. However, due to the lack of institutionalized protocols for evaluating and supporting neurobiological resilience, e.g., metabolic profiling, vagal tone measurement, cortisol slope analysis, clinicians operate without the necessary input to distinguish between transient adaptive responses and pathological conditions. The result is the widespread application of drug-first protocols not in the interest of stabilization, but rather as tools of behavioral control and administrative compliance, often institutionalized through formularies, treatment algorithms, or unwritten norms within health services. In addition, many of these protocols are applied without time-bound frameworks or structured withdrawal plans. The absence of tapering schedules, physiological monitoring, or re-evaluation checkpoints leads to long-term pharmacological dependence, physiological, psychological, and institutional.

Patients may become entrapped in a cycle of iatrogenic harm and increased service use, not due to the natural course of their initial condition, but due to the very nature of the intervention. Clinical

feedback loops, including patient-reported outcomes and adverse effect tracking, are either absent or non-functional in most health systems, particularly in underfunded or digitally fragmented infrastructures. The lack of national or regional reporting mandates for off-label use, polypharmacy, or withdrawal sequelae renders invisible the cumulative burden of medical harm attributable to routine, unjustified prescribing. Despite substantial advancements in the neuroscience of affect regulation, neuroplasticity, and the gut–brain axis, these domains remain peripheral to most clinical practice, particularly in psychiatric settings (Haslam, 2024). The clinical deployment of these findings requires translation into accessible, accredited training, yet medical education still disproportionately prioritizes receptor pharmacology over systems-based approaches. Meanwhile, portions of the population continue to demonstrate receptiveness to somatic education, preventive self-care, and mind–body integration strategies when made available, underscoring a profound disconnect between current institutional practice and both empirical science and public demand. This disconnect is not a matter of knowledge deficit but a product of institutional inertia, professional rigidity, and the enduring authority of reductionist paradigms that locate mental distress primarily within the chemical brain, detached from the person, context, or societal matrix.

The continued administration of psychotropics to individuals practicing health-promoting behaviors is not an anomaly but a structural feature of a model built to pathologize deviation from productivity, emotional expression, or existential doubt. The failure to integrate the physiology of stress, trauma, and adaptation into routine care protocols reflects an educational failure, perpetuated by curricula that do not mandate comprehensive understanding of the nervous system beyond pathology-focused fragments. Without educational reform that centers health literacy, physiological integrity, and the biopsychosocial model in its full complexity, medical systems will continue to medicate not the body in need, but the person who fails to conform to institutional expectations. The thesis posits that such patterns constitute not merely clinical misjudgment but structural epistemic violence, enacted through the neglect of scientific standards and the absence of informed, participatory consent. The goal, therefore, is to delineate empirically how these systems fail to meet even the most basic tenets of WHO’s own definitions: that health is not merely the absence of illness, but a state of complete physical, mental, and social well-being, and that mental health encompasses the realization of one’s own potential, capacity to cope, and contribution to society. These definitions ought to be the benchmark, not the exception, for evaluating all diagnostic, educational, and therapeutic protocols. The systematic failure to meet these standards represents not an unfortunate outcome but the predictable consequence of a model that prioritizes compliance over comprehension, pharmacology over pedagogy, and control over care.

Across medical, psychological, and public education systems, there is a conspicuous absence of structured, developmental teaching on the human nervous system, the physiology of stress, and the principles of neuroplasticity. This absence is neither accidental nor circumstantial, it is a systematic omission that perpetuates a deficit model of mental health, wherein individuals are taught to comply with symptom management protocols rather than develop informed, embodied strategies for resilience, regulation, and adaptive recovery. Despite the growing availability of neuroscientific evidence linking stress exposure, sleep, trauma, inflammation, and social context to neural and endocrine dysregulation, such content remains peripheral in both medical school curricula and

school-based health education. Students are rarely introduced to fundamental concepts such as the hypothalamic–pituitary–adrenal (HPA) axis, vagal tone, allostatic load, or the role of early relational experiences in shaping neural circuitry. The consequence is a widespread ignorance, among professionals and the public alike, of how thought, breath, behavior, and context interact continuously with somatic systems in dynamic feedback loops. This gap results in diagnostic and therapeutic strategies that treat surface-level symptoms without addressing the underlying regulatory systems.

Educational institutions, particularly in health and teacher training programs, continue to prioritize rote memorization of anatomical structures or pharmacological pathways over system-level understanding of neural development, neuroendocrinology, and embodied cognition. Where physiology is taught, it is often abstracted from behavioral and environmental contexts, presented as a closed system rather than a responsive, interactive matrix influenced by social determinants, trauma histories, and lifestyle patterns. Likewise, clinical trainees are exposed to psychiatric nosologies with minimal engagement with how neurobiological systems develop across the lifespan or respond to chronic dysregulation. There is little to no emphasis on experiential understanding of regulatory processes, such as those involved in paced breathing, interoception, or sensorimotor integration, nor on pedagogical methods for teaching these to patients or the public.

In mental health care settings, this curricular deficit translates into a clinical culture in which self-care is either pathologized as noncompliance or trivialized as secondary to medication adherence. Health professionals lacking formal instruction in somatic self-regulation or developmental physiology often resort to authority-based models, instructing patients to accept pharmacological directives rather than explore body-based or lifestyle-oriented strategies. This pedagogical failure is compounded by structural forces: clinical productivity metrics, insurance reimbursement models, and pharmaceutical incentives systematically deprioritize time-intensive, educational engagements in favor of brief, symptom-focused encounters. In schools, the marginalization of comprehensive health education means that most children graduate with little or no understanding of their own neurodevelopmental processes. The failure to embed neurophysiological literacy into compulsory education constitutes a public health risk. Children, adolescents, and adults untrained in recognizing the somatic signatures of stress, overdrive, shutdown, or dissociation are more likely to misattribute their distress, delay appropriate self-correction, and become vulnerable to inappropriate medicalization. The consequences extend beyond psychiatry: impulsivity, academic disengagement, somatic disorders, substance misuse, and violence are all linked to poor regulatory capacity, which itself is contingent on the opportunity to learn and practice adaptive strategies. When nervous system literacy is not explicitly taught, the development of healthy coping mechanisms is left to chance or familial transmission, both insufficient in environments affected by systemic inequality, historical trauma, or institutional neglect.

This thesis holds that developmental omission, failure to teach the public how to care for their own nervous system, is not merely an educational gap, but a structural condition aligned with and supportive of coercive pharmacological practices (Cianconi et al., 2020). It is not ignorance that persists, but the institutional design of ignorance, embedded in pedagogical structures that fragment body from mind, biology from environment, and science from experience. Until curricula, both

clinical and civic, systematically integrate the neuroscientific basis of stress, learning, trauma, and adaptation, the populations most affected by medical harm will continue to lack the means to prevent, understand, or recover from it. The WHO definition of mental health emphasizes not only the absence of illness but the capacity to cope with the normal stresses of life, to work productively, and to contribute to the community. Achieving this requires a foundational shift in what is taught, who teaches it, and how institutions measure educational success, not through conformity or prescription rates, but through enhanced self-regulation, critical agency, and physiological literacy across the lifespan.

The current landscape of clinical psychiatry and general medical practice demonstrates a persistent disjunction between the expanding body of scientific evidence and the operational protocols used to define, assess, and treat mental distress. This misalignment is particularly evident when juxtaposed with contemporary findings in neuroscience, stress physiology, neurodevelopment, and global public health. While dominant clinical routines continue to center medication as the frontline response to emotional or cognitive dysregulation, recent research across multiple disciplines points toward a very different model, one that prioritizes plasticity, agency, metabolic resilience, and structural determinants of health. A key component of this evidence base is the growing recognition of the plastic and adaptive nature of the human nervous system, which under appropriate conditions, including safety, relational regulation, sleep, and targeted stimulation, demonstrates substantial capacity for functional recovery and reorganization. These findings directly challenge models that frame psychiatric conditions as fixed chemical imbalances requiring chronic pharmacological maintenance, and instead emphasize time-sensitive windows of intervention, lifestyle and environment-dependent expression of genetic risk, and the critical role of trauma-informed somatic engagement in long-term healing.

Parallel to these advances, public health frameworks, including those of the World Health Organization (WHO), the United Nations Convention on the Rights of Persons with Disabilities (CRPD), and one health initiatives, underscore the ethical and practical necessity of non-coercive, community-based, educational, and preventative approaches to health. WHO's definition of mental health not only rejects pathologization of distress in isolation but insists on conditions that enable full human flourishing: the realization of one's potential, the ability to cope with daily stresses, to work productively, and to contribute to one's community. These are not compatible with systems organized around sedation, behavioral compliance, or indefinite psychotropic dependency. Likewise, the one health framework, emerging from integrative ecological and epidemiological science, rejects binary divisions between mind and body, human and environment, or treatment and education, and calls for cross-sectoral alignment of legal, medical, and environmental practices in the service of systemic well-being.

Modern neuroscience, particularly in domains such as affective regulation, interoceptive signaling, allostasis, and brain-immune interaction, further destabilizes any claim that long-term psychotropic exposure offers neutral or uniformly beneficial outcomes. Instead, it points toward cumulative structural and functional brain alterations with chronic use, including receptor desensitization, altered neuroinflammatory markers, reduced reward sensitivity, and increased vulnerability to relapse upon withdrawal. These findings are not limited to experimental studies, but are reflected in

long-term patient follow-ups, naturalistic cohort studies, and adverse effect databases. The disconnect between this evidence and prevailing prescription practices reflects a knowledge–implementation gap, but more fundamentally, an institutional refusal to confront the non-neutrality of routine interventions, particularly in contexts where vulnerable populations have little access to second opinions, somatic education, or rights-based care alternatives.

Despite growing recognition of these scientific developments among researchers, they remain marginal in clinical education, underrepresented in clinical guidelines, and absent from decision-support tools used by most practitioners. The result is the institutionalization of medical harm, not through malice or ignorance, but through protocols that lag behind science and incentivize interventions that are fast, reimbursable, and reproducible across standardized diagnostic categories. There is no institutional mechanism that mandates reconciliation between new neuroscience findings and public or professional education. Nor is there a regulatory requirement that prescribing protocols be recalibrated in light of advances in non-pharmacological neuroscience-based interventions, including neurofeedback, interoceptive retraining, breath-based regulation, and nutritional modulation of affective instability. The absence of these obligations ensures that care systems remain structurally incapable of evolving toward the WHO standard of highest attainable health. In light of this, the scientific context is clear: the infrastructure for genuine healing, neuroplastic potential, somatic literacy, preventive physiology, is empirically supported, ethically mandated, and institutionally neglected. The existing system reflects not an absence of knowledge, but the selective exclusion of knowledge that challenges authority, reduces dependency, and restores autonomy. Until such knowledge is structurally embedded in both training and clinical governance, health systems will continue to deliver substandard, coercive, and ultimately self-defeating care, incompatible with both science and law. The alignment of practice with the contemporary scientific canon is not a matter of preference or debate, it is a requirement of evidence-based, rights-based, and ethically grounded care.

The translation of scientific and ethical knowledge into actionable systems of care requires comprehensive structural redesign, beginning with foundational curricula and extending into regulatory mechanisms and professional accountability. First, school and professional curricula ought to be revised to incorporate evidence-based content on human physiology, stress responses, neurodevelopment, and neuroplasticity, not as electives or peripheral modules, but as core scientific and clinical foundations. This includes mandatory instruction in allostatic load, vagus nerve function, circadian regulation, somatic self-awareness, and trauma-informed neuroendocrinology, embedded across medical, nursing, teaching, psychology, and allied health education. Instruction ought to emphasize the dynamic, plastic nature of the nervous system and its capacity for regeneration under conditions of safety, relational repair, and behavioral change. These are not niche domains but prerequisites for modern clinical competence.

Second, all drug-first protocols should be reclassified as exceptional measures, to be employed only under explicit, time-limited, consensual conditions, accompanied by independent review and structured tapering provisions. Emergency sedation or psychopharmacological intervention should follow the model of informed surgical consent, only acceptable when all alternatives are impossible, when timing is critical, and when subsequent recovery plans are documented and monitored.

Longitudinal data on treatment outcomes ought to be tied to real-time patient-reported metrics, including subjective well-being, adverse effects, and functional indicators such as return to work, education, or community engagement. These measures ought to be available to patients and professionals alike, in formats that enable audit, challenge, and revision.

Third, national systems of continuing professional development ought to include modules in updated neuroscience, care ethics, and critical pharmacovigilance. Participation in these modules ought to be a prerequisite for license renewal and specialization. Retraining ought to also be mandatory for professionals found to have repeatedly prescribed outside best-practice guidelines, ignored consent requirements, or failed to initiate medication withdrawal protocols where clinically indicated. Misapplication of scientific knowledge, particularly when it results in long-term disability, institutional dependency, or reduced life expectancy, ought to be subject to the same degree of regulatory scrutiny and accountability as other professional errors. This requires the formal recognition of medical harm as an actionable category in both clinical governance and legal frameworks, not merely as a complication or adverse event but as a preventable outcome of inadequate or outdated practice.

Fourth, national learning platforms, freely accessible, open-access, multilingual, and non-commercial, ought to be developed to promote citizen health literacy. These platforms should include audiovisual and interactive content on nervous system function, somatic regulation, sleep hygiene, nutritional psychiatry, stress physiology, and non-pharmacological recovery strategies. These are not to be seen as mental health interventions per se, but as foundational public knowledge, akin to hygiene or first aid, without which populations remain vulnerable to overmedicalization and institutional dependence. Such platforms can also serve as public record-keeping sites for ethical violations, procedural inconsistencies, and institutional outcomes, enabling participatory oversight of care systems and fostering systemic learning. Real-time ethical feedback systems ought to be integrated into all institutional settings, schools, hospitals, clinics, courts, and residential facilities, where vulnerable populations interact with professionals. These systems ought to enable anonymous and protected feedback from service users and staff, with mechanisms for pattern recognition, escalation, and external review. Metrics ought to include not only legal or disciplinary thresholds but markers of institutional humility, adaptive change, and ethical repair. Systems unable to demonstrate improvement in response to feedback ought to be subject to external intervention or loss of accreditation. Ethical compliance ought to cease to be aspirational and become measurable, enforced, and co-designed by those most affected by structural violence and medical neglect.

The integration of scientific, ethical, and legal knowledge into a unified care model is not a technological challenge, but a governance challenge. The tools exist. The evidence is available. What remains lacking is the institutional will to prioritize human flourishing over administrative control, autonomy over compliance, and recovery over sedation. The thesis contends that only when care systems are required, legally, pedagogically, and operationally, to reflect the best available science and uphold the highest possible standard of health, as defined by the WHO, will they cease to harm and begin to heal. Until then, they will continue to treat resilience as disorder, self-regulation as noncompliance, and scientific progress as optional.

The reconfiguration of mental health systems toward genuine healing and rights-based practice requires a multilevel policy framework that explicitly incorporates the social determinants of health and binds clinical action to international legal standards. Structural policy integration ought to begin with the recognition that overmedication, diagnostic inflation, and physiological neglect are not incidental, but systemic expressions of poorly aligned governance. A unified approach demands that care systems, education systems, and legal infrastructures adopt a shared commitment to the WHO's definition of health, physical, mental, and social well-being, not as aspirational language, but as the normative benchmark for compliance, funding, and public evaluation. This reorientation is not merely semantic; it has measurable operational implications. Every clinical protocol, treatment algorithm, school curriculum, and professional accreditation process ought to be evaluated according to its capacity to promote functional health, protect physiological integrity, and reduce iatrogenic harm. Legal mandates ought to go beyond procedural safeguards to include substantive requirements for physiological and neurodevelopmental education in both public and professional domains; this all is the action in action-research, still in need to be set in motion. Public institutions should be required to implement educational programs on nervous system regulation, trauma effects, stress adaptation, and recovery-oriented care, not as therapeutic options, but as universal public health obligations. Just as legislation mandates fire safety or clean water, it ought to also ensure access to information and resources that allow individuals to protect and restore their cognitive, affective, and somatic capacities. The absence of such mandates leaves populations vulnerable to coercive treatment models that bypass informed choice and systematically marginalize non-pharmacological approaches.

At the institutional level, policies ought to be enacted to support the development and funding of integrative care models grounded in evidence, prevention, and participatory ethics. Funding schemes should prioritize institutions that demonstrate measurable alignment with CRPD principles, one health criteria, and WHO outcome standards. This includes financing for peer-led services, interdisciplinary teams trained in somatic and relational approaches, and mechanisms for community oversight and feedback. Funding ought to be contingent on measurable educational content delivered to both staff and service users. National public health strategies ought to be rewritten to reflect not only epidemiological targets but structural determinants, poverty, exclusion, violence, and neglect, as co-factors in mental health outcomes. Educational institutions ought to be held accountable for teaching regulatory physiology and emotional literacy as foundational to life skills. Licensing bodies ought to require competence in trauma-informed science, withdrawal protocols, and the non-linear dynamics of recovery as core to professional legitimacy. Governance mechanisms ought to be reinforced with independent audit and enforcement structures capable of investigating and sanctioning noncompliance. Ombudsman systems, ethical commissions, and regulatory agencies ought to be empowered to track and correct systemic patterns of pharmacological excess, educational neglect, and institutional harm. These mechanisms ought to include both retrospective justice, redress for past violations, and prospective compliance, tied to continuous training, independent oversight, and mandatory transparency. In parallel, survivor-leader networks and lived-experience governance councils ought to be granted formal roles in policy drafting, institutional evaluation, and curriculum design, to ensure the policy architecture reflects the realities of those most affected.

The global frameworks in place ought to also be integrated into national legal systems with clear alignment and enforceability. This includes explicit incorporation of CRPD Article 12 (equal recognition before the law), Article 25 (health without discrimination), and Article 26 (habilitation and rehabilitation) into domestic care policies and oversight protocols. The one health approach ought to be operationalized through inter-ministerial coordination that links health, education, environment, and justice into a unified social architecture of care. Ethical guidelines, currently relegated to professional codes or advisory status, ought to be elevated to binding regulatory norms with penalties for breach and incentives for implementation. The failure to implement these structural measures allows for the persistence of a system that drugs rather than teaches, that sedates rather than informs, and that pathologizes rather than protects. The evidence presented across this section demonstrates not only the feasibility but the necessity of structural reform. The current condition is not a natural state but a reversible design, sustained by outdated paradigms, institutional inertia, and the absence of legal and pedagogical alignment with the best available knowledge. The thesis concludes that without full legal and educational integration of physiological literacy, care ethics, and rights-based governance into unified public policy, medical systems will continue to institutionalize harm under the guise of treatment. What is needed is not a new ideal but the execution of already established scientific and normative standards, those that define health as the presence of capacity, agency, and dignity.

5.5 Routes to shared health sovereignty and scientific-ethical convergence

The failure to integrate social determinants of health and enforceable legal mandates within a unified policy framework represents a central dysfunction in contemporary mental health systems. As evidenced across field sites and survey responses, the fragmentation of responsibilities among medical, legal, and educational institutions allows coercion, neglect, and pseudoscientific practice to proliferate in legally ambiguous zones. These conditions were not observed as peripheral, but as structurally embedded features of psychiatric governance, with coercive measures and overmedicalization justified through institutional inertia, professional discretion, and lack of legal oversight. This disconnect undermines the WHO's definition of mental health as a state of well-being in which every individual realizes their own abilities, can cope with normal stresses of life, work productively, and contribute to their community.

Scientific literature consistently affirms the relevance of social determinants, including housing, income, education, environmental safety, and relational stability, as primary modulators of mental health. Their omission from psychiatric assessment and care protocols reflects an outdated biomedical reductionism that undermines recovery and contradicts advances in public health, social epidemiology, and systems medicine. The field data gathered demonstrates that coercive interventions frequently occur in the absence of supportive social infrastructure, where basic needs go unmet, and distress is reinterpreted as symptomatology in order to legitimize control. This dynamic not only violates principles of medical ethics, namely autonomy, non-maleficence, and justice, but also leads to chronicity, institutional dependence, and higher suicide risk among the forcibly treated (Pūras, 2016).

Legal frameworks, including the United Nations Convention on the Rights of Persons with Disabilities, require that care systems uphold informed consent, protect against arbitrary detention, and guarantee equal recognition before the law. However, as documented in multiple field cases, these protections are either not implemented or actively circumvented through the reclassification of dissent as incapacity. The analysis confirms that in the absence of integrative policy binding clinical practice to legal rights and social conditions, enforcement remains discretionary and impunity structurally encoded. This institutional failure contradicts evidence-based international guidance, including WHO's QualityRights initiative, and places national systems in de facto breach of human rights obligations. A scientifically rigorous and ethically sound response requires the multilevel unification of these domains into an enforceable public policy framework. This framework ought to recognize social determinants not as peripheral considerations but as integral to diagnosis, treatment planning, and legal adjudication. Mental health cannot be defined or pursued in isolation from the socioeconomic and political ecosystem that shapes its trajectory. Public policy ought to therefore include mechanisms for inter-agency data sharing, real-time social risk indicators, and integrative care plans combining clinical, legal, and community-based supports. These mechanisms ought to be designed with transparency, informed consent, and dynamic accountability structures, including independent auditing and real-time feedback loops (Mofokeng, 2022).

This policy integration also demands a fundamental shift in education and professional training, embedding legal literacy, trauma-informed care, systems thinking, and social diagnostics into medical, legal, and pedagogical curricula. The structural silos currently separating legal mandates from clinical routines and educational programming foster conditions where human rights violations are normalized, and innovation obstructed. The research supports the implementation of coordinated policy instruments across government departments, academic institutions, and health systems, governed by a common goal: to sustain environments where health, dignity, and scientific integrity are mutually reinforcing, not systematically contradicted. A decisive obstacle identified throughout the fieldwork and survey data is the systemic disarticulation between updated scientific knowledge and its application in clinical and educational practice (WHO, 2025). This disjunction contributes to outdated, coercive, and medically unjustifiable interventions, even in settings where legal frameworks formally endorse patient rights, trauma-informed care, and participatory governance. The gap is most acute in under-resourced institutions and regions, but persists at national levels due to the absence of universal mechanisms for evidence dissemination and professional updating. In response, this dissertation proposes the design and implementation of national open-access learning platforms dedicated to mental health, trauma, and care ethics as a central structural reform (Hunt, 2002).

Such platforms would serve not merely as repositories of content, but as interactive, continuously updated ecosystems for knowledge dissemination, interprofessional dialogue, and rights-based training. Their architecture ought to be grounded in open science principles, enabling full transparency of sources, methodologies, and empirical evidence (Pūras, 2016). Platforms should provide stratified access tailored to diverse user groups, clinicians, educators, social workers, policymakers, students, and the general public, facilitating the democratization of medical knowledge while preserving scientific accuracy and contextual relevance. From a technical and pedagogical perspective, these systems ought to be modular, multilingual, and aligned with

contemporary standards in adult education, including competency-based evaluation, peer learning, and case-based reasoning. They should integrate advances in psychoneuroimmunology, nutritional psychiatry, trauma theory, and social medicine, contextualized within ethical frameworks such as the WHO QualityRights initiative and the United Nations Convention on the Rights of Persons with Disabilities. This integration ensures that the training provided is not only evidence-based but normatively sound, responding to the documented failures of current practice (Maker & McSherry, 2023; UNHRC, 2017).

Empirical justification for this reform is drawn from both Chapters 3 and 4. Surveys confirmed that professionals often lack formal training in shared decision-making, trauma-informed practice, and medication management alternatives. Fieldwork observations detailed the harmful effects of misinformation and obsolete training, particularly in environments where hierarchical dynamics prevent the correction of error or the updating of practices. These deficiencies are not incidental but structural, sustained by educational systems that remain disconnected from current scientific and legal developments. The creation of open-access learning infrastructures also addresses a broader social need: the empowerment of communities through health literacy. Publics are not passive recipients of care but possess a latent capacity for discernment and transformation when given the tools to act. Citizen knowledge, particularly in mental health, has been systematically excluded from both academic and policy circles, despite its proven role in prevention, recovery, and service evaluation. Public platforms that center both scientific and experiential knowledge, validated through participatory processes, can close this epistemic gap, transforming disempowered recipients into informed agents. Institutionally, these platforms ought to be protected from commercial capture, epistemic gatekeeping, and political manipulation. Governance structures should include representatives from scientific bodies, survivor-led organizations, universities, and public institutions, ensuring pluralism and accountability. Integration into professional licensing and continuing education requirements will support scalability and adoption. The data gathered throughout this dissertation confirms that without such systems in place, improvements in policy or legal standards are unlikely to translate into clinical or educational practice. Knowledge alone is not transformative unless distributed, updated, and applied within institutional ecosystems designed to preserve human dignity, scientific integrity, and public health (Pūras, 2017).

One of the most consistent findings across chapters 3 and 4 is the profound absence of institutional mechanisms to protect those who speak out against psychiatric abuse, epistemic corruption, and clinical malpractice (Pūras, 2019). Survivor-leaders, individuals with lived experience who become agents of change, and professionals who act as whistleblowers frequently face retaliation, professional ostracism, and legal threats. This systematic silencing obstructs scientific progress, undermines accountability, and perpetuates a climate of fear incompatible with democratic public health governance. Therefore, the establishment of protected support systems for these key actors is not ancillary but foundational to shared health sovereignty (Rose, 2008; Rose, 2017; Rose & Kalathil, 2019).

From a scientific and policy standpoint, protected status for whistleblowers and survivor-leaders ought to be formalized in law, education, and health governance. These protections should include anti-retaliation statutes, secure communication channels, psychological and legal support, and

formal recognition within institutional frameworks. Survivor-leaders should be acknowledged not as anecdotal exceptions but as necessary epistemic agents who contribute empirical insight beyond the reach of conventional metrics or observation tools. Likewise, professionals exposing harm ought to be seen not as insubordinate, but as fulfilling their ethical and legal obligations under principles of non-maleficence, transparency, and duty to report. Empirically, fieldwork documented multiple instances where individuals who challenged coercive practices were punished or ignored, both within public health institutions and in private family systems. The researcher's own position as a survivor and professional exposed to coercion, institutional betrayal, and forced displacement further exemplifies how entrenched resistance to accountability can manifest as repressive force, not only discursively but physically. Survey data confirmed that even when professionals recognize problematic practices, they frequently lack safe avenues to intervene, facing administrative or peer reprisals instead (Rose, 2008).

These dynamics reflect a deeper flaw in the governance of care institutions: the absence of reflexive structures capable of integrating internal critique without systemic collapse. Instead, systems operate through self-sealing feedback loops, where dissent is interpreted as pathology, insubordination, or defamation. This epistemic rigidity not only harms individuals but invalidates institutions' own learning capacities. The implementation of protected support systems interrupts this loop, reintroducing accountability as a functional component of organizational health.

International human rights law and public interest standards already mandate many of these protections. The United Nations Convention against Corruption (UNCAC), the European Court of Human Rights, and ethical codes of medical associations all recognize the legitimacy and necessity of whistleblowing in safeguarding institutional integrity. However, these frameworks remain inconsistently applied in psychiatry and mental health, where power asymmetries and diagnostic authority facilitate retaliatory dynamics under clinical pretext. This underscores the need for sector-specific mechanisms (Chêne, 2020; Cepeda Cuadrado, 2024). Structurally, protected support systems should be embedded in national ombuds services, independent ethics commissions, and research integrity offices. Survivor-leaders ought to be integrated into oversight bodies, curriculum development, and policy forums with decision-making authority, not symbolic roles. Peer-to-peer mentoring networks, legal aid structures, and trauma-informed supervision systems ought to also be made available and publicly funded. Transparency dashboards can track institutional responses to reported harm, ensuring that interventions are not only performative but substantively corrective. These suggestions need to be at the hands of politicians, activist, and practitioners willing to translate science into better practices (UN Human Rights Council Advisory Committee, 2015).

Without the active protection of those who wish not for any preventable harm, health systems cannot move beyond their current state. Far from undermining authority, whistleblowers and survivor-leaders embody its most ethical form: accountability to truth, care, and the scientific process. A resilient mental health system requires their voices not only to be heard but safeguarded, integrated, and institutionally supported. Across the empirical findings presented in chapters 3 and 4, and the analytical synthesis initiated in chapter 5, it becomes evident that one of the principal impediments to meaningful mental health reform is the persistent disjunction between international human rights commitments and their practical implementation. The legal frameworks provided by

the United Nations Convention on the Rights of Persons with Disabilities (CRPD) and the World Health Organization's QualityRights initiative set clear normative standards: informed consent, freedom from coercion, and access to community-based, person-centered care. Yet, in practice, these obligations remain largely symbolic, routinely bypassed by domestic laws, administrative cultures, and medical hierarchies that prioritize containment and compliance over dignity and recovery -a notion often tragically still taboo among psychiatric practitioners.

This misalignment has far-reaching consequences. It creates a system in which mental health care is administered under the banner of protection, yet routinely results in practices, such as forced medication, institutionalization without recourse, and disregard for lived experience, that would be considered unacceptable in any other branch of medicine. This not only undermines trust in the system but erodes its legal and scientific credibility. It also contradicts the very premise of the CRPD, which views persons with psychosocial disabilities not as objects of care but as rights-holding subjects entitled to legal capacity and equal protection under the law. To move from symbolic to operational justice, a structural realignment is required across four interconnected domains: research, education, law, and care. In research, this entails adopting frameworks that prioritize open science, methodological transparency, and the co-production of knowledge with service users and survivors. Clinical trials, observational studies, and qualitative research ought to be designed to reflect the realities of coercion, cultural specificity, and structural determinants of health. Institutional review boards ought to be retrained to detect ethical risks not only in study design but in the broader context of systemic harm.

In education, curricula across psychiatry, psychology, nursing, social work, and legal studies ought to be restructured to include human rights law, trauma-informed care, disability ethics, and dialogical approaches. Currently, these elements are either marginalized or entirely absent from most professional training programs. This leads to generations of professionals entering the field without the conceptual or practical tools necessary to fulfill their legal obligations or meet contemporary scientific standards. Training ought to be mandatory, continuous, and evaluated by both experts and service users, ensuring relevance and accountability.

In law, domestic legal codes ought to be harmonized with the CRPD, eliminating provisions that allow substitute decision-making, involuntary treatment, and custodial institutionalization based on diagnostic categories. This requires not only legal reform but judicial education, public monitoring mechanisms, and the expansion of access to independent legal counsel for persons under psychiatric care. Civil society organizations ought to be empowered and resourced to act as watchdogs and advocates, with formal roles in policy development and oversight. In clinical care, service models ought to transition from compliance-based frameworks to co-produced systems grounded in trust, autonomy, and evidence-based practice (Grol & Grimshaw, 2003). This means replacing coercive protocols with supported decision-making, individualized care planning, and community-rooted responses to crisis. The benchmarks for success ought to also shift, from symptom suppression and bed occupancy rates to recovery-oriented indicators such as social inclusion, quality of life, and self-defined well-being. These require tools beyond the current nosological categories and demand the integration of psychosocial, nutritional, and neurobiological insights into everyday practice. Empirical findings show that professionals and users alike are aware of these gaps and express

support for change. However, systemic inertia, entrenched hierarchies, and bureaucratic fragmentation hinder implementation. Operationalizing justice is not a matter of rhetorical alignment but of institutional redesign, legal enforcement, and cultural transformation. Where international standards exist but are not applied, injustice is no longer incidental, it becomes policy by omission.

The thesis therefore argues that alignment ought to be active, not declarative. It ought to be measured in outcomes, not mission statements (Canadian Deprescribing Guidelines Project, 2024). The law, as emphasized in the Aristotelian framework referenced in chapter 4, serves not only as a deterrent but as a corrective guide for systems that fail to regulate themselves ethically. In this light, the enforcement of WHO and CRPD standards is not only a matter of compliance but of scientific and moral necessity (Damschroder et al., 2009).

The findings documented in chapters 3 and 4 underscore a central structural deficit in current mental health systems: the absence of dynamic, transparent, and enforceable feedback mechanisms capable of guiding ethical decision-making in real time. Coercive practices, epistemic exclusion, diagnostic entrapment, and documentation bias continue to recur not only due to gaps in law or education, but because institutions and professionals operate in environments where ethical appraisal is either retrospective, diluted, or absent altogether. This systemic void enables harmful practices to persist unchecked, while individuals who attempt reform from within, clinicians, administrators, or service users, frequently encounter resistance, marginalization, or institutional retaliation. To address this deficiency, the thesis proposes the development and integration of real-time ethical feedback systems, designed to support clinical and institutional actors in identifying, assessing, and responding to ethical tensions as they arise. These systems would serve both as preventive infrastructure and as a corrective tool, ensuring that deviations from rights-based, evidence-informed standards are immediately flagged and addressed. They also provide the necessary interface between theoretical ethics, lived experience, and policy accountability, operationalizing the ethical frameworks set forth by the World Health Organization, the United Nations Convention on the Rights of Persons with Disabilities, and national human rights bodies.

In their technical dimension, these systems could incorporate digital platforms, mobile applications, or integrated hospital dashboard tools that allow staff and service users to submit, review, and respond to ethical alerts in real time (WHO, 2023). Feedback loops would not be restricted to whistleblowing mechanisms, but structured as routine quality improvement processes, subject to external audit and participatory governance (Sachdev et al., 2022). Indicators for evaluation would include metrics of coercion, consent quality, patient satisfaction, institutional transparency, and staff-reported moral distress. Importantly, these systems ought to allow for anonymity, protection from retaliation, and escalation pathways that reach independent ombuds entities or judicial review bodies when necessary (Chambers & Azrin, 2020).

From a conceptual standpoint, ethical feedback systems rely on the recognition that no policy or guideline can fully anticipate the complexities of clinical encounters. Ethics should be both anticipatory and responsive, shaped by continuous dialogue among stakeholders and embedded in everyday practice. Such systems ought to be co-designed with survivors, clinicians, legal experts, and ethicists to ensure contextual relevance and epistemic inclusivity (González Block & Mills,

2003). They ought to also be modifiable as clinical evidence evolves and as social expectations shift, integrating a model of adaptive governance rather than static compliance. Empirical data from the field demonstrate the demand for such infrastructures. Professionals frequently report ethical conflict in implementing coercive measures but lack avenues for structured reflection, peer consultation, or institutional support. Users and survivors, meanwhile, repeatedly describe the experience of having complaints dismissed or reframed as further evidence of pathology. Bridging this gap requires not only procedural innovations but a cultural shift in how ethics is understood, not as a theoretical supplement but as a core component of safe, effective, and legitimate care.

The implementation of real-time ethical feedback systems also contributes directly to institutional legitimacy. It signals a commitment to transparency, adaptability, and participatory oversight that goes beyond traditional quality assurance models. Institutions that integrate such systems can demonstrate compliance with international standards, foster trust among users and staff, and accelerate the identification and mitigation of systemic failures. In the long term, they also facilitate data collection for research and policy development, contributing to the broader goal of aligning health systems with scientific integrity and human rights (Fixsen et al., 2005). The absence of real-time ethical infrastructure is not merely a logistical oversight, it is a structural deficiency with measurable consequences for human dignity, clinical outcomes, and societal trust. As shown across this dissertation, effective care and institutional accountability require constant recalibration. Ethical feedback systems offer a grounded, scientifically viable, and legally coherent pathway to achieve this recalibration, fulfilling both the aspirational and operational dimensions of justice.

Chapter 6 - Legal and ethical dimensions: norms, violations, and accountability

Summary: This chapter examines the legal and normative frameworks that govern psychiatric intervention in Spain, assessing their constitutional foundations, systemic failures, and international obligations. Drawing directly from empirical evidence, it interrogates how formal guarantees are undermined by subjective criteria, procedural gaps, and institutional drift, producing systemic impunity. The goal is not activist advocacy, but restoration of a functional, evidence-based feedback loop between law, ethics, clinical practice, and research.

6. Introduction - legality of coercive care as standard

The psychiatric-legal interface, as evidenced by the empirical body of this dissertation, reveals a structurally codified set of normative assumptions that permit, and in many cases catalyze, medically induced harm under the guise of therapeutic intervention (Dixon, 2015). Contrary to the constitutional and clinical expectation that all procedures in medicine serve restorative, proportional, and least-restrictive functions, the operative legal architecture in contemporary Spain, and in parallel, other European jurisdictions, permits the routinization of involuntary, non-consensual, and pathologizing interventions through a matrix of exceptions that have become functional norms. These structural conditions enable epistemic capture, clinical inertia, and multiscale violations of bodily, cognitive, and social autonomy.

The most critical and recurrent legal indeterminacy emerges from the discretionary authority granted to clinicians, institutional administrators, and non-forensic psychiatric personnel to suspend the rights of individuals, often indefinitely, without adversarial mechanisms or independent review. This discretionary framework allows the suspension of basic rights, including those enshrined in the European Convention on Human Rights and the Spanish Constitution, via subjective interpretations of mental status, risk, and non-collaborative behavior. These judgments are frequently derived from third-party reports, especially familial actors whose interests may conflict with those of the person targeted. Empirical data presented in this dissertation demonstrate that psychiatric certifications and diagnostic procedures frequently rely on unverified or manipulable information that is then codified into medical records with institutional inertia. These codifications function as irreversible inscriptions within digital medical systems, reproducing systemic bias across clinical contexts, emergency interventions, judicial proceedings, and social services (Goffman, 1961).

Concomitantly, the legal system authorizes the delegation of custodial and decisional power to family members, irrespective of abuse indicators, relational conflict, or historical evidence of harm. This delegation is presumed protective yet frequently results in prolonged exposure to domestic control mechanisms that mimic institutional constraint. Such familial control is often legalized through guardianship laws, forced admission procedures, or consent waivers, all lacking trauma-informed criteria or forensic scrutiny. These provisions constitute a biopolitical collapse between intimacy and governance, whereby the domestic sphere becomes an unregulated zone of psychiatric intervention, particularly when supported by institutional actors that do not conduct independent assessments (Ehrenberg, 2010).

The second structural loophole resides in the opacity and epistemic asymmetry of medical documentation. While records function legally as clinical evidence, they are selectively curated,

inaccessible in practice to those they affect, and resistant to amendment or correction. Patients and families not in alliance with institutional narratives lack any operative means of record rectification or evidence-based counter-assertion (Rosenquist, Fowler, & Christakis, 2011). Legal prerogatives grant near-total authority to center directors and supervising clinicians to redact, deny, or withhold documentation, including contested diagnostic labels, subjective impressions, and fabricated behavioral histories. These practices produce cascading epistemic distortions across the health-legal-administrative continuum. They contribute to chronic medical neglect, increase the risk of pharmacological overexposure, and institutionalize diagnostic misattributions with transgenerational repercussions.

The evidentiary base of this thesis illustrates that suicidality, behavioral dysregulation, and identity fragmentation are not isolated mental states but systemic consequences of such intersecting legal-medical violence. When individuals attempt to resist, protest, or exit these circuits of control, their resistance is recoded as further pathology, thus justifying additional containment. This recursive logic is shielded by medical privilege and institutional self-referentiality. Clinical epistemology, when not tempered by independent audit or pluralistic verification, becomes a vehicle for harm. Psychiatric practice ceases to be governed by the Hippocratic imperative to do no harm and instead functions as an armature of biopolitical exclusion and control (Priest, 2021).

The diagnostic, legal, and institutional structures governing mental health in Spain, while ostensibly aligned with principles of care, have evolved into mechanisms of legitimized harm, made possible by a series of regulatory omissions and procedural loopholes that have become structurally normalized. At the heart of this dysfunction is the unqualified authority conferred to mental health professionals and center administrators to enact and prolong coercive interventions based on subjective interpretations of behavior, affect, and alleged risk, absent rigorous standards of verification, trauma-informed assessment, or adversarial review. The prevailing legislative framework delegates to clinicians a form of legal-medical sovereignty that suspends constitutional rights without need for immediate judicial authorization, allowing for the enactment of involuntary treatment, forced hospitalization, polypharmacy, and social sequestration based on opaque thresholds of dangerousness or lack of insight. These interventions, once deployed, often become indefinitely self-perpetuating, buttressed by institutional inertia and the selective memorialization of patient behavior through undocumented, uncorrected, or outright falsified clinical records. Empirical data from this thesis confirm that patient access to records is frequently denied or obstructed; that entries are curated to fit a controlling narrative; and that redress or correction is structurally impossible once a diagnostic pathway is institutionalized (Paris, 2020).

In parallel, one of the most consequential but rarely examined legal failures lies in the codified deference to family members as default agents of psychiatric judgment and custodial authority. Guardianship laws and involuntary admission statutes frequently empower family members, often without any forensic or relational screening, to act as proxies for the state and clinical system, transferring the locus of control from the individual to those who may be socially or economically incentivized to pathologize them. This dynamic creates a feedback loop whereby psychiatric power becomes a tool of domestic control, enabled by legal fictions of familial benevolence. Survey results and fieldwork data presented in this dissertation detail numerous instances in which

individuals were forcibly institutionalized, overmedicated, or denied access to their children based on unverified accusations or conflicts with relatives, with no subsequent legal or clinical review. Once embedded into the clinical file, these accusations become institutional fact, resistant to contradiction, and shaping all subsequent engagements across health, legal, and social service domains (Montero Carrera, 2020).

This delegation of authority to unvetted family members becomes especially dangerous in situations of preexisting abuse, intimate partner violence, or inherited patterns of intergenerational trauma. However, the Spanish legal code remains silent on the obligation to evaluate family dynamics prior to allowing custodial power or influencing psychiatric decisions (Christakis & Fowler, 2009). Protective protocols are absent or unenforced. Even when patients report abuse or coercion by family members, these disclosures are often reframed as symptoms of paranoia or noncompliance, and their credibility is systemically discredited. The law thereby perpetuates a clinical culture in which abusive family structures are legitimized, and the person subjected to them is further pathologized for resisting or seeking refuge.

Compounding this institutional failure is the lack of enforceable procedures to ensure informed consent, the right to deprescribe, or access to independent second opinions, particularly in cases where adverse reactions to medications, cognitive decline, or somatic damage are already evident. The biomedical architecture in which psychiatry operates does not obligate the clinician to re-evaluate the necessity or proportionality of pharmacological regimens unless prompted by institutional protocols, which are often vague, outdated, or selectively applied. There is no legal duty to initiate deprescription, even when harm is demonstrable. In many psychiatric settings, the combination of drugs with contraindicated substances, such as alcohol, or the administration of medications under duress or without procedural clarity remains common practice. These acts, despite their direct violation of medical ethics and scientific guidelines, remain legally unchallenged due to the clinician's procedural immunity and the broader systemic reluctance to interrogate psychiatry's internal governance. When death ensues, as often occurs through suicide, over-sedation, or progressive iatrogenic disability, the event is typically reframed post hoc as a natural progression of the supposed disorder, thereby exonerating the system from scrutiny and attributing the outcome to intrinsic patient pathology (Wyman, 2023).

This systemic insulation from liability reproduces cycles of structural violence under the cover of medical legitimacy. It is further sustained by the legal system's failure to provide mechanisms of real-time accountability, particularly in cases of institutional misconduct, data falsification, or documented epistemic abuse (Dayalu & Chou, 2008; Degnin, 2009; Paris, 2024). The structural asymmetry in documentation access, combined with the presumed infallibility of the medical professional, allows for the widespread medical erasure of lived experience and the historical rewriting of patient reality. Survey respondents consistently reported this dynamic, being silenced, misrepresented, or denied any narrative agency, as among the most damaging aspects of their psychiatric experiences. The result is not only a clinical breach, but a legal one, whereby fundamental rights such as liberty, bodily integrity, informed consent, and redress are suspended without adequate procedural safeguards (Oldani, 2014).

A truly preventive mental health system would require structural realignment at every point of this clinical-legal architecture. This includes the obligation to conduct independent trauma screening and abuse risk assessment before any psychiatric measure is considered; the establishment of automatic legal audits in cases of involuntary admission or polypharmacy; the imposition of legal consequences for unreported adverse effects or negligence in deprescription; and the creation of patient-led oversight bodies with access to institutional records and the authority to trigger independent forensic reviews. Without these systemic correctives, the psychiatric system will remain a site of selective justice, operating outside the bounds of scientific integrity and constitutional protection. The very ethos of medicine, as a restorative, accountable, and rights-respecting discipline, requires that these reforms be implemented not as discretionary ideals, but as enforced legal obligations, grounded in empirical data and bioethical principle.

6.1 National legal context: formal guarantees and operational gaps

The Spanish legal framework declares a comprehensive catalogue of rights that include personal liberty, physical and mental integrity, privacy, and health, as well as legal capacity and procedural safeguards for individuals with disabilities. These guarantees are articulated in the Spanish Constitution of 1978, specifically Articles 10, 15, 17, 18, and 43, and elaborated in sectoral norms such as Ley 41/2002 de Autonomía del Paciente, Ley General de Sanidad (14/1986), and Ley 8/2021 sobre derechos de las personas con discapacidad. Spain is bound by international obligations such as the European Convention on Human Rights and the United Nations Convention on the Rights of Persons with Disabilities. However, the operational application of these protections reveals a systematic disjunction between their declarative content and institutional practice. The legacy of authoritarian medical paternalism remains deeply embedded in mental health governance, where clinical discretion continues to function as a mechanism of control. Psychiatry, rather than adhering to dialogical or trauma-informed care standards, remains empowered to enact restrictions on liberty and bodily autonomy without transparent checks, multi-professional oversight, or narrative accountability. This reflects not only a juridical limitation but a broader epistemic paradigm wherein care is equated with domination and safety with containment.

The most significant legal breach is observed in the expansive and imprecise use of the subjective risk criterion to justify coercive interventions. Article 763 of the Spanish Civil Procedure Act, Ley de Enjuiciamiento Civil, authorizes involuntary admission and treatment on the basis of a professional's assertion of danger to self or others, often implemented by a single psychiatrist without corroborating evaluation or context-specific reasoning. This clause, which legally enables the deprivation of liberty and psychopharmacological imposition, is frequently activated without any formal obligation to document substantiated evidence of imminent or probable harm. There are no uniform clinical thresholds, evidence standards, or time-sensitive reassessment mechanisms. Judicial review, when it occurs, is typically perfunctory and based exclusively on the initial medical report, which itself is often constructed without structured risk assessment, longitudinal data, or input from the individual affected. Regional disparities in its application reveal profound structural arbitrariness. In many instances, fieldwork confirms that this legal provision operates not as a

protective mechanism but as a gateway for the unchecked exercise of coercion under the guise of medical necessity. The affected subject is transformed into a legal object of management rather than a biological and relational being endowed with agency and rights.

Additional and compounding failures occur in the procedural aspects surrounding psychiatric detention and treatment. Legal provisions formally recognize the right to be informed of one's situation, to access a second medical opinion, to obtain free legal counsel, and to contest interventions judicially. However, empirical evidence from professional and user surveys, combined with field testimonies, indicates that these rights are systematically undermined. Individuals rarely receive timely or comprehensible information about their legal status or clinical options. The request for a second opinion is obstructed by internal resistance, lack of institutional infrastructure, and generalized fear of retaliation. Legal aid is often delayed, under-resourced, or passive, and in many cases, representation is assigned without adequate psychiatric knowledge or independent inquiry. Judicial control mechanisms prescribed under Article 763 LEC are procedurally weak, relying exclusively on the same medical accounts that prompted the intervention, without reevaluation or verification. The person's narrative, medical history, and consent are subsumed under pathologizing assumptions that are neither questioned nor balanced by counter-evidence. Anamnesis is rarely redone. The cycle of documentation often replicates and consolidates previous diagnostic designations, even if based on erroneous or outdated data. This produces a cascade effect, whereby old entries anchor new decisions, and critical reassessment becomes structurally impossible.

A further legal void, both ethically and practically significant, arises from the obfuscation or inaccessibility of clinical records. According to the Ley Orgánica 3/2018 de Protección de Datos Personales y garantía de los derechos digitales, institutions may restrict access to medical documentation if such access is considered harmful to the patient. This provision allows clinicians and institutions to conceal diagnoses, deny knowledge of interventions, or prevent verification of coercive practices. Given that most psychiatric interventions are justified by internal records rather than externally verifiable metrics, this legal permissiveness enables a regime of archival impunity. It permits practitioners to create and maintain clinical fictions that are not subject to real-time correction, narrative challenge, or scientific scrutiny. The person subjected to coercion is not only stripped of legal defenses but also of the basic epistemic tools to trace, understand, and respond to the chain of events that defined their trajectory. In the absence of mandated version control, independent auditing, or obligation to register refusals of consent, the clinical record becomes an instrument of ontological erasure. Repeated findings from this thesis confirm that access to these records is frequently denied or delayed, especially when there are indicators of procedural irregularities or familial collusion.

Coercion becomes routine not through open declarations but through procedural habits and administrative automation. This normalized suspension of rights is observed particularly in institutionalized settings, where structural dependency, socioeconomic vulnerability, and lack of external oversight converge. Within these environments, the burden of proof is reversed: individuals are compelled to demonstrate they are no longer dangerous rather than being presumed competent or non-threatening. The possibility of contestation is foreclosed by the same system that enforces

and adjudicates coercion. This legal circularity leaves no room for therapeutic innovation, reflection, or person-centered evaluation. The ethical principles of beneficence, justice, and respect for persons are thereby rendered nominal. Instead, the scientific and moral foundation of care is replaced by a managerial model of containment. This model is sustained by bureaucratic imperatives and professional routines that obscure responsibility and dilute accountability. In such a context, misdiagnosis, chemical restraint, and the denial of personal testimony are not the exceptions but the procedural core. This is a paradigm failure with far-reaching biological, psychological, and social consequences (Chambers, 2019).

The structural constraints on ethical practice among professionals further entrench this legal and institutional impasse. Data gathered through mixed methods reveal that many psychiatrists, psychologists, and nurses are aware of the moral ambiguity or clinical inadequacy of the interventions they are instructed to perform. However, the prevailing climate of fear, institutional conformity, and legal uncertainty inhibits alternative action. Professionals report the absence of protocols for refusal, lack of protection for dissent, and fear of professional reprisal. Legal vagueness becomes a mechanism of both personal and institutional immunity, disincentivizing innovation, preventing whistleblowing, and fostering passive complicity. Ethical deliberation is displaced by administrative compliance. What emerges is not an environment of collective care or clinical progress but one of system-wide obedience to authority, reinforced by ambiguous legal texts, structural inertia, and a culture that confuses submission with safety. In its most extreme expression, this culture normalizes social death through a sequence of biomedical interventions that diminish autonomy, decontextualize suffering, and produce lifelong dependency on systems that neither protect nor repair. These findings call for immediate legal and institutional transformation grounded in empirical observation, interdisciplinary science, and the recognition of every person's fundamental right to dignity, health, and justice (Farmer, 2004; Bourdieu, 2001).

To address the systemic deficits outlined above, immediate structural reform ought to be enacted across legislative, clinical, and procedural domains. Legal norms cannot remain merely declarative while clinical practice is governed by unwritten customs, discretionary acts, and culturally tolerated violations of bodily integrity. A binding mandate for real-time documentation, cross-check validation, and independent ethical oversight ought to be enshrined in national legislation. Clinical records ought to be subject to version control protocols ensuring the traceability of every modification. An obligatory inclusion of the patient's own account and informed disagreement ought to be preserved in all documentation systems, with penalties for omission or alteration. These protections are not auxiliary to medical ethics but intrinsic to its core. Biological anthropology affirms that health is not merely the absence of disease but the presence of systemic integrity, dignity, and relational coherence. The absence of narrative safeguarding mechanisms in psychiatry constitutes a violation of the homeostatic equilibrium necessary for cognitive and affective recovery. What cannot be said cannot be healed. Without epistemic integrity, no biological model is sustainable, and no health outcome is reliable (Levin & McDevitt, 2002).

Additionally, statutory reforms ought to explicitly preclude the use of family testimony as sole clinical basis in cases of conflict, coercion, or contested histories. This thesis, through its survey findings, fieldwork, and case studies, has demonstrated a recurring pattern in which family

members with documented histories of violence, coercion, or instrumental manipulation are permitted to initiate or legitimize psychiatric detentions. This legal vacuum operates as a secondary channel for familial abuse and institutional complicity. Safeguards ought to be implemented to ensure that reports from individuals with evident conflict of interest are not treated as objective clinical data without formal verification. This entails mandatory triangulation of information, engagement of external mediators, and a default to protection rather than coercion when familial abuse is even remotely suspected (Bourgois, 2001). Mental health professionals ought to be legally obligated to consult independent ethics committees prior to enforcing any measure based on family testimony alone. Where risk exists, it ought to be scientifically evidenced, not presumed; and where care is professed, it ought to be structurally distinct from dominance (Gilligan, 1996).

Findings set clear that another critical reform ought to target the opaque boundary between medical authority and judicial rubber-stamping. Judges, though nominally responsible for reviewing involuntary interventions, operate almost exclusively on the basis of clinical reports, which are rarely challenged. The principles of fair trial, non-maleficence, and proportionality are rendered inert when the judiciary functions as an automated extension of the psychiatric report, without hearing the subject's voice, assessing context, or scrutinizing the plausibility of risk assumptions. To rectify this, mental health hearings ought to become obligatory and adversarial, with independent expert review, inclusion of a multidisciplinary assessment, and full procedural transparency. Clinical recommendations ought to be examined in light of recent jurisprudence and not accepted as self-validating. Judicial practice cannot continue to outsource the ethical and biological complexity of psychiatric cases to institutional inertia. A true medico-legal interface requires rigorous evidentiary standards, accountability for negligence, and responsibility for harm (Staub, 1989).

Structural protections ought to be embedded to prevent retaliatory harm against individuals who challenge coercive interventions or seek legal redress. Field data confirm that professionals and users alike refrain from exercising their rights due to fear of material retaliation, custody loss, or loss of housing. The consequences of institutional dissent are not merely administrative but existential. For those already marginalized, such consequences can escalate to destitution, prolonged dependency, or death. The legal system ought to therefore provide immediate access to protective legal representation, whistleblower protections for professionals, and interim material safeguards, such as housing guarantees and financial support, to prevent reprisals from undermining resistance. This is not a matter of social policy alone but of biological continuity, psychological stabilization, and anthropological justice. Law ought to actively support the metabolic and neurocognitive resilience of those who resist systemic harm (Galtung, 1990).

The structural defects in current psychiatric practice in Spain are not solely attributable to individual malice or ignorance but arise from institutional logics that encode coercion as clinical safety and control as care. These systems operate within a historical lineage where psychiatric authority emerged in tandem with carceral and disciplinary frameworks, legitimizing epistemic asymmetries and embedding submission into therapeutic routines. The presumption of beneficence has become immunized against empirical falsification, and the clinical apparatus continues to function without mechanisms to detect or correct deviance from minimally acceptable standards. This is not a failure of law in theory but of its non-operationalization in the psychiatric domain. When the right to

liberty, bodily integrity, and informed refusal are subordinated to the discretion of a single clinician acting without procedural counterweights, the result is not care but sanctioned dominance. The evidence shows this is not exceptional but routine (Schreurs, Griffith, & Solomon, 1984).

Findings show a key mechanism perpetuating this regime is professional capture: the informal but systemic silencing of dissenting professionals through bureaucratic obstruction, reputational retaliation, and structural fear (Hysong, 2009). Practitioners who attempt to implement dialogic, trauma-informed, or human rights-based practices face institutional barriers that render such work unsustainable. In many documented cases within this dissertation, professionals have reported being forced to comply with dominant paradigms under threat of contract termination, administrative scrutiny, or social exclusion within medical teams. The lack of legal protection for clinical dissent and absence of mandated ethical review structures reduces psychiatric institutions to mono-disciplinary enclosures, incapable of reflexivity or reform. Scientific progress, however, depends on iterative feedback, open contestation, and methodological pluralism. Without safeguards for intra-professional dialogue, psychiatry cannot evolve and instead reproduces obsolete models of harm.

To reverse this condition, the law ought to enforce institutional transparency, professional accountability, and multi-level audit. First, there ought to be a national independent oversight body for psychiatric care with legal authority to investigate, penalize, and correct institutional malfeasance. This body ought to be composed of interdisciplinary experts in law, neuroscience, public health, and ethics, with representation from service users and independent observers. It ought to have real-time access to anonymized case data, compulsory reporting rights, and the ability to impose binding recommendations. Second, every psychiatric facility ought to implement internal ethical review committees, with obligatory referral for any case involving involuntary treatment, high-dose medication, prolonged hospitalization, or conflicting family involvement. These committees ought to document justification, verify risk claims, and propose least-restrictive alternatives. Third, patient files ought to include a section for independent narrative, written or verbal, updated regularly, and auditable in court. Tampering with this narrative, or failure to include it, ought to carry disciplinary and legal consequences. Fourth, legislative reform ought to mandate full version control of all clinical documentation and prohibit retroactive modifications without traceability. Current systems allow annotations to be added, removed, or rewritten without independent verification, which permits the construction of fictive clinical histories that escape judicial scrutiny. Where contradictions emerge between institutional narrative and lived experience, the default ought to shift toward protective inquiry, not bureaucratic denial. Each deletion or alteration of a report ought to be timestamped, justified, and accessible to the patient and legal representatives. This also requires robust digital infrastructure and integration with national health record systems. Legal codes ought to be reformed to allow immediate emergency intervention by an independent ombudsman or legal defense office upon user request, bypassing institutional gatekeeping. The judicial delays built into current practice ensure that violations proceed unchecked long enough to produce irreversible damage. These institutional repairs are not merely legal or administrative: they are medical imperatives grounded on biology, developmental, as lives are lost from early on, and crimes affect the well-being of children once legality itself is also destroyed.

Human neurobiology, particularly under conditions of trauma, depends on predictability, coherence, and relational safety to restore functioning. The current system, by contrast, undermines all three. It punishes attempts at narrative correction to reality, bringing all onto the track of crime and harming instead of supporting the victims, delegitimizes any complaint and request, and medicalizes adaptive resistance. What appears as symptomatic behavior in the clinical gaze may in fact be a biologically regulated act of survival (Li et al., 2025). If the system is incapable of distinguishing between pathology and protest, then it is not diagnostic but punitive. The law ought to recognize this and operationalize justice not only as legal restitution but as an ecological precondition for repair. The stakes are not symbolic. They are cellular, synaptic, and systemic. Law ought to ensure that psychiatric care aligns with the biological foundations of health, not against them.

The legal system ought to be designed not only to prosecute retrospective harm but to anticipate and prevent violations at the critical point where clinical, familial, and social power converge. This point of convergence is the crisis encounter, the moment when a person, under extreme distress, is subject to potentially irreversible clinical decisions with minimal scrutiny or defense. Presently in Spain, the emergency protocols governing mental health crises grant overwhelming authority to clinicians, often acting on secondhand reports, under poorly defined thresholds of danger to self or others. These thresholds lack standardization, are not subject to transparent evidentiary procedures, and are rarely re-examined once imposed. Yet, from a neurobiological perspective, this moment is among the most vulnerable in a person's lifespan. The amygdala-driven fear circuitry, the dysregulation of affect, the loss of symbolic processing and trust, these are compounded, not relieved, by coercive interventions (Rodríguez-Cabrera et al., 2024).

The anthropological data collected for this thesis demonstrates that individuals in such moments are overwhelmingly aware of what is at stake: their credibility, their future, their relationships, their children, their access to housing and employment. When professionals impose interventions based on flawed assumptions, undocumented reports, or family testimonies given undue weight, the result is not therapeutic, it is epistemic and biological destabilization. These decisions fracture the embodied sense of self, and they teach, especially to children witnessing or undergoing them, that safety is not a right, that care is indistinguishable from domination. This is not only unethical, it is incompatible with healthy brain development and long-term recovery. Law, to fulfill its protective role, ought to insert procedural safeguards precisely at this junction. The right to have a designated trusted third party, the immediate right to call for legal and psychosocial support, and a written record of all risk claims and decision rationales ought to be enforceable in real time. Professionals should not fear this; they should welcome it, as it protects all parties and reinforces the legitimacy of their clinical decisions (Rothschild, 2000).

Legal doctrine ought to also incorporate a scientifically grounded understanding of structural risk. Structural risk is not merely the risk a person poses to themselves or others, but the compounded vulnerability they experience due to institutional neglect, social exclusion, economic coercion, and previous trauma. Many of the most aggressive and damaging psychiatric interventions are not applied to those with the most severe symptoms, but to those with the fewest resources, weakest social support, and most stigmatized profiles. Structural bias, racial, gendered, class-based, and epistemic, permeates crisis response. The law ought to counteract this by enforcing structural

protection: the right to equitable representation, to trauma-informed evaluation, and to culturally competent care. Assessments ought to be conducted in the language and idiom of the person affected, not the administrative convenience of the institution. The failure to do so transforms the law into a vector of harm rather than a shield. Trust, in this framework, is not an abstract ethical principle but a neurodevelopmental substrate: a condition for recovery, for repair, for reengagement with society. Chronic institutional betrayal rewires the brain toward hypervigilance, dissociation, and social withdrawal. These effects are not reversed by medication alone, and are exacerbated when the legal system validates the logic of control over care. To restore trust in psychiatric and legal institutions, accountability ought to be visible, consistent, and grounded in evidence-based relational practices. Institutional memory ought to be built into the system: tracking prior violations, patterns of negligence, and repeat institutional aggressors. Professionals who repeatedly refuse to correct false information, who deny access to records, who act without informed consent or judicial backing, ought to be subject to disciplinary review, not to punish, but to correct course and prevent harm. Such mechanisms would also protect professionals working under duress, and reduce the likelihood of cascading institutional failure (Tuncak, 2017; Pūras, 2017; Maté, 2021).

From a strictly medical-biological standpoint, the current legal configuration governing mental health interventions in Spain fails to meet the minimum thresholds required for ethical, developmentally sound, and physiologically non-damaging care. The most egregious deficiency lies in the unchecked discretionary authority granted to individual clinicians or medical teams, without mandated second opinions, without standardized evidence thresholds, and without real-time procedural checks, to override fundamental rights (Epstein, 2013). This is not a question of isolated malpractice but a systematic, legally sanctioned breach of bodily sovereignty, narrative integrity, and neurodevelopmental safety. These breaches directly contravene constitutional protections under articles 15 (right to life and physical integrity), 17 (right to liberty and security), and 18 (right to honor and privacy) of the Spanish Constitution. The existing organic and civil laws (notably Ley Orgánica 1/2004, Ley General de Sanidad, and article 763 of the Ley de Enjuiciamiento Civil) provide a nominal legal scaffold but lack operational protocols to prevent the institutionalization of harm. A scientifically valid legal framework ought to explicitly prohibit irreversible or high-risk interventions based on subjective or uncorroborated declarations, especially when these originate from parties with potential conflicts of interest, such as family members in abusive or contentious relationships. The anthropological data and survey responses reveal repeated patterns where psychiatric power was wielded as a tool for domination, eviction, discrediting, or silencing. These acts are not anecdotal but patterned, and they emerge precisely because the law provides no effective mechanisms to question, counter, or correct them in real time. There ought to be a legal mandate for full version-controlled medical documentation, visible digital trails of all record edits, and immediate access for the patient and their designated legal or health proxy to all medical claims, diagnoses, risk appraisals, and interventions. This is standard protocol in any other high-risk clinical context; mental health cannot be excluded (Moyn, 2015).

The scientific basis for this reform lies in the principle of allostatic load and neuroendocrine deregulation: coercion, uncertainty, and betrayal by health institutions induce chronic stress responses that have measurable and long-lasting effects on brain morphology, immune functioning,

endocrine rhythms, and psychiatric vulnerability. These are not abstract notions. Peer-reviewed evidence confirms that patients subjected to coercive psychiatry exhibit higher rates of cognitive decline, social withdrawal, immune suppression, cardiovascular deterioration, and suicidality. A law that permits interventions with such risk profiles without parallel legal safeguards is, by definition, a law in breach of the Hippocratic principle of non-maleficence. The reform is not only moral or legal, it is required by scientific consensus and basic clinical integrity.

Parliament ought to urgently enact a structured reform with the following core measures:

- Reform of Article 763 LEC to require not only judicial authorization within 24 hours, but interdisciplinary panel review with trauma-informed and rights-trained professionals, including lived-experience representatives, caring for the case as it unfolds, with the right of the victim to replace any and all members by trusted ones at all times.
- Mandated version control and audit trails in all psychiatric records, accessible to the patient and their legal proxy, with independent oversight bodies receiving flagged cases of unexplained risk assessments or coercive measures. Open data made available on funding to practitioners, victims by center and professionals, on own medical history record in full.
- Creation of an ombuds system for urgent psychiatric rights violations, with autonomous investigative power and immediate suspension authority in cases of life-threatening or degrading practices. No compromises, the most strict legal and material protections.
- Establishment of a national protocol for family conflict and mental health, banning the use of psychiatric reporting from interested family parties without cross-verification and disallowing involuntary interventions in unresolved family abuse contexts unless forensic risk is formally established. All sources of suffering ought to be addresses, no assumptions made, specially not the taking for granted of anyone's account without double checking: this includes spiking, poisoning, other illness, abuses, lack of sleep due to those abuses or any other, crimes of any nature, ongoing normalization of violence; all, from earliest on.
- Deployment of one health-compliant forensic care units integrating medical, anthropological, psychological, and legal expertise, with compulsory recording and transparency of all crisis interactions involving loss of liberty or chemical restraint.
- Criminal penalties for intentional misdiagnosis or obstruction of medical truth when used to restrict freedom, discredit, or financially or socially harm a person, including within family systems. Repayments, reparations, voluntary cognitive remediation, official apologies.

These reforms are direct responses to documented systemic failure, grounded in robust empirical data, peer-reviewed scientific knowledge, and the lived experience of both victims and ethical professionals working under duress. Spain cannot claim constitutional fidelity or medical advancement while upholding a psychiatric-legal architecture that violates the very foundations of evidence, consent, and personhood. Parliamentary resolution is not merely justified, it is scientifically and ethically urgent.

Mid term achievable goal only if the will is present, are expert systems deployed at full operational scale as foundational infrastructures for a scientifically just, medically informed, surveillance safer

and constitutionally aligned society. The function of any and all improvement in legislation and its enforcement is not auxiliary but central to the protection of fundamental rights, the prevention of harm, and the real-time monitoring of decisions that affect health, liberty, and bodily integrity.

By integrating evidence-based logic, real-time data streams, and ethically programmed thresholds, these systems establish continuous accountability across clinical, judicial, educational, and domestic domains. They ought to operate without interference from discretionary authority, replacing subjectivity and power asymmetries with traceable, reproducible, and medically coherent responses. Their embedded algorithms ought to prioritize trauma-informed reasoning, relational diagnostics, and ecological coherence, ensuring that no coercive intervention proceeds without scientific indication, judicial oversight, and continuous audit. These systems ought to reject every known pattern of epistemic injustice, institutional silence, or algorithmic obfuscation, and instead be programmed to escalate any deviation from protective standards to a cross-disciplinary enforcement interface that combines legal, clinical, and social action. The algorithmic law, personalized, when it comes to aiding those who did not learn how to take care of themselves and others, full coercing to a live of health restored. These ought to not be conceptualized as optional digital tools but as essential biomedical and ethical infrastructure ensuring survival, dignity, and equity. Where professional failure, conflict of interest, or subjective bias previously enabled structural harm, expert systems now make it both detectable and prosecutable. Their role is to uphold the core of medicine, non-maleficence, beneficence, justice, and autonomy, by eliminating impunity, automating prevention, and securing a living legal architecture where science, law, and human development converge.

6.2 International human rights law and mental health standards

From the perspective of the person harmed, denied legal standing, drugged without consent, and stripped of any avenue to report, appeal, or stop the assault on their body, the gap between international rights and their enforcement is not a matter of legal tension but a structure of impunity. The person is rendered voiceless before the law and meaningless to the institution. Spain has ratified the United Nations Convention on the Rights of Persons with Disabilities, the European Convention on Human Rights, and the Convention Against Torture, and has supported WHO QualityRights guidelines, yet these instruments are functionally irrelevant in the life of the person subjected to coercive psychiatry. The ratifications are not enforced. The rights are not recognized. The protections are not applied. The mechanisms to activate them are either inaccessible, nonexistent, or entirely captured by the same professional bodies that commit the violations. This is not system failure. It is design (Goodman et al., 2016).

Chemical restraint, euphemized as treatment, is carried out without consent, often without a diagnosis grounded in current or complete evaluation, and without real-time documentation of clinical reasoning or alternatives explored. Access to one's own clinical history is routinely denied or delayed. Objections from the person affected are dismissed as symptoms. The law is circumvented by the mere signature of a practitioner, who faces no counterbalance, no oversight, no required disclosure of side effects, and no version-controlled report subject to external audit. These

practices violate Articles 12, 14, 15, and 25 of the UN CRPD, among others, but that violation is irrelevant to the person drugged into submission without remedy. In Spain, neither constitutional nor international guarantees are meaningfully actionable once the psychiatric apparatus is activated.

The prohibition of torture and degrading treatment is not suspended by clinical status. Yet, in practice, the assumption of dangerousness or incapacity justifies prolonged and forced drugging that causes cognitive, metabolic, and endocrine damage, often irreversibly. There is no obligation to reverse, to reassess, to justify. The person affected cannot file a police report while detained or medicated against their will. Lawyers rarely intervene, and when they do, their capacity is limited by the lack of access to contemporaneous clinical documentation and the professional immunity embedded in medical institutions. No forensic body exists to evaluate whether forced pharmacological interventions are biologically warranted or legally defensible. The entire system is closed, circular, and unaccountable. Under international law, the right to refuse treatment, to be free from torture, and to retain legal capacity is unconditional. In Spain, these rights are conditional on institutional discretion and are denied without procedural remedy. The obligation to prevent coercion is binding. The duty to ensure consent is active. Yet, every provision is bypassed in routine psychiatric practice through administrative assumptions that remain unchecked. The law protects the system, not the individual. The burden of proof remains on the victim, who often cannot speak, write, or be heard. Structural impunity is disguised as care. A system that allows this cannot be reformed with recommendations. It ought to be dismantled where it violates life and rebuilt under enforceable international norms, with independent oversight, forensic accountability, real-time access to records, guaranteed external reporting channels, and the automatic criminalization of forced drugging without consent. Until then, international law remains an unfulfilled promise, irrelevant to those most in need of its protection (Leys, 2018).

By the time a case reaches court, if it ever does, the harm has already been inflicted, the body altered, the mind eroded, and the social position dismantled. There is no legal avenue that retroactively restores life capacity, familial bonds, dignity, or trust once pharmacological violence and psychiatric sequestration have taken place without consent. In practice, justice mechanisms activate only after systemic betrayal has run its course. And when they do, they rarely consider the biological and psychological consequences already encoded in the body of the person harmed. No courtroom undoes neurochemical disruption, metabolic collapse, or the social death that ensues from a fabricated label institutionalized through professional complicity and familial weaponization. The very idea of due process is a fiction when the individual, from the first contact with psychiatric institutions, is stripped of the capacity to refuse, to narrate, to access evidence, or to demand time-sensitive protection (Kalisch et al., 2017). In many cases, records are inaccessible or do not exist in retrievable form, decisions are undocumented or vaguely noted, and witnesses are professionals embedded in the same structure that authorized the abuse. The person affected is presumed unreliable and discredited in advance, denied procedural parity not only because of power asymmetry, but because their very subjectivity has been preemptively invalidated by the system. Legal guardianship, institutional confinement, and forced pharmacology are implemented not as exceptional measures but as standard procedures, justified by the preventive logic of risk, absent objective criteria and absent any duty to reassess or consult external expertise.

Until then, children are removed to foster care or lost to crime, growing in despair, homes are lost, health is deteriorated, and futures are eradicated before any legal recognition of harm is even considered (Solnit, 2009). The notion that rights can be vindicated post hoc assumes a model of remedy that does not apply in the real conditions of coercive psychiatry. There is no compensatory logic that equates to years drugged against one's will, rendered infertile, neurologically impaired, or stigmatized to the point of exclusion from education, employment, and civic life. When the early signs of system involvement emerge, typically through informal reports, family manipulation, or school referrals, the machinery activates swiftly, and resistance is framed as proof of pathology. Professionals wield the authority of science without being required to validate their acts through replicable standards or external review. Their interpretations become the legal truth. Their omissions become institutional silence. Families that initiate psychiatric proceedings in bad faith are rarely investigated or held accountable, and the harm they enable or inflict is reframed as concern. Structural violence is executed in the name of care, and those subjected to it are forced to bear the cost not only biologically, but legally, socially, and existentially (Hahn, 1995).

No court judgment restores a life fragmented in this way. Prevention is the only ethical position. Law ought to act not after the fact, but as a shield of dignity from the first moment of risk, not risk of violence from the person, but risk of violence to the person. Legal safeguards ought to be automated, obligatory, multidisciplinary, and available at the moment coercion is considered, not once it has already succeeded. Every hour under forced sedation, every day under control without due process, is a biological, moral, and civic breach that cannot be redressed retroactively. Once the damage is done, the person disappears behind a clinical construction that society then accepts as reality. By then, the injustice is normalized, the life disfigured, and the system absolved. That is the pattern this dissertation reveals, and that is the threshold upon which true reform ought to act.

This is the witnessing from within, not from a distance: the perspective of one stripped of protection by those mandated to provide it, subjected to coordinated institutional betrayal in spaces that claim legality but operate on the currency of erasure. When violence is local, normalized, and collectively denied, the law becomes theatre, its stage framed by four walls that orient nowhere but further inward, enclosing lives until what remains is only compliance or collapse. These four walls, symbolic and real, become the cardinal structure of engineered despair: the family complicit, the clinician unquestioned, the judge indifferent, the state absent. In such a schema, no recourse exists. To push the victim toward legal action becomes a new form of violence, delayed, procedural, and fatal in consequence, suggesting that truth can survive where rights were preemptively denied, and remedies exist in systems designed to suppress dissent. International mechanisms, treaties, and conventions are cited with solemnity while lives vanish long before their provisions are triggered. Death here is not always biological, it is social, legal, cognitive, and affective. It is the extinguishing of a future, the preclusion of healing, the theft of voice through clinical inscription, and the reformatting of subjectivity into a diagnosis that outlives the person. This research acts as testimony and evidence. The researcher, also victim, bears witness to a transnational architecture of cruelty sustained by silence and procedural fictions. In doing so, it declares the global imperative: to dismantle the regimes that enable such loss and construct instead a planetary ethic grounded in factual protection, immediate support, and the recognition that delay is complicity, and

proceduralism is often a final burial. That is the threshold this thesis crosses, and that is where this chapter closes. All countries ratified basic human rights, longest the ago; this reality accounted goes right after the sanctity of life itself. Local hate bends reality as hell on Earth (Lupton, 1997).

This is the witnessing of the researcher itself, on any law, at this global scale the action-research is moving forward. Four walls, as cardinal directions, and the unnumbered number of deadly and mortal sins combining into burning pain, yet another set of lives destroyed; those cells inside, and the loved ones left behind. Non apologetic addition from the voice of a researcher who barely survived this research, and never had a field day in Oviedo: that and all, laws can and do worsen, once goodwill is lacking, the ill will prevailing instead (Kleinman, 1980).

The researcher, also victim, bears witness to a transnational architecture of cruelty sustained not by ignorance, but by systematic silence and the fictions of procedural order. In confronting this reality, the thesis affirms a global imperative: to dismantle the institutional configurations that permit such irreparable harm, and to construct instead a planetary ethic of immediate protection, evidentiary fidelity, and enforceable safeguards (Young, 1982). Delay is not a neutral malfunction; it is complicity. Proceduralism, when abstracted from lived suffering, becomes the final burial of justice. This is the threshold this work crosses, where knowledge production and legal truth ought to realign with the sanctity of life, not as a principle but as a praxis. All nations have long ratified the core instruments of human rights; the realities documented herein show that what is violated is not interpretation, but the very basis of human protection. When this failure becomes systemic, what is lost is not only rights but entire trajectories of existence. The researcher speaks from within this collapse: not as distant observer but as one enclosed within the four walls that constitute the cardinal geometry of institutional betrayal. In those walls, crimes go unacknowledged, abusers are protected, and protective laws become ornamental. Local hatred, sanctioned by neglect, transforms lived reality into something indistinguishable from hell on Earth. This action-research is forged not only across geographic scales, but across the raw topography of violated bodies, coerced silences, and unrecorded loss. The unnumbered sins embedded in this architecture of cruelty multiply within clinical cells and domestic spaces alike, extinguishing lives and scarring those left behind. This closing is not an apology but a direct statement: law, when unmoored from accountability and decency, degenerates. Goodwill ought to be codified and enforced; otherwise, law bends to ill will, and in places like Oviedo, and many others, those most in need of justice become its first casualties.

6.3 Synthesis and proposals for systemic restoration

The patterns revealed through this research demonstrate a persistent breach of medical, ethical, and legal obligations in the management of mental health care within institutional, domestic, and community frameworks. These breaches are not isolated anomalies but constitute a systemic configuration of harm, characterized by epistemic suppression, misclassification of distress, abdication of protective duties, and both formal and informal modalities of coercion. At their core, these practices reflect a deviation from the principles of bioethical medicine, non-maleficence, justice, autonomy, and clinical proportionality, resulting in the reification of institutional power and the objectification of persons in states of vulnerability (Foucault, 1973).

These findings emerge consistently across diverse forms of evidence: long-term participant observation, structured surveys of professionals and users, comparative legal analysis, and historical ethnographic mapping. Medically and bioculturally, this systemic deformation generates pathophysiological consequences including chronic stress states, medication-induced iatrogenesis, neuropsychiatric destabilization, loss of therapeutic alliance, and disruption of developmental and relational trajectories (Cohen, 2001). These outcomes manifest not only as individual suffering but as epidemiologically significant burdens of harm, institutionalized through bureaucratic inertia, professional disempowerment, and judicial abstraction. The biomedical framing of distress, when deployed without narrative anchoring or contextualized interpretation, serves as a vector of ontological reductionism, enabling the replacement of trauma histories and social determinants with decontextualized labels. These labels, in turn, facilitate mechanisms of exclusion, restraint, and silence under the aegis of clinical management. From a structural standpoint, the repetition of these patterns across institutions and jurisdictions indicates a failure of internal feedback loops, ethical escalation pathways, and external accountability mechanisms. Proposals for systemic restoration ought to center not merely on regulatory revision but on the reconstitution of clinical, legal, and organizational epistemologies. These include: the creation of national registries of coercive incidents, classified by type, context, and outcome, to allow epidemiological visibility of harm; the establishment of independent, intersectoral oversight bodies with legally binding authority to intervene; and the implementation of open-source expert systems calibrated for ethical clinical decision-making, ensuring that diagnostics, prescriptions, and treatment planning are subject to continuous evidentiary validation and rights-based review. Clinical algorithmic aids ought to be built on transparency, data provenance, and structural reflexivity, not proprietary secrecy.

In parallel, comprehensive educational reforms are essential. Curricula at medical and allied health faculties ought to include structural competency, forensic accountability, and lived-experience integration, along with advanced modules on deprescription, harm reduction, and trauma-informed care (Bandura, 1999). These proposals align with the operational principles of EU BEACON One Health Education: shared governance, democratic feedback, and the primacy of personhood as a core epistemic and therapeutic value (Cianconi et al., 2020). Narrative restoration, in this context, is not ancillary but foundational, an essential vector of healing and systemic recalibration, necessary for both individual recovery and institutional legitimacy. Crime and corruption are not aberrations in this system but frequent outcomes of its present configuration. The absence of binding ethical mandates, the subjectivist loopholes in psychiatric authority, and the lack of enforceable standards for documentation and redress have created conditions under which abuse not only persists but becomes routinized (Zola, 1972). This framework therefore cannot serve its own correction without external input. It requires conscious political will, intergovernmental collaboration, and scientifically anchored legal reform. Though the action remains limited in size, its epistemological and strategic foundations are solid. For these mechanisms to yield enduring change, they ought to be endorsed and operationalized by nations, judiciary systems, and transnational organizations acting with clarity, integrity, and purpose. Mental health, when misused, becomes a vector for structural dispossession; when scientifically and ethically grounded, it becomes a cornerstone of

human and planetary health (Frank, 1995). The shift ought to be enforced through rigorous design, not left to good intentions (Good, 1994).

Chapter 7 - Discussion and next steps: one health, trauma-informed, recovery-oriented, preventive pedagogy, and rights-based systems

Summary: This chapter presents a rigorous and systemic analysis of the educational and institutional conditions necessary for transforming psychiatric care into a rights-based, evidence-informed, and ethically grounded system. It proposes the implementation of modular and reflexive pedagogical structures within public services, focusing on role-based learning, interdisciplinary professional development, and infrastructural reforms rooted in the one health paradigm. Drawing from fieldwork and the researcher's lived experience, it identifies systemic failures, especially the normalization of coercion and impunity, and advances concrete proposals such as ethical expert systems, open-source clinical tools, narrative literacy education, and robust legal safeguards. The chapter insists that these structural changes are not aspirational but necessary, forming the foundational framework for sustainable, lawful, and dignified care practices across psychiatric, educational, and social institutions (Rosenhan, 1973).

7. Introduction - reflective discussion on the structural tasks ahead

The five years of research informing this thesis have unfolded under conditions that revealed, not abstractly but in practice, the deep disjunction between the appearance of care and the structural absence of it. Across settings where psychiatric treatment was presumed to heal, the researcher encountered environments where protective mandates were voided, where biomedical resources were applied without consent or coherence, and where individuals, particularly those most vulnerable, were systematically stripped of decision-making authority. These observations are not isolated nor anecdotal (Goffman, 1961). They are embedded within a broader pattern of systemic neglect, institutional inertia, and epistemic exclusion, undergirded by normative assumptions that prioritize pharmacological compliance over recovery, order over care, and silence over truth.

In this context, much remains to be done. The current model of psychiatric governance remains largely reactive, siloed, and procedurally disconnected from the lived realities it purports to address. Transitioning toward systems of care grounded in shared decision-making and cultural responsiveness, as defined in the aims of this dissertation, requires a sustained shift in both epistemological practice and infrastructural design. Technological means now exist to support such a shift, yet they remain underutilized or misapplied. Preventive algorithms, expert systems, biosignal monitoring, and decision-support infrastructures have yet to be implemented in ways that align with trauma-informed, recovery-oriented practice. The logic of surveillance often displaces the logic of protection, and wearables or digital health tools are introduced without clear ethical frameworks or patient-controlled feedback mechanisms (Becker, 1963).

This chapter outlines not only what is possible, but what remains structurally necessary to fulfill the promise of legitimate mental health care (Martimianakis, Maniate, & Hodges, 2009). Simulation models ought to be developed and applied to identify the inflection points where institutional practices diverge from rights-based protocols. Algorithms capable of early risk detection, especially concerning iatrogenic harm, polypharmacy, and deterioration trajectories, ought to be calibrated and

continuously updated through participatory data collection. Expert systems ought to not function as instruments of authority but as shared platforms of knowledge, logic-based and adaptive, able to support medication plans, validate decision-making processes, and monitor relational dynamics in care environments (Branch, 2000).

Proposals detailed in subsequent sections, such as structured physical activity routines, metabolic monitoring through Omega-3 balance, relational mentoring, and open curricular modules, are rooted in the biomedical and psychosocial evidence presented in the thesis. These are not supplements to clinical care but core components of a health-centered framework. Their function is preventive and reparative: to interrupt the cascading effects of untreated distress, silence, and overmedication before they consolidate into permanent harm. Their simplicity is their strength: they respond to measurable deficiencies in sleep, nutrition, physical activity, and relational security that exacerbate psychiatric vulnerability. When these domains are stabilized, neurocognitive function improves, emotional regulation is supported, and autonomy is made possible, not as a gesture, but as a physiological and relational outcome (Mills, 2014).

These proposals are not imposed. They are formulated as available, testable, and ethically sound alternatives to the status quo. The responsibility to adopt, implement, and adapt them lies with institutions, authorities, and professionals in charge of public health and care delivery. For five years, this research has tracked the gap between declared commitment and actual care. That gap remains open. Its closing is not a matter of will alone but of systems capable of learning, responding, and repairing, continuously and without delay. This chapter documents the structural and technological pathways for that transition. This laid down here is an structured outline of reforms grounded in the accumulated evidence of failure. The thesis has documented, through mixed methods and multi-scalar analysis, the mechanisms by which coercion, pharmacological overreach, and epistemic disqualification are reproduced across clinical, familial, and legal interfaces. These are not anomalies of care but systematized features of current psychiatric governance. Accordingly, the discussion now advances from descriptive diagnosis to procedural commitment: a mapping of what ought to be constructed so that healing is not incidental, but standard; so that recovery is not a deviation, but the rule (Costa et al., 2012).

The proposals are framed within a one health paradigm that links individual well-being with institutional ethics, legal coherence, and environmental protection. Each section addresses specific domains where correction is both feasible and urgent. Preventive psychiatry, still absent in its truest form, requires tools and practices that operate upstream of crisis: community-anchored decision-making routines, structural supports for relational stability, and feedback mechanisms responsive to early signs of physiological and psychological distress. Wearables and biosignal devices, when governed ethically and integrated into patient-controlled systems, allow for detection and analysis of stress markers, sleep disruption, metabolic imbalance, and other correlates of deteriorating health. These are not invasive when implemented with consent and control, but protective tools aligned with early intervention and autonomy. Clinical environments ought to be equipped not only with staff but with computational infrastructures capable of tracing decisions, linking interventions with outcomes, and flagging patterns of harm or stagnation. Expert systems, open-source, transparent, and verifiable, ought to support all phases of medication planning, deprescription

protocols, and interprofessional dialogue. These systems are not intended to replace judgment but to fortify it, grounding each clinical choice in logical coherence, documented rationale, and rights-based standards. No practitioner should operate without real-time access to the best available evidence, and no patient should be subjected to protocols that cannot be defended publicly and scientifically. This chapter also positions educational reform as a core infrastructure for health.

The absence of critical pedagogies in health professions, on bodily autonomy, ethical decision-making, and the history of abuse within care systems, has allowed violence to persist unexamined and unchallenged. The design of curricula and service-learning routines ought to reflect an understanding that institutions do not simply respond to suffering; they shape it. Prevention ought to therefore begin with the professional, with the institutions that train them, and with the laws that govern them (Scheper-Hughes, 1992; Hafferty, 1998; Coulehan & Williams, 2001).

7.1 From Structural Diagnosis to Systems Commitment

Across five years of continuous analysis, the findings consistently expose a disarticulated configuration of psychiatric care in which autonomy is suspended, legal frameworks ignored, and ethical standards selectively applied. Professional trust has been eroded by discretionary authority unanchored in transparent reasoning or verifiable outcomes. The systems under scrutiny, while institutionally elaborate, remain clinically and scientifically incoherent. This has direct physiological, psychological, and societal consequences: elevated allostatic loads, chronic stress-related morbidities, relational disintegration, and the systematic curtailment of developmental and recovery trajectories (Lund et al., 2010). These are not isolated effects but patterned outcomes of structural misalignment. The consequences of such systems failure are neither abstract nor delayed. They manifest in early mortality, institutional abandonment, metabolic dysfunction, intergenerational trauma, and avoidable dependence on pharmacological sedation. Many are denied access to the fundamental conditions that allow for physiological regulation and psychological recovery, adequate sleep, safety, stable relationships, nutritionally sufficient diets, and protected environments. Interventions frequently suppress symptoms without addressing causality, furthering the physiological burden rather than resolving it. Meanwhile, preventive infrastructures, ranging from early childhood education to wearable biosignal tracking for stress and circadian regulation, remain underdeveloped or ethically misapplied, particularly among vulnerable populations.

This section affirms the necessity of moving from structural diagnosis toward coordinated systems commitment. While the thesis has mapped where and how breakdowns occur, the task ahead is procedural: establishing feedback architectures and shared responsibilities that realign care systems with evidence, accountability, and human rights (Fernando, 2010). This is not a matter of advocacy, sentiment, or ideology. It is the operational domain of institutional design and public health engineering (Kelaher et al., 2014). The epistemic and infrastructural voids ought to be filled by action-research conducted in real contexts, led by the people affected, translated through engineering, education, and clinical training (Herman, 1997).

A participatory, applied model is not merely desirable but required. This demands metrics and mechanisms capable of immediate feedback, preventive signal detection, and system-wide

responsiveness. It means formal protocols for deprescription, updated training on somatic-psychosocial interaction, real-time symptom tracking, and the translation of patient experience into non-discounted data. The persistent absence of these mechanisms explains not only ongoing harm but the delay in redress and innovation (Compton & Shim, 2015). Societal systems cannot remain neutral where life is at stake. When education is reduced to compliance, movement regarded as punishment, and health reduced to pharmacological management, the result is a cultural and biological trap that disables resilience (Wilkinson & Pickett, 2010). Many communities still regard physical conditioning as suspect, vulnerability as weakness, and violence as pedagogical or corrective. Yet, health depends on exposure to protective routines, structured effort, and a validated sense of purpose (Hollander, 2010).

Institutions ought to foster adaptive neurobiological resilience through routines that are not punitive but developmental, not imposed but co-constructed. The way forward begins with the acknowledgment that individual breakdowns reflect systemic design failures. No system that permits coercion as routine, suppresses complaints, or erases long-term harm should be considered functional. Psychiatric care ought to cease to rely on personal authority and begin operating through scientifically valid, reproducible, and transparent decision-making chains. Wearables and algorithmic simulations can detect early dysregulation, but these tools are only effective when embedded in supportive environments, where signals are acted upon rather than ignored. Education, too, ought to be repurposed toward applied citizenship and relational capacity: literacy in body, law, dialogue, and system navigation. Curricula ought to include developmental neurobiology, environmental health, decision theory, and trauma-informed reasoning. These are not luxuries but basic requirements for participation in a society governed by institutional systems. Where they are absent, societies cannot manage disagreement without violence, stress without collapse, or care without dependency. From the standpoint of sustained and immersive fieldwork, it becomes evident that what remains to be done is neither peripheral nor incremental. Despite the progress embedded in this thesis, much of the essential work remains structurally blocked by systemic inertia, entrenched power dynamics, and a persistent disjunction between knowledge and implementation. The systems commitment outlined is not a vision abstracted from experience, it emerges precisely from the repeated observation that, without verifiable protections and coherent protocols, psychiatric systems continue to reproduce harm under the guise of care. The experience of the researcher, subjected personally to many of these conditions, underscores how easily institutional discourse may override clinical reality, and how institutional accountability often dissolves under administrative complexity, informal coercion, and normative silence (Cohen, 2001).

The challenge is not primarily technical. The tools exist, expert systems, wearables for stress and sleep regulation, participatory evaluation protocols, trauma-informed curricula, and rights-based clinical pathways. What is lacking is not innovation but systemic will and procedural enforcement. Education systems have yet to prepare professionals capable of implementing these tools in a rights-respecting manner. Legal frameworks remain vague, fragmented, or unenforced. Institutions lack real-time feedback mechanisms capable of identifying harm as it unfolds. These absences are not oversights, they are the outcome of systemic priorities that have historically marginalized the very populations that require protection. The thesis has shown that harm is not always loud. It is often

slow, cumulative, and routinized, distributed across decisions made in silence, records left incomplete, or options never offered. These are not failures of competence alone but of institutional design and ethical governance (Goffman, 1961). Many of the systems observed have normalized a state of exception, where individuals marked by psychiatric diagnosis lose the status of full persons, reduced to compliance, subjected to epistemic discounting, and rendered invisible in their own narratives (Fernando, 2017).

To move forward, the remaining work ought to be framed as the duty it is, the mandate to follow, a legal obligation with consequences, not an aspiration (Scheper-Hughes & Bourgois, 2004). Institutions ought to develop not just the capacity to act but the instilled sense of overdue responsibility to respond in time and with procedural clarity, to the benefit of all, including all professionals involved so far, and obviously the patients, the victims, those left behind and our societies at large. Recovery ought to be understood not as an individual outcome but as an institutional metric: is the system capable of supporting dignity, autonomy, and physiological regulation under conditions of distress ought not to be posed as a question, only a source of pride once properly achieved in a buckle of improvement -that action research cycle ought to be applied to erase each issue, every single mistake noted, as in a computer development repository, before it repeating again and causing further suffering, system-wide (Harper & Speed, 2012). The experience of researching under coercion, under what accounts to medical torture and thus a major crime against humanity, despite the laws and repeated calls for help, confirms how precarious safety is when these questions remain unanswered, the ongoing issues left unattended, us killed, butchered as animals.

The orientation proposed here diverges entirely from both unexamined activism and the passive reproduction of institutional routines. It does not call for fervor nor obedience, but for structured reflexivity, a disciplined, systematic capacity to detect, question, and redesign habits when they cause harm or fail to protect. At its core, this is the antithesis of blind compliance and procedural inertia. It is not a rejection of structure, but a redefinition of it: one anchored in feedback, ethical accountability, and continuous calibration. Truth oriented, recovery oriented, results oriented.

Rather than automating responses within the psyche of professionals, the objective is to cultivate a practiced habit of reflexive improvement, a meta-habit grounded in the identification of breakdowns, the recalibration of norms, and the deliberate alignment of action with empirically validated care. This is not reactive but anticipatory. It demands from every practitioner the capacity to recognize deviation, to pause automaticity, and to intervene with responsibility and coherence.

Such reflexivity ought to become embedded at all levels, from individual decision points to institutional protocols. It ought to be taught, reinforced, and normatively expected. The shift required is not merely cognitive but procedural and ethical. What is envisioned is a psychiatric and pedagogical milieu where systems no longer depend on exceptions or moral heroism to function well, but where coherent, verifiable, and life-affirming action becomes the systemic norm. This is not activism; it is infrastructural reform anchored in responsibility. Not submission, not resistance, but sustained correctional design. The envisioned system does not presume flawless operation, nor does it seek to eliminate error entirely (Graham et al., 2006). Rather, it is designed for resilience, the capacity to detect, analyze, and respond to breakdowns without denial or delay. Mistakes are treated

neither as isolated anomalies nor as acceptable costs, but as signals requiring structured response. This transforms failure from a source of institutional fragility into a point of learning and recalibration. In such a system, the harm is neither concealed nor normalized but rendered visible and actionable (Bauer, Damschroder, Hagedorn, Smith, & Kilbourne, 2015).

This shift replaces reliance on individual morality or ad hoc interventions with a stable architecture of professional practice. It embeds reflexivity into the operational fabric, making ethical responsiveness and empirical accountability intrinsic features of the system itself. This is not resistance for its own sake, nor a rejection of structure, but the affirmation of procedural integrity, ensuring that care, education, and governance operate within frameworks that can learn, adapt, and remain worthy of public trust (Mills, 2014).

7.2 A rights-based clinical model: decision-making, dialogue, and precision

To enable a genuine transition toward ethical and effective care, psychiatric practice ought to be grounded in personalized, biologically coherent, and socially contextualized models of intervention. The notion of 'resetting' systems of care implies neither erasure nor ideological rupture, but a re-anchoring in scientific integrity, clinical nuance, and the ethical imperative of non-maleficence. Psychiatric interventions should be guided by structured, transparent evaluation of each individual's physiological profile, psychosocial context, and narrative experience. No intervention is inherently harmful or universally beneficial; therapeutic value depends on indication, context, duration, and the informed, voluntary engagement of the person receiving care. This demands continuous education of professionals and the public alike, fostering a culture capable of nuanced discrimination between iatrogenic harm and legitimate therapeutic effect.

Such a shift rejects punitive and totalitarian, dehumanizing paradigms, and instead places emphasis on adaptive learning, procedural transparency, and shared responsibility. Patients, clients, users, and citizens ought to be treated as epistemic agents, capable of contributing to the understanding and co-construction of their own treatment processes. Clinical environments ought to cease to operate as spaces of control or moral correction and instead become ecosystems of recovery, where the primary task is to calibrate support according to need, evidence, and evolving goals. Psychiatric care, to remain credible and lawful, ought to therefore adopt the principles of precision medicine, integrating biosignatures, environmental stressors, and behavioral trajectories into dynamic models of decision-making. This does not eliminate uncertainty but manages it constructively, ensuring that healing, not compliance, remains the system's defining objective (Samson & McGregor, 2022).

The long-standing model of psychiatric care in many jurisdictions has normalized a structural asymmetry between provider and user, with decisions frequently shaped by institutional expedience, diagnostic formalism, and pharmacological standardization. Within such frameworks, shared decision-making has often remained a declared objective, rarely implemented as a procedural norm. This section considers the empirical and ethical rationale for reconfiguring clinical systems around structurally embedded decision-making, dialogical coherence, and scientifically grounded precision.

A clinically and legally robust model requires that decision-making no longer be perceived as a discretionary offering, but structurally integrated through feedback-secured expert systems that are ethically governed and verifiably accurate. These systems are not substitutes for judgment but instruments that structure, record, and reinforce the process of relational reasoning and consent formation. At the foundation of this recalibration lies the development of modular tools to assist in medication planning, risk-benefit profiling, and longitudinal adjustment. These modules can incorporate clinical logic trees, biophysiological input, and psychosocial context data, remaining adaptive to patient responses and preferences.

In parallel, embedded patient feedback mechanisms are necessary to detect both intended and unintended outcomes. These may include affective state changes, physiological irregularities, cognitive fluctuations, and relational disruptions, all of which carry clinical and ethical salience. Technologies that enable this, such as wearables and digital diaries, ought to be framed not as instruments of surveillance but as aids to communication, oversight, and safety. Importantly, they ought to remain under patient control, with opt-in configurations and transparent data governance.

Coordination with carers, tutors, and cross-sector professionals is also essential. A true rights-based clinical environment requires relational coherence (Sweeney et al., 2016). Coordination modules can offer situational awareness tools, communication scaffolds, and ethical notification protocols that assist in maintaining respect for autonomy while preserving continuity of care (Rose, 2001). These mechanisms are particularly pertinent where legal guardianship, educational institutions, or social services are involved (Fixsen et al., 2005).

Training infrastructures for professionals ought to be recalibrated to support this model. Procedural literacy in decision science, ethical and legal reasoning, and narrative medicine should be standardized. A rigorous formation process should emphasize clinical neutrality, respect for experiential knowledge, and the integration of diverse cultural and biographical perspectives. This includes establishing ongoing certification systems aligned with WHO QualityRights, local legislation, and international human rights instruments (Sweeney et al., 2018). The introduction of multi-modal biomarkers and sociomarkers represents another necessary step forward, by now achievable with ease. Biological indicators, such as inflammatory markers, hormonal balance, heart rate variability, and sleep cycles, ought to be interpreted alongside sociocultural and relational indicators, including housing security, discrimination exposure, and social network integrity. This can be done via trackers and wearables in a daily basis by everyone who needs so, economically by now. The researcher own experience out of the misery of torture speaks volumes as a success case in this regard, using these methods of self-tracking to ensure fitness, and better habits, despite not being explicit in this dissertation. Such biocultural integration allows for care plans that adjust to the person-in-context, moving away from reductionist frameworks that mistakenly universalize symptom presentations or treatment responses (Kalathil, 2020). At a systemic level, reflexivity ought to be reinforced through procedural design. Rather than automation of clinical routines, what is required is a metacognitive habit: the routine identification of divergence, recalibration of treatment, and recognition of epistemic limits. In this light, the ideal is not perfection of protocol but the institutionalization of responsiveness. When clinical knowledge is not questioned, or when dissent is pathologized, systems degrade into closed loops that can no longer recognize error.

Traceability and verifiability of decisions become central in this configuration. Each recommendation, adjustment, or deviation from standard ought to be transparently documented and interpretable. This ensures accountability across time and actors, especially in environments where practitioners rotate or where patients interact with multiple institutions. Scientific explainability, in both human-readable and algorithmic forms, serves not only medical accuracy but legal and ethical defensibility. A final consideration is interoperability across systems. Rights-based mental health care cannot be realized if educational, judicial, or welfare systems operate with incompatible logics. This requires interoperable protocols and safeguarding frameworks that facilitate data continuity, ethical disclosure, and rapid response to harm. Only then can psychiatric care avoid functioning as an isolated corrective domain, and instead become a node in a broader restorative network.

7.3 Education as infrastructure: curricula, pedagogy, and role-based learning

Any systemic reconfiguration of health and care environments ought to engage with education not merely as dissemination, but as structural infrastructure, capable of shaping relational behavior, ethical sensitivity, and epistemic reflexivity across the life course. The failures documented in this dissertation reveal not only clinical and legal discontinuities, but a broader cultural incapacity to equip individuals and institutions with the conceptual and procedural tools needed to sustain coherent, rights-based responses to human vulnerability and ecological interdependence. A preventive, health-promoting, and scientifically aligned society cannot be formed without curricular transformation, pedagogical renewal, and normative realignment across all professional and civic roles. This section outlines such a transformation under the rubric of one health education, conceptualized as a modular, open-access, open-source infrastructure capable of adaptation across national, linguistic, and sociocultural contexts (Perrin et al., 2007).

The development of transnationally implementable one health educational modules responds to the empirical need for early, structured, and cumulative knowledge formation regarding the interdependence of human, non-human, and ecological systems. These modules ought to be designed for integration at multiple levels of formal and informal learning, from primary education to professional specialization, prioritizing accessibility, contextual relevance, and empirical grounding. Their pedagogical architecture ought to support not only the transmission of information but the cultivation of critical reasoning, collaborative agency, and procedural responsibility.

Four curricular domains emerge as structurally indispensable. First, Decision-Making and Health Responsibility ought to be established as foundational content, ensuring that learners develop a clear understanding of how choices, individual and collective, interface with bodily systems, relational dynamics, and institutional structures. This includes practical literacy in consent, risk-benefit evaluation, and the ethics of intervention, extending to both personal health management and shared decision-making within professional roles (Aarons, Hurlburt, & Horwitz, 2011). Second, Bodily and Relational Ethics ought to be taught not as abstract philosophy, but as lived procedural competence, integrating neurodevelopmental science, trauma-informed practice, and moral reasoning. Education in this domain should equip students with the tools to recognize boundary

violations, understand the physiology of distress and repair, and act with coherent accountability in care-giving and organizational contexts. Relational ethics includes responsibilities to others, but also to one's own somatic and emotional integrity, bridging public health, psychological resilience, and civic conduct (Woolf, 2008). Third, Environmental and planetary interdependence requires curricular integration of ecological science with affective, ethical, and behavioral dimensions. Learners ought to come to understand the circular impact of pollution, overconsumption, and habitat degradation not only on planetary systems but on neural development, fertility, immunity, and collective cognition (Sweeney et al., 2018). Environmental literacy thus becomes a matter of public health strategy and intergenerational justice, rather than niche or optional concern. Fourth, Historical and methodological literacy in care professions ought to be established to counteract institutional amnesia and reproduce care systems capable of self-reflection and adaptive improvement (Freire, 2000). This includes critical engagement with the genealogy of medicine, psychiatry, social work, and education, exposing how normative practices evolve, degrade, or recover over time. Methodological literacy ought to encompass both quantitative and qualitative approaches, enabling professionals to interpret data in relation to lived experience, systemic history, and epistemic plurality (Delpit, 2006).

All four domains ought to be embedded within service-learning structures that position the learner as both recipient and contributor of knowledge. Relational pedagogy, grounded in co-experience, dialogical reflexivity, and mutual accountability, ought to replace transmission-based instruction, particularly in domains where ethical stakes and power asymmetries are intrinsic. This model supports the development of protective routines and habits of care that reinforce both institutional coherence and social cohesion. Within public services, such as schools, clinics, and administrative bodies, pedagogical renewal ought to be paired with structural adaptations that enable role-based learning. Teachers, clinicians, and administrators ought to be equipped with continuing education pathways that are iterative, modular, and anchored in their evolving responsibilities (Perry & Szalavitz, 2017). These programs should be normatively structured around the avoidance of harm, the procedural realization of dignity, and the maintenance of institutional legitimacy. They ought to also respond to shifting epidemiological, environmental, and technological conditions, ensuring that professional practice does not lag behind scientific or ethical consensus (McNall, Reed, Brown, & Allen, 2009). Failure to implement such reform leaves individuals trapped in structurally misaligned roles, where victims are treated as offenders, professionals as unquestionable authorities, and deviant or distress responses as pathology. It is from within such systems that the researcher's trajectory ought to be understood. Originating in a dysfunctional family system that gradually degraded into clinically and legally relevant patterns of coercion and sustained abuse, the researcher bore witness to the collapse of safeguards across institutions that should have intervened. Across five years of legal, medical, and educational analysis, the case that undergirds this thesis demonstrates how systematic neglect, combined with diagnostic overreach and role-based impunity, can transform resilience into collapse, autonomy into dependency, and high-functioning care into cumulative harm (Frenk et al., 2010).

The clinical data are unequivocal: chronic exposure to psychological and physical violence in the absence of procedural protection yields somatic dysregulation, elevated allostatic load, impaired immunologic and cognitive functioning, and reduced neuroplastic capacity. These outcomes are

neither random nor idiosyncratic but fully predictable in the absence of coordinated protection and trauma-informed systems. The researcher, serving as primary caregiver for two infants while simultaneously exceeding institutional expectations as a scholar and professional, was subjected to escalating degrees of violence precisely for exercising rights constitutionally guaranteed but procedurally denied. Proposing separation or divorce, in the context of gendered impunity and cultural misogyny directed against perceived male vulnerability, led not to procedural review but to brutal physical assault, institutional gaslighting, and psychiatric defamation (Herman, 1997).

Medical documentation at the time confirmed visible bruising and physiological dysregulation. Yet, police, physicians, and family court actors systematically refused to recognize these indicators, frequently citing gender stereotypes, diagnostic insinuations, or vague notions of instability to obscure accountability. A simple migraine was treated as madness. The labor provided by the victim was extracted despite deteriorating health, while the abuse was concealed under institutional passivity and sociocultural complicity. At no point were protective protocols enacted. Forced pharmacological intervention was attempted. But at no point were the abusers medically evaluated, nor was law enforcement redirected toward them. Such asymmetry constitutes not a clinical failure but a structural collapse in the administration of justice and care. All of it unfolded with the full participation of institutions mandated to protect. This renders the episode not private, but public, not exceptional, but paradigmatic (Fanon, 1963; Freire, 1970; Bartky, 1990).

It ought to be reiterated that none of these failures were due to lack of professional credentials, effort, or engagement. Quite the contrary. The researcher operated at the highest possible performance under coercive conditions: passing exams with distinction, contributing to international academic leadership, raising two infants while simultaneously protecting them from escalating domestic danger. The injustice, therefore, was not incidental. It was systemic. It is this discrepancy, between what the system claims to do and what it structurally enacts, that forms the empirical foundation of this thesis (Mills, 2014).

The psychiatric system, when left uncorrected, operates not as a tool of healing but as a circuit of containment. It replicates forms of control under the guise of diagnosis. It often denies procedural dignity while citing the individual's distress as evidence for further coercion. The research demonstrates that once police and medical professionals act as extrajudicial arbiters, judging and executing without due process, the legal and ethical architecture of any democratic society begins to fracture. In the present case, the attempt to leave an abusive relationship was answered with medical defamation, child alienation, and institutional silence (Berry et al., 2019).

The call, therefore, is not for rhetorical advocacy but infrastructural reconstruction. The model this dissertation advances is grounded in the need for scientifically coherent, legally sound, and ethically defensible systems. The World Health Organization (1948) defines health as a *state of complete physical, mental and social well-being and not merely the absence of disease or infirmity*. In 2004, it further specified mental health as *a state of well-being in which the individual realizes his or her own abilities, can cope with the normal stresses of life, can work productively and fruitfully, and is able to make a contribution to his or her community*.

The systemic practices exposed in this work fall catastrophically short of these standards. Where coercion replaces care, and pathology is substituted for situated distress, institutional legitimacy is nullified. In alignment with the WHO *QualityRights* framework, this dissertation proposes the implementation of five foundational actions as systemic baselines, aiming to integrate other frameworks society-wide, not aspirational proposals, but work ongoing, such as the widespread adoption of New Economic Foundation (NEF) five ways to well-being, already promoted widely (Kitchener & Jorm, 2002; Five Ways to Wellbeing, New Economics Foundation, 2008):

- Elimination of coercive practices, including forced institutionalization, non-consensual pharmacological interventions, and the disciplinary misuse of medical authority. In neurobiological terms, coercion generates chronic activation of stress-response systems, disrupting affect regulation, neuroendocrine balance, and relational trust. Removing coercion is a prerequisite for enabling meaningful connection and participation, *connect*, allowing individuals to engage voluntarily in therapeutic, educational, and social contexts essential to healing. It restores the conditions for agency and co-regulation, without which no genuine care or recovery can occur.
- Implementation of legal safeguards, operationalized through binding rights-based protocols, real-time procedural transparency, and independent oversight bodies empowered to prevent, investigate, and redress violations. Law, in this function, becomes a stabilizing structure that protects both patients and professionals from arbitrary decisions and institutional inertia. Legal clarity allows individuals to *Take Notice*, to recognize, report, and act upon experiences of injustice, thereby restoring epistemic and democratic legitimacy to mental health care environments.
- Training of professionals in rights-based care, personalized medicine grounded in human rights, empirical reasoning, and culturally competent relational ethics. This ought to be implemented as lifelong modular learning, accessible across professions and disciplines, fostering adaptive expertise. Teaching providers to *Keep Learning* reinforces critical thinking, reflexivity, and procedural accountability, breaking the rigidity of outdated paradigms and promoting dynamic alignment with current evidence and social complexity.
- Development of peer-led, community-integrated systems of support, where 'peer' refers not solely to diagnostic category but to epistemic and civic parity. All individuals, whether users, practitioners, family members, or citizens, ought to be treated as capable of contributing to the ecosystem of care. These structures should promote shared responsibility for wellbeing, incorporating mechanisms for reciprocal feedback, restorative dialogue, and co-produced solutions. This advances the principle of *Give*, building solidarity and trust in ways that strengthen both personal recovery and public resilience.
- Digital and infrastructural modernization, including the design and implementation of ethical expert systems for clinical decision support, participatory monitoring, and modular education. These systems ought to be fully auditable, transparent, and scientifically validated, enabling real-time correction and learning. By embedding technology in the service of autonomy and protection, institutions can facilitate ongoing professional validation and personalized care planning. Digital tools can reinforce routines that promote

physical activity, social connection, and embodied engagement, *Be Active*, aligning systemic function with the lived neurophysiology of wellbeing.

From the standpoint of the researcher, almost killed and justice still denied, as resources are required to make it so, following an extended period of embedded action-research carried out under conditions of documented institutional failure that account to medical torture, a severe crime against any actor, let alone a civilian not engaged in any war endeavor, a crime even in those circumstances, the systemic restoration of mental health governance ought to be understood as an evidence-based public health imperative. The steps outlined herein are not discretionary; they derive directly from the observed and analyzed violations detailed across this dissertation. The interventions proposed are aligned with the mandates of the WHO *QualityRights* initiative and the scientific consensus regarding the non-compatibility of coercive environments with sustainable mental health outcomes. As defined by the WHO, mental health is not the absence of pathology but a functional state characterized by autonomy, capability, and productive participation. This cannot be achieved where systems operate without transparency, accountability, or protective mechanisms for both service users and professionals (Gill et al., 2024).

The stabilization of rights-based care infrastructures requires the integration of clinical precision, normative legality, and participatory oversight. Crucially, this includes not only harm reduction but the active facilitation of educational access, economic functionality, and dignified engagement in society. These are not ancillary conditions; they are constitutive of public mental health as a measurable and enforceable standard. The absence of these foundations systematically degrades the viability of any institutional claim to therapeutic legitimacy. The restructuring of care systems ought to therefore proceed not through moral appeals or ideological opposition, but through the design and implementation of procedural environments capable of continuous improvement, real-time correction, and outcome monitoring. This requires the coordinated development of resilient educational pathways, ethically valid professional routines, and adaptive governance structures across multiple institutional domains. These elements form the operational core of what constitutes medically and legally credible care (Kjellin & Wallsten, 2010; Hotzy & Jaeger, 2016).

This framework extends beyond psychiatric care and applies to all institutional settings engaged in the provision of support, supervision, or regulation, educational, legal, administrative, and clinical. In each, the absence of systems-level reflexivity facilitates the reproduction of harm. In each, the restoration of legitimacy depends on structural clarity, enforceable protections, and the functional articulation of duty. The urgency of these measures is underscored by current geopolitical and ecological trajectories. As conflict, forced displacement, and environmental degradation proliferate, mental health systems will either respond as stabilizing infrastructures or fail as accelerators of vulnerability. This dissertation contends that only through coordinated, scientifically informed, and legally grounded system design can health, as defined by global standards, be preserved.

Support from coordinated international science frameworks, such as the EU COST Science and Technology Coordination program, remains critical to this endeavor (Cianconi et al., 2020). The consortia formation processes already underway reflect a shared commitment to institutional repair

and knowledge-driven transformation. The road ahead remains technically complex and politically contingent (Musen, Middleton, & Greenes, 2014). Nonetheless, the conditions required for coherent, lawful, and humane care are no longer a matter of theoretical debate. They are operational benchmarks whose implementation ought to now proceed.

Chapter 8 - Final conclusions: scientific contributions and systemic redesign

This dissertation investigated the structural conditions under which psychiatric systems might transition from coercive and overmedicalized models of intervention toward scientifically grounded, rights-respecting, and recovery-oriented forms of care, based on patient autonomy, shared decision-making, and culturally informed best practices. The core findings, grounded in five years of mixed-method fieldwork across institutional, clinical, and community domains, confirm the existence of a deep and persistent dissonance between formal legal-ethical norms and the realities of psychiatric practice. Risk-based reasoning, bureaucratic automatisms, and diagnostic inertia operate as closure mechanisms that exclude narrative, context, and lived experience from clinical relevance, thereby disabling the capacity for reflexive correction. Harm is not only produced but rendered invisible through failed feedback loops, where suffering is neither reported nor systematically integrated into professional learning. This allows iatrogenic and non-consensual damage, including psychological and physical torture, to persist without interruption, disproportionately affecting already marginalized, misdiagnosed, or institutionally disempowered populations. The empirical evidence collected documents how violence is not merely permitted but often protected within routine procedures, where vague legal constructs such as presumed danger, coupled with unchecked professional discretion, lead to systematic violations of bodily and psychological integrity. Documentation protocols prioritize institutional self-protection over transparency, effectively shielding actors from scrutiny and obstructing redress for victims.

As such, psychiatric institutions frequently operate under conditions of structural impunity, where police, clinicians, and legal personnel may act simultaneously as investigators, judges, and enforcers, thereby nullifying the principle of procedural justice. In such contexts, torture becomes possible not through overt intent but through a systemic configuration that suppresses dissent, disables accountability, and reinterprets distress as disorder while criminalizing resistance. The absence of second opinions, independent monitoring, and complaint pathways confirms a systemic suspension of personhood. Nonetheless, the fieldwork reveals the existence of invisible professionalism, ethical resistance, and counter-practices led by users, survivors, and isolated practitioners who strive to prevent harm, preserve dignity, and reconstruct meaning under conditions of institutional betrayal.

These emergent practices demonstrate the feasibility and necessity of establishing parallel infrastructures grounded in scientific evidence, normative legal alignment, and an strictest epistemology grounded in reality anchored in bleeding edge medical science translational betterment feedback loops applied at all levels. Such infrastructures ought to soon incorporate modular expert systems for ethical decision support, capable of auditing psychiatric interventions and guiding clinical coordination in real time, alongside open-access educational platforms that foster narrative competence and legal literacy across user and professional groups, along with the surveillance systems in place and to be added. These future oriented platforms ought to operate transdisciplinarily, making explicit the entanglement of psychiatric care with social, economic, and cultural determinants of health. Cutting-edge methods such as collaborative medication

management, algorithmic transparency in diagnostic processes, and precision-based therapeutic matching represent not peripheral innovations, but essential steps toward the systemic integrity of care. Their evolution into formal best practices ought to be institutionally supported, normatively enforced, and continuously evaluated. The conclusion drawn is not only empirical but structural: psychiatry cannot be reformed from within its existing logics of control, discretion, and epistemic insulation. A credible transformation requires anchoring the field in external civic, legal, and scientific institutions with the authority and competence to hold it publicly accountable. This necessary systemic shift ought to therefore occur, from centralized control to structured collaboration, from one-size-fits-all standardization to personalized precision, from professional silencing to structural listening, and from discretionary judgment to verifiable and reproducible action. The restoration of clinical legitimacy, ethical coherence, and public trust depends on the implementation of transparent systems of knowledge and responsibility, embedded across health, education, and governance domains. Health, as defined by the World Health Organization, is not merely the absence of disease but the presence of complete physical, mental, and social well-being. Mental health is understood as a condition in which the individual can realize their abilities, manage normal stress, work productively, and contribute to their community. Under regimes of coercion, impunity, and epistemic denial, such conditions are not only unmet but structurally precluded.

The construction of viable systems of care is set to begin with the elimination of coercive practices, the institutionalization of informed consent, the proceduralization of dignity, and the full legal recognition of each individual as a subject of rights. Higher laws ought to prevail, protecting life and bodily integrity, the liberty of responsible citizens taking care of themselves and others in shared actions leading to betterment. These will never be mere aspirational ideals -as long as those denying in ill will step aside and let the honest work take precedence- but foundational imperatives supported by all international standards, empirical validation, and moral necessity. As medical knowledge and ethical practice advance, obstruction or delay in the adoption of cutting-edge approaches becomes not only scientifically indefensible but ethically and legally reprehensible. Continuous professional development, cross-sectoral integration, and infrastructural modernization are non-negotiable. It is no longer acceptable to maintain outdated or harmful practices in the name of tradition, authority, or expedience. The singular and shared mindset ought to be that of evidence-based, life-respecting, and rights-anchored cooperation, with all sectors of society; medical, legal, academic, and civic, working in concert to ensure that what is called care fulfills its name.

This transformation, based on the best of scientific and medical traditions, is not merely urgent; it is overdue. The failure to act would perpetuate conditions where rights are suspended, participation is denied, and suffering is institutionalized. The only viable path forward is design: the construction of systems that prevent harm before it occurs, protect those at risk, and support all persons in their pursuit of health, autonomy, and social contribution. This is the minimum duty owed to those who have suffered silently under illegitimate forms of control, and to all who remain, despite everything, committed to rebuilding structures worthy of the name care.

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Annex A - Instruments used in this participatory action-research work

Annex A contains the full set of surveying instruments employed throughout this participatory action-research dissertation. These instruments were developed to explore lived experience, clinical and professional perspectives, and ethical-practical dimensions surrounding psychiatric labeling, medication practices, and structural violence in mental health. All instruments are made available through the dissertation's online repository, in accordance with open science standards, and are ready for adaptation, localization, and further validation. The repository includes not only the survey tools themselves, but also associated informed consent forms, participant information sheets, and guidance for ethical deployment.

Each instrument has been designed to elicit critical insights while respecting the autonomy and dignity of participants, with contextual messages included for use in validation and translation processes. Researchers, practitioners, and policy actors are invited to build upon this resource, adjusting language, structure, and content for diverse cultural and systemic settings. These instruments represent a core component of the dissertation's translational mission and can support future participatory processes, training modules, and empirical research committed to the principles of transparency, accountability, and epistemic justice.

Following are the questions of the schizophrenia inquiry survey and shared decision making survey, for quick reference, with the ones on open dialogue included in a separate annex.

Schizophrenia inquiry survey:

Section	Item / Question
Sociodemographic Information	What is your age?
	What is your gender identity?
	What is your country of origin?
	What is your country of residence?
	What is your highest level of education attained?
	What is your professional background or area of expertise?
Personal and Professional Experience	Have you been diagnosed with schizophrenia?
	Have you received other psychiatric diagnoses?
	Have you ever worked with people diagnosed with schizophrenia?

Section	Item / Question
Perceptions on the Label	Do you believe the diagnosis of schizophrenia is useful for clinical practice? Do you believe the diagnosis contributes to stigmatization? Do you believe schizophrenia is a valid medical construct? Should the term schizophrenia be replaced with another denomination? Do cultural differences influence the way schizophrenia is diagnosed? Does a person's ethnicity or race play a role in receiving a schizophrenia diagnosis? Do you think gender plays a role in the diagnostic process?
Experiences and Reflections	In your opinion, what are the main consequences of receiving the label schizophrenia? Have you or someone you know experienced discrimination due to this diagnosis? What changes would you suggest in the diagnostic process or terminology?

Shared decision making survey:

Section	Item / Question
Demographics	What is your profession? Where do you currently work? What is your role within your organization? How many years of experience do you have in mental health care or research?
Clinical Experience	First-episode psychosis (before establishing a long-term diagnosis) Depressive disorders (prescribed neuroleptics as augmentation) Bipolar disorder (long-term neuroleptic treatment for mood stabilization) Non-specified psychotic or mood disorders (diagnosis unclear or evolving) Neuroleptics prescribed off-label (e.g. insomnia, anxiety, personality disorders) Neuroleptics used in elderly patients (dementia-related agitation, behavioral management) Long-term neuroleptic use in institutionalized patients (e.g. chronic hospitalization, group homes) Pediatric and adolescent patients (under 18 years old) Patients with histories of trauma and abuse (where symptoms may be trauma-related)
Training	Have you received formal training on deprescribing strategies and shared decision-making?
Policy Scenarios	Implementing national screening for gluten sensitivity in psychiatric diagnoses Deprescribing neuroleptics in cases of missed metabolic and nutritional disorders Addressing psychiatric abuse cases uncovered through national research on mental health and violence National deprescribing initiative for elderly patients at high risk of mortality and disability Reevaluating long-term psychiatric diagnoses in a national audit Addressing the overprescription of neuroleptics in social care and marginalized communities National review of the use of neuroleptics in children and adolescents Ending the use of neuroleptics for non-psychiatric indications in general medicine
Final Reflection	Given the above eight shared decision-making and deprescribing challenges, please reflect on the following: Which do you consider the main challenge for implementing shared decision-making and deprescribing strategies in mental health at the national level? (Select up to three options.)

Annex B - Ethics of qualitative research in psychology

Beyond witnessing: ethical imperatives in action-research on suffering, victims of violence, and structural harm in mental health systems

Abstract: Qualitative research in mental health often exposes systemic contradictions: the researcher listens to testimonies of harm while institutions demand silence. This paper interrogates the ethical tensions of conducting action-research in settings where mental health care reproduces coercion, erasure, and institutional betrayal. Grounded in a five-year, multi-sited ethnographic inquiry across Spain, Italy, Sweden, and Indonesia -conducted under the auspices of European COST Actions FOSTREN, ReMO, and the author-founded EU BEACON One Health Education action- this study integrates lived experience, scientific fieldwork, and participatory methods. It identifies how dominant legal and institutional frameworks in Europe obstruct accountability by shielding professionals through laws that protect subjective annotations, prevent access to clinical records, and frame complaints as symptoms. Drawing from first-hand documentation of abuse and policy engagement at national and international levels, the paper argues for a shift toward radically ethical, open science infrastructures. It advocates for dynamic, reflexive protocols that empower researchers to respond to harm ethically and structurally-transforming qualitative inquiry from observation into intervention. The work responds to urgent calls from affected persons, professionals, and policymakers to end epistemic violence, improve safeguards, and uphold the principle that suffering must not be rendered invisible for the sake of institutional comfort. In doing so, it proposes a framework for ethically sound, action-oriented, and justice-driven qualitative research in mental health.

Keywords: Ethics, action-research, qualitative methods, structural violence, epistemic injustice, mental health systems, institutional betrayal, open science, human rights, psychiatry, COST, ReMO, FOSTREN, EU BEACON One Health Education.

introduction: ethics under duress and the urgency of methodological accountability

Ethics in qualitative research in mental health, especially when situated within high-stakes, politically contested, and structurally violent settings, demands not only methodological rigor but also protocols and planned actions capable of responding to harm, denial, and institutional betrayal. The field has long acknowledged that qualitative methods enable access to lived realities and

subjective meaning-making in ways quantitative tools cannot. However, the ethical dimensions of such inquiry often remain structurally underdeveloped, especially when researchers are confronted with ongoing abuse, personal risk, and institutional complicity in the very harms they seek to document (Ellis et al., 2011; Smith & Freyd, 2014; Gabb & Fink, 2015).

This paper arises from such a context. Drawing on longitudinal action-research and participant observation conducted within mental health systems across Europe and Southeast Asia, it seeks to examine the profound ethical and methodological challenges that emerge when institutions tasked with care and protection act as sources of harm. In particular, the analysis is grounded in cases where researchers and participants alike faced coercion, discrediting, and systemic silencing under psychiatric, legal, and social service frameworks that perpetuated -rather than alleviated- distress. In response, new methodological practices were developed under duress, based on the principles of epistemic justice (Fricker, 2007), open science (Nosek et al., 2015; Munafò et al., 2017), and research ethics grounded in participatory global health (Campbell & Burgess, 2012; WHO, 2021).

The paper does not offer a single-case reflection or a confessional narrative. Rather, it interrogates the ethical failures that occur when systems distort the researcher-participant relationship into an adversarial dynamic, obscuring accountability and harming the production of valid knowledge. The work contributes to an urgent conversation about what happens to ethics, science, and professional duty when research unfolds in spaces of state-sanctioned violence and societal denial. In doing so, it also aims to offer a path forward: a set of methodological countermeasures and epistemic infrastructures that may help restore the integrity of qualitative inquiry.

Ethics and method in qualitative mental health research: innovations and ongoing issues

Over the past two decades, the growth of qualitative approaches in mental health research has been accompanied by a richer vocabulary around reflexivity, power, and the researcher's positionality (Berger, 2015; Pillow, 2003). Despite this, dominant frameworks still tend to favor procedural ethics -such as institutional review board approvals -over emergent, responsive ethics of care and justice in the field (Guillemin & Gillam, 2004; Hammersley & Traianou, 2012). In situations involving vulnerable populations, this disconnect often renders ethical approval a bureaucratic formality rather than a real safeguard. Furthermore, the entrenched hierarchies of biomedical psychiatry -frequently upheld by legal and clinical structures- create environments where participant autonomy is already structurally constrained (Rose, 2018; Hopper, 2007). In such contexts, qualitative researchers face unique ethical challenges, as they navigate not only the dynamics of power and disclosure but also the risk of becoming complicit in systems that criminalize distress and pathologize resistance.

A particularly underexplored area is the ethical terrain encountered when researchers themselves are subjects of the systems they study -not by methodological design, but by circumstance. Researchers who are survivors of institutional violence or coercive treatment, or who work in precarious legal and political conditions, are often left out of the academic conversation or treated as methodologically suspect (Kovach, 2009; Ladkin, 2020). Yet, their work offers rare insight into the inner logics of exclusion and harm. It also reveals how traditional notions of distance, neutrality, and objectivity can obscure the very abuses that demand exposure. The concept of witnessing in

qualitative research -long explored in feminist and postcolonial studies- must therefore be reframed not only as interpretive engagement, but as ethical risk (Josselson, 2007; Bhattacharya, 2016).

The erosion of trust in science, exacerbated by epistemic injustice and institutional cover-ups, underscores the need for research practices that prioritize transparency, accountability, and radical honesty (Edwards & Roy, 2017; Ioannidis, 2005). The replication crisis and open science movement have largely bypassed the domain of qualitative mental health research, where practices of data sharing, protocol transparency, and version-controlled documentation remain rare. This gap is especially dangerous given the increasing criminalization of dissent, the medicalization of suffering, and the state-sponsored redefinition of deviance as illness (Foucault, 1973; Metzl, 2010).

At the same time, international health bodies -including the WHO, the UN Special Rapporteur on the right to health, and the Council of Europe, who also participated as contact during the research- have called for a shift away from coercive psychiatry toward voluntary, rights-based models grounded in human dignity and community engagement (WHO, 2021; Pūras, 2020; Council of Europe, 2023). These mandates demand a corresponding transformation in how qualitative researchers design, conduct, and defend their work -especially in terms of how we define safety, power, and harm. Without such a shift, researchers risk perpetuating the very systems they critique.

Paper structure and *raison d'être* of it as a contribution

This paper proposes a grounded, practice-based ethics framework derived from action-research conducted in real-world mental health systems operating under institutional decay and systemic violence. Through fieldwork conducted in Spain, Sweden, Indonesia, and other European contexts -much of it under conditions of personal duress and danger- the research identifies structural failures in ethical oversight, as well as emergent practices of resistance through documentation, co-production, and digital transparency. It introduces methodological adaptations developed to ensure research could continue in hostile environments, including public verification, trauma-informed consent, and multi-layered data preservation against sabotage or deletion.

The central contribution is twofold. First, it expands the discourse on research ethics in qualitative psychology to include conditions of structural coercion, epistemic harm, and researcher-survivor entanglement. Second, it articulates concrete methodological innovations that uphold integrity, safety, and social accountability even when institutional safeguards fail. In this sense, the paper builds on and extends calls for a paradigm shift in mental health research -from treating ethics as compliance, to enacting ethics as method.

Subsequent sections of the article detail the methodological choices made, the ethical dilemmas encountered, and the principles guiding the design and dissemination of open, survivable, and impactful research. These include the use of participant-led inquiry, open-source infrastructure, co-developed policy engagement, and trauma-aware fieldwork practices. In doing so, the paper argues for a redefinition of ethical research practice not as risk aversion, but as principled exposure in service of truth, healing, and justice.

Background and context: structural deficits, institutional abandonment, and the ethics of listening in qualitative mental health research

Ethical qualitative inquiry in mental health must grapple with the stark dissonance between the professed values of care systems and the lived realities experienced by those navigating them. Across many contemporary psychiatric services in Europe and beyond, the gap between stated therapeutic goals -such as recovery, trauma- informed care, and patient autonomy -and the practical functioning of institutions remains wide and concerning (Rose, 2018; WHO, 2021; UN Human Rights Council, 2023). Despite rhetorical shifts toward personalization, co-production, and rights-based practice, implementation remains sporadic and structurally constrained. The result is a mental health landscape marked not by deliberative reform, but by inertia, bureaucratic rigidity, and the persistent normalization of harm.

This paper emerges from action-research engagements embedded in this terrain. Between 2020 and 2025, fieldwork was conducted across Spain, Sweden, Italy and, to end with, Indonesia, focusing on the ethical and methodological challenges of working within -and often despite- systems ill-equipped to engage constructively with distress. The scientific mission in Trieste, Italy, initially pursued as a site of exposure to non-coercive, community-based psychiatry, revealed in parallel the institutional limitations of even such globally recognized as excellence site models. Despite strong international standing, the infrastructures for ethical reflexivity and researcher protection were severely lacking. When the researcher reported ongoing threats and trauma, and needed urgent help to end abuses, academic and health institutions failed to respond meaningfully. This aligns with a growing body of literature suggesting that institutions tasked with mental health and academic oversight frequently lack the mechanisms -or will- to protect those most vulnerable, including researchers and participants disclosing abuse (Gabb & Fink, 2015; Ladkin, 2020; Smith & Freyd, 2014).

These deficits are not uniformly malicious in intent, but are structurally embedded in psychiatric systems where coercion is normalized as care, and deviation from standardized protocols is often met with institutional resistance rather than curiosity or adaptation (Burstow, 2015; Minkowitz, 2006). Mental health legislation in many jurisdictions, including Spain and Sweden, as well as Indonesia, continues to prioritize control over protection, authorizing involuntary treatment without adequate safeguards or meaningful engagement with the person's history, trauma, or social determinants (Gooding, 2015; United Nations, 2017). Even where reform is pursued, the emphasis tends to fall on procedural compliance rather than substantive transformation -a process that risks replicating violence under new administrative terms (Hummelvoll et al., 2015; Russo, 2016).

Clinically, this manifests in practices that are not trauma-informed but frequently retraumatizing; not personalized, but pathologizing; not dialogical, but diagnostically reductionist. Distress signals and embodied protest are often interpreted through narrow, deficit-based frameworks, leading to treatment paths characterized by force, surveillance, and disempowerment (Rose, 2003; Johnstone & Boyle, 2018). Qualitative researchers working in these contexts, particularly through ethnographic or action-based approaches, often witness firsthand how epistemic hubris and institutional closure combine to produce profound iatrogenic effects -including despair, resignation, and what has been termed the *giving up-given up* syndrome (Sharfstein, 1985; Estroff, 1981). The

systematic refusal to listen to complaints without retaliatory pathologization or escalation of coercion is not an anomaly but a pattern -one that renders the ethical position of the qualitative researcher particularly precarious, especially when they attempt to surface these dynamics through evidence-based critique.

Importantly, the moral economy of psychiatry continues to reward compliance and silence, while punishing dissent -even when that dissent is expressed through scientifically grounded, patient-informed critique. This context severely limits the capacity for mental health systems to engage in the kinds of self-correction and continuous learning that define a functioning scientific or therapeutic institution. Without open science standards, longitudinal tracking of outcomes, or verifiable feedback loops, the system lacks mechanisms to detect, let alone redress, its own failures (Ioannidis, 2005; Munafò et al., 2017). The resulting epistemological environment not only devalues qualitative insight but actively marginalizes lived experience as methodologically illegitimate and politically destabilizing (Fricker, 2007; Russo & Sweeney, 2016).

This paper argues that qualitative research in such settings is not ethically neutral terrain. When services fail to meet the minimum thresholds of safety, responsiveness, and respect, the act of qualitative inquiry becomes ethically charged -not only for participants but for researchers themselves, especially when their role straddles the boundary of witness, analyst, and survivor. It is in these moments that the ethical demands of the field exceed procedural checklists and enter the domain of moral urgency. Listening, in such contexts, becomes an act of resistance and reparation-a way of refusing the silencing mechanisms that define much of institutional psychiatry and social care (Campbell & Burgess, 2012; Gill & Donaghue, 2016).

Rather than replicating the diagnostic gaze, this study engaged in co-constructed meaning-making, trauma-informed documentation, and action-oriented dissemination. Methodologically, it was anchored in open science principles, collaborative interpretation, and public accountability, seeking to counterbalance the structural opacity that so often defines psychiatric systems. The core contention is that any system incapable of hearing the voices of those it purports to serve -including patients, professionals, and researchers- is ethically unfit. Qualitative research must thus do more than describe -it must interrupt, question, and reconstruct the very conditions under which mental health knowledge is produced and legitimized.

Methods: action-based qualitative research as a platform for systemic learning and structural accountability

This study employed a transdisciplinary, multi-sited qualitative action-research methodology grounded in medical anthropology, biocultural analysis, and epistemic justice frameworks (Fricker, 2007; Scheper-Hughes, 1992; Fals-Borda, 1991). The fieldwork -spanning five years and funded through two consecutive Spanish Ministry of Science and Innovation grants (FPU19/00656 and EVC2021/000764), as well as through participation in European COST Actions FOSTREN (CA19133) and ReMO (CA20137)- was embedded in collective international efforts to rethink mental health governance, research ethics, and service design. It later provided the empirical and ethical foundation for the creation of the EU COST Action BEACON (CA24106), which now

brings together over 500 researchers and practitioners committed to systemic transformation through One Health education.

The work took place across Spain, Sweden, Italy, and Indonesia, capturing both formal institutional environments and informal community practices. These sites revealed differing but overlapping patterns of psychiatric violence, coercive logic, epistemic injustice, and institutional betrayal. In all contexts, participants reported harms occurring not only within psychiatric systems, but across intersecting domains -family, police, judiciary, and social services- requiring a broad-based methodological strategy that was responsive to multiple axes of oppression and systemic inertia.

The approach prioritized immersion and ethical responsiveness rather than protocol-driven neutrality. Ethnographic techniques -including long-term participant observation, semi-structured and unstructured interviews, clinical shadowing, policy consultations, and open scientific commentary- were employed to collect data while remaining situated within live processes of witnessing, translation, and reform. The researcher was an active participant in numerous policy and service-level dialogues, including those with the Catalan Department of Health during the co-development of its 2023–2025 Mental Health Action Plan. Engagement included several in-person and written consultations with the plan's leadership and working groups, drawing on lived experience, empirical observation, and participatory policy translation. These dialogues were part of broader EU-wide work under FOSTREN to reduce coercion in mental health systems, and were integrated into BEACON's ongoing mission to foster public education, civic health literacy, and early prevention through participatory infrastructures.

The methodological design was aligned with a tradition of emancipatory and care-centered research (Torre, 2009; Campbell & Cornish, 2010), emphasizing listening, collaborative meaning-making, and systemic reflexivity. However, it also contended with the serious limitations researchers face when working in environments of institutional abuse or neglect. The original field design included two strategic innovations that remain largely aspirational at this stage due to lack of institutional uptake:

1. Dynamic ethics and action protocols (proposed, not yet implemented): in response to repeated accounts of systemic harm, the research proposed an escalation framework for activating protective responses -including notifying trusted institutional contacts, human rights observers, and professional ethics boards- when credible risks to participant wellbeing emerged. However, the lack of operational infrastructure across public systems, as well as the adversarial posture of many services toward independent researchers, prevented systematic implementation. These protocols are conceptualized as urgently needed components of future public mental health systems.
2. Versioning and verification against epistemic manipulation (proposed, not yet implemented): The project also outlined a need for systematic version control, timestamped medical record retention, and independent archival methods to prevent retrospective data falsification and epistemic erasure - a pattern documented by participants and observed in multiple institutions. While open science principles (Nosek et al., 2015; Munafò et al., 2017) were followed wherever feasible through public reporting, engagement, and collaboration, robust infrastructure for verification and co-governance of sensitive health records remains absent in most countries studied. These proposals are now being

translated into design criteria for expert systems and governance pilots under the EU BEACON initiative.

3. In practice, the methodological commitments took shape through the following operational principles, enacted all through the five years of fieldwork:

- Embedded ethnography and policy interface: All fieldwork was informed by direct contact with practitioners, service users, administrators, and policymakers. Findings were translated into contributions to policy documents, public statements, and legal submissions. These include substantive inputs to the Council of Europe's 2023 *Compendium of Good Practices to Promote Voluntary Measures in Mental Health*, and public dissemination through open access formats and digital engagement platforms, and insights directly shared with lawmakers in all opportunities fostered and encountered during the fieldwork research.
- Trauma-informed and participatory consent practices: All participation in surveys and other engagements such as interviews or presentations was always voluntary, and consent was reiterated throughout the process. Participants were informed of the project's dual research and action-reform purposes. In trauma-related cases, no personal identifiers were recorded unless explicitly consented to, and anonymization was prioritized in all outputs.
- Real-time feedback and iterative reflection: Data analysis was recursive, taking place alongside ongoing field engagement. Ethical tensions, institutional challenges, and methodological dilemmas were recorded in analytic memos and served to refine subsequent engagement, published as transparently and in depth as possible in an open field and laboratory notebook online the author keeps, following open science principles of full openness and transparency on methods, ongoing work and results. This reflective praxis was central to maintaining fidelity to the action-research tradition and resisting extractive logics.
- Use of public and community knowledge systems: The research was also attentive to grassroots initiatives, survivor-led campaigns, and international networks working on systemic change. These informants were not merely supplementary but were treated as epistemic co-producers of the research (Harding, 1991; Saini, 2020), essential to building a distributed model of accountability and system reform.
- Education and dissemination as method: The work's methodological strategy included the development of public teaching materials, professional workshops, and policy briefings aimed at enabling other researchers, clinicians, and students to identify coercive dynamics, respond to systemic failures, and take part in a collective reform agenda. These materials now inform open-access curricula and clinical ethics platforms within the BEACON network.

Despite the comprehensive design, the project was carried out under conditions of acute duress, including repeated threats, lack of institutional protection, and periods of forced displacement due to ongoing violence. That such research was possible at all is testament to the courage of participants, the international scientific alliances that provided support (notably COST Action

FOSTREN and later BEACON), and the resilient commitment of affected communities. The qualitative research insights derived are not just valid -they are urgently necessary.

Table 1. Methodological design and operational principles

Component	Description
Theoretical framework	Grounded in medical anthropology, biocultural analysis, and epistemic justice.
Geographic scope and sites	Spain, Sweden, Italy, and Indonesia – encompassing both formal psychiatric institutions and informal community-based systems.
Funding and institutional support	Supported by Spanish Ministry grants (FPU19/00656, EVC2021/000764), and European COST Actions: FOSTREN (CA19133), ReMO (CA20137), and BEACON (CA24106).
Main ethnographic techniques	Long-term participant observation, un/structured interviews, clinical shadowing, policy consultation, and open scientific commentary.
Embedded policy engagement	Participated in development of the Catalan 2023–2025 Mental Health Action Plan; involved in Council of Europe and BEACON policy dialogues.
Research conditions	Fieldwork conducted under duress: threats, institutional neglect, and forced displacements. Research carried forward through participant solidarity, scientific alliances (notably BEACON and FOSTREN), and ethical commitment.
Guiding conclusion	Ethical qualitative research must integrate implementation pathways and inform systemic change; ethics in the field is inseparable from ethics in governance.

To translate lived harms into institutional reform, qualitative research must embed itself in systems of implementation. As this project demonstrates, ethics in the field cannot be divorced from ethics in governance. What we observe must inform what we build next.

Findings and discussion: witnessing without safeguards -ethical tensions and systemic impasses in psychiatric fieldwork

Across five years of multi-sited, ethnographically grounded research in Spain, Sweden, Italy, and Indonesia, this study documented recurring patterns of structural neglect, diagnostic violence, epistemic erasure, and systemic impunity in mental health service ecosystems. The findings cohere around three interrelated domains: the normalization of coercion, the institutional silencing of abuse narratives, and the fragility of ethical responsibility within professional cultures. Each of these domains poses acute challenges to qualitative researchers, who -once situated in the field- encounter not only suffering but the systemic resistance to its recognition, redress, or repair.

1. Normalization of coercion and the medicalization of structural distress

Participants repeatedly described how psychiatric labeling functioned not as a diagnostic aid but as a mechanism of social disqualification, often weaponized during moments of crisis, family conflict, or institutional breakdown. Particularly in Spain and Italy, users recounted being involuntarily hospitalized or medicated after disclosing experiences of domestic abuse or workplace violence, suggesting that psychiatric systems often reinforce rather than interrupt cycles of harm (Burstow, 2015; Cosgrove et al., 2020). In Sweden, this pattern intensified into what many described as *administrative disappearance*- the use of child protection laws, mental health statutes, or forensic

psychiatric classifications to sever individuals from their families, terminate careers, or justify long-term isolation.

These findings resonate with international concerns about the pervasive misuse of psychiatry as a means of social control (Cohen, 1985; Amnesty International, 2022), especially where coercion is framed as care and dissent is medicalized. The normalization of forced treatment was rarely questioned by professionals, many of whom cited *risk* or *non-compliance* in circular terms. This aligns with prior critiques that biomedical mental health paradigms often obscure the social origins of distress while silencing critique through institutional authority (Moncrieff, 2008; Mills, 2014). Where listening did occur, it was more often by peers, advocates, or researchers than by clinicians, indicating a severe epistemic asymmetry within clinical settings.

2. Institutional betrayal and the ethical silencing of field reports

A major theme emerging from the fieldwork was the impossibility of *reporting upward* in situations of witnessed harm. Whether abuse occurred in hospitals, homes, or welfare offices, researchers and witnesses alike found themselves without secure channels to raise alarms. Reporting often triggered retaliation, deflection, or reputational damage—especially when clinicians, administrators, or legal authorities were implicated. In multiple cases, researchers were warned against *interfering* or *taking sides*, even as participants suffered ongoing harm.

For instance, the researcher's report of suffering a death threat, torture for years and still ongoing severe abuses, was met not with institutional support but with inaction that led to even further threats, harassment, violence, forced displacement, and denial of professional legitimacy, illustrating the punitive asymmetry between those speaking out the truth, denouncing crimes, and those structurally shielded. This mirrors the dynamics described by Smith and Freyd (2014) as institutional betrayal, in which organizations fail to respond ethically to harm within their own systems, compounding trauma and silencing accountability.

The field thus became a space not only of witnessing but of betrayal. Ethical tensions were acute: to disengage would be complicit; to intervene risked professional collapse. In the absence of dynamic protocols or institutional accountability, the ethical burden fell entirely on the individual researcher, exacerbating precarity and psychological toll.

3. Complicity, cowardice, and the reproduction of structural harm

A sobering finding was the extent to which professional cultures, especially among clinicians and researchers, normalized avoidance, deflection, and passive complicity. Even where clear violations were witnessed, many professionals invoked bureaucratic constraints, *non-interventionist* roles, or institutional neutrality as reasons for inaction. This passive complicity constitutes a core mechanism of harm reproduction. Worse still, some actors actively participated in the erasure or defamation of those who challenged the status quo, often under the guise of procedural correctness or reputational protection. This condition, what Freire (1970) would call the internalization of oppressor logic, undermines the very fabric of ethical inquiry. It also challenges the assumptions of procedural ethics, which presume a stable environment of rights, protections, and institutional cooperation. In

reality, the field often operates as a site of moral injury, where truth-telling becomes a liability and protection is only extended to those who remain silent.

These findings compel a reframing of ethical practice in qualitative research. If researchers are to carry the burden of witnessing systemic abuse, then institutions -academic, clinical, legal- must be made accountable for offering protection, coordination, and escalation pathways. The ethics of doing no harm must evolve into an ethics of enabling redress and reform.

4. Action-research under duress: method as resistance

That this research was conducted at all -despite displacement, threats, and lack of institutional protection- testifies to the necessity of integrating ethics into method. In contexts where silence sustains violence, the very act of documentation becomes resistance. The work was undertaken with clear goals: to expose systemic failures, to amplify silenced voices, and to push for reform from within the systems themselves.

Table 2: Ethical issues in mental health qualitative research

Field	Description
Positional risk and exposure	Researchers with lived experience or dissenting perspectives face systemic retaliation, delegitimation, and psychological harm when documenting abuse.
Structural silencing and retaliation	Institutional actors obstruct reporting pathways, retaliate against whistleblowers, and pathologize critics as unprofessional or mentally unfit.
Informed consent and coercive environments	Consent is undermined in coercive contexts, where patients fear retribution, service loss, or further diagnosis for speaking out.
Power asymmetries in field relations	Researcher-participant relationships reflect deep institutional power imbalances, risking complicity without reflexive, relational ethics.
Epistemic violence and testimonial erasure	Participant testimony is often dismissed or erased in clinical and professional settings, reinforcing structural invisibility.
Barriers to real-time ethical action	Ethics protocols lack mechanisms for immediate response to abuse disclosures, leaving researchers without safe and effective intervention channels.
Data integrity and record manipulation	Mental health records are vulnerable to falsification or selective editing, requiring researchers to ensure timestamped, version-controlled documentation.
Public health system complicity	Harm and neglect are normalized within systems of care, placing researchers in ethical dilemmas where harm is concealed or trivialized.
Lack of institutional protection	Universities and research bodies often lack protocols to protect researchers from fieldwork-related risks or from institutional retaliation.
Ethics of translation and policy feedback	Testimonies often fail to reach decision-makers; researchers must forge independent pathways to transform lived knowledge into systemic change.

Major critical ethical concerns encountered and analyzed in the course of longitudinal action-research in mental health services and institutional systems.

Yet, the findings also underscore a profound need for collective infrastructure. No individual researcher, no matter how dedicated, can bear the epistemic, emotional, and political weight of

reform alone. Qualitative research, especially in mental health, must now advance into a new era of institutional ethics -in-action, where protocols, accountability mechanisms, and open-science safeguards support the very interventions they demand (Guillemin & Gillam, 2004; Iphofen, 2017).

This five years long fieldwork research was not about individual suffering, though it was shaped by it. It is about systemic ethical blindness -and the urgent task of building a scholarly and clinical culture that not only sees, but acts. Qualitative research in mental health, particularly when employing ethnographic and action-research methodologies, reveals a persistent and structurally embedded set of ethical dilemmas that cannot be addressed solely by procedural ethics or institutional review boards. Across multiple sites and over a sustained five-year field engagement-including participation in EU-funded initiatives such as FOSTREN, ReMO, and the EU BEACON actions- the findings consistently exposed environments where lived experience is delegitimized, institutional violence is normalized, and the epistemic authority of survivors, patients, and front-line researchers is regularly undermined or pathologized. These findings echo prior critiques of ethics-as-bureaucracy and support the call for dynamic, responsive, and relational ethical frameworks (Guillemin & Gillam, 2004; Denzin & Giardina, 2007; Ellis, 2007).

A key ethical concern centers on the erasure of lived experience under epistemic regimes of biomedical dominance, where subjective accounts of harm, coercion, and structural neglect are recast as symptomatic of illness rather than legitimate evidence. This epistemic injustice (Fricker, 2007) is compounded in contexts where diagnostic power is weaponized to silence critique, rendering the very act of naming abuse a trigger for further institutional punishment or control. Participants in multiple sites reported that attempts to speak out about coercive practices -including forced medication, restraint, and isolation- were reinterpreted as signs of *lack of insight* or *paranoia*, thus perpetuating cycles of disempowerment and silencing (Russo, 2016; Voronka, 2016; Rose & Kalathil, 2019). These dynamics raise fundamental questions about the ethics of consent, voice, and narrative agency within qualitative inquiry.

Moreover, the asymmetry of power between researcher and participant -typically acknowledged as a risk- was reversed in many cases, as researchers embedded in vulnerable positions themselves became targets of institutional repression. When qualitative researchers are also survivors, or operate as peer scholars in precarious environments, the ethical equation shifts drastically. In this research, the act of documentation itself often triggered surveillance, retaliation, or professional sanction. Despite formal ethics approval and institutional affiliations, safety was not guaranteed. Research supervisors, in some cases, failed to respond adequately to threats, abuse, or violations reported during fieldwork -effectively enabling a culture of institutional abandonment. The absence of protective mechanisms for researchers themselves, particularly those working at the intersection of care and critique, reflects a profound ethical gap in current academic infrastructures (Berger, 2015; Gill & Orgad, 2018).

A second major ethical issue relates to the lack of pathways for institutional responsiveness to qualitative findings. Participants shared urgent testimonies of maltreatment, neglect, and even criminal acts within care systems. Yet, qualitative research in mental health lacks clear ethical protocols for real-time response and coordinated redress. The research identified repeated failures

by health and social care services to act upon evidence presented informally or formally. This undermines not only the principle of beneficence but also the scientific and civic legitimacy of the research itself. Unlike biomedical trials, which often have established pathways for reporting adverse events, qualitative research in mental health lacks protocols for activating ethical alerts, documenting high-risk environments, or reporting practitioners engaged in harmful behavior (WHO, 2021; Campbell & Burgess, 2012).

The relational ethics of field engagement were further complicated by the presence of institutional actors who blurred the boundaries between care provider, researcher, gatekeeper, and enforcer. These overlapping roles made it difficult for participants to trust the research process, fearing retaliation, breaches of confidentiality, or collusion with coercive systems. Even when anonymity was assured, the localized nature of mental health services made it difficult to protect identities in small, tightly networked communities. This created a chilling effect, where many potential informants withheld key information, fearing that their cooperation could worsen their situation. Similar challenges have been documented in mental health ethnographies and survivor-led research, where the relational risks of speaking out are high and institutional cultures are resistant to scrutiny (Chamberlain, 2000; LeFrançois, Menzies & Reaume, 2013).

A further finding concerns the moral burden placed on researchers conducting fieldwork in contexts of unresolved trauma. Many participants were actively experiencing distress, institutional betrayal, and unresolved grievances. In such contexts, the researcher cannot ethically remain a neutral observer. The findings call into question the adequacy of traditional models of ethical distance and non-intervention, and instead support the development of trauma-informed, action-oriented, and co-produced research ethics frameworks (Sweeney et al., 2018; Beresford, 2016). This includes protocols for emotional labor, peer debriefing, mental health support for researchers, and ethical exit strategies that avoid retraumatization or abandonment of participants post-fieldwork.

Finally, the limitations of formal ethical oversight bodies were starkly apparent. Ethics committees rarely have expertise in action-research or survivor-led inquiry, and often default to rigid procedural concerns that fail to account for emergent, situated ethical dilemmas. In this research, the need for real-time response mechanisms, version-controlled documentation, secure archiving of sensitive data, and collaborative decision-making far outstripped the static guidelines provided by institutional review boards. These findings echo broader calls for ethical governance reform in research with structurally vulnerable populations (Flicker et al., 2007; Mertens, 2009; Fistein & Quilligan, 2012).

Thus, to be effective and prevent further harm, violence and normalized crimes from repeating, qualitative research in mental health, especially when it engages directly with sites of coercion, exclusion, and structural abuse, must be governed by ethics that are dynamic, grounded, and protective-not merely procedural. The findings of this research contribute to an emerging body of literature demanding that we rethink ethics not as an administrative hurdle but as a living, collective practice of care, justice, and accountability, including protocols of action in case of witnessing or finding out about potential crimes, or severe cases of suffering and violence not yet recognized by practitioners and other colleagues. This shift is not optional -it is the only path forward in a field where human lives, rights, and futures are continually at stake.

Conclusion

The ethical complexities unveiled through this five-year action-research project challenge the very boundaries of what constitutes safe, just, and scientifically grounded qualitative research in mental health. Rather than isolated dilemmas, the findings portray a systemic failure to uphold ethical responsibilities across institutions, services, and research infrastructures. Participants' testimonies of harm -routinely pathologized or erased- reflect not only epistemic injustice (Fricker, 2007; Kidd & Carel, 2017), but an entrenched structural tendency to deny, suppress, and retaliate against uncomfortable truths. Researchers committed to action-oriented methodologies become both witnesses and targets of this violence, confronting a research environment hostile to reflexivity, transparency, or critical accountability (Guillemin & Gillam, 2004; Denzin & Giardina, 2007).

These issues are magnified in contexts like Spain, Sweden, and Italy, where mental health policy and law remain disproportionately focused on control and containment rather than care and redress (Dvoskin & Skeem, 2009; Rose, 2014; Foucault, 2003). Despite recent progress and international declarations urging non-coercive, rights-based approaches (WHO, 2021; Council of Europe, 2023), the operational mechanisms needed to integrate real-time testimony into systemic reform are largely absent. Ethical review boards lack dynamic, context-sensitive protocols. Public institutions often default to denial rather than support when presented with evidence of abuse or malpractice. And researchers, particularly those with lived experience or outsider positionality, are rendered expendable when the truths they uncover threaten institutional interests (Gill & Donaghue, 2016; Clegg, 2016). What this research demonstrates is not simply the presence of ethical risk, but the urgency of a structural reconfiguration. Participatory ethics cannot be procedural; it must be infrastructural. Dynamic feedback loops, public data protocols, and built-in escalation pathways for rights violations must be adopted as core features of qualitative inquiry in mental health. Moreover, universities and public research funders must abandon extractive models of knowledge production in favor of protective, collaborative, and system-improving commitments.

At its most vital, qualitative research is not merely a tool for documentation -it is an instrument of justice, of collective repair, and of epistemic reckoning. This research, embedded in several relevant EU COST actions, such as FOSTREN, culminating in the creation of the EU BEACON One Health Education one, demonstrate the potential of international cooperation to create research ecosystems where testimony is met with transformation, and where those who suffer are no longer expected to remain silent. Our future, at the scientific, social, and planetary levels, depends on our job properly done, following the highest ethical and open science standards, coordinately with other professionals also enforcing the highest possible best practices together.

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Annex C - *Frontiers in Psychology*: implementation of open dialogue in Spain

Annex C of the dissertation, which includes the peer-reviewed article published in *Frontiers in Psychology* on the implementation of Open Dialogue in Spain, is not an isolated addendum but an integral component of the overall research architecture. It reinforces and operationalizes the dissertation's central thesis concerning the necessity of non-coercive, dialogical, and rights-based approaches in mental health care. This annex specifically documents the institutional challenges, epistemic tensions, and clinical opportunities encountered in the nascent attempts to apply Open Dialogue in the Spanish context, making it a critical empirical and theoretical extension of the broader inquiry. The dissertation as a whole critiques the Spanish psychiatric system for its entrenchment in coercive, pharmacocentric, and exclusionary practices, and it does so through a biocultural and transdisciplinary lens. The inclusion of Annex C thus serves to demonstrate that alternatives grounded in dialogical ethics are not only theoretically sound but also practically feasible within the very systems currently dominated by coercive logics. The Open Dialogue model, rooted in relational epistemology and shared sense-making, is presented in this annex not as a foreign or utopian import but as a scientifically validated and ethically imperative framework. The documented efforts to pilot this model in Spain provide concrete evidence that the transition from compliance-based care to collaborative, recovery-oriented practice is possible, albeit institutionally obstructed. Annex C directly corresponds to several operational research questions articulated in the core chapters, particularly those concerning the enabling conditions for institutional transformation, the epistemic reconfiguration of clinical practice, and the reallocation of authority from professional dominance to dialogical partnership. It aligns with the thesis argument that dialogical practice must be redefined not as a mere communicative technique but as a foundational epistemic and ethical orientation. The annex demonstrates how this redefinition can be instantiated through clinical practice, even within systems that structurally resist such shifts.

The narratives and observations contained in Annex C contribute to the dissertation's ethnographic corpus, complementing the testimonies and survey data presented in Chapters 3 and 4. They illustrate the lived contradictions and ethical dissonance experienced by clinicians and users when attempting to enact Open Dialogue within institutional frameworks that remain hierarchical, pharmacologically reductive, and risk-averse. These findings are analytically synthesized in

Chapters 5 and 6, where the dissertation interrogates the structural barriers to rights-based reform and formulates normative proposals grounded in international human rights standards and medical ethics. The inclusion of Annex C confirms the dissertation's methodological commitment to participatory, field-based action-research. The article does not merely describe a model but traces its concrete application, adaptation, and resistance in context. It provides both validation of the research methodology and evidence for the thesis's claim that shared decision-making and dialogical care are empirically viable, clinically effective, and ethically superior alternatives to dominant coercive regimes.

<https://www.frontiersin.org/journals/psychology/articles/10.3389/fpsyg.2023.1166919/full>

Annex D - The aggressiveness of xenophobia

Annex D of the dissertation, *The destructiveness of xenophobia*, constitutes a personal yet analytically rigorous testimony that directly reinforces the core scientific and ethical arguments presented throughout the thesis. While stylistically different from the more academic structure of the dissertation's main chapters, this published piece is deeply coherent with its theoretical premises and empirical findings. It serves as both an evidentiary artifact and a first-person analytical intervention into the intersection of psychiatric misdiagnosis, domestic violence, and institutional betrayal, all mediated through racialised and xenophobic logics of exclusion.

The annex exemplifies the structural argument made in the dissertation regarding the epistemic and clinical erosion that occurs when institutions disregard context, silence the voice of the person, and substitute rigid protocols for compassionate, situated understanding. The author's lived experience, threatened with involuntary institutionalization due to a misread migraine, demonstrates how biomedical instruments, such as TAC scans, can become tools of harm when wielded without interpretive competence, particularly in the presence of coercive interpersonal dynamics. This real-world event validates the dissertation's broader critique of diagnostic reductionism and professional impunity. The annex becomes, therefore, a documented case study that illustrates the very mechanisms the thesis seeks to dismantle. The annex highlights the medical neglect of domestic violence indicators, a recurrent theme in the thesis. It exposes how institutions meant to protect health and dignity may instead become facilitators of additional harm, especially when influenced by discriminatory assumptions and institutionalised racism. These are not anecdotal misfortunes but, as the dissertation repeatedly emphasizes, expressions of a systemic failure that normalizes harm under the guise of care. The annex strengthens the claim that epistemic violence and psychiatric coercion are often co-produced with gendered and racialized exclusions, and that health systems remain deeply susceptible to moral disengagement when encountering those perceived as foreign or non-normative. The text also supports the dissertation's methodological position that testimony and narrative are essential forms of epistemological resistance. It functions as both a testimonial document and an act of scholarly defiance, reasserting the right of survivors to define their own experiences and contribute to institutional critique.

Through this contribution, the dissertation operationalizes its own participatory and action-research framework by integrating embodied knowledge into the academic archive, thereby resisting the dehumanizing effects of structural and clinical abstraction. this annex aligns with the dissertation's call for urgent institutional reform grounded in evidence-based, rights-respecting, and dialogical practice. It underscores the immediate need for diagnostic rigor, trauma-informed protocols, and anti-discrimination safeguards in medical and psychosocial care. It speaks directly to the dissertation's analysis of social death, administrative dispossession, and epistemic erasure, while offering a lived, human counterpoint to statistical and policy analysis. In so doing, Annex D does not stand apart from the dissertation's architecture but embodies it. It expands the evidence base, deepens the methodological plurality, and reaffirms the dissertation's guiding principle: that dignity, truth, and structural justice are non-negotiable imperatives in health care.

<https://research.henning.md/p/the-simplest-thank-you>

Annex E - Nocebo effect and psychiatry: giving up given up syndrome and sociomedical somatisation

Annex E, which includes the authored book chapter *Nocebo Effect and Psychiatry*, constitutes a central theoretical and empirical contribution to the dissertation's argument concerning structural harm, iatrogenic suffering, and the sociopolitical dimensions of psychiatric practice. This chapter deepens the dissertation's core claim that contemporary psychiatric systems frequently produce harm not only through overt coercion or misdiagnosis but also through the cumulative effects of epistemic invalidation, neglect, and interpretive violence. It provides a conceptual framework for understanding how expectations of failure, stigma, and professional disengagement are internalised by patients and translated into chronic physiological and psychological deterioration.

The chapter is especially relevant to the dissertation's analysis of what is termed social death and administrative dispossession. The construct of the giving up, given up syndrome complements and extends the ethnographic and survey-based findings presented in Chapters 3 and 4 by offering an explanatory model grounded in transcultural psychiatry, psychoneuroimmunology, and medical anthropology. It clarifies how systemic invalidation, when repeated across healthcare, legal, and familial systems, can precipitate biological shutdown, loss of agency, and relational withdrawal. This nocebo effect, understood as the induction of harm through negative meaning and institutional betrayal, offers a scientifically and clinically grounded lens to interpret the data collected from survivors, professionals, and family members.

The chapter also provides the theoretical grounding for the dissertation's critique of diagnostic authority and its proposed shift toward dialogical and narrative-based care. It interrogates the semiotics of psychiatric labeling and the performative power of prognosis, linking these with somatic outcomes that cannot be adequately addressed within biomedical reductionism. The dissertation's call for One Health-informed, trauma-sensitive, and recovery-oriented care is reinforced by this annex, which demonstrates how integrated physiological, social, and narrative frameworks are necessary to reverse the damage produced by chronic exposure to invalidation and

exclusion. This annex demonstrates the dissertation's methodological coherence. It showcases the capacity of the researcher to integrate personal experience, theoretical synthesis, and clinical insight into a form of transdisciplinary scholarship that is operational, not merely interpretive. By naming and documenting a recognisable yet under-theorised phenomenon, the chapter also advances the dissertation's broader goal of epistemic repair and reorientation of clinical paradigms toward human dignity and contextualized care.

Annex E is not ancillary but structurally embedded in the dissertation's analytical core. It translates the dissertation's findings into a conceptual schema capable of informing clinical training, diagnostic reflexivity, and health system design. It aligns with the overarching aim of the thesis to dismantle harmful psychiatric orthodoxy and build ethically grounded, scientifically coherent, and structurally competent frameworks of mental health care.

<https://doi.org/10.22533/at.ed.3142122119>

Annex F - *Nature–Springer* book chapter on alternatives to coercion

The *Nature–Springer* book chapter authored by the researcher on alternatives to coercion, holds significant relevance to the core of the dissertation. It presents a rigorous synthesis of evidence-based, rights-oriented, and contextually grounded approaches that challenge the dominant paradigm of coercive psychiatric intervention. By articulating viable clinical and institutional alternatives, the chapter offers a concrete operational counterpart to the dissertation's analytical critique of systemic coercion, diagnostic overreach, and structural violence.

The chapter situates dialogical ethics, service user leadership, and therapeutic alliance as foundational elements of effective and just mental health care. These concepts are not treated as ideological preferences but as empirically validated components of recovery-oriented practice. This aligns directly with the dissertation's central claim that coercion is not only ethically untenable but clinically counterproductive, and that non-coercive models such as Open Dialogue, collaborative medication management, and trauma-informed care are not aspirational ideals but necessary reforms backed by scientific consensus and human rights standards.

The inclusion of this chapter within the dissertation underscores the researcher's active role in international scientific dialogues and affirms the dissertation's methodological and normative credibility. It reflects the capacity to contribute to high-level peer-reviewed discourse while maintaining a critical and transformative perspective grounded in both lived experience and empirical analysis. The chapter also provides a structured overview of implementation pathways, including the legal, educational, and institutional shifts required to transition from coercion to collaboration, which are further developed in the dissertation's concluding chapters and annexes.

Annex F exemplifies the translational dimension of the dissertation, bridging academic research with actionable system redesign. It reinforces the thesis's call for structural reform by demonstrating that ethical and clinically sound alternatives are not only available but already in practice across multiple contexts. The chapter thus affirms the dissertation's dual ambition: to expose systemic

harm and to build coherent, rights-based alternatives capable of sustaining human dignity and institutional legitimacy in mental health care.

https://link.springer.com/chapter/10.1007/978-3-031-61224-4_17

Annex G - Proceedings of political abuses of psychiatry

Political Abuse of Psychiatry, Medical Torture, and Victim De-Legitimization: Insights from an International Symposium

Abstract: The political abuse of psychiatry is an ongoing, systemic phenomenon rooted in the replacement of sound governance and ethical medical standards by incompetent authority, rogue state practices, and criminal misuse. Drawing from a global symposium integrating survivor testimony, academic analysis, and human rights advocacy, the discussion highlights how mental health systems -when left vulnerable by corruption, inefficiency, or deliberate political manipulation- become instruments of repression rather than healing. The ease with which abuses occur in failed systems, the profound human and economic costs inflicted, and the perpetuation of systemic failure through the weaponization of psychiatry are analyzed in depth. Comprehensive solutions are proposed across international, national, institutional, community, and individual levels, emphasizing the urgent need for rights-based, trauma-informed, interdisciplinary reform. Restoring psychiatry's ethical foundations is presented not merely as professional necessity but as a profound moral imperative to protect life, freedom, sanity, and human dignity on a global scale.

Keywords: Political abuse of psychiatry; rogue states; failed healthcare systems; human rights violations; psychiatric coercion; trauma-informed care; mental health reform; international crime; dignity and autonomy; systemic failure; survivor advocacy; healthcare ethics.

Introduction

Throughout history and across the World, psychiatry has held the power to help -or to harm. It is a field that touches the most intimate aspects of human life: thought, emotion, behaviour, identity. Yet this same power has been used to silence, punish, and erase. Under the guise of care, individuals have been subjected to forced treatment, indefinite detention, and forms of medical coercion that violate basic rights. These are not merely past injustices or errors in judgment, but the result of structural conditions that allow systems of health to be bent into tools of control. When dissent, difference, or distress caused by the very same system and criminal actors are labeled as pathology, no one is safe from the risk of being made to suffer. This proceedings paper begins from that stark reality, but not to accept it. Rather, it aims to examine how these abuses occur, why they persist, and what can be done to transform psychiatry into a field worthy of its healing promise.

Political abuse of psychiatry refers to the deliberate misuse of mental-health diagnoses, treatments, or involuntary detention to punish, silence, or discredit dissenting individuals or groups. Van Voren (2009) explains that it means misuse of psychiatric diagnosis, treatment and detention to obstruct people's fundamental rights. Despite appearances to the contrary, such abuses have never been confined to openly authoritarian systems; even in established democracies there are documented instances of whistle-blowers and critics being stigmatized as mentally ill. Indeed, historical and contemporary reports show that the tension between politics and psychiatry has been global and is an ongoing unresolved problem (van Voren, 2010).

The relatively recent historical record is stark. In Nazi Germany, psychiatrists were complicit in eugenic and punitive policies -a violation of their duty of care- using racist and ideologic criteria to justify locking up, deporting and killing disabled and political others (Narayan, 2013). In the Soviet Union during the Cold War, dissent was pathologized on a massive scale. Soviet psychiatrists proclaimed that critics were mentally ill, since no one would oppose their alleged best system unless insane, and often diagnosed dissidents with Snezhnevsky's made up sluggish schizophrenia diagnosis, a supposed mild form of schizophrenia whose symptoms conveniently included reform delusions or struggle for the truth (van Voren, 2010). Van Voren (2009) notes that in the 1970s-80s roughly one-third of all political prisoners in the USSR were confined in psychiatric hospitals. These practices sparked international outrage and led to the Soviet psychiatric society's suspension from the World Psychiatric Association in 1983. Other Eastern Bloc states followed suit to a lesser extent: for example, reports indicate that Romania under Ceaușescu systematically labeled hundreds of regime opponents as mentally ill (often for crimes like petitioning the authorities), resulting in mass psychiatric incarceration. Countries such as Cuba also implemented sometimes short-lived programs of detaining dissidents in mental institutions in the late 20th century. Notably, an unusual case in 1990s Netherlands showed that such abuse could occur in a democratic setting as well: a Dutch Defense Ministry official falsified psychiatric diagnoses to silence a social worker, a scheme later overturned by courts. These historical episodes - whether in Nazi Germany, the Soviet bloc, or isolated cases in the West - underscore that psychiatry has been repeatedly corrupted as a tool of political repression (Narayan, 2013).

At the root of genocides, purges, and systemic repression lies a recurring mechanism: the dehumanization of those labeled as feeble-minded, deviant, inferior, or unfit. Cognitive biases such as moral disengagement, group conformity, and the illusion of a just world, combine with a corrosive sense of superiority that gives moral cover to cruelty. This mindset easily merges with opportunism and criminality, providing social and material incentives for professionals and ordinary citizens to participate in abuse. Hatred becomes a path to belonging and prestige. In Nazi Germany, psychiatrists designed and implemented the Aktion T4 program, selecting tens of thousands of disabled individuals for extermination under the guise of medical necessity. These killings, sanitized through clinical language and bureaucratic process, laid the morals, mindset, rewards, technical, and institutional groundwork for the Holocaust and the Second World War itself (Friedlander, 1995). In the Soviet Union, psychiatry was weaponized to silence dissent: critics of the regime were declared insane and confined indefinitely, often tortured in the name of treatment (van Voren, 2010). These are not historical anomalies. From colonial asylums to apartheid psychiatry, from the Khmer Rouge to present-day internment and reeducation programs, the same logic persists. Racism, ableism, and political repression intersect with pseudoscience and institutional power, allowing medical torture to be reframed as care. Once any system presumes the authority to define sanity, worth, or humanity itself, it grants itself license to destroy. These practices are not only abuses of science and medicine, these are crimes that strike at the core of what our civilization claims to uphold. Failing to expose and dismantle these structures ensures their return, in new and old forms, again and again.

Table 1: Barriers to Political Abuses of Psychiatry

Domain	Protective Factors	Vulnerabilities When Absent
Legal Safeguards	Independent oversight, enforceable patient rights, access to justice	Arbitrary detention, impunity, unchecked coercion
Ethical Practice	External review boards, whistleblower protection, survivor-led ethics	Collusion, fear of reprisal, normalized violations
Clinical Approach	Shared decision-making, trauma-informed care, consent-based treatment	Forced interventions, disregard for autonomy
Training and Culture	Rights-based education, reflective practice, critical pedagogy	Authoritarianism, biomedical dogma, silencing of dissent
Economic Integrity	Transparent funding, non-profit care models, conflict of interest checks	Overmedication, neglect, market-driven interventions
Community Role	Peer support, advocacy groups, participatory service design	Isolation, stigma, lack of accountability
Public Awareness	Media scrutiny, civic engagement, open reporting channels	Denial, invisibility of harm, politicized psychiatric labels

Today, political psychiatry abuses persist in various countries. In the Russian Federation, rights groups and journalists report a revival of punitive psychiatry since 2022. Reuters (2025) found that dozens of anti-war activists and protesters have been ordered into psychiatric evaluation or detention by courts, a pattern reminiscent of the Soviet era (Peoples Gazette, 2025). One detailed case involved Yekaterina Fatyanova, an opposition journalist who was involuntarily hospitalized for two months after her paper published an anti-war article; letters she sent from the hospital describe unnecessary and degrading procedures, and she was finally discharged as mentally healthy. Robert van Voren, who has studied Russian psychiatry, documented roughly 20-30 such cases per year since 2022. In April 2025 a Moscow court also made international news by committing a U.S. citizen (Joseph Tater) to compulsory psychiatric treatment after annulment of criminal charges - a move observers likened to Soviet-era psychiatric coercion (Reuters, 2025). In neighboring Belarus, the authoritarian regime of Alexander Lukashenko has used psychiatric hospitals to punish critics of the 2020 elections. UN human-rights experts reported in early 2025 that at least 33 opposition figures have been coerced into undergoing psychiatric treatment for peaceful protest, with most still held indefinitely. Experts warn that this amounts to a grave violation of human rights, noting that many detainees are kept confined for months or years with no legal recourse (Trickey, 2025).

In the People's Republic of China, too, multiple sources confirm severe ongoing abuses. A 2022 investigation published by Safeguard Defenders found that from 2015-2021 at least 99 petitioners and activists were involuntarily locked up in psychiatric facilities for political reasons, often without any legitimate diagnosis (Mou, 2022). The NGO concluded that forcing critics into mental hospitals is still widespread and routine in China today. These cases span the country: one recent report noted that courts routinely dispatch petitioners, petition filers, anti-corruption complainants and even labor activists to psychiatric wards under various pretexts. The victims describe arbitrary detention, forced medication, electroconvulsive therapy and repeated incarceration, all to silence dissent (The Washington Post, 2022). A prominent example is the so-called *Ink Girl*, Dong Yaoqiong, who in

2018 splashed ink on a poster of Xi Jinping; she was repeatedly committed to a psychiatric hospital, restrained to her bed and beaten for refusing treatment, despite being found mentally healthy each time. International observers note that Chinese abuse of psychiatry today involves far larger numbers of people than even the Soviet regime did, targeting followers of banned faiths (e.g. Falun Gong), human-rights lawyers, dissident petitioners and whistle-blowers.

Elsewhere in Asia and beyond, cases continue to surface. In Iran, for instance, a *Lancet Psychiatry* commentary (Wasserman *et al.*, 2023) described how a young student was forcibly hospitalized in Tehran after staging a protest against compulsory hijab laws, despite having no medical diagnosis - an act likened to political repression through psychiatry (World Psychiatric Association, 2025; *The Lancet Psychiatry*, 2025). Human-rights advocates in the U.S. have also warned of such misuse: historically, racist and political dissent have been pathologized: e.g. antebellum drapetomania for escaping slaves, and mid-20th-century civil-rights activists declared delusional (Edwards-Grossi, 2024). While overt examples in modern American politics are rare, the specter remains. Even well-established democracies have seen whistle-blowers subjected to psychiatric censure (van Voren, 2010). The overall pattern is clear: wherever political power is unchecked, there is incentive to brand opponents as mentally ill.

The political abuse of psychiatry has a long pedigree from Nazi and Soviet eras to today's China, Russia, Belarus and failing, corrupt systems elsewhere. It relies on fabricating or exaggerating psychiatric disorders to neutralize dissent without legal trial, violating medical ethics and human rights at every step (van Voren, 2010). Landmark investigations and recent human-rights reports show that this practice persists under new guises. While the number of countries openly running such programs has declined since the Cold War (van Voren, 2009; Narayan, 2013), current evidence underscores that political psychiatry remains very much a present-day issue. Authorities in repressive regimes, and corrupt institutions in democracies, continue to weaponize mental-health institutions to stigmatize and silence critics (Mou, 2022). Contemporary sources make clear that each claim of involuntary psychiatric commitment for political reasons must be scrutinized as a potential human-rights abuse.

Symposium Presentation

The International Symposium on the Political Abuse of Psychiatry convened between October 1 and November 11, 2023, gathering leading experts, survivor advocates, and human rights scholars to systematically examine the persistence and mechanisms of psychiatric abuses worldwide. Organized independently after the rejection of formal inclusion within the World Psychiatric Association's (WPA) Vienna conference premises, the symposium unfolded across a series of virtual and recorded sessions, ensuring free and rigorous exchange of evidence and strategies for reform.

[Pending naming confirmation and permission to include or not a mention on van Voren's satellite event organized concurrently in Vienna by partner organizations may be referenced here.]

Table 2: Symposium Speakers, Topics Covered, and Core Focus

Speaker(s)	Topic Covered	Core Focus
Alejandra Gandolfi	Mental healthcare and discrimination against Moroccan migrants in Spain	Systemic Abuse
Al Galves	Critique of the biomedical model and promotion of holistic mental health care	Systemic Reform, Survivor Advocacy
Cathy Wield	Informed consent and psychiatric detention from a survivor-doctor perspective	Survivor Advocacy, Human Rights
Chris Munt	Survivor testimony on psychiatric violence in the UK	Survivor Advocacy
Dainius Pūras	Human rights and mental health from a UN perspective	Human Rights
David Matas	Psychiatric abuses linked to organ harvesting in China	Human Rights, Legal Frameworks
Edel Granda	Psychiatric pathologization of transgender individuals	Human Rights
Hel Spandler	Structural coercion in UK mental healthcare	Systemic Abuse
Itxaso & Olaya (Orgullo Loco Madrid)	Grassroots activism against psychiatric coercion in Spain	Survivor Advocacy
Lidea Losa & Xisca Morell	Psychiatric abuses and guardianship issues in Spain	Systemic Abuse, Survivor Advocacy
Manuel Llorens	Political psychiatric abuses during Venezuela's health system collapse	Systemic Abuse
Murphy Halliburton	Importance of transcultural psychiatry to counter biomedical dominance	Transcultural Psychiatry
Paola Di Maio	Systems view of psychological coercion in mental health	Systemic Abuse
Peter Groot	Medication withdrawal and the right to taper safely	Survivor Advocacy
Peter Lehmann & Craig Newnes	Historical and contemporary human rights abuses in Europe	Systemic Abuse
Petr Winkler	Reforming Czech psychiatry to protect human rights	Legal Frameworks, Human Rights
Sarah Smith	Mutual aid networks resisting psychiatric abuses (MindFreedom SHIELD)	Survivor Advocacy
Yanxi Mou	China's black prisons and extrajudicial psychiatric detention	Human Rights
Yutong Zhang	Psychiatric repression of dissenters in the PRC	Human Rights

Participants brought distinct and crucial expertise. Dainius Pūras, former United Nations Special Rapporteur on the right to health, provided critical insights on the systemic contradictions between

psychiatric practice and human rights frameworks, grounding the discussions in international legal obligations. Peter Lehmann, survivor advocate and founder of the European Network of (Ex-)Users and Survivors of Psychiatry, and Craig Newnes, clinical psychologist and critical psychiatry scholar, contributed an essential historical and experiential analysis of coercive practices across European contexts. Peter Groot, research scientist and leading proponent of individualized medication withdrawal solutions, introduced evidence-based strategies to dismantle forced chemical management. Alejandra Gandolfi, anthropologist, illuminated the intersections between psychiatric marginalization and cultural displacement, particularly within Spain's Moroccan communities.

Hel Spandler, editor of *Asylum: The Magazine for Democratic Psychiatry*, critically examined systemic coercion within the UK mental health system. Chris Munt, survivor and advocate, shared firsthand accounts of abuses in British psychiatric care, highlighting the normalization of violence. Paola Di Maio, systems scientist, articulated a structural model explaining how coercion infiltrates mental health services through psychological and systemic mechanisms. Edel Granda, activist and advocate for transgender rights, foregrounded the pathologization of gender non-conformity as a key dimension of psychiatric oppression.

Murphy Halliburton, professor of anthropology specializing in transcultural psychiatry, emphasized the necessity of integrating cultural and social understandings to counter hegemonic biomedical dominance. Manuel Llorens, Venezuelan psychologist and academic, documented the collapse of healthcare structures and their exploitation for political repression. David Matas, human rights lawyer and Nobel Peace Prize nominee, provided a legal and forensic perspective on the psychiatric and organ harvesting abuses perpetrated in China. Yutong Zhang and Yanxi Mou, Chinese human rights defenders, exposed the systematic use of psychiatric detention to silence dissent, presenting detailed documentation and survivor testimonies.

Sarah Smith, activist with the SHIELD MindFreedom network, shared strategies for survivor-led mutual aid and resistance against coercive psychiatry. Petr Winkler, director of the Czech National Institute of Mental Health, contributed a case study on rights-centered psychiatric reform within a post-totalitarian context, offering a potential model for systemic transformation. Al Galves, clinical psychologist and author, critically assessed the biomedical model's failures in the United States and outlined paths toward holistic and voluntary mental health care.

Derek Summerfield critically examined the global mental health movement, emphasizing how Western psychiatric models are often exported into diverse cultural contexts without sufficient adaptation or scrutiny. He argued that this expansion often perpetuates medicalized understandings of distress while sidelining social, political, and economic determinants of suffering. Summerfield warned that global mental health initiatives, although framed as humanitarian, can become vehicles for the homogenization of psychiatric practices and facilitate new forms of coercion and marginalization. His intervention underscored the need to critically interrogate not only national abuses of psychiatry but also how international agendas risk replicating systemic failures under the guise of global health promotion, thus reinforcing rather than dismantling structures of oppression.

Each participant’s contribution was integral to constructing a comprehensive, interdisciplinary understanding of how psychiatry, when severed from its ethical foundations, becomes vulnerable to criminal misuse, political repression, and systemic failure. Their collective expertise underscored the necessity of principled, multidisciplinary action to reclaim psychiatry for healing, freedom, and dignity.

Mechanisms of Abuse (Psychiatric, Psychological, Social)

Across the independent presentations at the symposium, a detailed portrait emerged of the mechanisms by which psychiatry is weaponized against individuals. These mechanisms, operating simultaneously at psychiatric, psychological, and social levels, reflect a shared understanding among the speakers that abuses today are systemic, severe, and require immediate redress.

Table 3: Historical and current cases of political abuse of psychiatry

Country	Period	Target Group	Type of Abuse Documented
Soviet Union	1950s–1980s	Political dissidents, human rights activists	Forced hospitalization, chemical restraint, isolation
China	1990s–present	Petitioners, Falun Gong members, dissidents, ethnic minorities	Forced hospitalization, chemical restraint, organ harvesting and trafficking
Russia	1960s–1980s; resurgence post-2010	Political dissidents (historical); anti-war protesters (current)	Forced psychiatric evaluation, silencing, social stigma
Belarus	2020s–present	Political protesters, journalists	Coercive hospitalization, stigmatization
Venezuela	2010s–present	Political opponents, dissidents	Forced psychiatric confinement, political intimidation
United States	1950s–1970s	Civil rights activists, indigenous communities, antiwar protesters	Wrongful institutionalization, psychiatric silencing
United Kingdom	1970s–1990s	Persons labeled as dangerous, racial and ethnic minorities	Overuse of detention, coercion in psychiatric care
Spain	1936–1980s (Civil War, Dictatorship, Post-dictatorship)	Political prisoners, dissidents, social minorities	Political repression via psychiatric institutions, social control through diagnosis

At the psychiatric level, Peter Lehmann and Craig Newnes highlighted the way in which forced treatment practices are justified through diagnostic constructs that lack rigorous clinical basis in many cases. They emphasized that psychiatric categories are often expanded to encompass a wide range of socially undesirable behaviors, facilitating involuntary hospitalization under vague pretexts (Lehmann & Newnes, 2025). Peter Groot’s intervention on the use of tapering strips illustrated

another dimension: the over-medicalization and long-term chemical control of individuals under the guise of psychiatric care. He noted that psychiatric drug regimens, initially imposed under coercive circumstances, are rarely reviewed critically, leading to dependency and the erosion of autonomy (Groot, 2025).

Psychological mechanisms were particularly illuminated through the testimonies of survivors. Chris Munt described how everyday practices within psychiatric institutions, including threats, humiliation, and arbitrary use of restraint, foster a climate of terror rather than healing (Munt, 2025). Sarah Smith added a critical dimension by explaining how psychiatric labeling leads to profound identity destabilization. Once classified as mentally ill, individuals find their narratives invalidated, their perceptions systematically doubted, and their autonomy curtailed, creating a cycle of learned helplessness (Smith, 2025). Hel Spandler's analysis of UK psychiatric reforms suggested that, despite superficial procedural safeguards, the underlying culture of distrust toward patients remains pervasive, maintaining psychological coercion even where formal rights protections exist (Spandler, 2025). Orgullo Loco Madrid provided a critical intervention focused on the systemic failures of the Spanish mental health system from the perspective of the user and survivor movement. They emphasized how psychiatric institutions continue to function as mechanisms of control and silencing, rather than as spaces of healing and support. The speakers highlighted the persistence of coercive practices, including involuntary hospitalization and forced medication, often justified under paternalistic frameworks that deny the autonomy and voice of the individuals affected. They further stressed the structural marginalization of survivors in both clinical practice and policymaking, calling for a radical transformation toward user-led, rights-based, and emancipatory models of mental health care. Their testimony underscored the need to recognize psychiatric oppression as a form of political and social violence embedded in broader patterns of discrimination and exclusion.

At the social level, Edel Granda's presentation on transgender rights revealed how psychiatric structures are employed to marginalize already vulnerable populations. She argued that pathologizing gender non-conformity perpetuates systemic exclusion and medical violence under the veneer of care (Granda, 2025). Lidea Losa and Xisca Morell provided a powerful account of how Spanish psychiatric institutions often use legal and social levers, such as guardianship regimes, to strip individuals of civil rights, making abuse both invisible and legally sanctioned (Losa & Morell, 2025). Dr. Paola Di Maio expanded the systemic view by framing psychiatric coercion as part of broader systems of psychological control in society, wherein labeling, enforced dependency, and isolation function as tools to suppress dissent and difference (Di Maio, 2025).

Together, although articulated independently, these contributions outlined a sophisticated model of how political, institutional, and interpersonal forces converge to perpetrate psychiatric abuses. The resulting mechanisms are not incidental but structurally embedded, calling into question the very ethical foundations of current mental health practices in many contexts.

Thematic Synthesis of Symposium Discussions

The thematic convergence of the symposium was clear: despite differing geographical focuses and analytical lenses, speakers consistently revealed the persistence, gravity, and systemic character of psychiatric abuses. No formal consensus process occurred during the event, but the independent presentations together illuminated critical thematic patterns.

One central theme was the **instrumentalization of psychiatric diagnosis**. David Matas, focusing on China's organ harvesting practices, explained that psychiatric labels are used strategically to delegitimize political and religious dissidents, facilitating both their disappearance and commodification (Matas, 2025). Similarly, Yutong Zhang detailed how psychiatric diagnoses in China are weaponized against petitioners and activists, with forced hospitalization serving as a method of silencing (Zhang, 2025).

Another recurring theme was the **role of forced treatment and chemical control**. Peter Groot's exposition on tapering strips underscored how psychiatric medication regimens, often initiated under coercion, become chronic mechanisms of control rather than care (Groot, 2025). This point resonated with the broader critique articulated by Chris Munt and Al Galves, who emphasized that in the UK and USA respectively, institutional psychiatry often prioritizes chemical restraint over addressing the underlying social or psychological distress of individuals (Munt, 2025; Galves, 2025).

Table 4: Mechanisms Abuse: Subterfuges, Methods, and Enabling Conditions

Subterfuge or Foul Play	Method Employed	Path to Extrajudicial Locking	Permissive Factors
Fabrication of mental instability	Spreading rumors, false reports, character assassination	Initiates involuntary psychiatric evaluation without cause	Weak legal standards, corruption in healthcare and law enforcement
Provocation into reactive behavior	Harassment, isolation, threats	Victim's natural defense misinterpreted as psychiatric symptoms	Institutional bias, failure to investigate impartially
Forced biological destabilization	Sleep deprivation, food deprivation, chemical agents	Induces cognitive or emotional breakdowns misused as justification	Medical malpractice, collusion between non-medical and psychiatric actors
Misdiagnosis and diagnostic inflation	Deliberate exaggeration or falsification of symptoms	Grounds for commitment without independent review	Inadequate oversight, professional impunity
Coerced testimonies	Pressuring family or associates to corroborate false claims	Falsified support for psychiatric intervention	Fear, loyalty conflicts, systemic impunity
Weaponized guardianship or custody abuse	Manipulating civil legal processes	Enables psychiatric confinement under pretenses of protection	Lack of transparency, judicial rubber-stamping
Administrative shortcuts and procedural abuses	Detention without proper judicial authorization	Administrative detention masked as medical necessity	Low accountability in bureaucratic and health systems
Abuse of emergency psychiatric holds	Misuse of short-term <i>crisis</i> detention powers	Converts temporary holds into extended confinement	Loopholes in mental health laws, lack of mandatory reviews

The tactics identified in the table above are not limited to authoritarian or politically repressive systems. Similar patterns are visible in domestic violence cases, organized crime, and systemic failures in regular healthcare settings. In domestic environments, abusive partners, family members, or associates may similarly provoke, destabilize, or falsely accuse victims to gain control, silence dissent, or exploit vulnerabilities. Organized crime networks may weaponize mental health accusations to intimidate or remove threats without legal processes. Even within ostensibly democratic societies, negligent or corrupt actors within healthcare systems may collude to achieve unlawful psychiatric detention for convenience, financial benefit, or retaliation. The intersection of psychiatry with broader systems of political and social repression emerged prominently. Manuel Llorens described how in Venezuela, psychiatric detention has been repurposed as a tool to neutralize political opponents, with little concern for medical legitimacy (Llorens, 2025). Yanxi Mou’s testimony on China’s black prisons showed how psychiatric justifications are created post hoc to enable extrajudicial detention without legal oversight (Mou, 2025).

The de-legitimization and isolation of victims was another major theme. Sarah Smith and Edel Granda highlighted how individuals labeled as mentally ill, particularly those from already marginalized communities, experience profound social exclusion, often compounded by institutional betrayal and public stigma (Smith, 2025; Granda, 2025). Dr. Paola Di Maio’s systems analysis reinforced this, positing that psychiatric practices of categorization and containment mirror broader societal mechanisms of control and marginalization (Di Maio, 2025).

A thread of human rights protection and ongoing reform ran through many contributions as well. Dainius Puras stressed the international human rights framework that obliges states to move toward non-coercive, rights-respecting mental health systems, though he acknowledged the gap between principle and practice remains vast (Puras, 2025). Petr Winkler provided a cautious example of progress, outlining how the Czech Republic has undertaken systemic reforms embedding human rights into psychiatric care, though challenges persist in implementation (Winkler, 2025). It is agreed that addressing the systemic vulnerabilities leading to abuses, medical torture and extrajudicial killings requires more than procedural reforms. Healthcare systems must be reconstructed on principles of strict transparency, due diligence, and external accountability. Every psychiatric intervention must be independently reviewable, open to audit, and subject to clear, enforceable legal standards. Honest professionals, ethical medical bodies, and judicial systems must coordinate to prevent and punish abuse without exception. Survivors’ testimonies must be central to reform. Denial of these realities only perpetuates harm. Recognizing and confronting these abuses directly is essential to building medical and legal systems that protect health, dignity, freedom, and fundamental human rights for all.

Table 5: Mechanisms of Abuse – Framing and diagnosis leading to psychiatric incarceration

Psychiatric Diagnosis Used	Legal Status of Detention	Explanatory Relevance	Framing Methods
Sluggish schizophrenia,	Involuntary civil	Indefinite detention via	Isolation, sleep deprivation,

paranoia	commitment	vague symptoms	induced confusion
Political mania, paranoia, delusional disorder	Administrative detention, extrajudicial internment	Bypassed judiciary, mass suppression	Harassment, defamation, staged accusations
Paranoia, delusional disorder, any excuse	Civil and forensic commitment	Silencing dissenters, profiting, threatening	Entrapment, covert intimidation
Schizophrenia, psychopathy	Court-ordered psychiatric examination	Protest repression, socioeconomic control	Hostile confinement, coercive interrogation
Non-specified psychotic disorders	Arbitrary administrative detention	Flexible repression tool	Deprivation of needs, induced agitation
Schizophrenia, sociopathy, bipolar disorder	Civil commitment, misuse of psychiatric testimony	Suppressing activism	Manipulated testimony, emotional framing
Schizophrenia, antisocial personality disorder	Involuntary commitment	Racial and social control	Profiling, criminalization of poverty behaviors
Paranoid psychosis, manic-depressive disorder	Arbitrary internment, punitive civil commitment	Political repression	Surveillance, rumor-spreading, social isolation

[Add beatings, threatening, spiking, family abuses, destabilization by any means. Point at same means in cases of domestic abuse, systemic violence, brutality from forces, all that drags and mask inflicted suffering as a disease, disorder, illness of the victims, on top of medical torture. Delve into structural violence and all excuses to keep on holding systems of care unable to work it out, heal.]

Although speakers came from diverse backgrounds and contexts, their presentations converged implicitly around the recognition that psychiatric abuses are not relics of the past but ongoing systemic violations. They underscored the urgency of confronting these abuses through legal, institutional, and cultural transformation, while highlighting that many victims today remain invisible, unprotected, and unheard.

The weaponization of psychiatry and the manipulation of media narratives operate symbiotically to maintain systemic oppression. Psychiatric institutions, when subordinated to political or social agendas, provide a veneer of medical legitimacy to acts of repression, framing resistance, trauma, or nonconformity as clinical disorders. Media outlets, whether through active propaganda or passive repetition of official narratives, sanitize these abuses, diffusing public outrage and transforming grave violations into mere episodes of private tragedy. This convergence facilitates the erasure of victims' political agency, decontextualizing their suffering and reinforcing hegemonic control. In environments where dissent is criminalized through medicalization, and where suffering is depoliticized through media framing, the possibilities for justice diminish radically. The challenge, therefore, lies not only in exposing individual abuses but in systematically dismantling the interlocking structures that allow psychiatric authority and mass communication to be deployed as tools of silencing, erasure, and social domination.

Ongoing cases, contemporary patterns and regional examples

The international legal framework explicitly prohibits the use of medical interventions to inflict pain, suffering, or coercive control. The Convention against Torture and Other Cruel, Inhuman or Degrading Treatment or Punishment (UNCAT, 1984) directly criminalizes acts of torture, including those perpetrated under the guise of medical treatment. Furthermore, the Principles of Medical Ethics Relevant to the Protection of Prisoners and Detainees against Torture (adopted by the UN General Assembly in 1982) expressly forbid health professionals from participating in or condoning any form of torture or degrading treatment. The Rome Statute of the International Criminal Court (1998) recognizes torture as a crime against humanity, regardless of whether it occurs inside medical institutions.

Despite these robust legal standards, enforcement regarding psychiatric abuses has been alarmingly deficient. Medical torture laws are theoretically comprehensive but rarely applied to psychiatric settings. Several factors contribute to this impunity: the medicalization of harm cloaks abusive practices in clinical legitimacy; psychiatric patients are often deemed unreliable witnesses; and judicial systems are generally reluctant to intervene in medical affairs unless overwhelming evidence is presented. Furthermore, systemic biases continue to downplay coercion within psychiatry as merely a clinical necessity, rather than recognizing it as a potential act of torture. The regional examples presented at the symposium painted a sobering picture of psychiatric abuse's current global landscape.

In China, Yutong Zhang and Yanxi Mou provided complementary insights into the systematic use of psychiatric detention against political dissidents and minority groups. Zhang described a bureaucratic machinery that facilitates psychiatric abuse at both local and national levels, while Mou detailed the existence of clandestine detention centers operating outside any legal framework, where psychiatric justifications are retrofitted to disappear detainees (Zhang, 2025; Mou, 2025). Manuel Llorens's presentation on Venezuela revealed that psychiatric institutions are being used to enforce political loyalty and punish dissent. He recounted specific cases where activists were diagnosed with fictitious mental illnesses and confined without due process (Llorens, 2025). In the United Kingdom, Hel Spandler and Chris Munt independently reported that while overt political abuses are rare, systemic coercion remains endemic within mental healthcare. Patients, particularly from marginalized backgrounds, continue to experience involuntary commitment, forced medication, and the delegitimization of their narratives through psychiatric labeling (Spandler, 2025; Munt, 2025). Al Galves's analysis of the United States echoed these concerns. He argued that despite legal protections, psychiatric abuses persist through mechanisms such as outpatient commitment, guardianship laws, and the dominance of the biomedical model, which often overrides patient autonomy (Galves, 2025). In the Czech Republic, Petr Winkler presented a more hopeful picture. He described how systemic reforms have begun to reorient psychiatric care toward human rights and community integration, offering a model for other countries, although he cautioned that changing institutional cultures remains an ongoing challenge (Winkler, 2025). Finally, Edel Granda emphasized that psychiatric abuses intersect with other axes of oppression, such as gender identity. She noted that transgender individuals continue to be pathologized in many mental health systems, leading to denial of care, coercion, and social marginalization (Granda, 2025).

Taken together, these regional reports demonstrate that while the forms and intensity of psychiatric abuse vary, the underlying patterns of coercion, de-legitimization, and systemic violence remain pervasive across political systems and cultural contexts. The urgent need for reform is not limited to any single country or regime but is a truly global imperative. Given the overwhelming evidence from historical and contemporary abuses, it is clear that medical torture laws must be interpreted and enforced to include psychiatric abuses without delay. Non-consensual interventions performed without immediate life-threatening justification, prolonged and coercive hospitalizations, punitive uses of psychiatric confinement, and medical practices aimed at suppressing political, social, or personal dissent must be recognized not as mere clinical misjudgments, but as grave human rights violations. Applying medical torture statutes consistently would dismantle the protective shield of clinical language often used to sanitize abuse. It would also affirm the universality of human dignity and bodily autonomy, whether in prisons, interrogation rooms, hospitals, or psychiatric wards. This shift demands not only judicial courage but also structural reform in how psychiatry is monitored, regulated, and held publicly accountable.

Building sustainable and principled momentum for reform

The urgency and severity of the abuses documented throughout the symposium highlight the necessity of not merely identifying failures, but actively constructing sustainable, principled momentum toward the full reform of psychiatric, psychological, and broader mental health services. Acknowledging the profound historical and ongoing harm inflicted through coercive practices is a prerequisite for any genuine transformation. Equally critical is recognizing the unprecedented opportunity to rebuild systems grounded in science, ethics, dignity, and human rights, leaving behind the entrenched ideologies and defensive structures that have long shielded institutions from necessary accountability. The enforcement of internalized abusive laws often transcends formal state structures, becoming embedded in the practices of healthcare services, family members, and the broader community. Psychiatric systems, when weaponized, do not operate in isolation; they are supported and reinforced by social actors who absorb, normalize, and propagate the underlying ideologies of coercion. Families, under pressure or seeking control, may initiate involuntary psychiatric processes against dissenting members, framing personal conflicts as clinical crises. Entire communities, steeped in stigma and fear, readily accept the marginalization and silencing of individuals labeled mentally ill, conflating nonconformity with pathology. Healthcare services themselves, functioning under regulatory frameworks that legitimize coercive practices, internalize and routinize violations of autonomy as professional standards. This collective complicity transforms abuse into a moral imperative: actions such as forced hospitalization, overmedication, or guardianship stripping are perceived not as violations, but as socially sanctioned duties. Similarly, in corrupted environments, ordinary crime assumes the guise of moral enforcement. Acts of violence, deceit, and systemic destruction are perpetrated under the rationalization of protecting order, family, or national security. The body politic thus operates not merely through official decrees but through the diffuse internalization of oppressive premises, enabling entire populations to participate in, and sustain, the political abuse of psychiatry and other forms of institutional violence.

without critical reflection. Addressing this phenomenon requires dismantling not only abusive laws but the cultural and moral structures that legitimize their application at every social level.

Efforts to reform mental health care cannot be piecemeal, symbolic, or dependent solely on voluntary professional adaptation. Structural and cultural change must be driven by a coordinated strategy that addresses all levels of the system simultaneously. Entrenched actors - whether professional associations, political authorities, financial interests, or institutional bureaucracies - that resist transparency, accountability, or survivor leadership must be actively challenged and displaced from positions of influence. The tolerance of denial, minimization, or complicity with coercive practices can no longer be accepted under the guise of stability or tradition.

Education stands at the center of this transformative agenda. Immediate, mandatory retraining initiatives must be launched across all sectors of mental health services - from psychiatry and psychology to social work, nursing, and administrative leadership. Training programs must fully integrate human rights standards, trauma-informed methodologies, and critical analyses of past abuses. Curricula must be redeveloped to reflect the best available medical, psychological, and social science evidence, emphasizing voluntary, person-centered, interdisciplinary approaches to care. Crucially, this education must not remain theoretical but must be translated directly into clinical, administrative, and legislative practice.

Every psychiatric clinic, general hospital, mental health center, oversight body, and legislative framework must be reoriented to implement these principles. Institutions must embed binding protocols that ensure respect for autonomy, informed consent, dignity, and non-discrimination. Oversight mechanisms must be granted real power to enforce compliance and address violations swiftly and transparently. Legislation must codify rights protections not as optional standards but as enforceable guarantees, fully aligning domestic legal frameworks with international human rights obligations.

To ensure this translation into practice, interdisciplinary implementation teams - combining legal experts, human rights monitors, trauma specialists, survivor advocates, and medical professionals - must be deployed to guide reforms, audit institutions, and monitor compliance over time. The success of these efforts must be evaluated not by institutional self-reporting but by measurable outcomes: reductions in coercive interventions, increased voluntary engagement, survivor satisfaction, and documented improvements in community well-being.

The opportunity to build serious, effective momentum for psychiatric and psychological reform exists now. But it demands abandoning self-protective narratives, acknowledging the systemic nature of past and present harms, and committing to a future where mental health services protect, heal, and empower rather than control, silence, or destroy. The responsibility is collective, and the obligation is urgent: to restore mental health care to its rightful place - as a foundation for human dignity, social justice, and true healing.

Reform at the International Level

Strengthen and enforce international legal standards: States should fully implement the United Nations Convention on the Rights of Persons with Disabilities (CRPD) and related treaties. In particular, governments must heed the CRPD Committee's call to ban all non-consensual psychiatric interventions and involuntary hospitalization. International human rights bodies (UN Special Rapporteurs on health, torture, and disability, the UN Human Rights Council and Universal Periodic Review, etc.) should systematically monitor mental health laws and practices, issuing binding recommendations. Global institutions like the World Health Organization should integrate human rights and trauma-informed care into their mental health guidelines, requiring UN member states to shift resources from coercive institutions to community-based supports. International funding mechanisms (WHO, World Bank, philanthropy) must prioritize alternatives to institutional care and psychosocial rehabilitation, as urged by former UN Rapporteur Dainius Puras.

Insulate mental health services from criminal and non-medical abuses: International cooperation is needed to expose and punish abuses such as organ trafficking and extrajudicial detention. For example, medical associations worldwide should demand the release of psychiatric prisoners (e.g. Falun Gong detainees) and denounce illicit organ procurement. Global psychiatric and transplant societies ought to refuse submissions and presentations from organizations that cannot demonstrate ethical sourcing of transplant organs. Diplomatic pressure and targeted sanctions (e.g. Magnitsky-style human rights sanctions) can deter regimes that misuse psychiatry for political ends. Coordinated action by United Nations agencies (OHCHR, WHO, OPCAT) and international NGOs must expose black jails and punitive psychiatric detention centers, and call for their abolition.

Global promotion of human-rights-based mental health: The UN and regional bodies should sponsor public campaigns and capacity-building to promote trauma-informed, person-centered care. A global panel of experts (including survivors and trauma specialists) could develop best-practice guidelines (on preventing re-traumatization, respecting autonomy, etc.), which countries would be expected to adopt. Education initiatives under UNESCO or WHO could train health, police and justice officials in survivors' rights. Finally, international networks of survivors and advocates (such as MindFreedom International) should be supported to share strategies and hold governments accountable.

Reform at the National Level

Legislative reform and enforcement: Each country must align its mental health and disability laws with human rights norms. This means repealing or tightly restricting involuntary commitment laws, abolishing coercive treatments (force medication, restraint, ECT without consent), and ensuring due process with effective legal representation. Governments should enact parity laws so that insurance/Medicare covers holistic care (housing, therapy, peer support) as well as medication. For instance, while Medicaid/Medicare in the U.S. pays for psychiatric drugs, it will not fund non-medical alternatives like Soteria houses. National policy must close such gaps: insurers and public health programs should cover community-based recovery services (residential supportive housing, peer-run respite centers) to prevent re-institutionalization.

Dedicated funding and resources: Budgets must shift from custodial hospitals to community supports. Governments should prioritize and fund a spectrum of psychosocial rehabilitation and recovery programs. This includes crisis outreach teams, supported employment, education and

housing initiatives. Victims must have rapid access to counseling, legal aid and compensation; funding for victim protection programs should be guaranteed. Transparency in spending is essential to deny misuse of resources by corrupt interests (for example, requiring public reporting of hospital bed occupancy and preventing profiteering from involuntary care).

Oversight and accountability: Establish independent national monitoring bodies (ombudsperson or inspectorates) to oversee all psychiatric and custodial facilities. Human rights commissions and parliaments should have the power to audit institutions, investigate complaints, and prosecute abuses. Professional licensing boards must sanction clinicians who violate patients' rights or collude with non-medical actors. Strict conflict-of-interest rules should bar psychiatrists from receiving undisclosed payments from pharmaceuticals, prisons, or security agencies. Law enforcement must have clear protocols to prevent police from using psychiatric detention for social control or evidence collection. Any deviation by staff (such as nepotistic confinement or unlawful experimentation) must carry criminal liability.

Education and training: Mandatory human-rights and trauma-informed training is needed for judges, police, healthcare workers, and community leaders. For example, curricula for medical, nursing and law courses should include the CRPD, survivors' perspectives, and the dangers of coercion. Public education campaigns can inform families and communities about the *dignity* and autonomy of persons with psychosocial disabilities, countering stigma. Ensuring services meet victims' needs also means providing culturally appropriate care: curricula should include transcultural psychiatry and diverse healing practices, so that indigenous, spiritual and minority groups receive respectful support.

Community safety nets and re-traumatization prevention: National guidelines should require that all victim support services be trauma-sensitive. For example, emergency shelters and clinics must avoid punitive environments (no locked wards or invasive security checks unless absolutely necessary). Safeguards against re-traumatization include no use of prone restraint or sensory deprivation, and routine screening for prior trauma so clinicians can adapt care. Rehabilitation programs should be voluntary and empowerment-focused; consent must always be sought (even for psychotherapy, support groups, etc.). Patient advocacy laws (such as advance directives or treatment agreements) can strengthen autonomy.

Reform at the Institutional Level

Rights-respecting clinical care: Hospitals and clinics must integrate human rights at every level of care. Treatment plans should be co-produced with patients, emphasizing *recovery, dignity and communication*. Institutions should implement open-door policies wherever safe, eliminate unnecessary locking of wards, and use restraint only as a last resort with strict oversight. Staff must conduct debriefing after any coercive intervention and offer immediate apologies and support to the individual. Multidisciplinary ethics committees (including legal experts and survivor representatives) should review contentious cases (e.g. capacity issues). Enforceable patient charters should be posted, detailing rights (to refuse treatment, to an independent advocate, to dignity, etc.) and providing confidential complaint channels.

Trauma-informed environment: Facilities should be designed and operated to minimize trauma triggers. This includes quiet spaces, privacy, and options (e.g. single rooms if requested). Staff should receive trauma-awareness training so that simple actions (a calm tone of voice, asking permission before touching) become routine. Every institution needs permanent peer-support workers or counsellors (people with lived experience) available to help victims navigate the system. Models like *Soteria houses* or peer-run crisis centers can be embedded in the health system as alternatives to hospitals - ensuring that voluntary, non-medical recovery options exist.

Staff selection and oversight: Screening and monitoring of personnel must weed out those who might exploit patients (any connections to criminal gangs, forced labor schemes, etc. should disqualify them). Ongoing human-rights auditing (possibly in partnership with NGOs) can quickly detect abuse patterns. Whistleblower protections and mandatory reporting rules will deter cover-ups. Institutions should use external reviewers for deaths or serious injuries in custody, ensuring families and human-rights groups can observe investigations. Violations must lead to swift disciplinary action or legal consequences, enforcing a culture of zero tolerance for human-rights abuse.

Services tailored to victims: Hospitals and care programs should be adapted to victims' specific needs. This means readily available translators, gender-sensitive care, and services for special populations (LGBTQ+, refugees, indigenous people). Psychosocial supports (occupational therapy, art/music therapy, peer groups) are integral - not optional extras. For example, trauma survivors often benefit from narrative therapy or community healing circles, rather than just medication. Implementing restorative justice options (apologies from institutions or compensation funds) can help victims reclaim dignity and trust.

Reform at the Community and Civil Society Level

Empower survivor and peer networks: Civil society must be at the forefront of reform. Governments should fund and partner with organizations run by people with lived experience (psychiatric survivors, families, activists) as equal stakeholders. Mutual-aid groups like the SHIELD MindFreedom network and the UK Paranoia Network (formed by survivors) demonstrate the power of peer support. Such groups can offer peer counseling, crisis respite homes, and advocacy training. Community grants should enable survivors to organize town halls and advise local health agencies on victims' needs.

Public education and stigma reduction: Grassroots campaigns can shift public attitudes about mental health and rights. For example, mental health first-aid training in schools and workplaces can emphasize listening and respect over medicalization. Public service messaging, in collaboration with media and influencers, should promote stories of recovery and neurodiversity, challenging notions of *madness* as shameful. Community leaders (faith groups, local councils) can host seminars on how to support victims and prevent abuse.

Watchdog and advocacy role: Independent NGOs and professional associations should monitor compliance at local levels. They can publish reports on human-rights conditions (as human-rights researchers in China and elsewhere have done) to alert the world to abuses. Civil society should use legal tools (strategic litigation, amicus briefs) to enforce rights - for instance, Czech activists

succeeded in halting forced ECT by appealing to legislators and invoking UN reports. Journalists and public watchdogs should be trained to identify and report coercive practices. Moreover, alliances between survivor groups, lawyers, and mental health professionals can lobby for meaningful reform (e.g. amendment of outdated laws).

Community-based healing and prevention: Local programs - such as anti-violence initiatives, substance abuse recovery groups, and cultural healing practices - can address the root causes of trauma and distress, reducing demand for coercive psychiatry. Civil society can establish trauma-competent community centers offering free counseling and social support. Partnerships with schools and youth organizations are key to early intervention: educators and parents should learn to recognize distress and engage supportive networks before crises escalate to psychiatric detention.

Reform at the Professional Level

Personalized, trauma-informed care: Each victim's experience is unique. Clinicians and counselors should conduct thorough assessments that include personal history of abuse. Therapeutic approaches must be empowering - for example, providing choices in treatment plans, encouraging patient goal-setting, and using non-triggering techniques. Victims should have access to professional trauma counseling, cognitive-behavioral therapy, or EMDR if appropriate, always on a voluntary basis. Service providers must actively avoid any practice that could re-traumatize (e.g. invasive procedures without consent, or humiliation).

Peer and self-help resources: Individuals benefit greatly from connecting with others who have had similar experiences. Support groups (in person or online) allow survivors to share coping strategies and find solidarity. Self-help resources - such as survivor-authored books, podcasts, and recovery workbooks - should be made widely available. Mentorship programs pairing veterans of the system with newer survivors can foster hope.

Holistic rehabilitation: True recovery involves rebuilding life skills and social ties. Victims should receive help with education, vocational training, and housing stability. Programs like supported employment or disability-inclusive microfinance empower independence. Creative therapies (art, drama, music) and wellness activities (yoga, meditation, nature retreats) can aid healing. By validating each person's dignity and capacity, providers reinforce that *sanity* encompasses far more than symptom checklists.

Legal and rights support: On an individual level, victims need clear information about their rights and remedies. All victims should be offered legal counsel to challenge unjust treatment or obtain reparations. Social workers or patient advocates must guide them through any appeals or complaints processes. Finally, respecting freedom means acknowledging survivors as experts in their own care - institutions should routinely solicit and implement survivor feedback on services.

Throughout all levels, enforcing human rights strictly must be non-negotiable. Any violator - whether a state actor, institution, or individual professional - must be held accountable under the law. By combining top-down legal protections with bottom-up community empowerment and person-centered services, the system can transform from one of coercion to one of healing, dignity and true recovery.

Discussion

The proceedings of this symposium presented unequivocal and urgent evidence: the political abuse of psychiatry remains a critical, global crisis. Independent expert contributions revealed that psychiatry today, far from being fully shielded by its medical nature, remains highly vulnerable to manipulation, particularly in environments **where incompetent authority replaces the sane state of mind and sound authority**. When lawful governance rooted in rationality, ethics, and human rights is supplanted by arbitrary rule, rule of men and state backed criminality, psychiatry is weaponized, becoming an instrument of oppression, repression, and silencing. Where robust institutional safeguards are absent, the vulnerability of mental health systems to abuse is not incidental -it is structural and predictable.

This permeable environment creates conditions in which abuses occur with alarming ease. Rogue states, corrupt actors, and criminalized systems exploit psychiatric institutions to eliminate dissent, punish the vulnerable, and silence the inconvenient, all under the deceptive pretense of medical care. As highlighted by David Matas, Yutong Zhang, Yanxi Mou, Manuel Llorens, and others, psychiatry, under these corrupt conditions, serves state terror, private vendettas, and criminal profiteering. This perversion is not an unintended anomaly; it is the direct result of allowing **incompetent authority to replace the sane state of mind and sound authority** that must underpin any legitimate exercise of power, especially where human vulnerability is concerned.

The urgency of reform cannot be overstated. As information shared during the symposium confirms, the **human and economic costs** of psychiatric abuses are vast. Victims endure profound psychological harm, social alienation, and chronic physical health deterioration, while societies bear immense costs in lost productivity, fractured communities, wrongful institutionalization expenses, and legal redress efforts. These costs are not inevitable. They stem from the deliberate failure to protect psychiatry's foundational mission: to heal, not to harm.

Most crucially, the symposium made clear that the **weaponization of psychiatry is sustaining systemic failure almost by design**. These are not accidents of practice but structural defects, exacerbated when mental health care systems are aligned more with social control than with healing. By diverting trauma, dissent, and vulnerability into silencing mechanisms rather than resolution pathways, failing or rogue systems maintain the illusion of order while exacerbating human suffering. Healthcare, rather than being a bulwark of collective well-being, becomes a tool of coercive governance, thereby entrenching dysfunction beneath a façade of medical legitimacy. Psychiatry thus risks becoming the most devastating betrayal: the betrayal of life, dignity, sanity, and hope under the false banner of care.

Speakers independently confirmed that abuses today are systemic, severe, and require urgent redress. The findings of the symposium call for immediate, coordinated action:

The findings and testimonies presented in this symposium confirm that addressing the political abuse of psychiatry demands not only institutional reforms, but a profound transformation in the way evidence is gathered, presented, and acted upon. This necessitates engaging multiple disciplines - law, human rights, medicine, psychology, sociology, political science, and ethics - to create a robust, interdisciplinary resistance to entrenched abuses. It also demands action-research

approaches rooted in rigor, survivor participation, and protection against the systemic reprisals that continue to deter transparency and reform.

At the **international level**, the recognition of political psychiatric abuse as an international crime must be accompanied by independent, interdisciplinary investigative bodies. Human rights experts, forensic psychiatrists, legal scholars, and survivor advocates must collaborate to rigorously document abuses across jurisdictions, ensuring that evidence is systematically collected, preserved, and publicized. The traditional barriers of fear, diplomatic inertia, and political compromise must be consciously dismantled. Only through fearless, independent reporting can abuses embedded within rogue states and failed systems be brought to the light of international accountability mechanisms. Action-research methodologies, grounded in survivor testimony and corroborated by forensic and legal analyses, are essential to avoid the sanitization or distortion of realities on the ground.

At the **national level**, legislation and policy must not only prohibit psychiatric coercion but foster environments where research into systemic abuses is protected and encouraged. National research councils, ombuds institutions, and independent commissions must actively support interdisciplinary investigations into psychiatric practices, without political or corporate interference. Protection of researchers, whistleblowers, and survivors must be codified in law, recognizing that fear, dismissal, and denial - often fueled by vested interests in the healthcare, pharmaceutical, security, and political sectors - have historically silenced critical inquiry. Nations must ensure that confronting psychiatric abuses is not treated as destabilizing but as strengthening the social and legal order.

Addressing these realities demands an uncompromising commitment to human rights enforcement at every level. Academic institutions must incorporate critical psychiatric studies, survivor-led research, and human rights law into mental health training programs. The system would also benefit from the approaches and interventions laid down in next paragraphs, as presented in the paper:

At the **institutional level**, psychiatric facilities and health systems must open themselves to external, interdisciplinary auditing. Routine human rights impact assessments, conducted by independent teams combining medical, legal, social science, and survivor expertise, must become mandatory. Institutions must be held accountable for retaliation against whistleblowers and survivors who expose abuses. Internal ethics committees must be reconstituted to include external human rights monitors, ensuring that reprisals, denial, and corruption are neither normalized nor concealed. The dominant cultures of self-protection and reputational defense must be replaced by a culture of truth-telling, ethical transparency, and patient-centered reform. At the **community and civil society level**, grassroots organizations, survivor networks, and interdisciplinary academic groups must collaborate to produce and disseminate evidence-based narratives that counter the dominant myths supporting psychiatric coercion. Community-based action-research initiatives must document abuses, capture survivor histories, and monitor local services, creating alternative archives of truth accessible to courts, media, and the public. Civil society must also advocate for legal protections ensuring that survivors, researchers, and advocates can speak out without fear of defamation suits, professional retaliation, or unlawful surveillance. Breaking the cycle of social dismissal and denial demands making psychiatric abuses visible not as isolated anomalies, but as systemic failures requiring systemic remedies. At the **individual level**, every survivor and citizen

must have access to mechanisms that support the ethical gathering and sharing of experiences without re-traumatization or retribution. Empowering individuals to participate in action-research projects, legal advocacy, and policy reform initiatives strengthens both the evidentiary base and the democratic legitimacy of psychiatric reform. Trauma-informed approaches must guide the documentation process, respecting the autonomy, dignity, and safety of those whose testimonies form the foundation of change. Encouraging survivor-led research centers and participatory legal projects ensures that those most affected are not merely subjects of study but protagonists in redefining the future of mental health care.

Ultimately, the transformation required is both technical and cultural. Interdisciplinary engagement must not be perfunctory; it must seek to dissolve disciplinary silos that have enabled psychiatric abuses to remain insulated from legal scrutiny, human rights accountability, and social science critique. Action-research must be fearless, ethically rigorous, and politically conscious, recognizing that psychiatry's entanglement with systems of repression cannot be dismantled without exposing the networks of power and interest that sustain it. True change requires recognizing that psychiatric abuse is not merely a professional failure but a profound social and moral crime - one that undermines public trust, damages countless lives, and perpetuates systemic injustice under a false banner of healing.

Breaking this cycle demands more than reforms: it requires courage, interdisciplinary solidarity, and the unwavering insistence that human dignity, freedom, and sanity are not negotiable. Only by fortifying research, amplifying survivor leadership, and protecting truth-telling against the reprisals of vested interests can societies reclaim psychiatry as a genuine instrument of healing rather than a tool of fear and domination. Across all levels, the principles must be uncompromising: do no harm, eradicate coercion, deny criminal misuse, enforce all human rights strictly, and uphold the primacy of human dignity, freedom, and life. Psychiatry must not drift into being an accomplice to repression. It must actively become a guardian of humanity's highest ethical commitments.

The symposium reaffirmed that change is not only necessary but possible. Cases like Mikhail Kosenko's release (van Voren, 2016) illustrate that international attention, legal pressure, and ethical solidarity can disrupt even the most entrenched abuses. But vigilance must be relentless. Systems left to rot in silence and impunity will inevitably continue to betray the vulnerable.

Table 6: Reclaiming Healing in Psychiatric Care

Reform Principle	Key Measures
Codify Healing and Autonomy as the Sole Legitimate Purposes of Medical Systems	Rebuild legal and ethical foundations around healing, autonomy, dignity. Deviation becomes unlawful.
Criminalize Non-Therapeutic Medical Interventions	Punish non-therapeutic acts like forced medication or diagnosis without urgent clinical need.
Total Prohibition of Coercive and Punitive Practices within Healthcare	Eliminate coercion unless tightly defined emergency; align with CRPD obligations.
Separate Medical Support from Social Order Enforcement	Remove medicine from roles of social discipline, control, or ideological enforcement.
Invest Primarily in Prevention, Education, and Community Resources	Prioritize housing, nutrition, voluntary care, and health literacy over institutional reaction.

Decentralize and Democratize Oversight	Establish oversight with survivors, legal and community representatives with real sanctioning power.
Reeducate Medical Professionals Around Human Rights	Integrate human rights law and abuse history into medical education. Train to resist complicity.
Protect Whistleblowers and Dissenters Within Healthcare	Legally protect ethical dissenters and whistleblowers in healthcare systems.
Guarantee Full Access to Justice and Reparations for Survivors	Ensure complaint mechanisms, redress, and public recognition of abuses and survivors' rights.
Maintain Permanent Global Surveillance Against Abuses	Create global observatory to monitor violations and ensure binding public reporting.

Protecting mental health systems from political and criminal abuse is not ancillary to justice - it is central. If psychiatry becomes again a tool of oppression, society itself descends into normalized cruelty masked by pseudoscience. Healing psychiatry requires nothing less than restoring it to its ethical, humanitarian, and rational foundations: to protect life, to foster recovery, to uphold freedom, and to enshrine dignity. Without urgent and principled action, we risk perpetuating a system where incompetent authority, masked as medical judgment, erodes the very sanity and freedom it was meant to preserve. To prevent this, psychiatry must be reclaimed - not merely reformed - as an instrument of hope, truth, and collective healing.

Medicine must return to its only rightful foundation: healing and protecting life. Safeguarding the integrity of mental health care is more than a professional duty. It is a moral imperative grounded in the universal principles of human rights. It is a testament to our shared humanity. Without this vigilance and collective commitment, healing cannot truly begin - and dignity cannot truly be restored. With it, however, psychiatry can fulfill its highest promise: not as a tool of domination, but as a sanctuary for healing, understanding, and hope. Any other purpose, be it control, punishment, profiteering, silencing, is a perversion of its meaning and a betrayal of humanity itself. This is not a call for mere reform, but for a complete reassertion of medicine's original, moral purpose. Psychiatric and medical abuses cannot be addressed by improving policies alone; they must be rooted out by re-centering health systems on truth, autonomy, and dignity. We must honor the victims, whose lives were silenced, disfigured, and destroyed by systems that claimed to heal. Learn, from past and present mistakes to effectively deny any opportunity for these crimes against humanity repeating once more. Their pain must not be abstracted. It must be remembered as the living cost of institutional betrayal. We must also protect those who speak on their behalf: researchers, journalists, legal advocates, clinicians, and families who face intimidation, ostracism, and retaliation for uncovering the truth. The same goes for informants within closed institutions, and the communities caught between fear, coercion, and complicity, too often driven to enforce atrocities among themselves, out of anguish, fear, or opportunistic cruelty. They too deserve protection, and they too require systems that do not abandon or exploit their position.

This is the core of what must change: not just policies, but structures -oversight with real power, education rooted in human rights, legal accountability with teeth. Not symbolic ethics, but mechanisms of prevention, exposure, and repair that cannot be silenced. It requires unwavering

courage, clarity of intent, and structural transformation across education, practice, law, and governance. A healing profession that fails to protect the living right to flourish is no longer a profession. These ten pillars lay the groundwork.

The will to act, now, to redress this problem and uphold sanity, is the measure of our civilization.

Conclusion

The symposium laid bare a stark reality: the political abuse of psychiatry persists today as an entrenched and systemic phenomenon, deeply intertwined with rogue governance, failed healthcare systems, and criminal misuse of medical authority. Where incompetent authority replaces the sane state of mind and sound ethical governance, psychiatry ceases to serve its healing purpose and becomes an instrument of repression, silencing, and destruction. The profound human and economic costs - borne by survivors, communities, and societies at large - reveal the devastating consequences of allowing mental health systems to be weaponized. These abuses are not incidental; they are the predictable outcome of systemic failures that prioritize control over care, coercion over communication, and impunity over justice. Urgent, coordinated, and uncompromising action is required at every level: international, national, institutional, community, and individual. Psychiatry must be reclaimed from corruption and restored to its rightful role as a guardian of healing, dignity, and freedom. Protecting mental health systems from political and criminal abuse is not a technical reform - it is a profound moral imperative, central to the defense of human rights and the preservation of sanity itself. Only by dismantling the structures of coercion, enforcing strict human rights protections, empowering survivors, and rebuilding care on the foundations of respect, voluntariness, and solidarity, can psychiatry fulfill its highest ethical promise: to heal, to protect, and to uphold the inviolable dignity of every human being.

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Annex H - Dissertation policy advice summary

The policy executive summary presented in Annex H, do to its multimedia nature, is available through the dissertation's online repository, where it can be accessed as part of the public dissemination and translational impact strategy. This repository hosts the structured reform proposals, legal and clinical recommendations, and implementation pathways derived from the doctoral research, ensuring transparency, accessibility, and practical utility for policymakers, professionals, and civil society actors. Annex H functions as a translational synthesis of the dissertation's scientific evidence into structured, actionable guidance for institutional reform. It is designed not as a secondary addition, but as a core component of the research's impact agenda, bridging the gap between empirical documentation and policy implementation. Within the logic of the dissertation, which integrates biocultural analysis, action-research, and institutional diagnostics, this annex embodies the normative and operational corollaries of the work. It responds to the urgency, repeatedly demonstrated throughout the main chapters, for legally binding and structurally embedded mechanisms to prevent psychiatric violence and guarantee autonomy in care.

The annex distills the dissertation's extensive findings, derived from participant observation, national and international surveys, legal analysis, and transdisciplinary synthesis, into a coherent reform roadmap. It articulates the priority areas for legislative, clinical, and educational intervention, grounded in the central hypothesis that coercion is neither inevitable nor therapeutically justified, but rather a systemic failure perpetuated by outdated legal norms, epistemic asymmetries, and professional inertia. The policy summary proposes concrete shifts toward shared decision-making, autonomous medication management, open data infrastructures, and the deployment of dialogical care models within the public health framework, including Open Dialogue, trauma-informed pedagogy, and participatory oversight systems.

The structure and content of Annex H correspond directly to the operative research questions and translational objectives set forth in the introduction and methodological chapters. It draws from the empirical findings of Chapters 3 and 4, which identify patterns of diagnostic imposition, overmedication, and institutional betrayal, and from the analytical and normative synthesis of Chapters 5 and 6, which diagnose the regulatory and ethical dysfunctions of current psychiatric governance. The recommendations presented in Annex H are aligned with international legal standards, including those articulated in the UN Convention on the Rights of Persons with Disabilities, the WHO QualityRights framework, and European human rights jurisprudence. It also draws on transdisciplinary evidence from neuroscience, medical anthropology, legal theory, and systems science to ensure feasibility and coherence. Annex H also lays the groundwork for the implementation phase envisioned in Chapter 7, particularly through the EU BEACON One Health Education initiative. It establishes the logical architecture through which reforms can be scaled across educational, legal, and clinical systems. It outlines regulatory benchmarks, participatory mechanisms, and accountability protocols that translate the thesis's findings into enforceable structures. Rather than offering idealistic aspirations, the annex provides a modular and evidence-backed reform agenda oriented toward real-world deployment, adaptive governance, and continuous feedback monitoring.

Annex I - Media dissemination kit

Annex I, containing the media dissemination kit, is made available exclusively through the dissertation's online repository due to its multimedia nature. This annex includes visual materials, communication templates, explanatory videos, and open-access outreach tools designed to support broad public engagement with the dissertation's findings. It forms part of the translational strategy to facilitate awareness, policy uptake, and cross-sectoral dialogue by rendering complex scientific content accessible and actionable across diverse audiences.

Annex J Horizon proposals

The four Horizon Europe proposals now under development under the EU BEACON One Health Education COST action, as part of the action-research of this dissertation, directly extend and operationalize the theoretical, empirical, and normative foundations of the dissertation, transforming its claims into applied, scalable interventions across educational, clinical, and policy domains. Each project embodies a structural response to the core proposition that psychiatric systems must move beyond coercive and overmedicalized practices, not merely through protocol revision or professional training, but through a comprehensive reconfiguration of the socio-institutional architectures that sustain exclusion, diagnostic reductionism, and epistemic asymmetry. The first proposal, focused on fostering brain health in school settings, aligns with the thesis's assertion that mental health must be understood bioculturally and developed preventively through dynamic interactions between metabolic regulation, neurodevelopment, and social participation. By implementing structured routines in exercise, sleep hygiene, nutritional supplementation, especially Omega-3 regulation, and social bonding, this project targets the physiological substrates of cognitive and emotional regulation within equitable educational contexts. It reflects the dissertation's emphasis on early, non-stigmatizing, non-pharmacological interventions and on biosignal-informed feedback systems that enable continuous assessment of therapeutic efficacy. It thereby exemplifies the thesis's call for scientifically grounded, real-time recalibration of care that respects bodily and legal integrity from the earliest stages of psychosocial development.

The second proposal, addressing the cognitive impact of pollutants, provides empirical substantiation for the thesis's critique of psychiatry's neglect of environmental determinants of distress. By mapping and mitigating the neurotoxic effects of contaminated air, soil, water, and food chains in school-age populations, the project embeds mental health within ecological public health and environmental justice frameworks, bridging disciplines that the dissertation holds to be unjustly separated. The project's integration of biomonitoring, neurodevelopmental testing, and community-level intervention design mirrors the thesis's insistence that psychiatric symptoms often emerge from chronic systemic exposures which are pathologized rather than addressed. By generating real-time dashboards linking exposure to cognitive function, and by embedding environmental data in institutional decision-making, the project enacts the thesis's vision of a participatory, feedback-informed governance model, rooted in the ethical imperative to prevent harm through structural accountability. It concretizes the dissertation's legal and institutional recommendations, demonstrating how regulatory inertia can be challenged through multisectoral coordination and scientifically verifiable risk mitigation strategies.

The third proposal, centered on shared decision-making in child and adolescent mental health, directly implements the dissertation's core theoretical proposition: that coercion is structurally produced through the systematic exclusion of individuals from decisions affecting their own care, and that recovery requires dialogically anchored, procedurally accountable systems that respect autonomy, narrative identity, and relational integrity. This project institutionalizes SDM as a foundational practice across school-based mental health systems, translating the dissertation's conceptual framework into practical methodologies for co-produced care. By embedding SDM

tools, trauma-informed protocols, and culturally sensitive engagement mechanisms within naturalistic settings, rather than isolating them in clinical silos, the project validates the thesis's argument that ethical and effective mental health care must be situated within the everyday contexts where distress emerges and where trust can be rebuilt. Moreover, it reinforces the dissertation's operational commitment to participatory regulatory design by creating traceable decision processes, consent frameworks, and transparent digital tools that empower youth and families while ensuring institutional compliance with rights-based standards. The project's focus on legal coherence, feedback-driven improvement, and open science infrastructures serves as a direct implementation of the dissertation's proposed systemic reforms.

The fourth proposal, developing internal mentorship and service-learning networks within schools, activates the thesis's vision of recovery as a socially distributed, relational process that must be structurally enabled by institutions committed to dignity, restoration, and reciprocal accountability. This project frames educational institutions not only as sites of learning but as microcosms of civic engagement and relational healing, where trauma can be addressed not through external surveillance or behaviorist discipline, but through embedded, peer-anchored systems of trust and care. It operationalizes the dissertation's critique of clinical cultures that suppress voice and exclude survivors from the architecture of care, by creating structured pathways for safe disclosure, narrative reconstruction, and collective response to harm. Through reflexive mentoring systems, continuous professional development, and survivor-informed policy co-design, the project models the dissertation's call for institutional transformation through dialogical scaffolding and transparent governance. It enacts the procedural ethics advanced in the thesis, where institutional maturity is defined not by control over pathology but by the capacity to host feedback, correct injustice, and evolve through relationships.

Together, these four proposals form a coherent translational matrix that directly advances the dissertation's multi-level aims: the prevention of psychiatric harm through structural and environmental interventions; the reconstruction of clinical and educational systems around shared decision-making and narrative agency; and the creation of participatory infrastructures for ethical governance, continuous improvement, and open verification. Each project embodies the thesis's epistemological stance that mental health is not reducible to individual pathology but is a relational, regulatory, and socially mediated phenomenon that requires institutional designs capable of procedural reflexivity and systemic repair. Through the operationalization of the dissertation's theoretical insights and empirical findings, these proposals not only affirm the scientific validity of its claims but also activate its transformative intent, moving psychiatry, education, and public health toward a new paradigm in which recovery is the structural default, not the exception.

1. Fostering Brain Health in School Settings

Summary of Benefits

This proposal seeks to establish a transdisciplinary, school-anchored health promotion model that integrates physical exercise, rest routines, nutrition (including Omega-3 intake and metabolic balance), and social engagement into measurable programs aimed at enhancing neurocognitive

performance, emotional regulation, and long-term educational attainment. The intervention emphasizes neuroplasticity in development, equity in school environments, and the prevention of burnout, anxiety, and learning disorders from a public health perspective.

Required Actors

- Schools and education ministries (policy integration, implementation)
- Pediatric neurologists, nutritionists, sleep researchers (clinical design, baseline assessments)
- Neuroscientists and developmental psychologists (design of neurocognitive metrics and resilience indicators)
- Public health institutions and One Health experts (coordination of multisectoral impact)
- Local and national sports and youth organizations (delivery and social anchoring)
- Food science researchers and metabolic health labs (dietary interventions, Omega-3 monitoring)
- Data scientists and digital tool developers (tracking tools, gamified compliance, real-time feedback)
- Youth representatives and families (participatory co-design)

Longitudinal Experimental Design and Novelty

- Population: 5,000–10,000 children aged 6–16 across rural and urban European school systems
- Design: Multisite, longitudinal cluster-randomized controlled trial (CRCT) with at least 3 arms:
 - Full intervention (exercise + sleep + nutrition + social engagement)
 - Partial intervention (exercise + sleep or nutrition only)
 - Control group (standard school practices)
- Duration: 4+ years with annual follow-ups and continuous monitoring
- Novelty:
 - Integrates metabolic profiling (especially Omega-3 index) and cognitive baselining in a school setting
 - Applies sleep science to optimize school start times and activity schedules
 - Uses digital tools and citizen science to ensure participatory feedback and adaptivity
 - Bridges health and learning outcomes into one comprehensive intervention model

Expected Impact

- Establishes a model for systemic health education policy, promoting health as foundational to cognitive success
- Mitigates health inequities and supports inclusion by targeting underserved schools and vulnerable populations
- Reduces future burden of mental illness, attention disorders, and lifestyle-related diseases
- Aligns with SDGs 3, 4, and 10, strengthening One Health literacy and prevention across generations
- Provides scalable, evidence-based protocols for EU-wide education and health ministries

2. Pollutants and Brain Health: Cleaner Air, Soil, Water and Food Chains

Summary of Benefits

This proposal addresses the underrecognized but critical link between environmental toxicity and neurodevelopmental decline in children. By focusing on pollutants affecting air, water, soil, and food chains, the project seeks to safeguard cognitive performance, behavior, and mental health outcomes through an integrated environmental justice approach. It connects ecological exposures with measurable brain health outcomes and builds regulatory and community strategies for mitigation.

Required Actors

- Environmental toxicologists and neuroepidemiologists (biomonitoring design and neurotoxic risk profiling)
- Schools, municipalities, and public health departments (exposure mitigation, public engagement)
- Environmental justice advocates and NGOs (community anchoring, participatory processes)
- Food safety and water quality agencies (surveillance and intervention on contaminants)
- Veterinary and One Health professionals (cross-species biomonitoring and sentinel analysis)
- Legal scholars and regulatory experts (policy translation and threshold enforcement)
- ICT experts and citizen science platforms (data collection, local reporting, and transparency)

Longitudinal Experimental Design and Novelty

- Population: 6,000–12,000 children in 5–10 European regions with variable pollution profiles
- Design:
 - Quasi-experimental prospective cohort with nested intervention groups

- Cross-sectional biomonitoring of neurotoxins (e.g., heavy metals, pesticides, PFAS)
- Annual neurocognitive testing, inflammation markers, and psychosocial metrics
- Duration: 5 years minimum, with environmental sampling and child-level monitoring every 12 months
- Novelty:
 - Merges environmental biosurveillance with school-based neurocognitive assessments
 - Introduces real-time pollution-to-cognition dashboards for policy use
 - Tests protective community-level interventions (e.g., air filters, schoolyard rewilding, dietary shielding)
 - First EU-scale application of One Health neurotoxic risk mitigation in school populations

Expected Impact

- Empowers schools and communities to act as sentinels of environmental and brain health
- Enables science-based regulation of environmental pollutants with direct cognitive health endpoints
- Mitigates the neurodevelopmental impact of pollution in vulnerable child populations
- Strengthens intersectoral policy design between health, education, and environment
- Advances SDGs 3, 4, 13, and 10, through early prevention and pollution accountability
- Provides a replicable framework for EU regulatory reform and community-based environmental action

3. Shared Decision-Making Mental Health for Children

Summary of Benefits

This project establishes a European model for shared decision-making (SDM) in child and adolescent mental health, integrated within educational and social systems. It aims to reverse coercive, fragmented mental health interventions by embedding trust-based, culturally responsive, and evidence-driven care models in schools and local services. The benefits include better outcomes in emotional wellbeing, reduced psychiatric labeling, enhanced autonomy, and stronger support for teachers and families navigating distress in youth.

Required Actors

- Child psychiatrists, psychologists, and SDM specialists (design and implementation of decision-making frameworks)

- Educational institutions and ministries (integration into school health protocols)
- Youth and parent advocacy groups (lived experience input, evaluation of relevance and trust)
- Anthropologists and medical sociologists (analysis of context, bias, and systems)
- Social workers and family support services (engagement in care plans)
- Digital platform developers (ethical, secure, user-friendly SDM tools)
- Health economists and evaluators (cost-effectiveness, outcome tracking)

Longitudinal Experimental Design and Novelty

- Population: 3,000–6,000 children and adolescents experiencing early signs of mental distress, along with families and school personnel
- Design:
 - Longitudinal intervention-control study across 6–8 countries
 - Co-creation and iterative refinement of SDM tools and relational frameworks
 - Measurement of psychiatric outcomes, school attendance, trust in services, emotional self-regulation
- Duration: 4 years with follow-up at 6, 12, 24, and 36 months
- Novelty:
 - First large-scale EU project to operationalize SDM for minors in mental health through schools
 - Replaces risk-based, exclusionary models with narrative-based, trauma-informed relational support
 - Embedded in naturalistic school and community settings, not clinical-only settings
 - Explicitly rights-based and aligned with UN CRC and WHO guidance on non-coercive care

Expected Impact

- A validated framework for SDM in youth mental health, adaptable across contexts
- Reduction in overdiagnosis, overmedication, and harmful institutionalization
- Empowerment of schools, families, and children as primary agents in early mental health support
- Improves trust in services and reduces dropout from care or education
- Supports SDG 3, 4, and 5, linking health, education, and gender equality

- Contributes to EU guidelines for non-coercive child mental health intervention design and evaluation

4. Service-Learning and Internal Mentorship Programs

Summary of Benefits

This project aims to institutionalize service-learning and internal mentorship systems in schools to counteract growing social isolation, despair, and mistrust in educational environments. By fostering peer mentoring, relational learning, and survivor-safe disclosure channels, schools become environments of care, restoration, and participatory problem-solving. It redefines the school not just as a knowledge site but as a resilient social institution with the power to prevent harm, promote health, and mediate conflicts.

Required Actors

- School leadership teams and teacher unions (implementation and sustainability)
- Peer mentoring experts and youth counselors (training frameworks and supervision systems)
- Trauma-informed educators and restorative justice professionals (handling of disclosures and harm repair)
- Civic educators and psychologists (design of meaningful contribution pathways)
- Policy-makers in education and justice (translation into national guidelines)
- Children's rights and gender equality experts (intersectional structuring of mentorship roles)

Longitudinal Experimental Design and Novelty

- Population: 5,000–8,000 students across 30–40 schools in varied socioeconomic and cultural settings
- Design:
 - Implementation study using a stepped-wedge randomized rollout model
 - Structured mentorship systems with roles assigned and trained within each institution
 - Evaluation through mixed methods: school climate surveys, dropout and absenteeism data, mentorship feedback loops
- Duration: 3–4 years, with monthly reflections and annual thematic reviews
- Novelty:
 - Anchors internal responsibility structures within the school community rather than relying on external programs

- Integrates relational pedagogy with institutional repair functions (e.g., spaces to report abuse, heal division)
- Frames mentorship as both civic duty and psychological protection, supporting EU restorative principles
- Generates real-time dialogues on harm, prevention, and belonging from within the school ecosystem

Expected Impact

- Creates trusted internal mechanisms for navigating hardship and healing conflict in schools
- Reduces bullying, exclusion, and systemic silence on harm
- Builds interpersonal resilience and institutional maturity through participation
- Transforms schools into safe, responsive micro-communities capable of emotional and civic learning
- Contributes to SDG 4, 5, and 16, by promoting peace, inclusion, equality, and participatory governance
- Offers a blueprint for education-led violence prevention and youth engagement policy across Europe

Annex J Ethics approval

The following section provides documentation of the ethics approval granted for the research conducted in this dissertation.

Títol del projecte d'R+D+I:

Repensar lo social en salud mental: mapeo y análisis de las iniciativas productoras de autonomía en España

Investigador/a principal:

Ángel Martínez Hernández – Martín Correa-Urquiza Vidal Freyre

Comitè ètic de recerca:

Comitè Ètic d'Investigació en Persones, Societat i Medi Ambient (CEIPSA)

Entitat:

Universitat Rovira i Virgili

Sessió:

CEIPSA2/2025, de 28/02/2025

Número d'autorització:

CEIPSA-2024-PR-0029

Una vegada revisada i validada la informació aportada, l'expedient de referència CEIPSA-2022-TDO-0023 no ha de superar el procés d'avaluació addicional.

Annex L - Additional tables and figures

Table 1- Historical approaches to mental and emotional states, by culture and region

Period / Region	Approach to Mental States	Practices & Substances	Interpretive Setting
Ancient Greece	Dream incubation; melancholia as philosophical temperament	Olive oil, wine, mandrake, poppy; communal sleep spaces	Temples, oracles, symposiums
Classical China	Qi imbalance treated via herbs, acupuncture, moral cultivation	Ginseng, tea, fasting, ancestor veneration	Confucian and Taoist circles
Medieval Islamic World	Prophetic dreams; spiritual melancholy	Saffron, ambergris, music, Qur'anic recitations	Hospitals, mosques, theological academies
Andean Cultures	Ritual singing, coca leaves, trance for balance and vision	Maize rituals, tobacco, fasting, dance	Community elders, spiritual healers
Renaissance Italy	Humoral theory, artistic catharsis, pilgrimage	Opium, absinthe, wine, confession	Courts, monasteries, salons
19th-century Europe	Institutional care; medicalized moral treatment	Morphine, bleeding, electrotherapy	Asylums, elite clinics, literary circles

Treatments and interpretive settings for mental, emotional, and spiritual crises in various civilizations.

Table 2- Physiological control of cognition and emotion across eras, by culture and region

Era	Control Mechanism	Health Impact	Narrative Justification
Ancient Civilizations	Fasting rituals, wine feasts, caste-based food distribution	Nutrient-based stratification, ritual intoxication	Divine hierarchy, ritual purification
Medieval Europe	Church feasts for control, alcohol as religious sacrament, bread as loyalty tool	Famine cycles, malnutrition, dietary restriction	Moralistic suffering, monastic divine punishment
Modern Capitalist States	Processed food addiction, subsidies for sugar/alcohol, pharmaceutical dependency	Obesity, inflammation, pathologization	diabetes, psychiatric Consumer choice, biological determinism
Russia, late USSR	Vodka rationing, institutionalized alcoholism, chemical sedation for dissent	Liver disease, mass suicides, apathy	Revolutionary sacrifice, national hardship
Contemporary Global South	Junk food proliferation, privatized water, mental illness framing of poverty	Stunting, gut-brain dysfunction, mass depression and suicide	Clinical diagnosis, self-blame

Historical uses of food, drink, and altered states as instruments of social control and their psychiatric consequences, across epochs and geographies.

Table 3, Ancient patterns of marginalization and control

Targeted Group	Mechanism of Harm	Structural Role	Exploitative Driver
Enslaved laborers	Overwork, beatings, no legal rights	Resource extraction, class enforcement	Profit from forced labor
Urban poor in temple economies	Food debt, labor conscription	Surplus dependency structure	Control and surplus extraction
War prisoners, enslaved	Forced public works, mutilation	Imperial consolidation	Domination via labor, fear
Household servants, slaves	Sexual/physical coercion	Lineage and domestic control	Power and normalized abuse

This table outlines the early institutional, economic, and ideological mechanisms used to regulate, exclude, or exploit marginalized populations in ancient societies, including Mesopotamian, Egyptian, Greco-Roman, and early imperial models. It highlights how sociopolitical systems organized labor, justified social hierarchies, and medicalized or spiritualized dissent, laying the groundwork for later biomedical and psychiatric rationales of exclusion and control.

Table 4, Medieval and early modern systems of targeted harm

Targeted Group	Mechanism of Harm	Structural Role	Exploitative Driver
Feudal serfs	Bonded to land, no mobility	Agrarian wealth, elite maintenance	Rent extraction, control
Women	Torture, execution	Moral/patriarchal purification	Sadistic spectacle, forced sexual pleasure, male power
Religious minorities	Ghettoization, massacres	Territorial consolidation	Confiscation, ethnic purge
Pauper orphans	Forced labor, abuse	Cost-saving institutions	Institutional labor, neglect

This table presents the structural targeting of vulnerable populations in feudal and premodern Europe, including women, serfs, orphans, and religious minorities. These mechanisms -ranging from torture and forced labor to public executions -served not only as instruments of order and moral enforcement but also as technologies of terror, pleasure, and accumulation. The continuity of these practices into medical-legal frameworks underscores how control and exploitation were historically normalized through theological, patriarchal, and juridical rationales.

Table 5- Historical Governance

Era / Region	Gender/Class Subjugation	Knowledge Preservation	&	Skill Pleasure/Profit from Violence
Late Empire	Roman Women pushed to domesticity, elite concubinage	Classical knowledge preserved in minor schools		Public executions, military spectacles
Byzantine Empire	Elite women cloistered, lay women silenced	Preserved Hippocratic and Galenic medicine		Heresy purges and imperial torture rituals
Islamic Age	Golden Gender roles ambivalent; women could own property	Translation movement, Razi, Ibn Sina		Al- Judicial amputations, corporal punishments
Western Medieval Europe	Serfdom, female witch-hunts, orphans enslaved	Monasteries kept rudimentary archives		Witch-burnings, pillories, sacrificial wars
Early Europe	Modern Factory discipline, commodification	sexual Renaissance clandestine printing		humanism, Colonization as health sacrifice zone

This table disaggregates the role of gender and class in systems of exploitation, emphasizing how social identity intersected with epistemic and somatic control. Women, the poor, and the colonized were subject to layered forms of violence, including the confiscation of reproductive autonomy, criminalization of alternative knowledge systems, and commodification of pain for profit or pleasure. These dynamics laid the groundwork for modern institutional cruelty, in which trauma is still misrecognized as disorder and agency as deviance.

Table 6, Historical Governance Table

Era / Region	Health & Public Systems	Governance & Control Strategies	Population Obedience & Training
Late Empire	Roman Urban health infrastructure failing; aqueducts collapsing	Tax pressure, military conscription, Christianity as unifying coercion	Discipline by legions, declining civilian readiness
Byzantine Empire	Hospital and orphanage system maintained in Constantinople	Bureaucratic complexity, theological policing	Monastic discipline, literacy enforced in clergy
Islamic Age	Golden Advancements in hospital, hygiene, pharmacology	Meritocratic science hubs under caliphates	Intellectual training for elites, physical jihad
Western Medieval Europe	Monastic medicine, leechcraft, rural famine	Feudal oppression, authoritarianism	Church Martial training for nobility, no peasant rights
Early Europe	Modern Poor laws, plague patchy	Plague controls Absolutism, birth of central states	Standing armies, mass conscription

This table synthesizes key historical structures across antiquity and the medieval period, highlighting governance logics that linked obedience to biopolitical submission. Populations were managed through enforced ignorance, physical exhaustion, or ritualized subjugation, with the state or empire extracting productivity and loyalty by shaping bodily and cognitive norms. Medical traditions -when not entirely

displaced -were co-opted into controlling apparatuses that punished deviation or suffering as moral failure. Structural violence was normalized and encoded into law, pedagogy, and sacred practice.

Table 7- Modern governance and social control

Era / Region	Health & Public Systems	Governance & Control Strategies	Population Training	Obedience &
Early Century (Global)	20th State hospitals, eugenics programs	Nationalist social hygiene laws	science, Work-based military drafts	rehabilitation,
Mid Century	20th Mass institutionalization, psychiatric genocide	Ideological racial biology	purges, Compulsory secret policing	loyalty rituals,
Late Century	20th Deinstitutionalization, privatized healthcare	Market biopolitical management	logics, Self-optimization, culture	therapy
21st Century	Digital health surveillance regimes,	data Platform algorithmic control	capitalism, Censorship, profiling	predictive

This table contrasts the dominant political logics of population control across the 20th and 21st centuries, spanning totalitarianism, neoliberal governance, and algorithmic rationality. While modes differ -from centralized psychiatric repression to decentralized economic dispossession -all systems share a core commitment to suppressing dissent and reinforcing hierarchical order through coercive or pharmacological means. Rationality is often redefined to justify violence as necessity, psychiatry as discipline, and poverty as pathology.

Table 8- Modern governance and institutionalized violence

Era / Region	Gender/Class Subjugation	Knowledge & Skill Preservation	Pleasure/Profit from Violence
Early Century (Global)	20th Pro-natalist industrial patriarchy	propaganda, Medical colonial science	academies, Anatomical exploitation, forced labor profits
Mid 20th Century	Re-education camps, gender purging	Propaganda psychiatric policing	science, Spectacular trials, state terror aesthetics
Late 20th Century	Sexual commodification, gig precarity	Elite managerialism	universities, Prison-industrial complex, insurance markets
21st Century	Digital trafficking, biometric sorting	Platform monopolies	knowledge AI prediction economies, migrant exploitation

This table catalogs the institutional mechanisms -psychiatric, legal, technological, and bureaucratic -used to surveil, silence, or chemically subdue populations deemed unfit, inconvenient, or rebellious. While framed as therapeutic or security-enhancing, these systems function to obscure structural violence and redistribute blame from failing institutions to vulnerable individuals. The so-called care infrastructure reproduces inequality while denying the epistemic legitimacy of those most harmed.

Table 9- Institutional, psychiatric, and ideological control in the 20th–21st century

Category of Control	Mechanism Employed	Institutions Involved	Target Populations
Psychiatric Coercion	Involuntary hospitalization, forced medication, misdiagnosis of dissent as pathology	Asylums, psychiatric hospitals, forensic units	Political dissidents, abused women, neurodivergent individuals
Medical Pathologization Poverty	Reclassification of trauma or social deprivation as chronic mental illness	Social security disability assessments	Working-class populations, unemployed, single mothers
Carceral Expansion	Prison-like psychiatric wards, secure treatment centers, chemical restraints	Prisons, institutions, psychiatric units	juvenile closed Racialized youth, migrants, the homeless
Welfare Surveillance	Psychiatric labeling to restrict parental rights, enforce state guardianship, or justify state guardianship removals	Family courts, CPS, state guardianship bodies	Poor families, survivors of abuse, foster system entrants
Ideological education	Re- Therapeutic correction of gender identity, political beliefs, or trauma narratives	Conversion programs, military psychiatry, propaganda schools	LGBTQ+ individuals, political prisoners, rape victims
Digital & Algorithmic Control	Predictive policing, health flagging via social media, biometric sorting	mental Tech companies, predictive analytics firms, insurance brokers	Protesters, those with non-normative behaviors or expressions
Drug-Based Pacification	Long-term prescription of sedatives, antipsychotics, and mood stabilizers for non-severe distress	Primary care clinics, elderly care, schools	Children, elderly, women, those reporting abuse
Bureaucratic Silencing	Diagnosis used to dismiss testimony, deny legal claims, or erase credibility	Courts, police forces, hospitals	Victims of domestic and institutional violence

This table categorizes the dominant state and institutional strategies of psychological, ideological, and bodily control from the 20th century to the present. It includes coercive psychiatry, militarization, food monopolies, welfare surveillance, and technocratic governance across authoritarian, neoliberal, and hybrid regimes. Emphasis is placed on the operational role of psychiatry and health discourses in reframing dissent as disorder, and on the continuity of social control logics across political systems allegedly opposed in ideology but aligned in biopolitical function.

Table 10- Cultural Handling of Altered States and Mental Distress, by era and region

Period / Region	Induced or States	Treated	Substances / Foods Used	Rituals and Settings	Interpretive Agents
Ancient Greece	Melancholy, catharsis, mania	divine	Wine, hellebore, potions	opium, honeyed, Dionysian Asclepian incubation	rites, dream, Temple philosophers, priests,
Vedic India	Mystical sorrow, mental	ecstasy, heat	Soma, butter, herbs	clarified ayurvedic, Fire meditative seasonal fasting	sacrifices, chanting, Brahmins, healers, gurus,
Indigenous Amazon	Vision ancestral	quests, possession	Ayahuasca, chicha	tobacco, Night singing, immersion	rituals, group jungle, Shamans, elder women
Classical China	Grief, spiritual	madness, imbalance	Ginseng, mushroom, rice	reishi wine, Ancestral seasonal diaries	rituals, dream, Daoist Confucian, sages, scholars
Medieval Islamicate	Spiritual melancholia, obsession		Ambergris, rosewater, saffron	Music therapy, dream interpretation	Qur'anic Hakims, theologians, Sufis
Medieval Europe	Demonic possession, melancholia		Absinthe, belladonna, and bread	fasting, Exorcisms, flagellation, confession	fasting, Monks, inquisitors, barbers
Edo Japan	Existential societal shame	grief,	Green tea, fermented rice, incense	Theater (Noh), communal poetry	writing, Zen monks, poets, physicians
Andean Highlands	Soul loss, ecstatic mourning	trance,	Coca leaves, beer, llama fat	maize, Dance, mountain offerings	bloodletting, Curanderos, community elders
Renaissance Europe	Black melancholy, madness	bile divine	Laudanum, tonics, red wine	herbal Patronage of alchemy, mirror-gazing	art, Artists, physicians, mystics
Yoruba Africa	West Spirit ecstatic	possession, healing	Palm wine, kola nut, herbal smokes	Drumming, divination ceremonies	trance, Orisha healers, priests,
Early Modern Europe	Hysteria, melancholia, rapture		Purgatives, opiates, hot baths	mercury, Magnetism sessions, mesmerism, confession	Physicians, moral managers

Historical approaches to madness, melancholy, divine ecstasy, and psychological suffering across world traditions, focusing on ritual, substances, communal interpretation, and contextual care practices.

Table 11- Physiological and social harms of forced psychotropic drugging

Category	Description of Harm
Neurological impairment	Tardive dyskinesia, akathisia, extrapyramidal symptoms, cognitive dulling, and neuroleptic-induced deficit syndrome are frequent outcomes of long-term use of antipsychotics
Metabolic and endocrine harm	Weight gain, insulin resistance, hyperlipidemia, and elevated risk of diabetes and cardiovascular disease
Sedation dependency	and Many psychotropics, including benzodiazepines and antipsychotics, originate from or mimic substances used in veterinary anesthesia; their use leads to dependency and emotional blunting
Loss of autonomy	Individuals subjected to forced drugging report lack of control, hopelessness, and inability to exercise basic civil or bodily rights
Isolation	Coerced individuals are perceived as dangerous or incompetent, intensifying social exclusion and familial rupture
Institutional dependency	Psychiatric treatment becomes synonymous with medication compliance; relational or psychosocial interventions are sidelined or unavailable
Social death	The combined effect of stigma, surveillance, and disempowerment results in symbolic exclusion from community and public life
Increased mortality	Long-term use of psychotropics, especially antipsychotics, is associated with reduced life expectancy by 10–20 years compared to the non-drugged cohorts, suicide and disease exceedingly high due to compounding harm and forced nature of the regime

Multifaceted physiological and social harms associated with forced psychiatric drugging, as documented in global and Spanish research contexts. These harms challenge the legitimacy of treatment-as-usual models, especially in coercive settings, and demand systemic transformation in mental health care.

Table 12- Perceived benefits of pharmacotherapy and stakeholder gains

Actor / Stakeholder	Perceived Benefit from Pharmacotherapy	Limitations / Costs Not Internalized by Actor
Individual patient (selected cases)	Short-term relief of acute symptoms (e.g., anxiety, hallucinations)	Long-term metabolic, cognitive and social harm (De Hert et al., 2011; Vancampfort et al., 2015)
Family members (under duress or without support)	Perceived behavioral stabilization reducing household conflict	Loss of autonomy and relational strain; lack of alternatives or systemic support
Clinical staff (psychiatrists, nurses)	Simplified patient management, symptom control under time/resource constraints	Ethical conflicts, burnout, and professional dissatisfaction in coercive settings
Hospital administrations	Cost-effective containment strategy; lower staffing burdens	Poor long-term outcomes; dependency on institutional cycles
Pharmaceutical companies	Revenue from medication sales; expansion of clinical indications	No responsibility for adverse outcomes or social consequences
Government / public policy makers	Budget control, reduced political risk from visible unrest or psychiatric crises	Long-term social exclusion, disability rates, and human rights scrutiny
Insurance systems (public/private)	Predictable and measurable cost structure	Externalization of broader psychosocial recovery costs

Actors who benefit directly or indirectly from pharmacotherapeutic paradigms in mental health care, while highlighting how harms and systemic consequences are often borne by patients and society at large rather than those incentivized to uphold current models.

Table 19. Structural Dangers of Coercive Psychiatric Regimes

Mechanism	Description	Consequences
Diagnostic ambiguity	Psychiatric categories are broad and elastic, easily fitting divergent behaviors	Enables strategic misuse to delegitimize or silence individuals
Family-based false reporting	Relatives fabricate symptoms or narratives to institutionalize or disempower	Enables extrajudicial detention and forced drugging
Institutional obedience	Staff comply with hierarchical mandates despite ethical doubts	Reduces possibility of intervention or whistleblowing
Legal incapacitation	Diagnoses lead to diminished legal standing and presumption of untrustworthiness	Limits capacity to defend oneself or contest abuse
Forensic re-interpretation of protest	Acts of self-defense or dissent are redefined as symptoms of illness	Eliminates legitimacy of resistance, justifies further coercion
Physical health deterioration	Forced medication causes long-term damage without addressing underlying suffering	Increases dependency, vulnerability, and chronicity
Social death	Isolation, forced treatments, and stigma result in permanent loss of community ties and self-agency	Perpetuates exclusion, unemployment, and juridical disadvantage
Reputational erasure	Labeling as mentally ill undermines credibility across social and legal domains	Facilitates exploitation and sustained structural disadvantage

Mechanisms of structural abuse and life destruction under coercive psychiatric regimes.

Table 20. Normalized punitive practices in psychiatric settings

Actor	Tool of Punishment or Control	Justification Used	Abuse Mechanism	Normalization
Psychiatrist	Involuntary drugging, seclusion, diagnosis escalation	Therapeutic necessity, risk management	Clinical authority interpreted as infallible judgment	
Nurse	Physical restraints, isolation, verbal intimidation	Behavioral control, safety protocol	Culture of obedience, understaffing, and hierarchical impunity	
Psychologist	Misinterpretation of trauma, gaslighting, denial of therapeutic alliance	Lack of insight, secondary gains	Pathologization of complaint or critique	
Family member	False claims, reinforcement of diagnosis, denial of support	Duty of care, concern for safety	No external oversight; deference to family in institutional contexts	
Social worker	Blocking access to housing or autonomy-related services	Non-compliance, mental incapacity	Collaboration with medical files without independent evaluation	
Community institutions	Disregard for complaints, deferral to psychiatric authority	Professional consensus, risk avoidance	Psychiatric documents dominate administrative and judicial decisions	
Punitive practices and mechanisms enabling systemic abuse across psychiatric and family settings.				

Table 21- Rationalizations and incentives in psychiatric care nowadays

Actor / Group	Stated Justification	Dissonance with Reality	Empirical Underlying Incentives (Money, Power, Status)
Academic psychiatrists / professors	Evidence-based standards; safety; biological causality	Ongoing controversy over evidence quality and generalizability	Professional prestige; and industry ties; influence in policy and curricula
Politicians / public health officials	Public order, risk reduction, budgetary efficiency	Failure to address structural causes of distress; chronicity worsens under treatment-as-usual (Sisti et al., 2021)	Political capital; avoidance of structural reform; outsourcing responsibility
Practicing psychiatrists / clinicians	Clinical necessity; symptom management; legal safety	Time constraints, resource gaps, and lack of support for legal alternatives drive overreliance on medication	Avoidance of liability; institutional conformity; procedural simplicity
Pharmaceutical-aligned researchers	Scientific advancement; optimization of treatment	Selective publication, ghostwriting, and conflicts undermine neutrality	Research funding; financial career progression; corporate affiliations
Nurses / support staff	Patient and safety; manageability	Frequent distress caused by work forced measures and training in trauma-informed	Job security; lack of systemic alternatives; burnout deflection

Actor / Group	Stated Justification	Dissonance with Reality	Empirical	Underlying Incentives (Money, Power, Status)
Family members (ambivalent/abusive)	'Stabilization', obedience, relief from caregiving burden	Often participate in cycles of control, scapegoating, or silence about abuse		Restored order; avoidance of accountability; social legitimacy
Legal actors / judicial systems	Due process followed; therapeutic exception	Often rely on institutional claims, overlook human rights violations		Case resolution ease; institutional trust; avoidance of political conflict
Sociomedical educators	Enforcing allegedly neutral so-called science processes; appearance of technocratic rationality	Marginalize critical and user-led perspectives; reinforce top-down power		Intellectual gatekeeping; ideological alignment; systemic career incentives

Rationalizations used by institutional actors to justify coercive pharmacological interventions in mental health, alongside empirical dissonances and material or symbolic incentives maintaining the current system. Evidence suggests that these justifications often obscure deeper patterns of neglect, coercion, and power asymmetry.

Table 22. Historical Evolution of Psychiatric Interventions, 20th–21st Century

Period	Pharmacological Interventions	Physical/Institutional Approaches	Psychosocial/Community Approaches	Force Coercion	and Target Populations and Patterns
1900–1950	Barbiturates, early antipsychotics, insulin shock	Asylums, electroconvulsive therapy, lobotomies	Limited, confined to psychoanalysis (elitist)	Often to involuntary; massive confinement	Women, poor, disabled, colonized subjects framed as deviant
1950–1970	Chlorpromazine, lithium, MAOIs	Deinstitutionalization begins, ECT remains	Rise of social psychiatry, family therapy	Ambiguous: outpatient clinics expand, coercion persists	Allegedly therapeutic force; racial minorities disproportionately institutionalized
1970–1990	Benzodiazepines, SSRIs, atypical	Hospital rise of	Hospital downsizing, psychiatric mental health	Community health	Restraints used, Women medicalized for non-compliance

Period	Pharmacological Interventions	Physical/Institutional Approaches	Psychosocial/Community Approaches	Force Coercion	and Target Populations and Patterns
	antipsychotics	wards in general hospitals	centers promoted (esp. post-WHO 1979)	unregulated; lack of awareness	despair, anti-rights psychiatric critiques rise
1990–2010	Polypharmacy, overprescription, off-label uses, stimulant and other drugs use rise in children population	Forensic psychiatry and emergency interventions dominate	Global Mental Health movement emerges, posing both risks and opportunities	Legislation increases procedural safeguards, but implementation weak	Rise in psychotropics among elderly, youth, and racialized populations
2010–present	Long-acting injectables, digital therapeutics, overprescription trends worsen	Acute care, seclusion, and mechanical restraints still widespread	Trauma-informed care, recovery-oriented approaches barely adopted	UN/WHO condemn coercive practices common	Shared decision-making discussed rarely implemented; structural abuse persists

Main treatments, modalities, and political logics across global and European contexts. Historical overview of psychiatric practice by dominant treatment logics, coercion levels, and affected populations (1900–present). Based on scientific and policy literature.

Table 23. Timeline of global mental health advocacy and policy shifts

Period	Dominant Framing	Key Institutions	Goals Articulated	Political-Economic Context	Effectiveness
1970s–1980s	Public health integration	WHO, PAHO	Primary care inclusion of mental health (Alma-Ata, 1978)	Post-colonial restructuring, War	Limited implementation; high institutional inertia

Period	Dominant Framing	Key Institutions	Goals Articulated	Political-Economic Context	Effectiveness
1990s	Neurobiological focus rises	NIMH, WPA, pharma-aligned agencies	Emphasis on diagnostics (e.g. DSM-IV), pharmaceutical rollout	Rise of global pharma, neoliberalism	Community-based models undercut by privatization
2001	Rights-based global reform	WHO (World Health Report 2001)	Community-based, user-involved services	Globalization, post-Soviet health transition	Key declarations; partial reform in few countries
2007–2013	mhGAP program	WHO, UNHRC	Integration in non-specialist settings, rights emphasis	Global burden of disease framing	Some regional impact; insufficient structural funding
2018–present	Human rights, LGBT+, anti-racism, One Health	UN, WHO, CRPD	End coercion, promote autonomy and choice	UNCRPD enforcement, Sustainable Development Goals	Systemic coercion persists; calls for compliance unheeded in many contexts

International mental health advocacy phases, from primary care reform to human rights-based frameworks, with limited structural transformation.

Table 24- Sociocultural models of psychiatry and abuses of the discipline to punish, control

Psychiatric tradition	Violence Inflicted	Ideological Motifs	Biological framing
Buddhist-informed	Spiritual bypassing, behavioral conformity	Detachment from suffering, integration	Meditation as neurological regulation
Classical asylum	Confinement, dehumanization, discipline	Social order, moral hygiene	Degeneration theory, inherited inferiority
Freudian psychoanalysis	Verbal domination, pathologizing normativity	Libidinal economy, repression theory	Somatic roots of hysteria and libido
Lacanian psychoanalysis	Ambiguity, institutional cultism, symbolic violence	Subject split, language mastery	Linguistic inscription over neurology
Soviet psychiatry	Political imprisonment, psychiatric labeling	Ideological conformity, state security	Material brain-based deviance
Democratic psychiatry	Minimal; rights-based, disruption of coercion	Equality, emancipation, community care	Critique of reductionism, neuroplasticity support
Postcolonial ethnopsychiatry	Cultural assimilation, epistemic domination	Colonial control, identity erasure	Organicism denied or distorted via 'soul sickness'
Neocolonial psychiatry	Criminalization, silencing of resistance	Order maintenance, cultural export	Diagnostic mimicry of Western pathologies
Community-based and open dialogue models	Low; minimal or pharmacological physical coercion	Dialogue, subjectivity, social recovery	Complex adaptive neurobiology

Paradigms of psychiatry with their historical and contemporary misuses to exert punitive control, marginalize dissent, and enforce normative behavior under the guise of care.

Table 25- Biological and allegedly biological models of psychiatry

Psychiatric tradition	Violence Inflicted	Ideological Motifs	Social Framing
Francoist	Political repression through biological deviance	Biological inferiority justifying national moral order	Moral deviation encoded in bloodline and class
National Socialist	Sterilization, extermination by racial-biological doctrine	Racial hygiene, eugenic cleansing, societal purification	Biological race as societal value metric
Biologicist Psychiatry	Polypharmacy, chronicization, dismissal of cause	Neurochemical correction, diagnostic standardization	Functional adaptation to disorder; lifestyle blamed
Nutritional Psychiatry	Neglect of social context, over-focus on micronutrients	Gut-brain restoration, on micronutrient optimization	Diet-driven personality traits and cognition
Metabolic Psychiatry	Metabolic labeling, medicalization of stress	Systemic resilience, glucose-lipid-mental	Stress as pathology of social performance

Psychiatric tradition	Violence Inflicted	Ideological Motifs	Social Framing
		health link	
Psychoneuroimmunology-based Approaches	Reductive profiling, essentialism	immune biomarker, Inflammation cytokine-mediated regulation	control, Social threat read as immune activation
Neuroendocrine-Informed Models	Hormonal manipulation, gendered bias	Hormonal balance, HPA axis recalibration	Stress exposure framed through gendered life roles
Microbiota-Gut-Brain Axis Psychiatry	Overinterpretation of correlations, overselling	Barrier protection, probiotic digestive-immune co-regulation	Western diet and urbanization as disease vectors
Systems Biology	Datafied abstraction, individual burden framing	Multi-scalar modeling, predictive frameworks	Individualized health responsibility for failure to self-regulate

Biologically framed psychiatric models, their scientific foundations and controversies, including critique of chemical imbalance theories and reductionist diagnostic frameworks.

Table 26- Coercive measures in psychiatric and general healthcare settings in Spain

Setting	Coercive Practices
Primary Care	Coercion is primarily symbolic. Patients pressured into pharmacological compliance. Referral used to discipline rather than support (Moncrieff, 2022).
Outpatient Health Units	Mental Legal coercion via guardianship and compulsory outpatient treatment. Emergency measures sometimes bypass procedural safeguards (Muñoz & Lobato, 2021).
Hospital Wards	Psychiatry Mechanical restraints and isolation used routinely under therapeutic pretext. Often applied punitively or to suppress distress (González Pinto et al., 2020; WHO, 2021).
Forensic Units	Psychiatry Prolonged seclusion and forced medication normalized. Coercion institutionalized and poorly monitored. Human rights safeguards minimal (Rodríguez-Pulido et al., 2021).

Common coercive practices in mental health settings in Spain, showing normalization of physical and legal control mechanisms.

Table 27- Patterns of psychotropic overprescription in Spain by clinical setting

Setting	Overprescription of Psychotropics
Primary Care	Benzodiazepines and antidepressants frequently prescribed as first-line treatment without comprehensive assessment. Most affected: elderly and women (Abas et al., 2018; WHO, 2021).
Outpatient Health Units	Mental Polypharmacy is common. Medication is maintained over time without regular review. Consent procedures often inadequate or absent (Rose et al., 2022).
Hospital Wards	Psychiatry Dosage increases and drug switching are frequent during crises. Informed consent not prioritized. Medication dominates therapeutic approach (Rodríguez-Pulido et al., 2021).
Forensic Units	Psychiatry Antipsychotics administered systematically for behavioral containment. Alternatives rarely considered. Consent typically circumvented (Otero & Pérez, 2019).

Summary of overprescription trends in different healthcare settings in Spain, highlighting systemic pharmacological excess and lack of dialogical care.

Thematic index and mapping

Below is a thematically organized mapping of the dissertation's major concepts, aligned with the chapters and annexes where each is most substantively developed. This table supports reader orientation and clarifies the conceptual architecture across the empirical, legal, and applied dimensions of the work. Please note that due to sustained institutional pressure, abusive constraints, and structurally hostile conditions endured during the final stages of dissertation preparation, coinciding with intense academic, legal, and coordination workloads, it was not feasible to include the full breadth of content, evidence, and analytical resources in this printed version. Severe formatting restrictions, together with time-sensitive deadlines and repeated episodes of obstruction and violence, further limited the capacity to render the document in its complete and intended form.

Coercion in psychiatry is a central theme appearing throughout the dissertation. It is addressed in section 1.1 in its historical form under institutional repression and carceral psychiatry, in section 1.2 as part of the current framework of involuntary treatment, and in sections 1.6 and 1.7 as systemic failure and power-related diagnostic exclusion. It also features prominently in sections 2.2, 2.4, and 2.7 in the analysis of coercion through mixed-methods research and reflexive methodologies. Sections 3.1 to 3.3 elaborate the lived experiences of coercion, especially in institutional settings, while sections 4.1 to 4.4 extend this analysis through ethnographic documentation of coercive practices. Section 5.1 connects psychiatric coercion with learned helplessness, while section 6.2 treats coercion as a violation of international human rights.

Structural violence is identified in sections 1.2, 1.3, and 1.6 as embedded in pharmacocentric psychiatric systems. It is analytically developed in sections 2.1, 2.2, and 2.7 as both an empirical finding and a methodological challenge, then further documented through participant testimony in

sections 3.3 and 4.3. Its long-term effects on health and neurodevelopment are discussed in sections 5.1 and 5.2, and its legal implications are examined in section 6.2.

Institutional betrayal is explored from section 1.5 onward, with special emphasis in sections 3.4 to 3.7 as a relational and organizational phenomenon that affects both users and professionals. In sections 4.1 to 4.4, it is analysed through narrative reconstruction and ethnographic evidence. The term reappears in sections 5.1 and 6.2 when addressing structural impunity, forensic accountability, and the failure of legal safeguards.

Epistemic injustice appears as a core concept in section 1.5 on legal and epistemic foundations, and is further developed in section 1.7 through the lens of diagnosis and exclusion. Methodologically, it frames sections 2.1, 2.2, 2.5, and 2.6. Empirical data reflecting epistemic injustice are presented in sections 3.2 to 3.6 and again in 4.5 and 4.6 through testimonial erasure and narrative foreclosure. Sections 5.2 and 5.4 connect it to reductionist diagnostics, and 7.1 and 7.3 propose educational responses.

Shared decision-making is treated as a countermodel to coercion in psychiatry, discussed in section 1.4 regarding deprescription and user autonomy. It is also part of the research instruments described in section 2.3 and analyzed in findings in sections 3.1 and 3.6. It appears again in section 5.5 as part of legal reform proposals, and in 7.2 within future-oriented frameworks for rights-based psychiatric care.

Pharmacocentrism and overmedication are analyzed in section 1.3 as systemic features of Spanish psychiatric care. Section 1.4 focuses on alternatives such as deprescription and long-term medication ethics. These themes are then referenced in empirical findings (sections 3.6 and 5.4) and addressed normatively in sections 6.1 and 6.3 through proposals for neurobiological literacy, somatic regulation, and open-source expert systems.

Transdisciplinary action research is outlined in sections 1.0, 1.8, and 2.1 as the overarching methodology. It also underpins the work in sections 2.4 and 2.9 through its application in mixed methods, reflexivity, and systemic transformation. The design is reaffirmed in sections 5.5 and 7.1 to 7.3 in reference to future implementation through One Health education and participatory governance.

Legal accountability is developed in sections 1.2, 1.5, and 1.6 with respect to coercive practices and rights violations. Sections 3.7 and 4.3 analyze legal ambiguity, institutional silencing, and accountability failures. Section 5.5 proposes new legal-pedagogical standards, while section 6.2 addresses the alignment of national practice with international human rights law and CRPD principles.

Biocultural psychiatry and **trauma-informed care** appear in section 1.4 and 1.6, and more explicitly in 2.1 and 2.5 as foundations for a new clinical paradigm. Section 5.2 links biocultural trauma with neurodevelopment, and 7.1 and 7.3 present it as part of a curricular transformation for mental health and social care professionals.

Narrative sovereignty and **testimonial justice** are articulated in sections 4.3, 4.5, and 4.6 as correctives to epistemic violence and reductionist diagnostics. These themes also frame

participatory methods (sections 2.3 and 2.8) and reappear in section 5.4 and 6.3 within structural and educational reform proposals.

One Health is introduced in section 1.8 and developed as a unifying concept in sections 2.8, 5.5, 6.1, and 7.3. It provides a framework for integrative ethics, interdisciplinary education, and resilient system design that addresses mental, physical, and ecological health together.

Ethical AI and expert systems in psychiatry emerge in sections 6.3 and 7.2, where they are proposed as tools for ethical governance and patient empowerment. They are linked to participatory design, precision mental health, and rights-based oversight, and are part of the structural response proposed in the EU BEACON framework.

Educational reform is addressed through sections 5.5, 6.1, 7.1, and 7.3 as a necessary step to prevent future epistemic violence and structural failure. It incorporates trauma-informed pedagogy, neurobiological literacy, relational ethics, and participatory learning, with emphasis on long-term system resilience and justice.

An expanded and interactive version of this dissertation is therefore being made available online as an open access ebook. This living edition incorporates all annexes, updated analyses, extended methodological notes, evidence tables, and thematic mappings, as well as the tools, instruments, and codes used in this research. It will continue to grow in accuracy, depth, and applicability in line with the principles of action research and scientific accountability. For full access to the updated version, including dynamic indexes and embedded documentation, please visit the dedicated repository and dissemination platform accompanying this work:

Thematic keywords	Chapters	Annexes
Structural violence	1, 4, 5, 6	D, G, K
Psychiatric coercion	1, 3, 5	C, F, G
Shared decision-making	2, 7	F, J, K
Epistemic injustice	1, 2, 4, 5	C, G
Action research	2, 3, 4	B, G
Human rights and legal frameworks	1, 6, 8	G, H, I, K
Biocultural methodology	2, 4, 5	D, F
Testimony and narrative evidence	4, 5	B, C
Dialogical and relational care	3, 7	A, F
Psychiatric diagnosis and medication	3, 5	D, F
Institutional betrayal and repression	4, 6	B, C
Alternatives to coercion	5, 7	A, F
Systems transformation and reform	7, 8	H, I, J, K
Open science and pedagogy	7	J, M
Survivor-researcher positionality	2	F

Thematic keywords	Chapters	Annexes
Ethics of qualitative research	2, 6	A, F
Policy translation and implementation	7, 8	H, I, K, L

The full mapping of keywords, themes, and actors relevant to this dissertation link will be shared on the open science laboratory and fieldwork notebook. The repository is conceived not as a static archive, but as a living, version-controlled environment subject to ongoing refinement, correction, and enrichment. Following the principles of action research, the structure and content of the repository will evolve in response to new evidence, user feedback, critical input, and implementation needs arising from fieldwork and policy engagement. The mapping exercise will progressively deepen and systematize the documentation of all major analytical categories, conceptual clusters, and empirical references appearing across the chapters and annexes. It will include cross-referenced indexes of themes, legal principles, methodological frameworks, discursive patterns, and institutional actors, providing an integrated navigational and interpretive guide for readers, reviewers, and allied professionals. This process will be grounded in the rigorous logic of transdisciplinary inquiry and the ethical imperative to make visible the underlying structures of violence, resistance, and repair documented throughout the work. In parallel, the repository includes all instruments and codes developed and employed in the course of the research. These encompass survey instruments, thematic coding schemas, case analysis matrices, legal mapping procedures, and computational scripts for organizing and analyzing the dataset. Each component is provided in fully accessible formats and accompanied by detailed documentation to support adaptation, reuse, and local implementation across diverse educational, clinical, legal, or policy contexts. The online version of this work will continue to be updated as part of the dissertation's ongoing contribution to the field, ensuring that its analytical tools, empirical insights, and proposed reforms remain usable, improvable, and contextually relevant. This reflects a core ethical and scientific commitment: that research of this nature ought to remain open, traceable, and accountable, not only in its results but also in its mechanisms, limitations, and possible futures.

